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**Child Care and Development Fund (CCDF) Plan
for
State/Territory Idaho**

FFY 2025 – 2027

Version: Initial Plan

Plan Status: Approved as of 2024-11-09 00:31:17 GMT

This Plan describes the Child Care and Development Fund program to be administered by the State or Territory for the period from 10/01/2024 to 9/30/2027, as provided for in the applicable statutes and regulations. The Lead Agency has the flexibility to modify this program at any time, including amending the options selected or described.

For purposes of simplicity and clarity, the specific provisions of applicable laws printed herein are sometimes paraphrases of, or excerpts and incomplete quotations from, the full text. The Lead Agency acknowledges its responsibility to adhere to the applicable laws regardless of these modifications.

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Overview

Introduction

The Child Care and Development Block Grant Act (CCDBG) (42 U.S.C. 9857 *et seq.*), together with section 418 of the Social Security Act (42 U.S.C. 618), authorize the Child Care and Development Fund (CCDF), the primary federal funding source devoted to supporting families with low incomes afford child care and increasing the quality of child care for all children. The CCDF program is administered by the Office of Child Care (OCC) within the Administration for Children and Families (ACF) at the U.S. Department of Health and Human Services and provides resources to State, Territory, and Tribal governments via their designated CCDF Lead Agency.

CCDF plays a vital role in supporting family well-being and child development; facilitating parental employment, training, and education; improving the economic well-being of participating families; and promoting safe high-quality care and learning environments for children when out of their parents' care.

As required by CCDBG, this CCDF Plan serves as the State/Territory Lead Agency's application for a three-year cycle of CCDF funds and is the primary mechanism OCC uses to determine Lead Agency compliance with the requirements of the statute and regulations. CCDF Lead Agencies must comply with the rules set forth in CCDBG and corresponding ACF-issued rules and regulations. The CCDF Plan is a fundamental part of OCC's oversight of CCDF and is designed to align with and complement other oversight mechanisms including administrative and financial data reporting, the monitoring process, error rate reporting, audits, and the annual Quality Progress Report.

Organization of Plan

In their CCDF Plans, State/Territory Lead Agencies must describe how they implement the CCDF program. The Plan is organized into the following sections:

1. CCDF Program Administration
2. Child and Family Eligibility and Enrollment and Continuity of Care
3. Child Care Affordability
4. Parental Choice, Equal Access, Payment Rates, and Payment Practices
5. Health and Safety of Child Care Settings
6. Support for a Skilled, Qualified, and Compensated Child Care Workforce
7. Quality Improvement Activities
8. Lead Agency Coordination and Partnerships to Support Service Delivery
9. Family Outreach and Consumer Education
10. Program Integrity and Accountability

Completing the Plan

This revised Plan aims to capture the most accurate and up-to-date information about how a State/Territory is implementing its CCDF program in compliance with the requirements of CCDF. In responding to plan questions, Lead Agencies should provide concise and specific summaries and/or bullet points as appropriate to the question. Do not insert tables or charts, add attachments, or copy manuals into the Plan. A State/Territory's CCDF Plan is intended to stand on its own with sufficient information to describe how the Lead Agency is implementing its CCDF program without need for added attachments, tables, charts, or State manuals.

OCC recognizes that Lead Agencies use different mechanisms to establish CCDF policies, such as State statute, regulations, administrative rules, policy manuals, or policy issuances. Lead Agencies must submit their CCDF Plan no later than July 1, 2024.

Review and Amendment Process

OCC will review submitted CCDF Plans for completeness and compliance with federal policies. Each Lead Agency will receive a letter approximately 90 days after the Plan is due that includes all Plan non-compliances to be addressed. OCC recognizes that Lead Agencies continue to modify and adapt their programs to address evolving needs and priorities. Lead Agencies must submit amendments to their Plans as they make substantial policy and program changes during the three-year plan cycle, including when addressing non-compliances.

Appendix 1: Implementation Plan

As part of the Plan review process, if OCC identifies any CCDF requirements that are not fully implemented, OCC will communicate a preliminary notice of non-compliance for those requirements via an emailed letter. OCC has created a standardized template for Lead Agencies to submit as their 60-day response to that preliminary notice. This template is found at Appendix 1: Lead Agency Implementation Plan. This required response via the Appendix will help create a shared understanding between OCC and the Lead Agency on which elements of a requirement are unmet, how they are unmet, and the Lead Agency's steps and associated timelines needed to fully implement those unmet elements.

CCDF Plan Submission

CCDF Lead Agencies will submit their Plans electronically through the Child Care Automated Reporting System (CARS). CARS will include all language and questions included in the final CCDF Plan template approved by the Office of Management and Budget (OMB). Note that the format of the questions in CARS could be modified from the Word version of the document to ensure compliance with Section 508 policies regarding accessibility to electronic and information technology for individuals with disabilities.

1 CCDF Program Administration

Strong organizational structures, operational capacity, and partnerships position States and Territories to administer CCDF efficiently, effectively, and collaboratively.

This section identifies the CCDF Lead Agency, CCDF Lead Agency leadership, and the entities and individuals who will participate in the implementation of the program. It also identifies the partners who were consulted to develop the Plan.

1.1 CCDF Leadership

The governor of a State or Territory must designate an agency (which may be an appropriate collaborative agency) or establish a joint interagency office to represent the State or Territory as the Lead Agency. The Lead Agency agrees to administer the program in accordance with applicable federal laws and regulations and the provisions of this Plan, including the assurances and certifications.

1.1.1 Designated Lead Agency

Identify the Lead Agency or joint interagency office designated by the State or Territory. OCC will send official grant correspondence, such as grant awards, grant adjustments, Plan approvals, and disallowance notifications, to the designated contact identified here.

- a. Lead Agency or Joint Interagency Office Information:
 - i. Name of Lead Agency: **Idaho Department of Health and Welfare**
 - ii. Street Address: **450 W. State St.**
 - iii. City: **Boise**
 - iv. State: **Idaho**
 - v. ZIP Code: **83702**
 - vi. Web Address for Lead Agency: **<https://healthandwelfare.idaho.gov/>**
- b. Lead Agency or Joint Interagency Official contact information:
 - i. Lead Agency Official First Name: **Alex**
 - ii. Lead Agency Official Last Name: **Adams**
 - iii. Title: **Director**
 - iv. Phone Number: **208-334-5500**
 - v. Email Address: **Alex.Adams@dhw.idaho.gov**

1.1.2 CCDF Administrator

Identify the CCDF Administrator designated by the Lead Agency, the day-to-day contact, or the person with responsibility for administering the State's or Territory's CCDF program. The OCC will send programmatic communications, such as program announcements, program instructions, and data collection instructions, to the designated contact identified here. If there is more than one designated contact with equal or shared responsibility for administering the CCDF program, identify the Co-Administrator or the person with administrative responsibilities and include their contact information.

- a. CCDF Administrator contact information:
 - i. CCDF Administrator First Name: **Aubrie**

- ii. CCDF Administrator Last Name: **Hunt**
- iii. Title of the CCDF Administrator: **Program Manager, Child Care and TANF**
- iv. Phone Number: **208-334-5686**
- v. Email Address: **Aubrie.Hunt@dhw.idaho.gov**
- b. CCDF Co-Administrator contact information (if applicable):
 - i. CCDF Co-Administrator First Name:
 - ii. CCDF Co-Administrator Last Name:
 - iii. Title of the CCDF Co-Administrator:
 - iv. Phone Number:
 - v. Email Address:
 - vi. Description of the Role of the Co-Administrator:

1.2 CCDF Policy Decision Authority

The Lead Agency has broad authority to administer (i.e., establish rules) and operate (i.e., implement activities) the CCDF program through other governmental, non-governmental, or public or private local agencies as long as the Lead Agency retains overall responsibility for the administration of the program. Administrative and implementation responsibilities undertaken by agencies other than the Lead Agency must be governed by written agreements that specify the mutual roles and responsibilities of the Lead Agency and other agencies in meeting the program requirements.

1.2.1 Entity establishing CCDF program rules

Which of the following CCDF program rules and policies are administered (i.e., set or established) at the State or Territory level or local level? Identify whether CCDF program rules and policies are established by the State or Territory (even if operated locally) or whether the CCDF policies or rules are established by local entities, such as counties or workforce boards.

Check one of the following:

- a. ☒ All program rules and policies are set or established by the State or Territory. (If checked, skip to question 1.2.2.)
- b. ☐ Some or all program rules and policies are set or established by local entities or agencies. If checked, indicate which entities establish the following policies. Check all that apply:
 - i. Eligibility rules and policies (e.g., income limits) are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
 - ii. Sliding-fee scale is set by the:
 - ☐ State or Territory.

- ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
- ☐ Other. Identify the entity and describe the policies the entity can set:
- iii. Payment rates and payment policies are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- iv. Licensing standards and processes are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- v. Standards and monitoring processes for license-exempt providers are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- vi. Quality improvement activities, including QIS, are set by the:
 - ☐ State or Territory.
 - ☐ Local entity (e.g., counties, workforce boards, early learning coalitions).
 - ☐ Other. Identify the entity and describe the policies the entity can set:
- vii. Other. List and describe any other program rules and policies that are set at a level other than the State or Territory level:

1.2.2 Entities implementing CCDF services

The Lead Agency has broad authority to operate (i.e., implement activities) through other agencies, as long as it retains overall responsibility for CCDF. Complete the table below to identify which entity(ies) implements or performs CCDF services.

Check the box(es) to indicate which entity(ies) implement or perform CCDF services.

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who conducts eligibility determinations?	☒	☐	☐	☐	☐ Describe:
Who assists parents in locating child care (consumer education)?	☐	☐	☐	☒	☐ Describe:

CCDF Activity	CCDF Lead Agency	TANF Agency	Local Government Agencies	CCR&R	Other
Who issues payments?	[x]	[]	[]	[]	[] Describe:
Who monitors licensed providers?	[x]	[]	[x]	[]	[] Describe:
Who monitors license-exempt providers?	[x]	[]	[x]	[]	[] Describe:
Who operates the quality improvement activities?	[x]	[]	[]	[x]	[] Describe:

1.2.3 Information systems availability

For any activities performed by agencies other than the Lead Agency as reported above in 1.2.1 and 1.2.2, identify the processes the Lead Agency uses to oversee and monitor CCDF administration and implementation activities to retain overall responsibility for the CCDF program.

Check and describe how the Lead Agency includes in its written agreements the required elements. Note: The contents of the written agreement may vary based on the role the agency is asked to assume or type of project but must include, at a minimum, the elements below.

a. Tasks to be performed.

[x] Yes. If yes, describe: CCR&R services are provided statewide through a contract with the University of Idaho, which employs staff and/or contracts with other agencies to carry out the scope of work. The scope of work for the CCR&R offices is outlined in a contract and includes:

(1) Working directly with families who receive child care assistance to offer the families support and assistance in making an informed decision about child care options to ensure families are enrolling their children in the most appropriate child care setting to suit their needs and that is of high quality as determined by the Department;

(2) Collecting data and providing information on the coordination of services and supports including services provided through the Individuals with Disabilities Education Improvement Act (IDEIA) for children with disabilities;

(3) Tracking attempts to contact providers for the provider information update process which collects data about a provider's current availability, hours, age groups served, etc. in local areas or regions of the state and submits such information to the Department for a market rate study;

(4) Engaging in services to increase the supply and quality of child care services in the state;

(5) As appropriate, coordinating their activities with the activities of the lead agency and/or local agencies that administer Child Care and Development Funds (CCDF); and

(6) Training providers to prepare disaster preparedness and response plans.

☐ No. If no, describe:

b. Schedule for completing tasks.

☒ Yes. If yes, describe: **Full implementation of the scope of work and responsibilities detailed within contracts held between IDHW and its contracted partners includes unique and specific timelines associated to each task or report. A variety of reports analyzed monthly, biannually, and annually evaluate the maintenance of ongoing tasks and compliance with assigned projects and initiatives. In addition, program monitoring visits are conducted at least twice per year to ensure contracted partners continue to meet the contract expectations.**

☐ No. If no, describe:

c. Budget which itemizes categorical expenditures in accordance with CCDF requirements.

☒ Yes. If yes, describe: **In the Cost/Billing Procedures section of the contract, it outlines the annual budget and itemizes the expenditures allowed.**

☐ No. If no, describe:

d. Indicators or measures to assess performance of those agencies.

☒ Yes. If yes, describe: **The Lead Agency monitors administrative implementation and responsibilities of the contract with University of Idaho through the Division of Welfare's Contracts and External Resource Management (CERM) team. CERM evaluates and ensures that the standards, techniques, and criteria identified in the scope of work are consistent and timely. Additionally, they ensure billing invoices are accurate and include backup documentation.**

☐ No. If no, describe:

e. In addition to the written agreements identified above, describe any other monitoring and auditing processes used to oversee CCDF administration. **The administrative and implementation responsibilities of our contracted partners are routinely monitored by our contract monitoring team (CERM). After reviewing the contract with IDHW program staff the CERM team develops contract monitoring tools used to document and calculate compliance, assess performance, and measure outcomes. Additionally, required monthly invoices are evaluated prior to approval for compliance with contract requirements and spending allocations. Financial audits are conducted based on state and federal requirements.**

1.2.4 Certification of shareable information systems.

Does the Lead Agency certify that to the extent practicable and appropriate, any code or software for child care information systems or information technology for which a Lead Agency or other agency expends CCDF funds to develop is made available to other public agencies? This includes public agencies in other States for their use in administering child care or related programs.

☒ Yes.

☐ No. If no, describe:

1.2.5 Confidential and personally identifiable information

Certification of policies to protect confidential and personally identifiable information

Does the Lead Agency certify that it has policies in place related to the use and disclosure of confidential and personally identifiable information about children and families receiving CCDF assistance and child care providers receiving CCDF funds?

☒ Yes.

☐ No. If no, describe:

1.3 Consultation in the Development of the CCDF Plan

The Lead Agency is responsible for developing the CCDF Plan, and consultation with and meaningful input and feedback from a wide range of representatives is critical for CCDF programs to continually adapt to the changing needs of families, child care programs, and the workforce. Consultation involves meeting with or otherwise obtaining input from an appropriate agency in the development of the State or Territory CCDF Plan. As part of the Plan development process, Lead Agencies must consult with the following:

- (1) Appropriate representatives of general-purpose local government. General purpose local governments are defined by the U.S. Census at https://www2.census.gov/govs/cog/g12_org.pdf.
- (2) The State Advisory Council (SAC) on Early Childhood Education and Care (pursuant to 642B(b)(1)(A)(i) of the Head Start Act) or similar coordinating body pursuant to 98.14(a)(1)(vii).
- (3) Tribe(s) or Tribal organization(s) within the State. This consultation should be done in a timely manner and at the option of the Tribe(s) or Tribal organization(s).

1.3.1 Consultation efforts in CCDF Plan development

Describe the Lead Agency's consultation efforts in the development of the CCDF Plan, including how and how often the consultation occurred.

- a. Describe how the Lead Agency consulted with appropriate representatives of general-purpose local government: **Lead Agency staff traveled the state and visited each municipality operating a child care licensing system to meet in-person reviewing the licensing requirements, the CCDF Plan, and soliciting feedback. Additionally, a copy of the CCDF State Plan with a request to provide feedback was distributed to local municipalities that operate their own child care licensing programs to give them an opportunity to provide feedback on the CCDF State Plan and its impact to child care providers in their communities.**
- a. Describe how the Lead Agency consulted with the State Advisory Council or similar coordinating body: **The Idaho Child Care Program (ICCP) Advisory Panel is a diverse group**

of child care providers and child care stakeholders convened quarterly to assist in promoting the continuous improvement of the services provided by the Idaho Child Care Program and to offer an external perspective to the Lead Agency in the development of the CCDF State Plan. The Lead Agency staff solicited feedback prior to drafting the plan at the October 2023 meeting. At the January 2024 meeting additional sections of the plan were discussed and additional feedback was solicited. During the April 2024 ICCP Advisory Panel the draft plan was announced and the updated federal rules were shared. Additionally, panel members received an emailed copy of the plan with a request to provide feedback and to participate at the public hearings.

- b. Describe, if applicable, how the Lead Agency consulted with Indian Tribes(s) or Tribal organizations(s) within the State: **Each tribal nation with borders neighboring Idaho received a copy of the CCDF State Plan and information on the dates and times of the public comment period and public hearings to have the opportunity to provide feedback in person, over the phone, via email, and through postal mail. The Idaho Department of Health and Welfare meets quarterly with the tribal nations to discuss program updates and provide information as necessary. A copy of the state plan was provided at the quarterly meeting.**
- c. Identify other entities, agencies, or organizations consulted on the development of the CCDF Plan (e.g., representatives from the child care workforce, or statewide afterschool networks) and describe those consultation efforts: **IdahoSTARS is a joint project between the University of Idaho's Center on Disabilities and Human Development and the Idaho Association for the Education of Young Children. The IdahoSTARS project is funded by the Idaho Department of Health and Welfare through the Child Care and Development Block Grant. The IdahoSTARS project oversees all child care resource and referral offices statewide and works directly with providers to improve the quality and training and education of our workforce. Experts on the IdahoSTARS project contributed to the development of the CCDF State Plan, reviewed draft plan language, and recommended changes in their areas of expertise.**

Additionally, representatives from the following child care partners, stakeholders, advocates, and networks received a copy of the CCDF State Plan and information on the dates and times of the public comment period and public hearings to have the opportunity to provide feedback in person, over the phone, via email, and through postal mail: Idaho Out of School Network, Idaho Association for the Education of Young Children, Idaho Voices for Children, Idaho Head Start Association, Health District lead staff from each region, Idaho State Department of Education, Treasure Valley YMCA, Community Council of Idaho, Idaho School Readiness Project, Idaho Children's Trust Fund, CACFP sponsor St. Vincent de Paul, the Idaho Workforce Development Council, Idaho Maternal, Infant, and Early Childhood Home Visiting Program. Additionally, the plan was shared with leadership from the following IDHW administered programs: Medicaid, TANF, Behavioral Health, Child Welfare, Navigation, 2-1-1 the Idaho Careline, Idaho Infant Toddler Program, and the Idaho Head Start Collaboration Director.

1.3.2 Public hearing process

Lead Agencies must hold at least one public hearing in the State or Territory, with sufficient Statewide or Territory-wide distribution of notice prior to such a hearing to enable the public to comment on the provision of child care services under the CCDF Plan.

Describe the Statewide or Territory-wide public hearing process held to provide the public with an opportunity to comment on the provision of child care services under this Plan.

- i. Date of the public hearing: **6/21/2024; 6/24/2024**

Reminder: Must be no earlier than January 1, 2024. If more than one public hearing was held, enter one date (e.g., the date of the first hearing, the most recent hearing date, or any hearing date that demonstrates this requirement).

- ii. Date of notice of public hearing: **5/31/2024**

- iii. Was the notice of public hearing posted publicly at least 20 calendar days prior to the date of the public hearing?

☒ Yes.

☐ No. If no, describe:

- iv. Describe how the public was notified about the public hearing, including outreach in other languages, information on interpretation services being available, etc. Include specific website links if used to provide notice **A notice of public hearing was published in statewide and local newspapers throughout Idaho. Social media was also utilized to spread the word about the public comment period.**

IdahoSTARS also included information about the public hearing in its newsletters and communications to providers. A draft of the state plan was published on the Idaho Department of Health and Welfare's public website, with the link to the plan distributed to various agencies and partners, ensuring they have all the background information in addition to the draft of the state plan. A variety of options for feedback and comments were made available leading to the day of the hearing, including a dedicated email box, mail, and fax options.

- v. Describe how the approach to the public hearing was inclusive of all geographic regions of the State or Territory: **The Lead Agency and contractors provided outreach to families and communities in every region on the details of the notice of public hearing. Notices of the state plan and public hearings were published in the following regional and statewide newspapers: Coeur d'Alene, Idaho Press Tribune, Idaho State Journal, Idaho Statesman, Lewiston Tribune, Post Register, Times-News. The hearings were hosted via WebEx, open to anyone in Idaho, so that no travel was necessary to attend.**

- vi. Describe how the content of the Plan was made available to the public in advance of the public hearing (e.g., the Plan was made available in other languages, in multiple formats, etc.): **Idaho's draft CCDF state plan was posted for public viewing on June 1, 2024 on the Lead Agency's website alongside the current state plan at this webpage:**

<https://healthandwelfare.idaho.gov/providers/child-care-providers/child-care-resources>

- vii. Describe how the information provided by the public was taken into consideration

regarding the provision of child care services under this Plan: **Written and oral comments were received and reviewed by IDHW staff and program leadership. All comments and feedback were compiled and recorded to accompany the plan in program documentation. Feedback was reviewed to determine if any amendments need to be made to the plan, and will be shared with the Idaho Child Care Program Advisory Panel.**

1.3.3 Public availability of final Plan, amendments, and waivers

Lead Agencies must make the submitted and approved final Plan, any approved Plan amendments, and any approved requests for temporary waivers publicly available on a website.

- a. Provide the website link to where the Plan, any Plan amendments, and waivers (if applicable) are available. Note: A Plan amendment is required if the website address where the Plan is posted changes. **<https://healthandwelfare.idaho.gov/providers/child-care-providers/child-care-resources>**
- b. Describe any other strategies that the Lead Agency uses to make submitted and approved CCDF Plan and approved Plan amendments available to the public. Check all that apply and describe the strategies below, including any relevant website links as examples.
 - i. ☒ Working with advisory committees. Describe: **The Idaho Child Care Program (ICCP) Advisory Panel is convened quarterly to assist in promoting the continuous improvement of the services provided by the Idaho Child Care Program. The Idaho Child Care Team presents CCDF Plan updates during these quarterly scheduled meetings. The Idaho Child Care Program (ICCP) Advisory Panel offers an external perspective to assist the Department with CCDF Plan development.**
 - ii. ☒ Working with child care resource and referral agencies. Describe: **Child Care Resource Centers (CCRC) contribute to the development of the CCDF Plan. Additionally, they work to keep child care providers informed about programmatic changes. CCRCs are regularly updated on CCDF Plan updates and development.**
 - iii. ☐ Providing translation in other languages. Describe:
 - iv. ☒ Sharing through social media (e.g., Facebook, Instagram, email). Describe: **The State of Idaho published the CCDF Plan and notification of the public comment period on the Idaho Department of Health and Welfare's public website. Additionally, the Idaho Child Care Team worked with IDHW staff and IdahoSTARS staff in charge of public relations and external communication to educate clients and child care providers on CCDF Plan updates and development via Facebook, Instagram, and an email list of all known child care provider email addresses.**
 - v. ☒ Providing notification to key constituents (e.g., parent and family groups, provider groups, advocacy groups, foundations, and businesses). Describe: **The CCDF State Plan draft was posted on the Idaho Department of Health and Welfare (DHW) website and shared with key constituents via email, social media, and staff in charge of external communications. Key constituents who received notice of the plan include Idaho Association for the Education of Young Children, Idaho Voices for Children, regional Health Districts, Head Start Collaboration Director, Idaho Infant Toddler Program, University of Idaho's (UIdaho) Center on Disabilities**

and Human Development (CDHD), Lee Pesky Learning Center, Idaho's United Way programs, Community Council of Idaho, Idaho State Department of Education, Idaho colleges and universities, and the following cities with daycare licensing: Boise, Chubbuck, Coeur d'Alene, Idaho Falls, Lewiston, Moscow, and Pocatello.

- vi. **[x]** Working with Statewide afterschool networks or similar coordinating entities for out-of-school time. Describe: **The CCDF State Plan draft was posted on the Idaho Department of Health and Welfare (DHW) website and shared with the director and staff of the Idaho Out of School Network.**
- vii. **[x]** Direct communication with the child care workforce. Describe: **The State of Idaho published the CCDF Plan and notification of the public comment period on the Idaho Department of Health and Welfare's public website. Additionally, the Idaho Child Care Team worked with IDHW staff and IdahoSTARS staff in charge of public relations and external communication to educate clients and child care providers on CCDF Plan updates and development via Facebook, Instagram, and an email list of all known child care provider email addresses.**
- viii. **[]** Other. Describe:

2 Child and Family Eligibility and Enrollment and Continuity of Care

Stable and reliable child care arrangements facilitate job stability for parents and healthy development of children. CCDF eligibility and enrollment policies can contribute to these goals. Policies and procedures that create barriers to families accessing CCDF, like inaccessible subsidy applications and onerous reporting requirements, interrupt a parent's ability to work and may deter eligible families from participating in CCDF.

To address these concerns, Lead Agencies must provide children with a minimum of 12 months between eligibility determinations, limit reporting requirements during the 12-month period, and ensure eligibility determination and redetermination processes do not interrupt a parent's work or school.

In this section, Lead Agencies will identify how they define eligible children and families and how the Lead Agency's eligibility and enrollment policies support access for eligible children and families.

2.1 Reducing Barriers to Family Enrollment and Redetermination

Lead Agency enrollment and redetermination policies may not unduly disrupt parents' employment, education, or job training activities to comply with the Lead Agency's or designated local entity's requirements. Lead Agencies have broad flexibility to design and implement the eligibility practices that reduce barriers to enrollment and redetermination.

Examples include developing strategies to inform families and their providers of an upcoming redetermination and the information that will be required of the family, pre-populating subsidy renewal forms, having parents confirm that the information is accurate, and/or asking only for the information necessary to make an eligibility redetermination. In addition, Lead Agencies can offer a variety of family-friendly methods for submitting documentation for eligibility redetermination that considers the range of needs for families in accessing support (e.g., use of languages other than English, access to transportation, accommodation of parents working non-traditional hours).

2.1.1 Eligibility practices to reduce barriers to enrollment

- a. Does the Lead Agency implement any of the following eligibility practices to reduce barriers at the time of initial eligibility determination? Check all that apply and describe those elements checked.
- i. ☐ Establishing presumptive eligibility while eligibility is being determined. Describe the policy, including the populations benefiting from the policy, and identify how long the period of presumptive eligibility is:
 - ii. ☒ Leveraging eligibility from other public assistance programs. Describe: **In Idaho, child care subsidy eligibility and payments are processed in the Idaho Benefit Eligibility System (IBES). This is also the eligibility system for Medicaid, Supplemental Nutrition Assistance Program (SNAP), Temporary Assistance for Needy Families (TANF), Aid to the Aged, Blind, and Disabled (AABD). Processing eligibility in a unified system means that changes and information shared with one program can be used to inform others at the request of the applicant. For example, income reported for a SNAP application can be used for a child care subsidy application with no additional effort on the part of the applicant. Applications are assigned to eligibility workers based on timeliness priorities, with work items queued accordingly. The system's design allows the Lead Agency to monitor the entire application flow, tracking which workers handle specific cases to assess timeliness and pinpoint areas for improvement. This structured approach facilitates timely eligibility determinations and continual service enhancement.**
 - iii. ☒ Coordinating determinations for children in the same household (while still ensuring each child receives 12 months of eligibility). Describe: **All subsidy participating children in the same household are determined eligible using the same verifications at the time of application. At this time, children added to an existing, approved benefits case have the same reevaluation date as the household they join. The lead agency will work to update this process within our automated eligibility system with future updates.**
 - iv. ☐ Self-assessment screening tools for families. Describe:
 - v. ☐ Extended office hours (evenings and/or weekends).
 - vi. ☒ Consultation available via phone.
 - vii. ☒ Other. Describe the Lead Agency policies to process applications efficiently and make timely eligibility determinations: **IDHW maintains a highly efficient benefit eligibility system we track and monitor the eligibility determination process to ensure that clients are served as quickly as possible. Idaho will process all ICCP applications within 30 days. (IDAPA 16.06.12.050)**
 - viii. ☐ None.
- b. Does the Lead Agency use an online subsidy application?
- ☐ Yes.
- ☒ No. If no, describe why an online application is impracticable. **IDHW offers multiple ways for a household to provide information addition to online document submission,**

including an option for in-person drop-off of documents, fax, verified collateral calls, and an email option to our customer support inbox. At this time, an online application for child care is under consideration, but an automation timeline has not yet been established due to the complexities of our integrated universal eligibility system.

c. Does the Lead Agency use different policies for families receiving TANF assistance?

☐ Yes. If yes, describe the policies:

☒ No.

2.1.2 Preventing disruption of eligibility activities

a. Identify, where applicable, the Lead Agency's procedures and policies to ensure that parents do not have their employment, education, or job training unduly disrupted to comply with the State's/Territory's or designated local entity's requirements for the redetermination of eligibility. Check all that apply.

i. ☒ Advance notice to parents of pending redetermination.

ii. ☒ Advance notice to providers of pending redetermination.

iii. ☒ Pre-populated subsidy renewal form.

iv. ☒ Online documentation submission.

v. ☒ Cross-program redeterminations.

vi. ☐ Extended office hours (evenings and/or weekends).

vii. ☒ Consultation available via phone.

viii. ☒ Leveraging eligibility from other public assistance programs.

ix. ☐ Other. Describe:

b. Does the Lead Agency use different policies for families receiving TANF assistance?

☐ Yes. If yes, describe the policies:

☒ No.

2.2 Eligible Children and Families

At eligibility determination or redetermination, children must (1) be younger than age 13; (2) reside with a family whose income does not exceed 85 percent of the State's median income (SMI) for a family of the same size and whose family assets do not exceed \$1,000,000; and (3)(a) reside with a parent or parents who are working or attending a job training or educational program (which can include job search) or (b) receive, or need to receive, protective services as defined by the Lead Agency.

2.2.1 Eligibility criteria: age of children served

Lead Agencies may provide child care assistance for children less than 13 years of age, including continuing to provide assistance to children if they turn 13 during the eligibility period. In addition, Lead Agencies can choose to serve children up to age 19 if those children are unable to care for themselves.

- a. Does your Lead Agency serve the full federally allowable age range of children through age 12?
☒ Yes.
☐ No. If no, describe the age range of children served and the reason why you made that decision to serve less than the full range of allowable children.
Note: Do not include children incapable of self-care or under court supervision, who are reported below in 2.2.1b and 2.2.1c.
- b. Does the Lead Agency extend eligibility for CCDF-funded child care to children ages 13 and older but below age 19 who are physically and/or mentally incapable of self-care?
☐ No.
☒ Yes.
 - i. If yes, the upper age is (may not equal or exceed age 19): **18.00**
 - ii. If yes, provide the Lead Agency definition of physical and/or mental incapacity:
Physically or mentally incapable of self-care as verified by a licensed mental health professional or licensed practitioner of the healing arts. (IDAPA 16.06.12.105.03.a.)
- c. Does the Lead Agency extend eligibility for CCDF-funded child care to children ages 13 and older but below age 19 who are under court supervision?
☐ No.
☒ Yes. If yes, and the upper age is (may not equal or exceed age 19): **18.00**
- d. How does the Lead Agency define the following eligibility terms?
 - i. “residing with”: **Child Custody: A child may move from one (1) parent's home to the other parent's home on a regular basis. The child may be a member of either household, but not both households. If the parents cannot agree on the child's household for the child care benefit, the child is included in the household with primary custody. Primary custody is determined by where the child is expected to spend fifty-one percent (51%) or more of the nights during a benefit period. When only one (1) parent applies for ICCP benefits, the child may be included in that parent's household even though they do not have primary physical custody of the child. (IDAPA 16.06.12.105.04)**
 - ii. “in loco parentis”: **Acting *in loco parentis* means a person who acts in place of a parent, assuming care and custody of a child by legal guardianship. (IDAPA 16.06.12.010.14)**

2.2.2 Eligibility criteria: reason for care

Lead Agencies have broad flexibility on the work, training, and educational activities required to qualify for child care assistance. Lead Agencies do not have to set a minimum number of hours for families to qualify for work, training, or educational activities, and there is no requirement to limit authorized child care services strictly based on the work, training, or educational schedule/hours of the parent(s). For example, the Lead Agency can include travel or study time in calculating the amount of needed services.

How does the Lead Agency define the following terms for the purposes of determining CCDF eligibility?

- a. Identify which of the following activities are included in your definition of “working” by checking the boxes below:
- i. ☒ An activity for which a wage or salary is paid.
 - ii. ☒ Being self-employed.
 - iii. ☐ During a time of emergency or disaster, partnering in essential services.
 - iv. ☐ Participating in unpaid activities like student teaching, internships, or practicums.
 - v. ☐ Time for meals or breaks.
 - vi. ☒ Time for travel.
 - vii. ☐ Seeking employment or job search.
 - viii. ☐ Other. Describe:
- b. Identify which of the following activities are included in your definition of “attending job training” by checking the boxes below:
- i. ☒ Vocational/technical job skills training.
 - ii. ☒ Apprenticeship or internship program or other on-the-job training.
 - iii. ☐ English as a Second Language training.
 - iv. ☒ Adult Basic Education preparation.
 - v. ☒ Participation in employment service activities.
 - vi. ☐ Time for meals and breaks.
 - vii. ☒ Time for travel.
 - viii. ☐ Hours required for associated activities such as study groups, lab experiences.
 - ix. ☐ Time for outside class study or completion of homework.
 - x. ☐ Other. Describe:
- c. Identify which of the following diplomas, certificates, degrees, or activities are included in your definition of “attending an educational program” by checking the boxes below:
- i. ☒ Adult High School Diploma or GED.
 - ii. ☒ Certificate programs (12-18 credit hours).
 - iii. ☒ One-year diploma (36 credit hours).
 - iv. ☒ Two-year degree.
 - v. ☒ Four-year degree.
 - vi. ☒ Travel to and from classrooms, labs, or study groups.
 - vii. ☐ Study time.
 - viii. ☐ Hours required for associated activities such as study groups, lab experiences.

- ix. ☐ Time for outside class study or completion of homework.
 - x. ☐ Applicable meal and break times.
 - xi. ☐ Other. Describe:
- d. Does the Lead Agency impose a Lead Agency-defined minimum number of hours of activity for eligibility?
- ☒ No.
- ☐ Yes.
- If yes, describe any Lead Agency-imposed minimum requirement for the following:
- ☐ Work. Describe:
- ☐ Job training. Describe:
- ☐ Education. Describe:
- ☐ Combination of allowable activities. Describe:
- ☐ Other. Describe:
- e. Does the Lead Agency allow parents to qualify for CCDF assistance based on education and training without additional work requirements?
- ☒ Yes.
- ☐ No. If no, describe the additional work requirements:
- f. Does the Lead Agency extend eligibility to specific populations of children otherwise not eligible by including them in its definition of “children who receive or need to receive protective services?”
- Note: A Lead Agency may elect to provide CCDF-funded child care to children in foster care when foster care parents are *not* working or are *not* in education/training activities, but this provision should be included in the Lead Agency’s protective services definition.
- ☐ No. If no, skip to question 2.2.3.
- ☒ Yes. If yes, answer the questions below:
- Provide the Lead Agency’s definition of “protective services” by checking below the sub-populations of children that are included:
- ☒ Children in foster care.
- ☐ Children in kinship care.
- ☐ Children who are in families under court supervision.
- ☒ Children who are in families receiving supports or otherwise engaged with a child welfare agency.
- ☐ Children participating in a Lead Agency’s Early Head Start - Child Care Partnerships program.
- ☐ Children whose family members are deemed essential workers under a

governor-declared state of emergency.

☒ Children experiencing homelessness.

☐ Children whose family has been affected by a natural disaster.

☐ Other. Describe:

- g. Does the Lead Agency waive the income eligibility requirements for cases in which children receive, or need to receive, protective services on a case-by-case basis?

☒ No.

☐ Yes.

- h. Does the Lead Agency waive the eligible activity (e.g., work, job training, education, etc.) requirements for cases in which children receive, or need to receive, protective services on a case-by-case basis?

☐ No.

☒ Yes.

- i. Does the Lead Agency use CCDF funds to provide respite care to custodial parents of children in protective services?

☒ No.

☐ Yes.

2.2.3 Eligibility criteria: deciding entity on family income limits

How are income eligibility limits established?

☒ There is a statewide limit with no local variation.

☐ There is a statewide limit with local variation. Provide the number of income eligibility tables and describe who sets the limits:

☐ Eligibility limits are established locally only. Provide the number of income eligibility tables and describe who sets the limits:

☐ Other. Describe:

2.2.4 Initial eligibility: income limits

- a. Complete the appropriate table to describe family income limits.

- i. Complete the table below to provide the statewide maximum income eligibility percent and dollar limit or threshold:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1	3758.00	57.00	2127.00
2	4914.00	59.00	2876.00

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
3	6070.00	60.00	3626.00
4	7227.00	61.00	4375.00
5	8383.00	61.00	5125.00

- ii. Does the Lead Agency certify that they use other funds if the income eligibility limit percent exceeds 85% SMI?

☒ Not applicable. The Lead Agency does not allow income eligibility limits above 85% SMI.

☐ Yes, the Lead Agency certifies that they use other funds (non-CCDF funds) for families with income that exceeds 85% SMI.

☐ No. The Lead Agency establishes income eligibility limits above SMI and includes CCDF funds to pay for families with income that exceeds 85% SMI. If checked, describe:

- b. Complete the table below if the Lead Agency has local variation in the maximum income eligibility limit. Complete the table for the region/locality with the highest eligibility limit, region/locality with the lowest eligibility limit, and the region/locality that is most populous:

- i. Region/locality with the highest eligibility limit:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

- ii. Region/locality with the lowest eligibility limit:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
3			
4			
5			

iii. Region/locality that is most populous:

Family Size	100% of SMI (\$/Month)	Maximum Initial Eligibility Limit (or Threshold) %	Maximum Initial Eligibility Limit (or Threshold) \$
1			
2			
3			
4			
5			

iv. Does the Lead Agency certify that they use other funds if the income eligibility limit percent exceeds 85% SMI?

☐ Not applicable. The Lead Agency does not allow income eligibility limits above 85% SMI.

☐ Yes, the Lead Agency certifies that they use other funds (not CCDF funds) for families with income that exceeds 85% SMI.

☐ No. The Lead Agency establishes income eligibility limits above 85% SMI and includes CCDF funds to pay for families with income that exceeds 85% SMI. If checked, describe:

c. How does the Lead Agency define “income” for the purposes of eligibility at the point of initial determination? Check all that apply:

- i. ☒ Gross wages or salary.
- ii. ☒ Disability or unemployment compensation.
- iii. ☒ Workers’ compensation.
- iv. ☒ Spousal support, child support.
- v. ☒ Survivor and retirement benefits.
- vi. ☒ Rent for room within the family’s residence.
- vii. ☒ Pensions or annuities.
- viii. ☒ Inheritance.

- ix. ☐ Public assistance.
- x. ☒ Other. Describe: **All gross earned and unearned income is counted in determining eligibility and the childcare benefit amount, unless specifically excluded. (IDAPA 16.06.12.071) The following sources of income are not counted as family income (IDAPA 16.06.12.072):**
- **Earned income of a dependent child under age 18, unless the child is a parent who is seeking or receiving child care benefits**
 - **Income received for person not residing with the family**
 - **All educational funds including grants, scholarships, an AmeriCorps Education Award, and federal and state work-study income**
 - **Assistance to meet a specific need from other organizations and agencies**
 - **Non-recurring lump sum income**
 - **Loans - money received that is to be repaid**
 - **TAFI and AABD benefits**
 - **Foster care payments**
 - **Living allowances, wages and stipends paid to AmeriCorps or VISTA volunteers under 42 U.C.S. 5044, P.L. 93-113, Title IV, Section 404(g)**
 - **Income tax refunds and earned income tax credits**
 - **Reimbursements from employers for work-related travel**
 - **Income received from a tribe for any purpose other than direct wages**
 - **Income of licensed foster parents when determining eligibility for a foster child**
 - **Adoption assistance payments**
 - **Wages paid by the Census Bureau for temporary employment related to U.S.Census activities; excluded for a time period not to exceed six (6) months during the regularly scheduled ten-year U.S. Census**
 - **Office of Refugee Resettlement assistance**
 - **Workforce Investment Act (WIA) benefits or Workforce Innovation and Opportunity Act (WIOA) benefits**

d. What is the effective date for these income eligibility limits? **October 01, 2023**

- e. Income limits must be established and reported in terms of current SMI based on the most recent data published by the Bureau of the Census, even if the federal poverty level is used in implementing the program.

What federal data does the Lead Agency use when reporting the income eligibility limits?

☒ LIHEAP. If checked, provide the publication year of the LIHEAP guideline estimates used by the Lead Agency: **2023**

☐ Other. Describe:

- f. Provide the direct URL/website link, if available, for the income eligibility limits.
<https://healthandwelfare.idaho.gov/services-programs/children-families/apply-child-care-assistance>

2.2.5 Income eligibility: irregular fluctuations in earnings

Lead Agencies must take into account irregular fluctuations in earnings in initial eligibility determination and redetermination processes. The Lead Agency must ensure that temporary increases in income, including temporary increases that can result in a monthly income exceeding 85 percent of SMI from seasonal employment or other temporary work schedules, do not affect eligibility or family co-payments.

Check the processes that the Lead Agency uses to take into account irregular fluctuations in earnings.

- i. ☒ Average the family's earnings over a period of time (e.g., 12 months).

Identify the period of time **If changes in income have occurred or are anticipated, past income cannot be used as an indicator of anticipated income. If income changes and income received in the past thirty (30) days does not reflect anticipated income, the Department can use the household income received over a longer period to anticipate income. If income changes seasonally, the Department can use the household income from the last season, comparable to the certification period, to anticipate income. (IDAPA 16.06.12.076)**

- ii. ☒ Request earning statements that are most representative of the family's monthly income.

- iii. ☐ Deduct temporary or irregular increases in wages from the family's standard income level.

- iv. ☐ Other. Describe the other ways the Lead Agency takes into account irregular fluctuations in earnings:

2.2.6 Family asset limit

- a. When calculating income eligibility, does the Lead Agency ensure each eligible family does not have assets that exceed \$1,000,000?

☒ Yes.

☐ No. If no, describe:

- b. Does the Lead Agency waive the asset limit on a case-by-case basis for families defined as

receiving, or in need of, protective services?

☒ No.

☐ Yes. If yes, describe the policy or procedure:

2.2.7 Additional eligibility criteria

Aside from the eligibility conditions or rules which have been described in 2.2.1 – 2.2.6, is any additional eligibility criteria applied during:

- a. ☒ Eligibility determination? If checked, describe: **Eligible Child (IDAPA 16.06.12.105): A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions:**

1. Immunizations Requirements - A child must be immunized in accordance with IDAPA 16.02.11, "Immunization Requirements for Children Attending Licensed Daycare Facilities in Idaho." Child care benefits can continue during a reasonable period necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

2. Citizenship or Alien Status Requirement - A child must be a citizen or living lawfully in the United States.

- b. ☒ Eligibility redetermination? If checked, describe: **Eligible Child (IDAPA 16.06.12.105): A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions:**

1. Immunizations Requirements - A child must be immunized in accordance with IDAPA 16.02.11, "Immunization Requirements for Children Attending Licensed Daycare Facilities in Idaho." Child care benefits can continue during a reasonable period necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

2. Citizenship or Alien Status Requirement - A child must be a citizen or living lawfully in the United States.

2.2.8 Documentation of eligibility determination

Lead Agencies must document and verify that children receiving CCDF funds meet eligibility criteria at the time of eligibility determination and redetermination.

Check the information that the Lead Agency documents and verifies at initial determination and redetermination and describe what information is required and how often.

Required at Initial Determination	Required at Redetermination	Description
[x]	[]	Applicant identity. Describe how you verify: For all benefits programs, identity verification is required only once, unless later information raises a question about the individual's identity. Idaho has an automated verification system that is used by Supplemental Nutrition Assistance Program and Medicaid program workers called eVerif-I. This system interfaces with other systems such as vital statistics, Social Security, Dept. of Labor, Child Support, and other automated systems that is used to verify income and identity of all family members.
[x]	[]	Applicant's relationship to the child. Describe how you verify: Household declaration on the application or with information in other benefit systems. Only once during certification period is this verified, at time of application.
[x]	[]	Child's information for determining eligibility (e.g., identity, age, citizen/immigration status). Describe how you verify: Idaho has an automated verification system that is used by SNAP and Medicaid program workers called e-Verif I. This system interfaces with other systems such as Vital Statistics, Social Security, Department of Labor, Child Support, and other automated systems that are used to verify income and identity of all family members.
[x]	[x]	Work. Describe how you verify: Qualifying activities are verified with a school schedule, training schedule, and/or most recent 30 days of wages and working hours. Activities are verified every 12 months with scanned copies of all verifications stored in eCase. Verifications will be requested after 12 months, or if a change is reported by the client. For foster children, staff in the Division of Family and Community Services (FACS) verify a foster parent's activity hours. Unless a discrepancy is found, child care eligibility staff do not request additional verifications.

Required at Initial Determination	Required at Redetermination	Description
[x]	[x]	Job training or educational program. Describe how you verify: Qualifying activities are verified with a school schedule, training schedule, and/or most recent 30 days of wages and working hours. Activities are verified every 12 months with scanned copies of all verifications stored in eCase. Verifications will be requested after 12 months, or if a change is reported by the client. For foster children, staff in the Division of Family and Community Services (FACS) verify a foster parent's activity hours. Unless a discrepancy is found, child care eligibility staff do not request additional verifications.
[x]	[x]	Family income. Describe how you verify: Family income is verified with most recent pay records and/or wage stubs, work verification forms completed by the employer, employer statements verifying income, collateral contacts, self-employment documents such as tax returns, and/or the Work Number. Income is verified every 12 months with scanned copies of all verifications stored in eCase. For foster children, household income is not counted.
[x]	[x]	Household composition. Describe how you verify: Household declaration on the application or with information in comprehensive benefit systems. This is verified every 12 months.
[x]	[x]	Applicant residence. Describe how you verify: Household declaration on the application or with information in other benefit systems. For foster children, staff in the Division of Family and Community Services (FACS) verify a foster family's residence. Unless a discrepancy is found, child care eligibility staff do not request additional verifications. This is verified every 12 months.
[]	[]	Other. Describe how you verify: NA

2.2.9 Exception to TANF work requirements

Lead Agencies must ensure that families with young children participating in TANF will be informed of their right not to be sanctioned under the TANF work requirement if the custodial parent has a demonstrated inability to obtain child care for a child under age six, in accordance with Section 407(e)(2) of the Social Security Act.

- a. Identify the TANF agency that established these criteria or definitions: **The Idaho Department of Health and Welfare (IDHW) is responsible for TANF.**

- b. Provide the following definitions established by the TANF agency:
 - i. **“Appropriate child care”:** A licensed provider or license exempt provider as outlined in IDAPA, or client preferred informal child care provided by family, friend, or neighbor, with available hours of operations that align with the participant’s work schedule.
 - ii. **“Reasonable distance”:** Total travel time from the participant’s residence to the child care facility or home by public or private transportation is within 1 hour.
 - iii. **“Unsuitability of informal child care”:** The suitability of an informal child care arrangement is determined on a case-by case basis, and may be considered unsuitable if the child care does not offer a safe and stable environment or adequate care and supervision.
 - iv. **“Affordable child care arrangements”:** To be ☐affordable,☒ a family’s out of pocket child care costs, after subsidy payments and any provider applied discounts, do not exceed 50% of the client’s cash benefit amount.
- c. How are parents who receive TANF benefits informed about the exception to the individual penalties associated with the TANF work requirements?
 - i. ☐ In writing
 - ii. ☒ Verbally
 - iii. ☐ Other. Describe:

2.3 Prioritizing Services for Vulnerable Children and Families

Lead Agencies must give priority for child care assistance to children with special needs, families with very low incomes (considering family size), and children experiencing homelessness. A Lead Agency has the flexibility to prioritize other populations of children.

Note: Statute defines children with disabilities, and CCDF rule gives flexibility to Lead Agencies to include vulnerable populations in their definition of children with special needs.

CCDF defines “child experiencing homelessness” as a child who is homeless, as defined in Section 725 of Subtitle VII-B of the McKinney-Vento Act (42 U.S.C. 11434a).

2.3.1 Lead Agency definition of priority groups

Describe how the Lead Agency defines:

- d. **“Children with special needs.” Any child with physical, mental, emotional, behavioral disabilities, or developmental delays identified on an individual education plan (IEP) or an individualized family service plan (IFSP). (IDAPA 16.06.12.011.10)**
- e. **“Families with very low incomes.” Families with very low incomes are families participating in Temporary Assistance for Families in Idaho (TAFI).**

2.3.2 Prioritization of child care services

Identify how the Lead Agency will prioritize child care services for the following children and families.

a. Complete the table below to indicate how the identified populations are prioritized.

Population Prioritized	Prioritize for enrollment in child care services	Serve without placing on waiting list	Waive co-payments as described in 3.3.1	Pay higher rate for access to higher quality care	Use grants or contracts to reserve spots	Other
Children with special needs	☐	☒	☐	☐	☐	☐ Describe:
Families with very low incomes	☐	☒	☒	☐	☐	☐ Describe:
Children experiencing homelessness, as defined by CCDF	☐	☒	☐	☐	☐	☒ Describe: Families experiencing homelessness may use their participation with any shelter, agency, or program supporting the development of self sufficiency or rehousing as a qualifying activity.
(Optional) Families receiving TANF, those attempting to transition off TANF, and those at risk of becoming dependent on TANF	☐	☒	☒	☐	☐	☐ Describe:

a. Does the Lead Agency define any other priority groups?

☒ No.

☐ Yes. If yes, identify the populations prioritized and describe how the Lead Agency prioritizes services:

2.3.3 Enrollment and grace period for children experiencing homelessness

Lead Agencies must allow (after an initial eligibility determination) children experiencing homelessness to receive CCDF services while required eligibility documentation is obtained.

Lead Agencies must establish a grace period that allows children experiencing homelessness and children in foster care to receive CCDF assistance while providing their families with a reasonable

time to take any necessary actions to comply with State, Territory, or local immunization and other health and safety requirements. The length of such a grace period must be established in consultation with the State, Territorial, or Tribal public health agency.

Note: Any payment for such a child during the grace period may not be considered an error or improper payment.

- a. Describe the strategies to allow CCDF enrollment of children experiencing homelessness while required eligibility documentation is obtained: **The Idaho Department of Health and Welfare (IDHW) Self-Reliance Specialists (SRS) gather and record information for Child Care to process applications. For families experiencing homelessness that are not working, training, or participating in education, community partners/agencies may provide families with the child care activity form showing that the family is participating in a work or training program offered by a partner or agency which allows approval of child care. Once verified, children may enroll in child care while other required components are completed and or obtained. According to the administrative code coordinated by our Division of Public Health (IDAPA 16.02.11) parents and guardians of children attending daycare have up to fourteen days to comply with the immunization requirements (16.06.12.101 Compliance). A child who has received at least one dose of each required vaccine and is currently on schedule for subsequent immunizations may conditionally attend daycare when a schedule of intended immunizations form is provided (16.02.11.102). Additionally, in Idaho, any parent or guardian may exempt their children from daycare immunization requirements with a signed statement (IDAPA 16.02.11.110.02).**
- b. Describe the grace period for each population below and how it allows them to receive CCDF assistance while providing their families with a reasonable time to take any necessary actions to comply with immunization and other health and safety requirements.
 - i. Provide the policy for a grace period for:

Children experiencing homelessness: **A child must be immunized in accordance with IDAPA 16.02.11, §Immunization Requirements for Licensed Daycare Facility Attendees.§ Immunization policies allow all children to receive care for up to 14 days before completing immunization requirements. (IDAPA 16.06.12.105.01)**

Children who are in foster care: **A child must be immunized in accordance with IDAPA 16.02.11, §Immunization Requirements for Licensed Daycare Facility Attendees.§ Immunization policies allow all children to receive care for up to 14 days before completing immunization requirements. (IDAPA 16.06.12.105.01)**
 - ii. Does the Lead Agency certify that the length of the grace period was established in consultation with the State, Territorial, or Tribal public health agency?

☒ Yes.

☐ No. If no, describe:
- c. Describe how the Lead Agency coordinates with licensing agencies and other relevant State, Territorial, Tribal, and local agencies to provide referrals and support to help families with children receiving services during a grace period comply with immunization and other health and safety requirements: **IdahoSTARS supports child care providers**

serving children participating in foster care or experiencing homelessness by offering resources to connect families to agencies that provide immunizations.

2.4 Lead Agency Outreach to Families Experiencing Homelessness, Families with Limited English Proficiency, and Persons with Disabilities

The Lead Agency must conduct outreach and provide services to families with limited English proficiency, families experiencing homelessness, and persons with disabilities.

2.4.1 Families with limited English proficiency and persons with disabilities: outreach and services

- a. Check the strategies the Lead Agency or partners utilize to conduct outreach and provide services to eligible families with limited English proficiency. Check all that apply.
 - i. ☒ Application in languages other than English (application and related documents, brochures, provider notices).
 - ii. ☒ Informational materials in languages other than English.
 - iii. ☒ Website in languages other than English.
 - iv. ☐ Lead Agency accepts applications at local community-based locations.
 - v. ☒ Bilingual caseworkers or translators available.
 - vi. ☒ Bilingual outreach workers.
 - vii. ☒ Partnerships with community-based organizations.
 - viii. ☒ Collaboration with Head Start, Early Head Start, or Migrant and Seasonal Head Start.
 - ix. ☒ Home visiting programs.
 - x. ☐ Other. Describe:
- b. Check the strategies the Lead Agency or partners utilize to conduct outreach and provide services to eligible families with a person(s) with a disability. Check all that apply.
 - i. ☐ Applications and public informational materials available in braille and other communication formats for access by individuals with disabilities.
 - ii. ☒ Websites that are accessible (e.g., Section 508 of the Rehabilitation Act).
 - iii. ☐ Caseworkers with specialized training/experience in working with individuals with disabilities.
 - iv. ☐ Ensuring accessibility of environments and activities for all children.
 - v. ☒ Partnerships with State and local programs and associations focused on disability- related topics and issues.
 - vi. ☐ Partnerships with parent associations, support groups, and parent-to-parent support groups, including the Individuals with Disabilities Education Act (IDEA) federally funded Parent Training and Information Centers.
 - vii. ☒ Partnerships with State and local IDEA Part B, Section 619 and Part C providers and agencies.

- viii. ☐ Availability and/or access to specialized services (e.g., mental health, behavioral specialists, therapists) to address the needs of all children.
- ix. ☐ Other. Describe:

2.4.2 Families experiencing homelessness: Outreach and technical assistance efforts

- a. Check, where applicable, the procedures used to conduct outreach for children experiencing homelessness and their families.
 - i. ☐ Lead Agency accepts applications at local community-based locations.
 - ii. ☒ Partnerships with community-based organizations.
 - iii. ☒ Partnering with homeless service providers, McKinney-Vento liaisons, and others who work with families experiencing homelessness to provide referrals to child care.
 - iv. ☐ Other. Describe:
- b. The Lead Agency must provide training and technical assistance (TA) to providers and appropriate Lead Agency (or designated entity) staff on identifying and serving children and families experiencing homelessness.
 - i. Describe the Lead Agency's training and TA efforts for providers in identifying and serving children and their families experiencing homelessness. **During the Initial Idaho Child Care Program Orientation, IdahoSTARS staff explain the responsibilities of childcare providers towards the families they serve, emphasizing the importance of offering support and being sensitive to the needs of families who are experiencing homelessness. Quality Child Care Consultants provide support and encourage programs to write policies around supporting all families including those who are experiencing homelessness.**
 - ii. Describe the Lead Agency's training and TA efforts for Lead Agency (or designated entity) staff in identifying and serving children and their families experiencing homelessness. **IdahoSTARS RISE Application has free eLearning. Training is available to anyone who creates an account in the system. The training catalog includes the training When Families Experience Homelessness/ Cuando las Familias Experimentan Falta de Vivienda: This training has a special focus on families experiencing homelessness and how child care providers can support them. The strategies identified during the training can help any family under stress. Objectives:**
 - 1. Recognize types of homelessness
 - 2. Explore Idaho homelessness statistics
 - 3. Identify strength-based strategies to support children and families. This eLearning training is available in English and Spanish in online modules and offered with interpreters in a live Zoom webinar format to meet the needs of other language groups.

2.5 Promoting Continuity of Care

Lead Agencies must consider children’s development and promote continuity of care when authorizing child care services and must establish a minimum 12-month period for each child, both at the initial eligibility determination and redetermination.

2.5.1 Children’s development

Describe how the Lead Agency’s eligibility, enrollment, reporting, and redetermination policies promote continuity of care in order to support children’s development. **The Idaho Department of Health and Welfare (IDHW) establishes an eligibility period of 12-months for all subsidy participating families. At redetermination a second eligibility threshold of 200% Federal Poverty Level (FPL) allows for opportunity for job growth and advancement while maintaining continuity of care for children as data from Idaho's current subsidy families showed that at the time of redetermination, most families served went over income yet stayed under 200% of FPL. Therefore, we believe that families served can continue making advancements while children remain in consistent care arrangements throughout the second year.**

2.5.2 Minimum 12-month eligibility

Lead Agencies must establish a minimum 12-month eligibility period for each child, both at the initial eligibility determination and at redetermination to support continuity in child care assistance and reduce barriers to families retaining eligibility. This requirement is:

- Regardless of changes in income, Lead Agencies may not terminate CCDF assistance during the minimum 12-month period if a family has an increase in income that exceeds the Lead Agency’s income eligibility threshold but not the federal threshold of 85 percent of SMI; and
- Regardless of temporary changes in participation in work, training, or educational activities.
 - a. Does the Lead Agency certify that their policies or procedures provide a minimum 12-month eligibility period for each child at initial eligibility determination?
☐ Yes.
☒ No. If no, describe: **This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address 12 month eligibility for families and 12 month eligibility for children added to an existing, eligible case. IDHW will request technical assistance regarding the implementation of this requirement.**
 - b. Does the Lead Agency certify that its definition of “temporary change” includes each of the minimum required elements?
 1. Any time-limited absence from work for an employed parent due to such reasons as the need to care for a family member or an illness.
 2. Any interruption in work for a seasonal worker who is not working between regular industry work seasons.
 3. Any student holiday or break for a parent participating in a training or educational program.
 4. Any reduction in work, training, or education hours, as long as the parent is still working or attending a training or educational program.

5. Any cessation of work or attendance at a training or educational program not listed above. In these cases only, Lead Agencies may establish a period of 3 months or longer.
6. Any change in age, including a child turning 13 years old during the minimum 12-month eligibility period.
7. Any changes in residency within the State or Territory.

☒ Yes.

☐ No. If no, describe:

- c. Are the policies different for redetermination?

☒ No.

☐ Yes. If yes, provide the additional/varying policies for redetermination:

2.5.3 Job search and continued assistance

- a. Does the Lead Agency consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination and/or at the minimum 12-month eligibility redetermination? (Note: If yes, Lead Agencies must provide a minimum of 3 months of job search.) Check all that apply:
- i. ☐ Yes. The Lead Agency does consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination. If yes, describe:
 - ii. ☐ Yes. The Lead Agency does consider seeking employment (engaging in a job search) as an eligible activity at redetermination. If yes, describe:
 - iii. ☒ No. The Lead Agency does not consider seeking employment (engaging in a job search) as an eligible activity at initial eligibility determination or redetermination.
- b. Does the Lead Agency continue assistance during the minimum 12-month eligibility period when a parent has a non-temporary loss or cessation of eligible activity?
- ☐ Yes. The Lead Agency continues assistance.
- ☒ No, the Lead Agency discontinues assistance.
- i. If no, describe the Lead Agency's policies for discontinuing assistance due to a parent's non-temporary change: **An eligible family who loses or ceases its qualifying activity, may continue to receive assistance for up to three months to engage in a job search and resume work, or resume attendance at a job training or educational program (IDAPA 16.06.12.202).**
 - ii. If no, describe what specific actions/changes trigger the job-search period after each such loss or cessation: **Losing or ceasing a qualifying activity triggers the job search period.**
 - iii. If no, how long is the job-search period where a family can continue assistance (must be at least 3 months)? **Three months**
- c. The Lead Agency may discontinue assistance prior to the next minimum 12-month redetermination in the limited circumstances listed below. Check and provide the policy for all circumstances in which the Lead Agency chooses to discontinue assistance prior to

the next minimum 12-month redetermination:

- i. ☐ Not applicable.
- ii. ☐ Excessive unexplained absences despite multiple attempts by the Lead Agency or designated entity to contact the family and provider, including the prior notification of a possible discontinuation of assistance.

Provide the Lead Agency's policy defining the number of unexplained absences identified as excessive:

- iii. ☒ A change in residency outside of the State or Territory.

Provide the Lead Agency's policy for a change in residency outside the State or Territory: **The family must live in the state of Idaho and have no immediate intention of leaving (IDAPA 16.06.12.102). Child care benefits will be closed if the family moves out of state.**

- iv. ☒ Substantiated fraud or intentional program violations that invalidate prior determinations of eligibility.

Provide the Lead Agency's definition of fraud/intentional program violations that lead to discontinued assistance: **An IPV is an intentionally false or misleading action or statement as identified below in Subsections 702.01 through 702.07 of this rule.**

An IPV is established when a family member or the child care provider admits the IPV in writing and waives the right to an administrative hearing, or when determined by an administrative hearing, a court decision, or through deferred adjudication. Deferred adjudication exists when the court defers a determination of guilt because the accused family member or child care provider meets the terms of a court order or an agreement with the prosecutor.

01. False Statement. An individual makes a false statement to the Department, either orally or in writing, in order to participate in the Idaho Child Care Program.

02. Misleading Statement. An individual makes a misleading statement to the Department, either orally or in writing, to participate in the Idaho Child Care Program.

03. Misrepresentation of Fact. An individual misrepresents one (1) or more facts to the Department, either orally or in writing, to participate in the Idaho Child Care Program.

04. Concealing Fact. An individual conceals or withholds one (1) or more facts to participate in the Idaho Child Care Program.

05. Non-Compliance With Rules and Regulations. An individual fails

**repeatedly or substantially to comply with this chapter of rules.
(16.06.12.702)**

2.5.4 Reporting changes during the minimum 12-month eligibility period

Lead Agencies may only require families to report changes that impact a family's eligibility, including only if the family's income exceeds 85 percent of the SMI, taking into account irregular fluctuations in income, or there is a non-temporary change in the parent's work, training, or education status, during the 12-month eligibility period. Lead Agencies may also require families to report that enable the lead agency to contact the family or pay providers, such as a new telephone number or address.

Note: The response below should exclude reporting requirements for a graduated phase-out, which are described in question 2.5.5.

Does the Lead Agency limit what families must report during the 12-month eligibility period to the changes described above?

☐ Yes.

☒ No. If no, describe: **As recorded in our preliminary notice dated April 2, 2024, Idaho Administrative Code (IDAPA) requires subsidy participants to report permanent income changes if the income exceeds 130% of the Federal Poverty Guides for a family of the same size. This was an administrative oversight during previous rulemaking activities. The Lead Agency anticipated requesting a rule change during the 2024 legislative session however we are now under a rulemaking moratorium. Idaho will need additional time to submit a rule change to our legislature for their review during the 2025 session.**

2.5.5 Policies and procedures for graduated phase-out of assistance at redetermination

Lead Agencies that establish initial family income eligibility below 85 percent of SMI must provide a graduated phase-out of assistance for families whose income has increased above the Lead Agency's initial income threshold at the time of redetermination but remains below the federal threshold of 85 percent of SMI.

Lead Agencies that provide a graduated phase-out must implement a two-tiered eligibility threshold, with the second tier of eligibility (used at the time of eligibility redetermination) to be set at:

- (i) 85 percent of SMI for a family of the same size; or,
- (ii) An amount lower than 85 percent of SMI for a family of the same size but above the Lead Agency's initial eligibility threshold that:
 - (A) Takes into account the typical household budget of a family with a low income
 - (B) Provides justification that the second eligibility threshold is:
 - (1) Sufficient to accommodate increases in family income over time that are typical for workers with low incomes and that promote and support family economic stability
 - (2) Reasonably allows a family to continue accessing child care services without unnecessary disruption

At redetermination, a child must be considered eligible if their parents are participating in an eligible activity even if their income exceeds the Lead Agency's initial eligibility income limit as

long as their income does not exceed the second tier of eligibility. Note that once determined eligible, the child must be considered eligible for a full minimum 12-month eligibility period, even if the parents' income exceeds the second tier of eligibility during the eligibility period, as long as it does not exceed 85 percent of SMI.

A child eligible for services via the graduated phase-out of assistance is considered eligible under the same conditions as other eligible children with the exception of the co-payment restrictions, which do not apply to a graduated phase-out. To help families transition from child care assistance, Lead Agencies may gradually adjust co-payment amounts in proportion to a family's income growth for families whose children are determined eligible under a graduated phase-out. Lead Agencies may require additional reporting on changes in family income but must still ensure that any additional reporting requirements do not constitute an undue burden on families.

Check and describe the option that best identifies the Lead Agency's policies and procedures regarding the graduated phase-out of assistance.

- a. ☐ Not applicable. The Lead Agency sets its initial eligibility threshold at 85 percent of SMI and therefore is not required to provide a graduated phase-out period. (If checked, skip to question 3.1.1.)
- b. ☐ The Lead Agency sets the second tier of eligibility at 85 percent of SMI. If checked, describe the policies and procedures:
 - i. ☐ Lead Agency adjusts the family's co-pay during the graduated phase-out period. If checked, describe how the Lead Agency gradually adjusts co-payment for families under a graduated phase-out period in proportion to a family's income growth. Include information on the percentage or amount of change made in the co-payment during graduated phase-out:
 - ii. ☐ Lead Agency requires additional reporting requirements during the graduated phase-out period. If checked, describe:
- c. ☒ The Lead Agency sets the second tier of eligibility at an amount lower than 85 percent of SMI for a family of the same size but above the Lead Agency's initial eligibility threshold. If checked, provide the following information:
 - i. Provide the income level (\$/month) and the percent of SMI for the second tier of eligibility for a family of three: **200% of Federal Poverty Limit or \$4,144 a month for a family of three, according to the 2023 Federal Poverty Guidelines. This is 68% of 100% of SMI.**
 - ii. Describe how the second eligibility threshold takes into account the typical household budget of a low-income family: **Idaho's second eligibility threshold, set at 200% of the Federal Poverty Limit (FPL), was determined through a review of income levels at time of initial eligibility and at the past year's redetermination. When determining the exit threshold, data showed that the majority of families served continued to be under 200% of the Federal Poverty Limit at redetermination, therefore, being able to support families continuously as they work toward increased wages and or improved education levels, leading to future work opportunities. The Idaho Department of Health and Welfare also excludes various income to support families as defined in IDAPA 16.06.12.072. 072.**

- iii. Describe how the second eligibility threshold is sufficient to accommodate increases in family income over time that are typical for low-income workers and that promote and support family economic stability: **The second eligibility threshold of 200% FPL allows for opportunity for job growth and advancement while maintaining continuity of care for children by accommodating increases in family income over time without exceeding the exit threshold.**
- iv. Describe how the second eligibility threshold reasonably allows a family to continue accessing child care services without unnecessary disruption: **Increases in household income are not acted on until redetermination period unless income exceeds 85 percent of SMI.**
- v. ☐ Lead Agency adjusts the family's co-pay during the graduated phase-out period. If checked, describe how the Lead Agency gradually adjusts co-payment for families under a graduated phase-out period in proportion to a family's income growth. Include information on the percentage or amount of change made in the co-payment during graduated phase-out:
- vi. ☐ Lead Agency requires additional reporting requirements during the graduated phase-out period. If checked, describe:

3 Child Care Affordability

CCDF subsidies make child care more affordable for eligible families, providing access to a greater range of child care options that allow parents to work, go to school, or enroll in training and they allow parents to access higher quality care options that better support children's development. CCDF requires some families participating in CCDF to pay an affordable co-payment set by the Lead Agency to cover a part of their care. But co-payments can be a significant and destabilizing financial strain on family budgets and a barrier to parent employment, and the CCDBG Act requires that the co-payment amount not be a barrier to families participating in CCDF. Lead Agencies may not set parent co-payments above 7% of family income regardless of gradual phase-out policies and regardless of the number of children receiving assistance. Lead Agencies are encouraged to set co-payments much lower than 7% to make child care more affordable for more families and have broad flexibility to waive co-payments for too many participants. Lead Agencies must ensure that the total payment to a child care provider is not reduced because of family's lowered or waived co-payment.

In this section, Lead Agencies will identify how they determine an eligible family's co-payment, the policies in place to waive or ensure co-payments are affordable for families, and how the Lead Agency improves access for children and families in economically and/or socially marginalized communities.

3.1 Family Co-payments

Lead Agencies must establish and periodically revise a sliding-fee scale for families receiving CCDF services that varies based on income and the size of the family to determine each family's contribution (i.e., co-payment) and does not create a barrier to receiving CCDF assistance. In addition to income and the size of the family, the Lead Agency may use other factors as appropriate when determining family contributions/co-payments. Lead Agencies may not use price of care or amount of subsidy payment in determining co-payments. Lead Agencies must

ensure that the total payment to a child care provider is not reduced because of family's lowered or waived co-payment.

3.1.1 Family co-payment

Lead Agencies may not charge any family more than 7% of a family's gross income, regardless of the number of children participating in CCDF.

- a. What is the maximum percent of a family's gross income any family could be charged as a co-payment? **At this time, co-payments in Idaho are a flat rate assessed per child based on household size, household income, and qualifying activity hours. While Idaho looks forward to updating our copayment calculations there is not currently a maximum percentage of a family's gross income.**
- b. Does the Lead Agency certify that their sliding fee scales are always based on income and family size (regardless of how many different scales they may use)?

☒ Yes.

☐ No. If no, describe:

3.1.2 Sliding fee scale

Provide the CCDF co-payments for eligible families in the table(s) below according to family size for one child in care.

- a. Is the sliding fee scale set statewide?
☒ Yes.
☐ No. If no, describe how the sliding fee scale is set:
- b. Complete the table below. If the sliding fee scale is not set statewide, complete the table for the most populous locality:

	<i>A</i>	<i>B</i>	<i>C</i>	<i>D</i>	<i>E</i>	<i>F</i>
Family Size	Lowest monthly income at initial eligibility where the family is first charged a co-pay (greater than \$0).	What is the monthly co-payment for a family of this size based on the income level in (A)?	What percentage of income is the co-payment in (B)?	Highest monthly income at initial eligibility where a family is charged a co-pay before a family is no longer eligible.	What is the monthly co-payment for a family of this size based on the income level in (D)?	What percentage of income is this co-payment in (E)?
1	0.00	10.00	200.00	2127.00	75.00	3.50
2	0.00	10.00	200.00	2876.00	75.00	2.60
3	0.00	10.00	200.00	3626.00	75.00	2.00
4	0.00	10.00	200.00	4375.00	75.00	1.70

	<i>A</i>	<i>B</i>	<i>C</i>	<i>D</i>	<i>E</i>	<i>F</i>
Family Size	Lowest monthly income at initial eligibility where the family is first charged a co-pay (greater than \$0).	What is the monthly co-payment for a family of this size based on the income level in (A)?	What percentage of income is the co-payment in (B)?	Highest monthly income at initial eligibility where a family is charged a co-pay before a family is no longer eligible.	What is the monthly co-payment for a family of this size based on the income level in (D)?	What percentage of income is this co-payment in (E)?
5	0.00	10.00	200.00	5125.00	75.00	1.40

- c. What is the effective date of the sliding-fee scale(s)? **October 01, 2023**
- d. Provide the link(s) to the sliding-fee scale(s):
<https://healthandwelfare.idaho.gov/providers/child-care-providers/child-care-resources>
- e. Does the Lead Agency allow providers to charge families additional amounts above the required co-payment in instances where the provider's price exceeds the subsidy payment?
- ☐ No.
- ☒ Yes.
- If yes:
- i. Provide the rationale for the Lead Agency's policy to allow providers to charge families additional amounts above the required co-payment, including a demonstration of how the policy does not provide a barrier and promotes affordability and access for families: **When defining policies for families and providers, we review balancing both continuity for children with policies that are fair to providers. The decision to allow additional rate above the required co-payment is a combination of both levers. Ensuring families have choice and access, while also allowing child care businesses to identify their costs as such to promote and provide quality care. Providers and advocacy groups continue to support the practice of allowing childcare providers to charge above the Local Market Rate, as demonstrated in comments provided as feedback to this plan. Families participating with the Idaho Child Care Program (ICCP) continue to access every type of child care provider, including high quality providers participating in Idaho's QRIS program and NAEYC accreditation. Ongoing Market Rate updates continue to ensure that ICCP participating providers will maintain their participation with ICCP, and families will continue to be able to access Idaho's best child care choices.**
- ii. Provide data (including data on the size and frequency of such amounts) on the extent to which CCDF providers charge additional amounts to families: **Idaho retains this information in our benefit eligibility system and is in the process of**

gathering and evaluating this information, an amendment will be made once the data evaluation is complete.

3.2 Calculation of Co-Payment

Lead agencies must calculate a family's contribution (or co-payment), taking into account income and family size, and Lead Agencies may choose to consider other factors in their calculation.

3.2.1 Family co-payment calculation

- a. How is the family's contribution calculated, and to whom is it applied? Check if the fee is a dollar amount or if the fee is a percent of income below, and then check all that apply under the selection, as appropriate.

- i. ☒ The fee is a dollar amount and (check all that apply):

☒ The fee is per child, with the same fee for each child.

☐ The fee is per child and is discounted for two or more children.

☐ The fee is per child up to a maximum per family.

☐ No additional fee is charged after a certain number of children.

☐ The fee is per family.

☐ The contribution schedule varies because it is set locally/regionally (as indicated in 1.2.1). Describe:

☐ Other. Describe:

- ii. ☐ The fee is a percent of income and (check all that apply):

☐ The fee is per child, with the same percentage applied for each child.

☐ The fee is per child, and a discounted percentage is applied for two or more children.

☐ The fee is per child up to a maximum per family.

☐ No additional percentage is charged after a certain number of children.

☐ The fee is per family.

☐ The contribution schedule varies because it is set locally/regionally (as indicated in 1.2.1). Describe:

☐ Other. Describe:

- b. Does the Lead Agency use other factors in addition to income and family size to determine each family's co-payment? (Lead Agencies may not use price of care or amount of subsidy payment in determining co-payments).

☐ No.

☒ Yes.

If yes, check and describe those additional factors below:

- i. ☐ Number of hours the child is in care. Describe:

- ii. ☐ Quality of care (as defined by the Lead Agency). Describe:
- iii. ☒ Other. Describe: **For families that qualify using hours of education, with working hours of 10 per week or less, the copayment is assessed at \$35 for part time hours, and \$75 for full time qualifying hours.**
- c. Describe any other policies the Lead Agency uses in the calculation of family co-payment to ensure it does not create a barrier to access. Check all that apply:
 - i. ☐ Base co-payments on only a portion of the family's income. For instance, only consider the family income over the federal poverty level.
 - ii. ☐ Base co-payments on the number of children in the family and reduce a portion of the co-payments as the number of children being served increases.
 - iii. ☐ Other. Describe:

3.3 Waiving Family Co-payment

3.3.1 Waiving family co-payment

The Lead Agency may waive family contributions/co-payments for many families to lower their costs and maximize affordability for families. Lead Agencies have broad flexibility in determining for which families they will waive co-payments.

Does the Lead Agency waive family contributions/co-payments?

☐ No, the Lead Agency does not waive any family contributions/co-payments. (Skip to question 4.1.1.)

☒ Yes. If yes, identify and describe which family contributions/co-payments waived.

- i. ☐ Families with an income at or below 100% of the Federal Poverty Level for families of the same size.
- ii. ☐ Families with an income above 100% but at or below 150% of the Federal Poverty Level for families of the same size.
- iii. ☐ Families experiencing homelessness.
- iv. ☐ Families with children with disabilities.
- v. ☐ Families enrolled in Head Start or Early Head Start.
- vi. ☒ Children in foster care or kinship care, or otherwise receiving or needing to receive protective services. Describe the policy: **Eligible families, except TAFI families participating in non-employment TAFI activities and guardians of foster children, must pay part of their child care costs (16.06.12.503)**
- vii. ☒ Families meeting other criteria established by the Lead Agency. Describe the policy: **Eligible families, except TAFI families participating in non-employment TAFI activities and guardians of foster children, must pay part of their child care costs (16.06.12.503)**

4 Parental Choice, Equal Access, Payment Rates, and Payment Practices

Core purposes of CCDF are to provide participating parents choice in their child care arrangements and provide their children with equal access to child care compared to those children not participating in CCDF. CCDF requirements approach equal access and parental choice comprehensively to meet these foundational program goals. Providing access to a full range of child care providers helps ensure that families can choose a child care provider that meets their family's needs. CCDF payment rates and practices must be sufficient to support equal access by allowing child care providers to recruit and retain skilled staff, provide high-quality care, and operate in a sustainable way. Supply-building strategies are also essential.

This section addresses many of the CCDF provisions related to equal access, including access to the full range of providers, payment rates for providers, co-payments for families, payment practices, differential payment rates, and other strategies that support parental choice and access by helping to ensure that child care providers are available to serve children participating in CCDF.

In responding to questions in this section, OCC recognizes that each Lead Agency identifies and defines its own categories and types of care. OCC does not expect Lead Agencies to change their definitions to fit the CCDF-defined categories and types of care. For these questions, provide responses that closely match the CCDF categories of care.

4.1 Access to Full Range of Provider Options

Lead Agencies must provide parents a choice of providers and offer assistance with child care services through a child care certificate (or voucher) or with a child care provider that has a grant or contract for the provision of child care services. Lead Agencies are reminded that policies and procedures should not restrict parental access to any type or category of care or provider (e.g., center care, home care, in-home care, for-profit provider, non-profit provider, or faith-based provider, etc.).

4.1.1 Parent choice

- a. Identify any barriers to provider participation, including barriers related to payment rates and practices, (including for family child care and in-home providers), based on provider feedback, public comment, and reports to the Lead Agency: **Of the child care programs known to IdahoSTARS through the data in our RISE Application, 53% participate in the Idaho Child Care Program (ICCP). This is not an exhaustive list for all child care programs in Idaho because there is no requirement to be in our RISE Application unless participating in ICCP or state licensed. A significant number of programs that are not participating with subsidy are part-day, part-year programs that may not provide a sufficient amount of child care to support a parent working 40 hours per week, or do not have operating hours reasonably related to parents' work schedules.**
Anecdotal provider feedback indicates that a small number of family and in-home childcare providers considering subsidy participation do not wish to comply with the minimum health and safety requirements and/or background check requirements for themselves or their household members. Health and safety are foundational to quality child care and at this time Idaho is not considering relaxing any of the health, safety, or background check standards in order to promote participation. Idaho's payment practices do not seem to be a barrier to subsidy participation. This topic has repeatedly been discussed with the ICCP Advisory Panel as well as during regular communication with providers. Providers seem satisfied with payment amounts, copays, and timeliness of

subsidy payments. One consistently discernable barrier to subsidy participation is lack of child care availability: Child care programs are easily able to maintain full enrollment and long waitlists without accepting subsidy participating families. Until access to child care improves, there will continue to be a cohort of child care programs unwilling to participate with the subsidy program simply because they do not need it.

- b. Does the Lead Agency offer child care assistance through vouchers or certificates?
☒ Yes.
☐ No.
- c. Does the Lead Agency offer child care assistance through grants or contracts?
☐ Yes.
☒ No.
- d. Describe how the parent is informed that the child care certificate allows the option to choose from a variety of child care categories, such as private, not-for-profit, faith-based providers; centers; family child care homes; or in-home providers: **The Idaho Department of Health and Welfare (IDHW) conducts "informed choice interviews" with families who come into local offices or who call or mail forms to apply for child care services. During these interviews, families are provided information that they can use to make decisions regarding services. For child care specifically, families are referred to IdahoSTARS to search for an Idaho Child Care Program (ICCP) provider.**
- e. Describe what information is included on the child care certificate: **Once child care is found and the family and provider have signed and returned (via mail, fax, email) the monthly dependent care form to DHW, a Notice of Action is generated and provided to the family, identifying child eligibility status, income used to determine eligibility, qualified activity used to determine eligibility, billed amount by the provider, total payment from the child care program to the provider, and total amount family pays directly to the provider.**

4.2 Assess Market Rates and Analyze the Cost of Child Care

To establish subsidy payment rates that ensure equal access, Lead Agencies must collect and analyze statistically valid and reliable data and have the option to conduct either a (1) market rate survey (MRS) reflecting variations in the price to parents of child care services by geographic area, type of provider, and age of child, or (2) an ACF pre-approved alternative methodology, such as a cost estimation model, which estimates the cost of care by incorporating both data and assumptions to estimate what expected costs would be incurred by child care providers and parents under different scenarios. All Lead Agencies must analyze the cost of providing child care through a narrow cost analysis or pre-approved alternative methodology.

Prior to conducting the MRS or pre-approved alternative, Lead Agencies must consult with the State Advisory Council on Early Childhood Education and Care (designated or established pursuant to the Head Start Act (42 U.S.C. 9837b(b)(1)(A)(i)) or similar coordinating body, local child care program administrators, local child care resource and referral agencies, and other appropriate entities; and organizations representing child care caregivers, teachers, and directors. Prior to conducting the MRS or pre-approved alternative methodology, Lead Agencies must consult with the State Advisory Council on Early Childhood Education and Care (designated or established

pursuant to the Head Start Act (42 U.S.C. 9837b(b)(1)(A)(i)) or similar coordinating body, local child care program administrators, local child care resource and referral agencies, and other appropriate entities; and organizations representing child care caregivers, teachers, and directors.

Note: Any Lead Agency considering using an alternative methodology instead of a market rate survey to set payment rates, is required to submit a description of its proposed approach to OCC for pre-approval in advance of developing and conducting the alternative methodology. Advance approval is not required if the Lead Agency plans to implement both an MRS and an alternative methodology to set rates at a percentile of the market rate, but a Lead Agency conducting a limited market rate survey and using it to inform their cost model would need pre-approval for this approach. In its request for ACF pre-approval, a Lead Agency must provide details on the following elements of their proposed alternative methodology:

- Overall approach and rationale for using proposed methodology
- Description of stakeholder engagement
- Data collection timeframe (if applicable)
- Description of the data and assumptions included in the methodology, including how these elements will yield valid and reliable results from the model
- Description of how the methodology will capture the universe of providers, and reflect variations by provider type, age of children, geographic location, and quality

4.2.1 Completion of the market rate survey or ACF pre-approved alternative methodology

Did the Lead Agency conduct a statistically valid and reliable MRS or ACF pre-approved alternative methodology to meet the CCDF requirements to assess child care prices and/or costs and determine payment rates? Check only one based on which methodology was used to determine your payment rates.

- a. ☒ Market rate survey.
- i. When were the data gathered (provide a date range; for instance, September – December 2023)? **May 2024**
- b. ☐ ACF pre-approved alternative methodology.
- i. ☐ The alternative methodology was completed.
- ii. ☐ The alternative methodology is in process.

If the alternative methodology was completed:

When were the data gathered and when was the study completed?

Describe any major differences between the pre-approved methodology and the final methodology used to inform payment rates. Include any major changes to stakeholder engagement, data, assumptions or proposed scenarios.

If the alternative methodology is in progress:

Provide a status on the alternative methodology and timeline (i.e., dates when the alternative methodology activities will be conducted, any completed steps to date, anticipated date of completion, and expected date new rates will be in effect using the alternative methodology).

c. Consultation on data collection methodology.

Describe when and how the Lead Agency engaged the following partners and how the consultation informed the development and execution of the MRS or alternative methodology, as appropriate.

- iii. State Advisory Council or similar coordinating body: **In January 2024, the Idaho Child Care Program (ICCP) Advisory Panel members were notified that the Local Market Rate study was to begin that month. Panel members were encouraged to ask questions and review the state and federal requirements related to the MRS. Additionally, panel members were asked to provide input on any considerations to be made in the development and conduct of the study, as well as any guidance in rate setting. In April 2024, the ICCP Advisory Panel members were provided presentations on the process, status, and initial and results of the Market Rate Survey. Members were given the opportunity to review their previous recommendations, ask questions, and provide comments and feedback.**
- iv. Local child care program administrators: **In late Fall 2023, IDHW child care staff toured the state meeting on location with child care providers and hosting evening meetings for all child care providers and stakeholders in the following cities: Boise, Meridian, Caldwell, Coeur d'Alene, Idaho Falls, Pocatello, Twin Falls, Moscow, and Lewiston. At each meeting the Local Market Rate Survey was reviewed, data collection was discussed, and all attendees were encouraged to participate with the survey, ensure their information was accurate in RISE, and review the results of the survey. At each meeting time was dedicated to soliciting feedback on the market rates and ICCP payment practices. Additionally, child care administrators from the following locations are members of the ICCP Advisory Panel and received additional information and opportunities for feedback as described above: Boise, Rexburg, Nampa, Twin Falls, Pocatello, Coeur d'Alene, and Moscow.**
- v. Local child care resource and referral agencies: **Idaho's child care resource and referral offices are contractually required to gather, track, and report data specific to the Local Market Rate study. The statewide staff are actively involved in developing outreach and communication plans to collect current data from each provider. They contribute to the quality assurance procedures each year for collecting accurate and timely data for providers and conduct outreach when fixes are needed in data.**
- vi. Organizations representing child care caregivers, teachers, and directors from all settings and serving all ages: **In January 2024, the Idaho Child Care Program (ICCP) Advisory Panel members were notified that the Local Market Rate study was to begin that month. Panel members were encouraged to ask questions and review the state and federal requirements related to the MRS. Additionally, panel members were asked to provide input on any considerations to be made in the development and conduct of the study, as well as any guidance in rate setting. In April 2024, the ICCP Advisory Panel members were provided presentations on the process, status, and initial and results of the**

Market Rate Survey. Members were given the opportunity to review their previous recommendations, ask questions, and provide comments and feedback. Members of this group include leadership from the Idaho Association for the Education of Young Children, Idaho Voices for Children, Community Council of Idaho, the State Department of Education, the Idaho Infant Toddler Program, the Idaho Out-of-School Network, and the Idaho Head Start Association.

vii. Other. Describe: **NA**

d. An MRS must be statistically valid and reliable.

An MRS can use administrative data, such as child care resource and referral data, if it is representative of the market. Please provide the following information about the market rate survey:

- i. When was the market rate survey completed? **6/28/2024**
- ii. What was the time period for collecting the information (e.g., all of the prices in the survey are collected within a three-month time period)? **All known providers are asked to maintain rate data in the RISE database, facilities receive automated reminders to update their data quarterly, upon their login. Data used for the survey was downloaded May 2024.**
- iii. Describe how it represented the child care market, including what types of providers were included in the survey: **The providers included in the analysis were required to be active and located in the state of Idaho, and the type of care provided was limited to child care centers, group care, and family care. Both providers participating with ICCP and non-ICCP providers in all jurisdictions (licensed by cities) were included in the data system.**
- iv. What databases are used in the survey? Are they from multiple sources, including licensing, resource and referral, and the subsidy program? **The Market Rate data were provided by IdahoSTARS, the agency responsible for the state's Child Care Resource and Referral System. Statewide provider data for all state licensed and child care providers participating in the Idaho Child Care Program (ICCP) are maintained by IdahoSTARS using data from the RISE database.**
- v. How does the survey use good data collection procedures, regardless of the method for collection (mail, telephone, or web-based survey)? **The provider data required for the market rate analysis were retrieved from the RISE database and included basic provider information and full-time rates. Data provided in the study is collected every six months from all known providers to ensure accuracy and current rates for each providers in RISE.**
- vi. What is the percent of licensed or regulated child care centers responding to the survey? **100.00**
- vii. What is the percent of licensed or regulated family child care homes responding to the survey? **100.00**
- viii. Describe if the survey conducted in any languages other than English: **The Market Rate data were provided by IdahoSTARS, the agency responsible for the state's Child Care Resource and Referral System. Statewide provider data for all state licensed and child care providers participating in the Idaho Child Care Program**

(ICCP) are maintained by IdahoSTARS using data from the RISE database. The RISE database is available in Spanish. Additionally, non-English speaking providers are contacted by an interpreter to verify data.

- ix. Describe if data were analyzed in a manner to determine price of care per child: The RISE database allows for child care pricing to be entered in several modes, depending upon how the child care provider sets its prices. The modes are hourly, daily, weekly, monthly, and annually. Providers are encouraged to enter data for all of the modes they use. For example, one provider may have both a daily rate and a monthly rate that their customers can pay. Because there are multiple modes, and the providers are encouraged to use the modes that best suit them, child care rates are not standard among all providers. To provide an estimate of the market rate across the entire industry, one must either perform separate rate analyses by pricing mode or convert the pricing data to one consistent mode. In the interest of assessing the market rates for the entire population of providers at one time, the latter option was chosen. Based on information from the database, the two most common modes for rates are daily and monthly. To provide a single analysis for the entire industry, and to be consistent with Department of Health and Welfare's (DHW) current policy to pay providers on a monthly basis, the monthly rate was chosen to be the basis for the market rate analysis. For converting the rates, the preference was to use monthly rates, followed in order by weekly rates, daily rates, and hourly rates. The multiplicative factors used to convert non -monthly rates to monthly rates were:
- 4.28 weeks per month;
 - 21.4 days per month (5 days per week $\times$ 4.28 weeks per month);
 - 171.2 hours per month (40 hours per week $\times$ 4.28 weeks per month).
- x. Describe if data were analyzed from a sample of providers and if so, how the sample was weighted: **No, all known licensed and subsidy participating child care providers are required to enter their information in to the provider data management system, RISE and their data was used in the rate calculations.**

- e. Price variations reflected.

The market rate survey data or ACF pre-approved alternative methodology data must reflect variations in child care prices or cost of child care services in specific categories.

- i. Describe how the market rate survey or pre-approved alternative methodology reflected variation in geographic area (e.g., county, region, urban, rural). Include information on whether parts of the State or Territory were not represented by respondents and include information on how prices or costs could be linked to local geographic areas. **To evaluate the market structure, three geographic levels were examined (zip code, county, and region) as the basic unit for the analysis, and county was selected as the unit that best allowed differentiation between units without a large number of units with missing information. Some areas have higher overall rate structures than other areas due to differences in population density and differences in economic conditions. Following methodology used by several other states in their market rate analyses, principal components and cluster analyses were performed to divide counties into groups so that the counties within a group had similar rate structures while counties in different**

groups had differing rate structures. These analyses resulted in identifying three groups of counties. (2024 Idaho Child Care Market Rate Analysis). Using a single state-wide rate would not allow the Idaho Department of Health and Welfare to account for differences in rates due to geographic location, while using separate rates for each provider would be overly complex and would not account for similarities in rates within geographic regions. The process of determining a set of geographic regions for the market rate analysis required four steps:

1. Select the initial geographic units to use in the market rate structure analysis
2. Reduce the provider-level data to unit-level data
3. Determine the best measurements to use to compare and combine the units
4. Determine the best number of groups and to assign the geographic regions to the appropriate group.

ii. Describe how the market rate survey or pre-approved alternative methodology reflected variation in type of provider (e.g., licensed providers, license-exempt providers, center-based providers, family child care home providers, home based providers). **The Market Rate Analysis was restricted to three types of care: 1. Child care centers 2. Group care 3. Family (including relative) care. This is inclusive of all licensed child care providers and nearly all subsidy participating providers. Rates for in-home child care providers were excluded as the sample size was too small for reasonable rate setting. The decision was made to combine family and group care together for the market rate analysis with child care centers in a separate Market Rate Analysis.**

iii. Describe how the market rate survey or pre-approved alternative methodology reflected age of child (e.g., infant, toddler, preschool, school-age): **With respect to age, children are divided into four categories in the IdahoSTARS database, these groups align with state licensing child to adult ratio requirements:**

1. 0 - 23 months
2. 24 through 35 months
3. 36 through 59 months
4. 60 through 156 months.

Within each of the age categories, rates are collected by IdahoSTARS for two "usage" categories: full-time and part-time. The Market Rate Structure Analysis incorporated these four categories and the market rate analysis provided percentiles for each of these four categories.

- iv. Describe any other key variations examined by the market rate survey or ACF pre-approved alternative methodology, such as quality level: **NA**

4.2.2 Cost analysis

If a Lead Agency does not complete a cost-based pre-approved alternative methodology, they must analyze the cost of providing child care services through a narrow cost analysis. A narrow cost analysis is a study of what it costs providers to deliver child care at two or more levels of quality: (1) a base level of quality that meets health, safety, staffing, and quality requirements, and (2) one or more higher levels of quality as defined by the Lead Agency. The narrow cost analysis must estimate costs by levels of quality; include relevant variation by provider type, child's age, or location; and analyze the gaps between estimated costs and payment rates to inform payment rate setting. Lead agencies are not required to complete a separate narrow cost analysis if their pre-approved alternative methodology addresses all of the components required in the narrow cost analysis.

Describe how the Lead Agency analyzed the cost of child care through a narrow cost analysis or pre-approved alternative methodology for the FFY 2025–2027 CCDF Plan, including:

- a. How did the Lead Agency conduct a narrow cost analysis (e.g., a cost model, a cost study, existing data or data from the Provider Cost of Quality Calculator)? **DHW utilized the data available in the RISE Database in order to obtain the provider wage data. By using the data from RISE, DHW was able to complete an analysis that represented all types of providers in all geographic regions of Idaho. The dataset was parsed by the type of care provided (i.e. Center, Group, or Family) and by higher quality care by Idaho's quality rating system.**
- b. In the Lead Agency's analysis, were there any relevant variations by geographic location, category of provider, or age of child? **DHW utilized the data available in the RISE Database in order to obtain the provider wage data. By using the data from RISE, DHW was able to complete an analysis that represented all types of providers in all geographic regions of Idaho. The dataset was parsed by the type of care provided (i.e. Center, Group, or Family) and by higher quality care by Idaho's quality rating system.**
- c. What assumptions and data did the Lead Agency use to determine the cost of care at the base level of quality (e.g., ratios, group size, staff compensations, staff training, etc.)? **DHW utilized the data available in the RISE Database in order to obtain the provider wage data. By using the data from RISE, DHW was able to complete an analysis that represented all types of providers in all geographic regions of Idaho. The dataset was parsed by the type of care provided (i.e. Center, Group, or Family) and by higher quality care by Idaho's quality rating system.**
- d. How does the Lead Agency define higher quality and what assumptions and data did the Lead Agency use to determine cost at higher levels of quality (e.g., ratio, group size, staffing levels, staff compensation, professional development requirements)? A Lead Agency can use a quality improvement system or other system of quality indicators (e.g., accreditation, pre-Kindergarten standards, Head Start Program Performance Standards, or State-defined quality measures). **Idaho defines "higher quality" as any program participating with our Steps to Quality program in addition to any Head Start or nationally accredited facility. Steps to Quality participating providers meet a base level of quality focusing full implementation of enhanced health and safety practices in addition to enhanced professional development requirements for staff. Providers that deliver higher**

levels of quality verified at steps 3 through 6 implement additional quality practices related to lower adult to child ratios, ongoing professional development, staff qualifications, learning environment, curriculum and assessment use, and relationships amongst teachers and children.

- e. What is the gap between cost and price, and how did the Lead Agency consider this while setting payment rates? Did the Lead Agency target any rate increases where gaps were the largest or develop any long-term plans to increase rates based on this information? **The estimated average cost to run a Center based facility was \$343,598. The state of Idaho had an average reimbursement rate during that time of \$72,480, which accounts for about 21% of their total costs. The estimated average costs to run a Group of Family based facility was \$54,308. The state of Idaho had an average reimbursement rate of \$26,640, which accounts for about 49% of their total costs.**

Idaho will continue to review data available in RISE to improve our understanding of the cost of providing child care in Idaho. Data collected will be used to inform all future rate updates as funding is available to insure that rates paid are sufficient to address the cost of providing care.

4.2.3 Publicly available report on the cost and price of child care

The Lead Agency must prepare a detailed report containing the results of the MRS or ACF pre-approved alternative methodology and include the Narrow Cost Analysis if an ACF pre-approved alternative methodology was not conducted.

The Lead Agency must make this report widely available no later than 30 days after completion of the report, including posting the results on the Lead Agency website. The Lead Agency must describe in the detailed report how the Lead Agency took into consideration the views and comments of the public or stakeholders prior to conducting the MRS or ACF pre-approved alternative methodology.

a. Describe how the Lead Agency made the results of the market rate survey or ACF pre-approved alternative methodology report widely available to the public by responding to the questions below.

- i. Provide the date the report was completed: **8/6/2024**
- ii. Provide the date the report containing results was made widely available (no later than 30 days after the completion of the report): **8/20/2024**
- iii. Provide a link to the website where the report is posted and describe any other strategies the Lead Agency uses to make the detailed report widely available: **<https://healthandwelfare.idaho.gov/providers/child-care-providers/child-care-resources>**
- iv. Describe how the Lead Agency considered partner views and comments in the detailed report. Responses should include which partners were engaged and how partner input influenced the market rate survey or alternative methodology: **In late Fall 2023, IDHW child care staff toured the state meeting on location with child care providers and hosting evening meetings for all child care providers and stakeholders in the following cities: Boise, Meridian, Caldwell, Coeur d'Alene, Idaho Falls, Pocatello, Twin Falls, Moscow, and Lewiston. At each meeting the**

Local Market Rate Survey was reviewed, data collection was discussed, and all attendees were encouraged to participate with the survey, ensure their information was accurate in RISE, and review the results of the survey. At each meeting time was dedicated to soliciting feedback on the market rates and ICCP payment practices. Attendees at the local meetings included local child care program administrators, owners, and operators, child care staff, regional staff from the child care resource and referral agency, and organizations representing child care staff.

In January 2024, the Idaho Child Care Program (ICCP) Advisory Panel members were notified that the Local Market Rate study was to begin that month. Panel members were encouraged to ask questions and review the state and federal requirements related to the MRS. Additionally, panel members were asked to provide input on any considerations to be made in the development and conduct of the study, as well as any guidance in rate setting. In April 2024, the ICCP Advisory Panel members were provided presentations on the process, status, and initial and results of the Market Rate Survey. Members were given the opportunity to review their previous recommendations, ask questions, and provide comments and feedback. Members discouraged the implementation of tiered reimbursements based on QRIS participation, and echoed the regional meeting attendees' recommendation to set rates as high as the budget would allow to continue to improve subsidy participating children's access to the full diversity of child care options. IDHW acknowledges and has met both of these recommendations.

4.3 Adequate Payment Rates

The Lead Agency must set CCDF subsidy payment rates in accordance with the results of the current MRS or ACF pre-approved alternative methodology and at a level to ensure equal access for eligible families to child care services comparable with those provided to families not receiving CCDF assistance. Lead Agencies are also required to provide a summary of data and facts to demonstrate how payment rates ensure equal access, which means the Lead Agency must also consider the costs of base level care and higher quality care as part of its rate setting. Finally, the Lead Agency must re-evaluate its payment rates at least every 3 years.

The ages and types of care listed in the base payment rate tables are meant to provide a snapshot of the categories of rates and are not intended to be comprehensive of all categories that might exist or to reflect the terms used by the Lead Agency for particular ages. If rates are not statewide, please provide all variations of payment rates when reporting base payment rates below.

Base rates are the lowest, foundational rates before any differentials are added (e.g., for higher quality or other purposes) and must be sufficient to ensure that minimum health, safety, quality, and staffing requirements are covered. These are the rates that will be used to determine compliance with equal access requirements.

4.3.1 Payment rates

- a. Are the payment rates that the Lead Agency is reporting in 4.3.2 set statewide by the Lead Agency?

☒ Yes.

i. If yes, check if the Lead Agency:

☐ Sets the same payment rates for the entire State or Territory.

☒ Sets different payment rates for different regions in the State or Territory.

☐ No.

ii. If no, identify how many jurisdictions set their own payment rates:

b. Provide the date the current payment rates became effective (i.e., date of last payment rate update based on most recent MRS or ACF pre-approved alternative methodology as reported in 4.2.1). **10/1/2024**

c. If the Lead Agency does not publish weekly rates, then how were the rates reported in 4.3.2 or 4.3.3 calculated (e.g., were daily rates multiplied by 5 or monthly rates divided by 4.3)? **To provide a single analysis for the entire industry, and to be consistent with Department of Health and Welfare's (DHW) current policy to pay providers on a monthly basis, the monthly rate was chosen to be the basis for the market rate analysis. For converting the rates, the preference was to use monthly rates, followed in order by weekly rates, daily rates, and hourly rates. The multiplicative factors used to convert non-monthly rates to monthly rates were:**

- **4.28 weeks per month;**

- **21.4 days per month (5 days per week × 4.28 weeks per month);**

- **171.2 hours per month (40 hours per week × 4.28 weeks per month).**

4.3.2 Base payment rates

a. Provide the base payment rates in the tables below. If the Lead Agency completed a market rate survey (MRS), provide the percentiles based on the most recent MRS for the identified categories. If the Lead Agency sets different payment rates for different regions in the State or Territory (and checked 4.3.1aii), provide the rates for the most populous region as well as the region with payment rates set at the lowest percentile. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

The preamble to the 2016 final rule states that a benchmark for adequate payment rates is the 75th percentile of the most recent MRS. The 75th percentile benchmark applies to the base rates. The 75th percentile is the number separating the lowest 75 percent of rates from the highest 25 percent. Setting rates at the 75th percentile, while not a requirement, would ensure that eligible families can afford three out of four child care providers. In addition to reporting the 75th percentile in the tables below, the Lead Agency must also report the 50th percentile and 60th percentile for each identified category.

If the Lead Agency conducted an ACF pre-approved alternative methodology, provide the estimated cost of care for the identified categories, as well as the percentage of the cost of care covered by the established payment rate. If the Lead Agency indicated it sets different payment rates for different regions in the State or Territory in 4.3.1.a, provide

the estimated cost of care and the percentage of the cost of care covered by the established payment rate for the most populous region as well as the region with rates established at the lowest percent of the cost of care.

For each identified category below, provide the percentage of providers who are receiving the base rate without any add-ons or differential payments.

Provide the full-time weekly base payment rates in the table below. If weekly payment rates are not published, then the Lead Agency will need to calculate its equivalent.

i. Table 1: Complete if rates are set statewide. If rates are not set statewide, provide rates for most populous region. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Infants (6 months)	1039.00 Per Month	100.00	241.00	27.00	1177.00	1234.00	1317.00		
Family Child Care for Infants (6 months)	685.00 Per Month	100.00	159.00	50.00	685.00	700.00	850.00		
Center Care for Toddlers (18 months)	1039.00 Per Month	100.00	241.00	27.00	1177.00	1234.00	1317.00		
Family Child Care for Toddlers (18 months)	685.00 Per Month	100.00	159.00	50.00	685.00	700.00	850.00		

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Preschoolers (4 years)	877.00 Per Month	100.00	204.00	26.00	1010.00	1053.00	1176.00		
Family Child Care for Preschoolers (4 years)	650.00 Per Month	100.00	151.00	34.00	675.00	697.00	800.00		
Center Care for School-Age (6 years)	798.00 Per Month	100.00	186.00	31.00	890.00	963.00	1044.00		
Family Child Care for School-Age (6 years)	642.00 Per Month	100.00	149.00	32.00	650.00	660.00	749.00		

ii. Table 2: Do not complete if rates are set statewide. If rates are not set statewide, provide rates for region with payment rates set at the lowest percentile. Percentiles are not required if the Lead Agency also conducted an ACF pre-approved alternative methodology but must be reported if the Lead Agency conducted an MRS only.

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Center Care for Infants (6 months)	1039.00 Per Month	100.00	241.00	27.00	1177.00	1234.00	1317.00		
Family Child Care for Infants (6 months)	685.00 Per Month	100.00	159.00	50.00	685.00	700.00	850.00		
Center Care for Toddlers (18 months)	1039.00 Per Month	100.00	241.00	27.00	1177.00	1234.00	1317.00		

Care Type	Base payment rate (specify unit, e.g., per day, per week, per month)	% of providers receiving Base rate	Full-Time Weekly Base Payment Rate	What is the percentile of the rate? (MRS)	What is the 50th percentile of the rate? (MRS)	What is the 60th percentile of the rate? (MRS)	What is the 75th percentile of the rate? (MRS)	What is the estimated cost of care? (Alternative Methodology)	What percent of the estimated cost of care is the rate?
Family Child Care for Toddlers (18 months)	685.00 Per Month	100.00	159.00	50.00	685.00	700.00	850.00		
Center Care for Preschoolers (4 years)	877.00 Per Month	100.00	204.00	26.00	1010.00	1053.00	1176.00		
Family Child Care for Preschoolers (4 years)	650.00 Per Month	100.00	151.00	34.00	675.00	697.00	800.00		
Center Care for School-Age (6 years)	798.00 Per Month	100.00	186.00	31.00	890.00	963.00	1044.00		
Family Child Care for School-Age (6 years)	642.00 Per Month	100.00	149.00	32.00	650.00	660.00	749.00		

- b. Does the Lead Agency certify that the percentiles reported in the table above are calculated based on their most recent MRS or ACF pre-approved Alternative Methodology?

☒ Yes.

☐ No. If no, what is the year of the MRS or ACF pre-approved alternative methodology that the Lead Agency used? What was the reason for not using the most recent MRS or ACF pre-approved alternative methodology? Describe:

4.3.3 Tiered rates, differential rates, and add-ons

Lead Agencies may establish tiered rates, differential rates, or add-ons on top of their base rates as a way to increase payment rates for targeted needs (e.g., a higher rate for serving children with special needs).

- a. Does the Lead Agency provide any rate add-ons above the base rate?

☐ Yes. If yes, describe the add-ons, including what they are, who is eligible to receive the add-ons, and how often are they paid:

☒ No.

- b. Has the Lead Agency chosen to implement tiered reimbursement or differential rates?

☐ Yes.

☒ No. Tiered or differential rates are not implemented.

If yes, identify below any tiered or differential rates, and, at a minimum, indicate the process and basis used for determining the tiered rates, including if the rates were based on the MRS or an ACF pre-approved alternative methodology. Check and describe all that apply:

- i. ☐ Differential rate for non-traditional hours. Describe:
- ii. ☐ Differential rate for children with special needs, as defined by the Lead Agency. Describe:
- iii. ☐ Differential rate for infants and toddlers. Note: Do not check if the Lead Agency has a different base rate for infants/toddlers with no separate bonus or add-on. Describe:
- iv. ☐ Differential rate for school-age programs. Note: Do not check if the Lead Agency has a different base rate for school-age children with no separate bonus or add-on. Describe:
- v. ☐ Differential rate for higher quality, as defined by the Lead Agency. Describe:
- vi. ☐ Other differential rates or tiered rates. For example, differential rates for geographic area or for type of provider. Describe:
- vii. If applicable, describe any additional add-on rates that you have besides those identified above.

Does the Lead Agency reduce provider payments if the price the provider charges to private-pay families not participating in CCDF is below the Lead Agency's established payment rate?

☒ Yes. If yes, describe: **Payment will be based on the lower of the provider's usual and customary rates or the Local Market Rate (LMR) (IDAPA 16.06.12.502.01).**

☐ No.

4.3.4 Establishing payment rates

Describe how the Lead Agency established payment rates:

- a. What was the Lead Agency's methodology or process for setting the rates or how did the Lead Agency use their data to set rates? **Idaho has conducted a 2024 Market Rate Study, however due to budget constraints Idaho is not able to update our payment rates. Payment rates established using the 2021 rate study will be maintained until the next state fiscal year. At that time, the plan will be amended with the updated payment rates using the current study. Idaho continues to prioritize setting rates with the intention to fund the actual cost of childcare while maintaining program sustainability and subsidy access for children and families.**
- b. How did the Lead Agency determine that the rates are adequate to meet health, safety,

quality, and staffing requirements under CCDF? **The updated market rate and resultant reimbursements focused on ensuring the ability of providers to meet the health, safety, and staffing requirements under the CCDF law. Child care providers rely on the Idaho Child Care Program (ICCP) for consistent, timely payments that sustain their businesses. Additionally, allowing the providers to bill ICCP in the same amount and methodology that they invoice their cash paying clients serves as a stable stream of income for their business. Current and future reimbursement rates were based on the actual charges reported by providers, regardless of their participation in ICCP. While receiving those reimbursement rates, providing participating in ICCP consistently demonstrated full compliance with health, safety, and supervision requirements. Additionally, ICCP funds the cost of health and safety inspections for all participating child care providers.**

- c. How did the Lead Agency use the cost of care, either from the narrow cost analysis or the ACF pre-approved alternative methodology to inform rate setting, including how using the cost of care promotes the stabilization of child care providers? **Rates are determined using Local Market Rate Survey, which is inclusive of all Idaho child care providers. Rates were gathered in real time and were inclusive of any rate increases charged by providers. After thorough review of the Local Market Rate Survey, rates are set with the intention to fund the actual cost of childcare while maintaining subsidy access for children and families. This encourages families to access a wide range of child care providers who participate in subsidies, limits out-of-pocket expenses for families, and maintains a sustainable program. Idaho has conducted a 2024 Market Rate Study, however due to budget constraints Idaho is not able to update our payment rates. Payment rates established using the 2021 rate study will be maintained until the next state fiscal year. At that time, the plan will be amended with the updated payment rates using the current study.**
- d. How did the Lead Agency account for the cost of higher quality while setting payment rates? **While Idaho's current QRIS participation rates are too small to provide statistically valid cost/quality comparisons, QRIS participation continues to improve, and we are piloting the use of the Cost of Quality Calculator with some QRIS participating programs to better understand their unique expenses. Additionally, when completing Idaho's Local Market Rate Survey all programs considered to be high quality were included in the rate calculation. High quality facilities with rates high enough to be considered outliers were not excluded. The higher rates charged by those facilities were intentionally included to ensure that rates were statistically valid and inclusive of the higher cost of high quality child care.**
- e. Identify and describe any additional facts (not covered in responses to 4.3.1 – 4.3.3) that the Lead Agency considered in determining its payment rates to ensure equal access. **When defining policies for families and providers, we review both balancing continuity for children and policies that are fair to providers. The decision to allow additional rate above the required co-payment is a combination of both levers. Ensuring families have choice and access, while also allowing child care businesses to identify their costs as such to promote and provide quality care. Providers and advocacy groups continue to support the practice of allowing child care providers to charge above the Local Market Rate, as demonstrated in comments provided as feedback to this plan. Families participating with the Idaho Child Care Program (ICCP) continue to access every type of child care provider, including high quality providers participating in Idaho's QRIS program and NAEYC accreditation. Ongoing Market Rate updates continue to ensure that ICCP participating**

providers will maintain their participation with ICCP, and families will continue to be able to access Idaho's best child care choices.

4.4 Payment Practices to Providers

Lead Agencies must use subsidy payment practices that reflect practices that are generally accepted in the private pay child care market. The Lead Agency must ensure timeliness of payment to child care providers by paying in advance or at the beginning of delivery of child care services. Lead Agencies must also support the fixed cost of child care services based on paying by the child's authorized enrollment, or if impracticable, an alternative approach that will not undermine the stability of child care programs as justified and approved through this Plan.

Lead Agencies must also (1) pay providers based on established part-time or full-time rates rather than paying for hours of service or smaller increments of time, and (2) pay for reasonable, mandatory registration fees that the provider charges to private-paying parents. These policies apply to all provider types unless the Lead Agency can demonstrate that in limited circumstances the policies would not be considered generally-accepted payment practices.

In addition, Lead Agencies must ensure that child care providers receive payment for any services in accordance with a payment agreement or an authorization for services, ensure that child care providers receive prompt notice of changes to a family's eligibility status that could impact payment, and have timely appeal and resolution processes for any payment inaccuracies and disputes.

4.4.1 Prospective and enrollment-based payment practices

Lead Agencies must use payment practices for all CCDF child care providers that reflect generally-accepted payment practices of providers serving private-pay families, including paying providers in advance or at the beginning of the delivery of child care services and paying based on a child's authorized enrollment or an alternative approach for which the Lead Agency must demonstrate paying for a child's authorized enrollment is not practicable and it will not undermine the stability of child care programs. Lead Agencies may only use alternate approaches for subsets of provider types if they can demonstrate that prospective payments and authorized enrollment-based payment are not generally-accepted for a type of child care setting. Describe the Lead Agency payment practices for all CCDF child care providers:

- a. Does the Lead Agency pay all provider types prospectively (i.e., in advance of or at the beginning of the delivery of child care services)?

☐ Yes. If yes, describe:

☒ No, it is not a generally-accepted payment practice for each provider type. If no, describe the provider type not paid prospectively and the data demonstrating it is not a generally-accepted payment practice for that provider type, and describe the Lead Agency's payment practice that ensures timely payment for that provider type: **Payments are issued directly to eligible providers via check or direct deposit on the second business day of the month following the month when care was provided. For example, payment for care provided in September would be made on the second business day of October. Idaho is developing a plan for prospective payments in alignment with the 2024 CCDF Rules. Idaho intends to apply for a transitional waiver while we work to implement prospective payments.**

- b. Does the Lead Agency pay based on authorized enrollment for all provider types?

☐ Yes. The Lead Agency pays all providers by authorized enrollment and payment is not altered based on a child's attendance or the number of absences a child has.

☐ No, it is not a generally-accepted practice for each provider type. If no, describe the provider types not paid by authorized enrollment, including the data showing it is not a generally-accepted payment practice for that provider type, and describe how the payment policy accounts for fixed costs:

☒ It is impracticable. Describe provider type(s) for which it is impracticable, why it is impracticable, and the alternative approach the Lead Agency uses to delink provider payments from occasional absences, including evidence that the alternative approach will not undermine the stability of child care programs, and thereby accounts for fixed costs: **Idaho requires that rates charged by the child care provider must not exceed the usual and customary rates charged for child care to persons not entitled to receive benefits under ICCP. If a child care provider's established rates are not altered based on a child's attendance the subsidy payment is not altered. Idaho intends to apply for a transitional waiver as we develop additional strategies to pay by authorized enrollment not linked to a child's number of absences.**

4.4.2 Other payment practices

Lead Agencies must (1) pay providers based on established part-time or full-time rates rather than paying for hours of service or smaller increments of time, and (2) pay for reasonable, mandatory registration fees that the provider charges to private-paying parents, unless the Lead Agency provides evidence that such practices are not generally-accepted for providers caring for children not participating in CCDF in its State or Territory.

- a. Does the Lead Agency pay all providers on a part-time or full-time basis (rather than paying for hours of service or smaller increments of time)?

☒ Yes.

☐ No. If no, describe the policies or procedures that are different than paying on a part-time or full-time basis and the Lead Agency's rationale for not paying on a part-time or full-time basis:

- b. Does the Lead Agency pay for reasonable mandatory registration fees that the provider charges to private-paying parents?

☒ Yes. If yes, identify the fees the Lead Agency pays for: **One-time fees for registering a child in a child care facility are payable above the local market rate if the fee is charged to all who enroll in the facility. Reimbursement cannot exceed \$250 and must be usual and customary rates charged to all families. Registration fees are separate from local market rates. (IDAPA 16.06.12.500.02).**

☐ No. If no, identify the data and how data were collected to show that paying for fees is not a generally-accepted payment practice:

- c. Describe how the Lead Agency ensures that providers are paid in accordance with a written payment agreement or an authorization for services that includes, at a minimum, information regarding provider payment policies, including rates, schedules, any fees charged to providers, and the dispute-resolution process: **All Idaho Child Care Program**

(ICCP) child care providers sign an 'ICCP Provider Agreement' annually which describes the provider's and the State's requirements and responsibilities while participating in the Idaho Child Care Program. Additionally, all new providers complete a subsidy orientation with an IdahoSTARS Resource Specialist to review the participation requirements billing practices for ICCP. The ICCP Provider Agreement and the orientation both reference Child Care IDAPA and ensure providers know, read and understand the rights and responsibilities, which include the dispute resolution process as identified as 'PROVIDER NOTIFICATION' in IDAPA 16.06.12.752. The ICCP Provider Agreement also reminds providers that they are subject to, and protected by, the entire chapter of IDAPA 16.06.12 "Rules Governing the Idaho ChildCare Program (ICCP)". This chapter explains payment policies (16.06.12.500.-505), and the dispute-resolution process (16.06.12.003 and 16.06.12.752).

- d. Describe how the Lead Agency provides prompt notice to providers regarding any changes to the family's eligibility status that could impact payments, and such a notice is sent no later than the day that the Lead Agency becomes aware that such a change will occur: **Providers are notified of projected payment as soon as the client's eligibility is determined, if a family's projected payment amount changes, if the case closes, or requires the completion of a reevaluation to remain eligible. Providers receive a monthly statement of all issued payments with the payment.**
- e. Describe the Lead Agency's timely appeal and resolution process for payment inaccuracies and disputes: **Administrative Appeals - (IDAPA 16.06.12.003) All administrative appeals are governed by provisions of IDAPA 16.05.03, "Rules Governing Contested Cases Proceedings and Declaratory Rulings."** Clients have 30 days from the date of a decision to request a fair hearing. Fair hearing officers review and responds to all requests within five calendar days of the "Fair Hearing Requested" task being set. They communicate the status of reinstatement/continuance of benefits pending the hearing result to the customer within five calendar days of the "Fair Hearing Requested" task being set, then work with the client to coordinate and manage all aspects of a fair hearing to completion. (Benefits Process Manual: "Fair Hearing Process"). **Provider Notification - (IDAPA 16.06.12.752) "When the Department determines actions defined in Sections 701 through 705, 750, and 751 [Overpayment, Intentional Program Violation, Denial of Payment, Funding Restrictions, Termination of Provider Status, Refusal to Enter Into An Agreement] of these rules are appropriate, it will send written notice of the decision to the provider or person. The notice will state the basis for the action, the length of the action, the effect of the action on that person's ability to provide services under state and federal programs, and the person's appeal rights."**
- f. Other. Describe any other payment practices established by the Lead Agency:

4.4.3 Payment practices and parent choice

How do the Lead Agency's payment practices facilitate provider participation in all categories of care? **Providers and advocacy groups continue to support the practice of allowing child care providers to charge above the Local Market Rate, as demonstrated in feedback provided for this plan. This policy encourages and maintains a majority of Idaho's child care providers' voluntary participation with the Idaho Child Care Program (ICCP). Idaho's payments are timely, and providers receive accurate payment information each month. Providers have quick and easy access to DHW staff via email, phone call, or in-office visit to assist with any payment questions or**

issues. Families participating in ICCP continue to access every type of child care provider, including high quality providers participating in Idaho's QRIS program and NAEYC accreditation. More than 10% of all children participating in child care subsidy in Idaho are being served in programs considered to be high quality as defined in Idaho's Quality Ratings and Improvement System. Ongoing Market Rate updates continue to ensure that providers participating in ICCP will maintain their participation with ICCP, and families will continue to be able to access Idaho's best child care choices.

4.5 Supply Building

Building a supply of high-quality child care that meets the needs and preferences of parents participating in CCDF is necessary to meet CCDF's core purposes. Lead Agencies must support parent choice by providing some portion of direct services via grants or contracts, including at a minimum for children in underserved geographic areas, infants and toddlers, and children with disabilities.

4.5.1 Child care services available through grants or contracts

Does the Lead Agency provide direct child care services through grants or contracts for child care slots?

☐ Yes, statewide. Describe how the Lead Agency ensures that parents who enroll with a provider who has a grant or contract have choices when selecting a provider:

☐ Yes, in some jurisdictions, but not statewide. Describe how many jurisdictions use grants or contracts for child care slots and how the Lead Agency ensures that parents who enroll with a provider who has a grant or contract have choices when selecting a provider:

☒ No. If no, describe any Lead Agency plans to provide direct child care services through grants and contracts for child care slots: **Currently, IDHW does not pay direct child care services through grants and contracts through child care slots. IDHW will request a waiver to obtain technical assistance on how to implement this requirement.**

If no, skip to question 4.5.2.

i. If yes, identify the populations of children served through grants or contracts for child care slots (check all that apply). For each population selected, identify the number of slots allocated through grants or contracts for direct service of children receiving CCDF.

☐ Children with disabilities. Number of slots allocated through grants or contracts:

☐ Infants and toddlers. Number of slots allocated through grants or contracts:

☐ Children in underserved geographic areas. Number of slots allocated through grants or contracts:

☐ Children needing non-traditional hour care. Number of slots allocated through grants or contracts:

☐ School-age children. Number of slots allocated through grants or contracts:

☐ Children experiencing homelessness. Number of slots allocated through grants or contracts:

☐ Children in urban areas. Percent of CCDF children served in an average month:

☐ Children in rural areas. Percent of CCDF children served in an average month:

☐ Other populations. If checked, describe:

- ii. If yes, how are rates for slots funded by grants and contracts determined by the Lead Agency?

4.5.2 Care in the child's home (in-home care)

The Lead Agency must allow for in-home care (i.e., care provided in the child's own home) but may limit its use.

Will the Lead Agency limit the use of in-home care in any way?

☒ Yes.

☐ No.

If yes, what limits will the Lead Agency set on the use of in-home care? Check all that apply.

- i. ☒ Restricted based on the minimum number of children in the care of the in-home provider to meet the Fair Labor Standards Act (minimum wage) requirements. Describe: **The Department limits the approval of all in-home child care under ICCP to the following circumstances:**
There are three (3) or more ICCP eligible children in the home who are not in school at any time during the day and require child care. (16.06.06.12.400.01)

Parents are responsible to pay persons providing care in the child's home the minimum wage, as required by the Fair Labor Standards Act (29 U.S.C. 206a) and other applicable state and federal requirements. (IDAPA 16.06.12.502.03)

- ii. ☒ Restricted based on the in-home provider meeting a minimum age requirement. Describe: **Age of Provider - (IDAPA 16.06.12.0802.01) All child care providers providing services must be 18 years old or older. Persons sixteen or seventeen years old may provide child care if they have direct, on-site supervision from a licensed child care provider who is at least 18 years old.**
- iii. ☒ Restricted based on the hours of care (i.e., certain number of hours, non-traditional work hours). Describe: **Requirements for In-Home Care Under ICCP - (IDAPA 16.06.12.400.02.a and b) Parents must contact the Idaho Department of Health and Welfare (DHW) to request approval of in-home child care. Only parents who have qualified activities outside their home will be considered for in-home care approval. DHW limits the approval of all in-home child care under ICCP for fewer than three children in the home. If there are fewer than three children in the home who are eligible for ICCP and require child care, in-home care will be approved by DHW only when one of the following special circumstances are met: 1. Parents' qualifying activity occurs during times when**

out-of-home care is not available. If child care is needed during any period when out-of-home care is not available, in-home care will be approved for the entire time care is needed. A family is not expected to change between out-of-home and in-home care 2. The family lives in an area where out-of-home care is not available.

- iv. ☐ Restricted to care by relatives. (A relative provider must be at least 18 years of age based on the definition of eligible child care provider.) Describe:
- v. ☒ Restricted to care for children with special needs or a medical condition. Describe: **Requirements for In-Home Care Under ICCP - (IDAPA 16.06.12.400.02.c) Parents must contact the Idaho Department of Health and Welfare (DHW) to request approval of in-home child care. Only parents who have qualified activities outside their home will be considered for in-home care approval. DHW limits the approval of all in-home child care under ICCP to fewer than three children in the home. If there are fewer than three children in the home who are eligible for ICCP and require child care, in-home care will be approved by DHW only a child has a verified illness or disability that would place the child or other children in an out-of-home facility at risk.**
- vi. ☐ Restricted to in-home providers that meet additional health and safety requirements beyond those required by CCDF. Describe:
- vii. ☐ Other. Describe:

4.5.3 Shortages in the supply of child care

Lead Agencies must identify shortages in the supply of child care providers that meet parents' needs and preferences.

What child care shortages has the Lead Agency identified in the State or Territory, and what is the plan to address the child care shortages?

- a. In infant and toddler programs:
 - i. Data sources used to identify shortages: **Facility data, including age ranges of children served and slots available, is collected in the RISE Application with regular reminders to facility administrators to update data. Data is also collected on the direct referrals to IdahoSTARS for infant and toddler care. For quality efforts, IdahoSTARS is expanding Infant Mental Health Endorsement for Infant Educators, tracking progress through endorsement and number of Infant Educators endorsed in the state. Pyramid Model implementation with programs and providers serving children birth to three years of age includes data collection on the programs' Benchmarks of Quality, results of fall and spring Teaching Pyramid Infant-Toddler Observation Scales (TPITOS), and two mid-year surveys examining impact and participant experience through implementation of the Pyramid Model practices.**

IdahoSTARS collects and analyzes specific data related to childcare availability, including information on programs that have opened or closed. When a program closes, IdahoSTARS assists families in finding alternative childcare that meets their needs, if available, through referrals and follow-up surveys to ensure they were able to secure care. While IdahoSTARS' primary role is increasing access to high-

quality child care, project activities do support broader understanding of infant toddler shortages such as data collection and reporting, along with professional development offerings that can influence availability and sustainability of infant and toddler care.

Childcare programs report their capacity and waitlist information in the RISE system, which is used in referrals to help families locate available childcare. This data also allows IdahoSTARS to identify gaps in care, particularly for infant and toddler care.

Current data indicates that there is a shortage of infant and toddler care, as families often report difficulty in finding suitable care for these age groups.

- ii. Method of tracking progress: Facility data is entered directly into the RISE application by program administrators. Referral data is entered into RISE by IdahoSTARS Resource Specialists. Quality improvement and professional development data is housed in RISE and within secure project management and survey software.
- iii. What is the plan to address the child care shortages using family child care homes In collaboration with statewide partners, IdahoSTARS provides supports to those interested in starting a child care business helping to ensure their successful start-up. In addition, quality support with aligned grants are available to new programs. IdahoSTARS also provides additional program incentives to family child care homes expanding their support and engagement in quality improvement activities

The specific data elements that identify infant and toddler care shortages include waitlists, reported openings/closures, and families who report being unable to find care through referrals. These indicators point to existing shortages in the availability of infant and toddler care.

Additionally, Business Practice Cohorts help child care providers stay in business by offering modules on budgeting, record-keeping, and marketing. IdahoSTARS also supports home providers by encouraging them to become ICCP certified and assists them in expanding their childcare businesses.

- iv. What is the plan to address the child care shortages using child care centers? In collaboration with statewide partners, IdahoSTARS provides supports to those interested in starting a child care business helping to ensure their successful start-up. In addition, quality support with aligned grants are available to new programs. IdahoSTARS also provides additional program incentives to child care centers expanding their support and engagement in quality improvement activities.

Implementing the Pyramid Model in childcare settings can improve the quality of care provided to infants and toddlers by equipping caregivers with strategies to support the social-emotional development of young children. By enhancing

caregivers' skills, the model helps create more stable and supportive environments, making these settings more attractive for both caregivers and parents, potentially reducing shortages.

Integrating ITMH principles into childcare practices ensures that caregivers are trained to recognize and support the mental health needs of infants and toddlers. This training can lead to higher-quality care, better child outcomes, and increased job satisfaction among caregivers, which could help retain staff and reduce shortages.

In the context of infant-toddler care shortages, direct referrals can help ensure that available spots in quality programs are filled efficiently, reducing the gap between supply and demand. Additionally, referrals to specialized programs or training can support caregivers in developing the skills needed for infant-toddler care, potentially increasing the number of providers willing and able to care for this age group.

By improving business practices, childcare providers can create more sustainable operations. For example, better financial management might allow a provider to offer competitive wages or benefits, which could attract and retain caregivers for infant-toddler care. Additionally, efficient staffing practices can ensure that more spots are available for infants and toddlers, helping to alleviate shortages.

- b. In different regions of the State or Territory:
 - i. Data sources used to identify shortages: **Facility data, including age ranges of children served, slots available, and program location is collected in the RISE Application with regular reminders to facility administrators to update data. Data is also collected on the direct referrals by region to IdahoSTARS for child care.**
 - ii. Method of tracking progress: **IdahoSTARS sets quality improvement targets which are regularly reviewed for recruitment and retention of programs into the state quality rating and improvement system and the Idaho Child Care Program. Strategic progress is tracked and reported to the Idaho Department of Health and Welfare.**
 - iii. What is the plan to address the child care shortages using family child care homes? **IdahoSTARS will continue to incentivize family child care homes for their engagement in quality initiatives, and will continue to provide technical assistance to new child care programs along with opportunities to receive start-up grant funding.**

The efforts described by IdahoSTARS address child care supply in different regions by focusing on a few key strategies:

Incentivizing Family Child Care Homes: By providing incentives for family child care homes to engage in quality initiatives, IdahoSTARS is encouraging existing child care providers to improve and maintain high standards. This can lead to more reliable and quality child care options across various regions, as providers are motivated to meet these higher standards.

Technical Assistance to New Child Care Programs: Offering technical assistance to

new child care programs is crucial for increasing the supply of child care in underserved or growing regions. By helping new providers navigate the complexities of starting a child care business, IdahoSTARS ensures that these programs are more likely to succeed and sustain their operations, thereby increasing the overall availability and sustainability of child care.

Start-Up Grant Funding: Providing opportunities for start-up grant funding helps reduce the financial barriers to opening new child care facilities. This is particularly important in regions where child care supply is low, as it makes it easier for new providers to enter the market and serve communities that may lack sufficient child care options.

- iv. What is the plan to address the child care shortages using child care centers? IdahoSTARS will continue to incentivize child care centers for their engagement in quality initiatives, and will continue to provide technical assistance to new child care programs along with opportunities to receive start-up grant funding.

The efforts mentioned focus on addressing regional shortages in child care by identifying and responding to gaps in child care availability and supporting child care providers in those areas. Here's how the efforts are structured to focus on regional shortages:

Identifying Shortages: IdahoSTARS uses data from waitlists, reported openings/closures, and feedback from families who were unable to find care to identify regions across the state where there are significant shortages in child care, particularly in child care centers.

Incentivizing Quality: IdahoSTARS incentivizes family child care homes to engage in quality initiatives, which can improve the overall availability and quality of care in regions where shortages are identified.

Supporting New Programs: IdahoSTARS provides technical assistance and start-up grant funding to help establish new child care programs in regions with shortages. This directly addresses the lack of available child care by encouraging the development of new providers.

Sustaining Existing Providers: Through Business Practice Cohorts, IdahoSTARS offers targeted support to help existing child care centers stay in business, especially in areas where child care options are limited. These cohorts cover essential business skills like budgeting, record keeping, staffing, and marketing, which are critical for the sustainability of these centers.

Encouraging ICCP Certification: By encouraging center directors and owners who do not currently accept Idaho Child Care Program (ICCP) subsidies to become certified, IdahoSTARS aims to increase the availability of subsidized child care in regions where there are shortages, making care more accessible to low-income families.

- c. In care for special populations:
 - i. Data sources used to identify shortages: Data is collected by IdahoSTARS on direct **Enhanced** referrals, those that necessitate additional support and follow up due to special child and family needs.

- ii. Method of tracking progress: **Data is collected in the RISE Application by Resource Specialists on enhanced referral.**
- iii. What is the plan to address the child care shortages using family child care homes? **IdahoSTARS is expanding their quality efforts to increase the availability of quality programs better able to meet the diverse needs of young children through implementation of the Pyramid Model. For enhanced referrals, the Child Care Resource Center staff, including Child Care Health Consultants, will be available to support successful access and inclusion of children with disabilities into child care settings.**
- iv. What is the plan to address the child care shortages using child care centers? **IdahoSTARS is expanding their quality efforts to increase the availability of quality programs better able to meet the diverse needs of young children through implementation of the Pyramid Model. For enhanced referrals, the Child Care Resource Center staff, including Child Care Health Consultants, will be available to support successful access and inclusion of children with disabilities into child care settings.**

4.5.4 Strategies to increase the supply of and improve quality of child care

Lead Agencies must develop and implement strategies to increase the supply of and improve the quality of child care services. These strategies must address child care in underserved geographic areas; infants and toddlers; children with disabilities, as defined by the Lead Agency; and children who receive care during non-traditional hours.

How does the Lead Agency identify any gaps in the supply and quality of child care services and what strategies are used to address those gaps for:

- a. Underserved geographic areas. Describe: **According to the Idaho Needs Assessment completed in April 2024 by the Idaho School Readiness Project (with funding from the Preschool Development Birth-5 grant), Idaho's supply of child care is especially difficult to measure:**

"The potential demand for early childhood care and education (ECCE) in Idaho based on American Community Survey (ACS) data on the count and percentage of children ages birth through 5 with all parents in the labor force and, therefore, potentially needing care. To estimate the availability of ECCE relative to the potential demand, researchers employ a variety of methodologies to identify "child care deserts" where the quantity of ECCE slots does not meet the potential demand for care. 10 For all methodologies, it is necessary to have reliable early care licensed capacity data to compare with the counts of young children potentially needing care. As indicated above, it is only possible to provide broad-range estimates of licensed capacity for Idaho child care, and these estimates are only possible for children ages birth through 12. Therefore, it is not possible, with currently available data, to accurately assess the adequacy of ECCE capacity in the state." (Idaho Needs Assessment Report page 47)

The report also notes:

"A basic methodology for determining child care deserts is calculating the gap between licensed slots and the number of children with all parents in the labor force (as a proxy for

the number of children potentially needing care). The Center for American Progress definition is more nuanced: "A child care desert is any census tract with more than 50 children under age 5 that contains either no child care providers or so few options that there are more than three times as many children as licensed child care slots." In their 2017 study, they estimated that 49% of people in Idaho lived in a child care desert. This was similar to the national rate of 50%." (Idaho Needs Assessment Report page 47)

With the outcome of this most recent report unable to provide specificity related to underserved geographic areas, Idaho will continue to refine data collection efforts utilizing technical assistance and partners to continue our research in this area. With an understanding that nearly half the state lacks adequate access to needed child care, Idaho continues to have regionally located child care consultants available to provide on-site and over-the-phone technical assistance to new and existing providers considering providing child care in underserved geographic areas. Consultants have knowledge and resources available to assist providers serving children in rural and remote communities, as well as in the start-up and expansion of child care businesses. Start up grants are available to newly enrolled child care providers. The grant is a reimbursement grant that may cover such expenses as background checks for the provider or items needed to pass their health inspection.

- b. Infants and toddlers. Describe: IdahoSTARS delivers professional development specific to infants and toddlers. In addition, IdahoSTARS provides supports and funding for Infant Mental Health Endorsement, to include reflective consultation and other provider retention activities.

Based on data analysis from our RISE application, we have identified a significant shortage of infant and toddler care providers. Our findings indicate that many infant and toddler teachers frequently transition to preschool positions or leave the field altogether. In response to this issue, we have recognized the need for targeted professional development tailored specifically to the unique needs and challenges of infant and toddler care. The Infant Mental Health endorsement addresses this need by formally recognizing providers' expertise in this specialized area, thereby enhancing their marketability. Additionally, the endorsement supports reflective practice as a retention strategy, offering ongoing support to caregivers to address challenges and improve their practice.

- c. Children with disabilities. Describe: IdahoSTARS is working with state partners to expand access to child care slots for children with disabilities through enhanced referrals, collaborative developmental screening activities, through professional development on specific disabilities and inclusive practices, and through implementation of the Pyramid Model and aligned behavioral supports.

The gap in access to child care slots for children with disabilities can be identified through a series of data collection, analysis, and stakeholder engagement activities, particularly focusing on the following areas:

Enhanced Referrals:

Data Collection: Identify the current number and types of referrals made for children with disabilities to child care providers. This includes understanding the rate of successful

placements versus referrals that do not result in placement.

Gap Identification: A gap may be identified if there is a significant number of referrals that do not lead to child care placements or if there is a lack of providers available to accept these referrals. Additionally, a disparity in referrals between children with and without disabilities could indicate a gap.

Collaborative Developmental Screening Activities:

Data Collection: Assess the current availability and usage of developmental screening tools in child care settings and the follow-up actions taken when a screening indicates a potential disability.

Gap Identification: A gap may be identified if there are insufficient screening activities, if screenings are not reaching all children (especially those in rural or underserved areas), or if there is a lack of follow-up resources or services when a developmental concern is identified.

- d. Children who receive care during non-traditional hours. Describe: Anecdotal information indicates that in Idaho a variety of professions such as law enforcement and the military are seeking child care solutions for care during non-traditional hours. According to a July 18, 2023 press release from Idaho Senator Mike Simpson, The National Law Enforcement Foundation is partnering to build a childcare center for law enforcement officers in the Treasure Valley to provide "safe and secure childcare and education coverage that matches the needs of law enforcement families and would meet a widely acknowledged need within Idaho law enforcement." The Idaho National Guard in Boise has also constructed a child care facility to meet the non-traditional hour child care needs of Guardsmen during their drill weekends. Additionally, the recently completed statewide needs assessment is informing future strategies for increasing access and quality of child care during non-traditional hours.

In the meantime, Idaho continues to have regionally located child care consultants available to provide on-site and over-the-phone technical assistance to new and existing providers considering providing child care during non-traditional hours. Consultants have knowledge and resources available to assist providers serving children during non-traditional hours. Start up grants are available to newly enrolled child care providers. The grant is a reimbursement grant that may cover such expenses as background checks for the provider or items needed to pass their health inspection.

- e. Other. Specify what population is being focused on to increase supply or improve quality. Describe: IdahoSTARS is implementing internal professional development support for our coaching team to improve their ability to build responsive relationships in coaching. This activity is intended to increase the quality of coaching relationships for providers of all race/ethnicities, preferred languages, and facility types.

4.5.5 Prioritization of investments in areas of concentrated poverty and unemployment

Lead Agencies must prioritize investments for increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment and do not currently have sufficient numbers of such programs.

Describe how the Lead Agency prioritizes increasing access to high-quality child care and development services for children of families in areas that have significant concentrations of poverty and unemployment and that do not have access to high-quality programs. **An analysis was performed so that the Lead Agency could highlight the poverty information (through census data) and the child and provider level data to get an idea of where children participating in the Idaho Child Care Program (ICCP) were being served and what types of providers were serving the most children in correlation to which of those counties had poverty rates. Unemployment data was gathered through the Department of Labor to contribute to the analysis. Idaho plans to use this data to work with IdahoSTARS to target the areas of need and increase recruitment of providers in those high need areas. The report will also be a tool to review data during the process of conducting the market rate study. Idaho defined areas with significant concentrations of poverty and unemployment by comparing number of children in poverty by county, per number of providers available. These data were derived from the Census. Idaho does not have a waiting list for subsidized care, so all children/families in areas of significant concentrations of poverty and unemployment would have access to the Idaho Child Care Program subsidy as an option pending other eligibility criteria are met. The IdahoSTARS project, through the University of Idaho, targets child care providers who currently run businesses in identified high poverty communities to increase technical assistance and recruit current providers into enrolling in the Steps to Quality, Idaho's Quality Rating Improvement program. In efforts to recruit more providers, the CCRC staff offer start up grants to assist providers in starting their business.**

5 Health and Safety of Child Care Settings

Child care health and safety standards and enforcement practices are essential to protect the health and safety of children while out of their parents' care. CCDF provides a minimum threshold for child care health and safety policies and practices but leaves authority to [Lead Agencies](#) to design standards that appropriately protect children's safety and promote nurturing environments that support their healthy growth and development. Lead Agencies should set standards for ratios, group size limits, and provider qualifications that help ensure that the child care environment is conducive to safety and learning and enable caregivers to promote all domains of children's development.

CCDF health and safety standards help set clear expectations for CCDF providers, form the foundation for health and safety training for child care workers, and establish the baseline for monitoring to ensure compliance with health and safety requirements. These health and safety requirements apply to all providers serving children receiving CCDF services – whether the providers are licensed or license-exempt, must be appropriate to the provider setting and age of the children served, must include specific topics and training on those topics, and are subject to monitoring and enforcement procedures by the [Lead Agency](#). CCDF-required annual monitoring and enforcement actions help ensure that CCDF providers are adopting and implementing health and safety requirements.

Through child care licensing, [Lead Agencies](#) set minimum requirements, including health and safety requirements, that child care providers must meet to legally operate in that State or Territory. In some cases, CCDF health and safety requirements may be integrated within the

licensing system for licensed providers and may be separate for CCDF providers who are license-exempt.

This section addresses CCDF health and safety requirements, [Lead Agency](#) licensing requirements and exemptions, and comprehensive background checks.

When responding to questions in this section, OCC recognizes that each [Lead Agency](#) identifies and defines its own categories of care. OCC does not expect [Lead Agencies](#) to change their definitions to fit the CCDF-defined categories of care. For these questions, provide responses that best match the CCDF categories of care.

5.1 Licensing Requirements

Each Lead Agency must ensure it has in effect licensing requirements applicable to all child care services provided within the State/Territory (not restricted to providers receiving CCDF funds).

5.1.1 Providers subject to licensing

For each category of care listed below, identify the type of providers subject to licensing and describe the licensing requirements.

- a. Identify the center-based provider types subject to child care licensing: **A daycare center license is issued for a place or facility providing daycare where 13 or more children, regardless of relationship to the person or persons providing the care, are in attendance. If a city or county within its respective jurisdiction has adopted an ordinance for regulation and/or licensing of daycare services, then the provisions of this chapter shall not apply with such city or county unless the ordinance is subsequently repealed. Cities and counties are granted authority and may adopt ordinances for regulation and/or licensing of daycare services. The following municipalities have exercised their right to local option licensing programs: Boise, Chubbuck, Coeur d' Alene, Idaho Falls, Lewiston, Moscow, and Pocatello. Center-based providers must be licensed to participate in the Idaho Child Care Program (ICCP).**

Are there other categories of licensed, regulated, or registered center providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

- b. Identify the family child care providers subject to licensing: **A family daycare home, caring for 6 children or less, is not required to be licensed. However, a family daycare home may voluntarily elect to be licensed by the Idaho Department of Health and Welfare. Idaho provides licensing of a Group Daycare Facility, which may be located within a home/residence setting. A group daycare facility license is issued for a place or facility providing daycare, where seven to twelve children, regardless of relationship to the person or persons providing the care, are in attendance. If a city or county within its respective jurisdiction has adopted an ordinance for regulation and/or licensing of daycare services, then the provisions of this chapter shall not apply with such city or county unless the ordinance is subsequently repealed. A family daycare home, caring for 6 children or less, is not required to be licensed to participate in the Idaho Child Care Program (ICCP). Group Daycare Facilities must be licensed to participate in the Idaho Child Care Program**

(ICCP).

Are there other categories of regulated or registered family child care providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

- c. Identify the in-home providers subject to licensing: **When a child is cared for in their own home, there is no state licensing requirement.**

Are there other categories of regulated or registered in-home providers the Lead Agency does not categorize as license-exempt?

☐ Yes. If yes, describe:

☒ No.

5.1.2 CCDF-eligible providers exempt from licensing

Identify the categories of CCDF-eligible providers who are exempt from licensing requirements, the types of exemptions, and describe how these exemptions do not endanger the health, safety, and development of children. -Relative providers, as defined in CCDF, are addressed in subsection 5.8.

- a. License-exempt center-based child care. Describe by answering the questions below.

- i. Identify the categories of CCDF-eligible center-based child care providers who are exempt from licensing requirements. **Idaho does not have license-exempt center-based child care (IDAPA 16.06.02.301.03).**
- ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **Idaho does not have license-exempt center-based child care (IDAPA 16.06.02.301.03).**
- iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **Idaho does not have license-exempt center-based child care (IDAPA 16.06.02.301.03).**

- b. License-exempt family child care. Describe by answering the questions below.

- i. Identify the categories of CCDF-eligible family child care providers who are exempt from licensing requirements. **A family daycare home is not required to be licensed because under Idaho Law, if a provider services less than seven children, they are considered family. However, a family daycare home may voluntarily elect to be licensed by the Idaho Department of Health and Welfare. CCDF eligible family child care providers must fully comply with IDAPA 16.06.12. and complete the following:**
 - A provider orientation
 - Sign an ICCP Provider Agreement annually
 - Meet all health and safety criteria detailed in IDAPA 16.06.12, including background checks on household members
 - Complete an annual health and safety inspection as detailed in IDAPA 16.06.12

- Meet all training criteria detailed in IDAPA 16.06.12
- Maintain records as detailed in IDAPA 16.06.12
- Demonstrate compliance with local codes and ordinances

- ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **The occasional or irregular care of a neighbor's, relative's or friend's child or children by a person not ordinarily in the business of providing daycare is also exempt from licensing.**
- iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **A family daycare home that serves six or fewer children is exempt from licensure but may voluntarily elect to be licensed. Exempt providers commonly provide care in their home or the home of the parent or guardian. Any person providing daycare for four to six children in a family daycare home must complete and pass a Criminal History and Background Check to ensure the safety and well-being of children in their care.**
- c. In-home care (care in the child's own home by a non-relative). Describe by answering the questions below.
 - i. Identify the categories of CCDF-eligible in-home care (care in the child's own home by a non- relative) providers who are exempt from licensing requirements. **When a child is cared for in their home, there is no licensing requirement.**
 - ii. Describe the exemptions based on length of day, threshold on the number of children in care, ages of children in care, or any other factors applicable to the exemption. **When a child is cared for in their home, there is no licensing requirement.**
 - iii. Describe how the exemptions for these CCDF-eligible providers do not endanger the health, safety, and development of children. **Annually each in-home care provider is responsible to ensure that health and safety requirements are met for children being cared for in the children's own home, as defined in Section 802 of these rules. (16.06.12.401)**

5.2 Ratios, Group Size, and Qualifications for CCDF Providers

Lead Agencies must have child care standards for providers receiving CCDF funds, appropriate to the type of child care setting involved, that address appropriate staff:child ratios, group size limits for specific age populations, and the required qualifications for providers. Lead Agencies should map their categories of care to the CCDF categories. Exemptions for relative providers will be addressed in subsection 5.8.

5.2.1 Age classifications

Describe how the **Lead Agency** defines the following age classifications (e.g., Infant: 0 – 18 months).

- a. Infant. Describe: **A child under the age of 24 months**
- b. Toddler. Describe: **A child 24 months to under 36 months of age**

- c. Preschool. Describe: **A child 36 months to under 5 years**
- d. School-Age. Describe: **A child five-twelve years of age**

5.2.2 Ratio and group size limits

Provide the ratio and group size limits for settings and age groups below.

- a. Licensed CCDF center-based care:
 - i. Infant.

Ratio: **One adult to six infants, aged less than 24 months.**

Group size: (IDAPA 16.06.03.101, 16.06.03.300, 16.06.03.360) **Each type of child care setting is limited by their provider agreement or daycare license to the total number of children in their care. The ratio point system limits the number of children per provider based on the children's ages, an important developmental consideration when evaluating mixed age child care programs. The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:**

- 1. Each child in attendance under the age of 24 months equals two points**
- 2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points**
- 3. Each child in attendance from 36 months to under five years of age equals one point**
- 4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.**

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios. When monitoring group size, the Idaho Child Care Program (ICCP) provider agreement, daycare license type, occupancy load, and child ratio points are all considered, and the lowest number of children is the most allowed. For example, if a provider requests a center daycare license for 50 children, but the total maximum occupancy load for the facility is set at a maximum of 30 people on site by the local fire marshal, the provider would be limited to the occupancy load of no more than a total of 30 people (adults and children) onsite, and ratio points may further limit the amount of children allowed in the provider's care based on the children's ages. Cities and local municipalities may elect to administer more stringent group size requirements. The maximum occupancy load for a child care facility is required to be determined prior to the issuance of a daycare license. Occupant loads are determined by the local fire official according to the current Idaho Fire Code, which is based on the International Fire Code.

(<https://codes.iccsafe.org/content/IFC2021P2>)

Currently, Idaho does not limit group sizes based on children's age groups. This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address group sizes without hindering the important program flexibilities that our mixed-age childcare providers rely on. IDHW will request technical assistance regarding the implementation of this requirement.

ii. Toddler.

Ratio: One adult to eight toddlers, aged 24 months to under 36 months

Group size: (IDAPA 16.06.03.101, 16.06.03.300, 16.06.03.360) Each type of child care setting is limited by their provider agreement or daycare license to the total number of children in their care. The ratio point system limits the number of children per provider based on the children's ages, an important developmental consideration when evaluating mixed age child care programs. The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios. When monitoring group size, the Idaho Child Care Program (ICCP) provider agreement, daycare license type, occupancy load, and child ratio points are all considered, and the lowest number of children is the most allowed. For example, if a provider requests a center daycare license for 50 children, but the total maximum occupancy load for the facility is set at a maximum of 30 people on site by the local fire marshal, the provider would be limited to the occupancy load of no more than a total of 30 people (adults and children) onsite, and ratio points may further limit the amount of children allowed in the provider's care based on the children's ages. Cities and local municipalities may elect to administer more stringent group size requirements. The maximum occupancy load for a child care facility is required to be determined prior to the issuance of a

daycare license. Occupant loads are determined by the local fire official according to the current Idaho Fire Code, which is based on the International Fire Code.
(<https://codes.iccsafe.org/content/IFC2021P2>)

Currently, Idaho does not limit group sizes based on children's age groups. This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address group sizes without hindering the important program flexibilities that our mixed-age childcare providers rely on. IDHW will request technical assistance regarding the implementation of this requirement.

iii. **Preschool.**

Ratio: One adult to twelve preschoolers, aged 36 months to less than 5 years

Group size: (IDAPA 16.06.03.101, 16.06.03.300, 16.06.03.360) Each type of child care setting is limited by their provider agreement or daycare license to the total number of children in their care. The ratio point system limits the number of children per provider based on the children's ages, an important developmental consideration when evaluating mixed age child care programs. The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

- 1. Each child in attendance under the age of 24 months equals two points**
- 2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points**
- 3. Each child in attendance from 36 months to under five years equals one point**
- 4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.**

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios. When monitoring group size, the Idaho Child Care Program (ICCP) provider agreement, daycare license type, occupancy load, and child ratio points are all considered, and the lowest number of children is the most allowed. For example, if a provider requests a center daycare license for 50 children, but the total maximum occupancy load for the facility is set at a maximum of 30 people on site by the local fire marshal, the provider would be limited to the occupancy load of no more than a total of 30 people (adults and children) onsite, and ratio points may further limit the

amount of children allowed in the provider's care based on the children's ages. Cities and local municipalities may elect to administer more stringent group size requirements. The maximum occupancy load for a child care facility is required to be determined prior to the issuance of a daycare license. Occupant loads are determined by the local fire official according to the current Idaho Fire Code, which is based on the International Fire Code.
(<https://codes.iccsafe.org/content/IFC2021P2>)

Currently, Idaho does not limit group sizes based on children's age groups. This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address group sizes without hindering the important program flexibilities that our mixed-age childcare providers rely on. IDHW will request technical assistance regarding the implementation of this requirement.

iv. School-Age.

Ratio: One adult to 24 school age children, aged 5 years to under 13 years

Group size: (IDAPA 16.06.03.101, 16.06.03.300, 16.06.03.360) Each type of child care setting is limited by their provider agreement or daycare license to the total number of children in their care. The ratio point system limits the number of children per provider based on the children's ages, an important developmental consideration when evaluating mixed age child care programs. The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios. When monitoring group size, the Idaho Child Care Program (ICCP) provider agreement, daycare license type, occupancy load, and child ratio points are all considered, and the lowest number of children is the most allowed. For example, if a provider requests a center daycare license for 50 children, but the total maximum occupancy load for the facility is set at

a maximum of 30 people on site by the local fire marshal, the provider would be limited to the occupancy load of no more than a total of 30 people (adults and children) onsite, and ratio points may further limit the amount of children allowed in the provider's care based on the children's ages. Cities and local municipalities may elect to administer more stringent group size requirements. The maximum occupancy load for a child care facility is required to be determined prior to the issuance of a daycare license. Occupant loads are determined by the local fire official according to the current Idaho Fire Code, which is based on the International Fire Code.
(<https://codes.iccsafe.org/content/IFC2021P2>)

Currently, Idaho does not limit group sizes based on children's age groups. This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address group sizes without hindering the important program flexibilities that our mixed-age childcare providers rely on. IDHW will request technical assistance regarding the implementation of this requirement.

v. Mixed-Age Groups (if applicable).

Ratio: Mixed Age Group Ratio - (39-1109, Idaho Code), The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios.

Group size: (IDAPA 16.06.03.101, 16.06.03.300, 16.06.03.360) Each type of child care setting is limited by their provider agreement or daycare license to the total number of children in their care. The ratio point system limits the number of children per provider based on the children's ages, an important developmental consideration when evaluating mixed age child care programs. The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios. When monitoring group size, the Idaho Child Care Program (ICCP) provider agreement, daycare license type, occupancy load, and child ratio points are all considered, and the lowest number of children is the most allowed. For example, if a provider requests a center daycare license for 50 children, but the total maximum occupancy load for the facility is set at a maximum of 30 people on site by the local fire marshal, the provider would be limited to the occupancy load of no more than a total of 30 people (adults and children) onsite, and ratio points may further limit the amount of children allowed in the provider's care based on the children's ages. Cities and local municipalities may elect to administer more stringent group size requirements. The maximum occupancy load for a child care facility is required to be determined prior to the issuance of a daycare license. Occupant loads are determined by the local fire official according to the current Idaho Fire Code, which is based on the International Fire Code.
(<https://codes.iccsafe.org/content/IFC2021P2>)

Currently, Idaho does not limit group sizes based on children's age groups. This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address group sizes without hindering the important program flexibilities that our mixed-age childcare providers rely on. IDHW will request technical assistance regarding the implementation of this requirement.

- b. If different, provide the ratios and group size requirements for the license-exempt center-based providers who receive CCDF funds under the following age groups:
- i. ☒ Not applicable. There are no differences in ratios and group size requirements.
 - ii. Infant:
 - iii. Toddler:
 - iv. Preschool:

- v. School-Age:
- vi. Mixed-Age Groups:
- c. Licensed CCDF family child care home providers:
 - i. Infant (if applicable)

Ratio: **Mixed Age Group Ratio - (39-1109, Idaho Code)**, The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios.

Group size: Six children (See IDAPA 16.06.12.101.03 "Family child care is for six (6) or fewer children.")

- ii. Toddler (if applicable)

Ratio: **Mixed Age Group Ratio - (39-1109, Idaho Code)**, The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children

in attendance and for determining compliance with child-to-staff ratios.

Group size: Six children (See IDAPA 16.06.12.101.03 "Family child care is for six (6) or fewer children.")

iii. Preschool (if applicable)

Ratio: Mixed Age Group Ratio - (39-1109, Idaho Code), The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios.

Group size: Six children (See IDAPA 16.06.12.101.03 "Family child care is for six (6) or fewer children.")

iv. School-Age (if applicable)

Ratio: Mixed Age Group Ratio - (39-1109, Idaho Code), The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:

1. Each child in attendance under the age of 24 months equals two points
2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points
3. Each child in attendance from 36 months to under five years of age equals one point
4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios.

Group size: **Six children (See IDAPA 16.06.12.101.03 "Family child care is for six (6) or fewer children.")**

v. **Mixed-Age Groups**

Ratio: **Mixed Age Group Ratio - (39-1109, Idaho Code), The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:**

1. Each child in attendance under the age of 24 months equals two points

2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points

3. Each child in attendance from 36 months to under five years of age equals one point

4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios.

Group size: **Six children (See IDAPA 16.06.12.101.03 "Family child care is for six (6) or fewer children.")**

d. **Are any of the responses above different for license-exempt family child care homes?**

☒ No.

☐ Yes. If yes, describe how the ratio and group size requirements for license-exempt providers vary by age of children served.

☐ Not applicable. The Lead Agency does not have license-exempt family child care homes.

e. **Licensed in-home care (care in the child's own home):**

i. **Infant (if applicable)**

Ratio: **Idaho does not license in-home care in the child's home.**

Group size: **Idaho does not license in-home care in the child's home.**

ii. **Toddler (if applicable)**

Ratio: **Idaho does not license in-home care in the child's home.**

Group size: **Idaho does not license in-home care in the child's home.**

iii. **Preschool (if applicable)**

Ratio: **Idaho does not license in-home care in the child's home.**

Group size: **Idaho does not license in-home care in the child's home.**

- iv. School-Age (if applicable)
Ratio: **Idaho does not license in-home care in the child's home.**
Group size: **Idaho does not license in-home care in the child's home.**

- v. Mixed-Age Groups (if applicable)
Ratio: **Idaho does not license in-home care in the child's home.**
Group size: **Idaho does not license in-home care in the child's home.**

- f. Are any of the responses above different for license-exempt in-home care?

☐ No.

☒ Yes. If yes, describe how the ratio and group size requirements for license-exempt in-home care vary by age of children served. **Mixed Age Group Ratio - (39-1109, Idaho Code), The maximum allowable child-to-staff ratio is based on a 12-point-per-staff member using the following point system:**

1. Each child in attendance under the age of 24 months equals two points

2. Each child in attendance from 24 months to under 36 months of age equals one and one-half (1-1/2) points

3. Each child in attendance from 36 months to under five years of age equals one point

4. Each child in attendance from five to under thirteen years of age equals one-half (1/2) point.

Each child in attendance shall be counted by the department for purposes of calculating maximum allowable points, counting the number of children in attendance and for determining compliance with child-to-staff ratios.

5.2.3 Teacher/caregiver qualifications for licensed, regulated, or registered care

Provide the teacher/caregiver qualifications for each category of care.

- a. Licensed center-based care

- i. Describe the teacher qualifications for licensed CCDF center-based care (e.g., degrees, credentials, etc.), including any variations based on the ages of children in care: **(16.06.03.200, 16.06.12)**

Lead and Assistant Teachers:

1. All Idaho Child Care Program (ICCP) child care providers offering services must be 18 years old or older

2. Persons 16 or 17 years old may provide child care if they have direct, on-site supervision from a licensed child care provider who is at least 18 years old

3. Each provider must certify that they do not have a communicable disease or

any physical or psychological condition that might pose a threat to the safety of a child in their care

Employees, Volunteers, and Other Individuals:

1. All employees, volunteers, and individuals aged 13 or older who have direct contact with or provide care to children eligible for ICCP benefits must successfully receive clearance from a comprehensive criminal history and background check
2. Each provider must certify that they do not have a communicable disease or any physical or psychological condition that might pose a threat to the safety of a child in their care

Owner/Operator:

1. For licensed providers, each owner or operator of a daycare center licensed by the Idaho Department of Health and Welfare (DHW) must receive and ensure that each staff member receives and completes four hours of on-going training every 12 months after the staff member's date of hire. For ICCP subsidy participating providers, each staff member who provides child care receives and completes twelve (12) hours of ongoing training every twelve (12) months after the staff member's date of hire.
2. Required trainings must be related to continuing education in child development, does not include their pediatric CPR and pediatric first aid training, and must be documented and maintained in each staff member's record
3. Each owner or operator of a daycare center, group daycare facility, or family daycare home voluntarily licensed by DHW must receive and ensure that each staff member receives and completes Pediatric Rescue Breathing, Infant-Child CPR, and Pediatric First Aid Treatment certification from a certified instructor.

- ii. Describe the director qualification for licensed CCDF center-based care, including any variations based on the ages of children in care or the number of staff employed: **(16.06.03.200)** No specific qualifications are required of directors. Each owner or operator of a daycare center licensed by the Idaho Department of Health and Welfare (DHW) must receive and ensure that each staff member receives and completes four hours of on-going training every 12 months after the staff member's date of hire. Required trainings must be related to continuing education in child development, does not include their pediatric CPR and pediatric first aid training, and must be documented and maintained in each staff member's record. Additionally, each owner or operator of a daycare center, group daycare facility, or family daycare home voluntarily licensed by DHW must receive and ensure that each staff member receives and completes Pediatric Rescue Breathing, Infant-Child CPR, and Pediatric First Aid Treatment certification from a certified instructor.

b. Licensed family child care

Describe the provider qualifications for licensed family child care homes, including any variations based on the ages of children in care: **(16.06.03)**

Lead and assistant teachers:

- 1. All Idaho Child Care Program (ICCP) child care providers offering services must be 18 years old or older**
- 2. Persons 16 or 17 years old may provide child care if they have direct, on-site supervision from a licensed child care provider who is at least 18 years old**
- 3. Each provider must certify that they do not have a communicable disease or any physical or psychological condition that might pose a threat to the safety of a child in their care**

Employees, Volunteers, and Other Individuals:

- 1. All employees, volunteers, and individuals aged 13 or older who have direct contact with or provide care to children eligible for ICCP benefits must successfully receive clearance from a comprehensive criminal history and background check**
- 2. Each provider must certify that they do not have a communicable disease or any physical or psychological condition that might pose a threat to the safety of a child in their care**

Owner/Operator:

- 1. For licensed providers, each owner or operator of a daycare center licensed by the Idaho Department of Health and Welfare (DHW) must receive and ensure that each staff member receives and completes four hours of on-going training every 12 months after the staff member's date of hire. For ICCP subsidy participating providers, each staff member who provides child care receives and completes twelve (12) hours of ongoing training every twelve (12) months after the staff member's date of hire.**
- 2. Required trainings must be related to continuing education in child development ,does not include their pediatric CPR and pediatric first aid training, and must be documented and maintained in each staff member's record**
- 3. Each owner or operator of a daycare center, group daycare facility, or family daycare home voluntarily licensed by DHW must receive and ensure that each staff member receives and completes Pediatric Rescue Breathing, Infant-Child CPR, and Pediatric First Aid Treatment certification from a certified instructor.**

- c. Licensed, regulated, or registered in-home care (care in the child's own home by a non-relative)

Describe the provider qualifications for licensed, regulated, or registered in-home care providers (care in the child's own home) including any variations based on the ages of children in care: **1. All Idaho Child Care Program (ICCP) child care providers offering services must be 18 years old or older**

- 2. Each provider must certify that they do not have a communicable disease or any physical or psychological condition that might pose a threat to the safety of a child in their care**
- 3. All employees, volunteers, and individuals aged 13 or older who have direct contact with or provide care to children eligible for ICCP benefits must successfully receive clearance from a comprehensive criminal history and background check**
- 4. Each owner or operator of a daycare center, group daycare facility, or family daycare home voluntarily licensed by DHW must receive and ensure that each staff member receives and completes Pediatric Rescue Breathing, Infant-Child CPR, and Pediatric First Aid Treatment certification from a certified instructor.**

5.2.4 Teacher/caregiver qualifications for license-exempt providers

Provide the teacher/provider qualification requirements (for instance, age, high school diploma, specific training, etc.) for the license-exempt providers under the following categories of care:

- a. License-exempt center-based child care. **Idaho does not have license-exempt center-based child care (16.06.03.101.03).**
- b. License-exempt home-based child care. **(16.06.03.200, 16.06.12)**

Lead and Assistant Teachers:

- 1. All Idaho Child Care Program (ICCP) child care providers offering services must be 18 years old or older
- 2. Persons 16 or 17 years old may provide child care if they have direct, on-site supervision from a licensed child care provider who is at least 18 years old
- 3. Each provider must certify that they do not have a communicable disease or any physical or psychological condition that might pose a threat to the safety of a child in their care

Employees, Volunteers, and Other Individuals:

- 1. All employees, volunteers, and individuals aged 13 or older who have direct contact with or provide care to children eligible for ICCP benefits must successfully receive clearance from a comprehensive criminal history and background check
- 2. Each provider must certify that they do not have a communicable disease or any physical or psychological condition that might pose a threat to the safety of a child in their care

Owner/Operator:

- 1. For licensed providers, each owner or operator of a daycare center licensed by the Idaho Department of Health and Welfare (DHW) must receive and ensure that each staff member receives and completes four hours of on-going training every 12 months after the staff member's date of hire. For ICCP subsidy participating providers, each staff member who provides child care receives and completes twelve (12) hours of ongoing training every twelve (12) months after the staff member's date of hire.
- 2. Required trainings must be related to continuing education in child development, does not include their pediatric CPR and pediatric first aid training, and must be documented and maintained in each staff member's record
- 3. Each owner or operator of a daycare center, group daycare facility, or family daycare home voluntarily licensed by DHW must receive and ensure that each staff member receives and completes Pediatric Rescue Breathing, Infant-Child CPR, and Pediatric First Aid Treatment certification from a certified instructor.

- c. License-exempt in-home care (care in the child's own home). **Teacher/Caregiver**

Qualifications:

- 1. All Idaho Child Care Program (ICCP) child care providers offering services must be 18 years old or older
- 2. Each provider must certify that they do not have a communicable disease or any

physical or psychological condition that might pose a threat to the safety of a child in their care

3. All employees, volunteers, and individuals aged 13 or older who have direct contact with or provide care to children eligible for ICCP benefits must successfully receive clearance from a comprehensive criminal history and background check

5.3 Health and Safety Standards for CCDF Providers

Lead Agencies must have health and safety standards for providers serving children receiving CCDF assistance relating to the required health and safety topics as appropriate to the provider setting and age of the children served. This requirement is applicable to all child care programs receiving CCDF funds regardless of licensing status (i.e., licensed or license-exempt). The only exception to this requirement is for relative providers, as defined by CCDF. Lead Agencies have the option of exempting certain relatives from any or all CCDF health and safety requirements.

Exemptions for relative providers' standards requirements will be addressed in question 5.8.1.

Describe the following health and safety standards for programs serving children receiving CCDF assistance on the following topics (note that monitoring and enforcement will be addressed in subsection 5.5):

5.3.1 Prevention and control of infectious diseases (including immunizations) health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the prevention and control of infectious diseases for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:**
05. Child's Health Information.
 - a. Immunization record or waiver of exemption form or statement;
 - b. Any medical conditions or allergies that could affect the care of the child; and
 - c. Medications the child is taking or may be allergic to.

IDAPA 16.06.03.380. HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

01. Food. Must be from an approved source under IDAPA 16.02.19, *Idaho Food Code*. Food must not be served past expiration or "use by" date.

02. Food Preparation. Food for use in daycare facilities must be prepared and served in a sanitary manner with sanitized utensils and on surfaces that have been cleaned, rinsed, and sanitized prior to use to prevent cross-contamination.

a. Frozen food must be thawed in the refrigerator, under cold running water, or as part of the cooking process and cooked to proper temperatures under IDAPA 16.02.19, *Idaho Food Code*.

b. Individuals preparing food must use proper hand-washing techniques, minimize

bare hand contact with food, and wear clean clothes.

03. Food Temperatures. Potentially hazardous foods must be kept refrigerated at forty-one degrees Fahrenheit (41°F) or below, held hot at one hundred thirty-five degrees Fahrenheit (135°F) or more, and reheated or cooled at safe temperatures under IDAPA 16.02.19, *Idaho Food Code*. Refrigerators must be equipped with an accurate thermometer.

04. Food Storage. All food that is served in daycare facilities must be stored in such a manner that protects it from potential contamination. There must be no evidence of pests present in the daycare facility.

05. Food Contact Surfaces. Must be kept clean and sanitized, including counters, serving tables, high chair trays, and cutting boards.

06. Dishwashing Sanitizing. Dishes, glasses, utensils, silverware, and all other objects used for food preparation and eating must be sanitized using appropriate sanitizing procedures.

07. Utensil Storage. Clean utensils must be stored on clean shelves or drawers and not subject to recontamination, and sharp knives and other sharp objects be kept out of reach of children.

08. Garbage. Must be kept covered or inaccessible to children.

09. Hand Washing. Children and facility staff must be provided with individual or disposable towels for hand drying, and the hand washing area be equipped with soap and warm and cold running water.

10. Diaper Changing. Must be conducted in such a manner as to prevent the spread of communicable diseases, be separate from food preparation and serving areas, and have easy access to a hand washing sink.

11. Sleeping Areas. Children sleeping at the facility must have separate cots, mats, or beds and blankets.

13. Restrooms, Water Supply, and Sewage. All daycare facilities must have restrooms.

a. Each facility must have at least one (1) flushable toilet and one (1) hand washing sink with warm and cold water per restroom.

b. Plumbing and bathroom fixtures must be in good condition.

c. All daycare facilities and homes must comply with IDAPA 24.39.30, *Rules of Building Safety (Building Code Rules)*.

14. Water Supply. The facility's water supply must meet one (1) of the following requirements:

a. Be from a public water system that is maintained under IDAPA 58.01.08, *Idaho Rules for Public Drinking Water Systems*, at the time of initial or renewal application; or

b. Be from a private source, such as well or spring, be tested annually for bacteria and nitrate, and be approved by the Department.

c. Water used for consumption at a daycare facility is from an acceptable source. Temporary use of bottled water or boiled water may be allowed for a period specified by the Department.

15. Sewage Disposal. Facility sewage must be disposed of through a public system, or in the absence of a public system, in a manner approved by the local health authority under IDAPA 58.01.03 *Individual/Subsurface Sewage Disposal Rules*.

21. Adequate Heat, Light, and Ventilation. A daycare facility must have adequate

heat, light and ventilation. Windows and doors must be screened if used for ventilation.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

05. Animals and Pets. Any pet or animal present at the facility, indoors or outdoors, must be in good health, show no evidence of carrying disease, and be a friendly companion of the children. The operator must maintain the animal's vaccinations and vaccination records which will be made available to the Department upon request.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to.

IDAPA 16.06.03.380. HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

01. Food. Must be from an approved source under IDAPA 16.02.19, *Idaho Food Code*. Food must not be served past expiration or *use by* date.

02. Food Preparation. Food for use in daycare facilities must be prepared and served in a sanitary manner with sanitized utensils and on surfaces that have been cleaned, rinsed, and sanitized prior to use to prevent cross-contamination.

a. Frozen food must be thawed in the refrigerator, under cold running water, or as part of the cooking process and cooked to proper temperatures under IDAPA 16.02.19, *Idaho Food Code*.

b. Individuals preparing food must use proper hand-washing techniques, minimize bare hand contact with food, and wear clean clothes.

03. Food Temperatures. Potentially hazardous foods must be kept refrigerated at forty-one degrees Fahrenheit (41°F) or below, held hot at one hundred thirty-five degrees Fahrenheit (135°F) or more, and reheated or cooled at safe temperatures under IDAPA 16.02.19, *Idaho Food Code*. Refrigerators must be equipped with an accurate thermometer.

04. Food Storage. All food that is served in daycare facilities must be stored in such a manner that protects it from potential contamination. There must be no evidence of pests present in the daycare facility.

05. Food Contact Surfaces. Must be kept clean and sanitized, including counters, serving tables, high chair trays, and cutting boards.
06. Dishwashing Sanitizing. Dishes, glasses, utensils, silverware, and all other objects used for food preparation and eating must be sanitized using appropriate sanitizing procedures.
07. Utensil Storage. Clean utensils must be stored on clean shelves or drawers and not subject to recontamination, and sharp knives and other sharp objects be kept out of reach of children.
08. Garbage. Must be kept covered or inaccessible to children.
09. Hand Washing. Children and facility staff must be provided with individual or disposable towels for hand drying, and the hand washing area be equipped with soap and warm and cold running water.
10. Diaper Changing. Must be conducted in such a manner as to prevent the spread of communicable diseases, be separate from food preparation and serving areas, and have easy access to a hand washing sink.
11. Sleeping Areas. Children sleeping at the facility must have separate cots, mats, or beds and blankets.
13. Restrooms, Water Supply, and Sewage. All daycare facilities must have restrooms.
- a. Each facility must have at least one (1) flushable toilet and one (1) hand washing sink with warm and cold water per restroom.
 - b. Plumbing and bathroom fixtures must be in good condition.
 - c. All daycare facilities and homes must comply with IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).
14. Water Supply. The facility's water supply must meet one (1) of the following requirements:
- a. Be from a public water system that is maintained under IDAPA 58.01.08, [Idaho Rules for Public Drinking Water Systems](#), at the time of initial or renewal application; or
 - b. Be from a private source, such as well or spring, be tested annually for bacteria and nitrate, and be approved by the Department.
 - c. Water used for consumption at a daycare facility is from an acceptable source. Temporary use of bottled water or boiled water may be allowed for a period specified by the Department.
15. Sewage Disposal. Facility sewage must be disposed of through a public system, or in the absence of a public system, in a manner approved by the local health authority under IDAPA 58.01.03 [Individual/Subsurface Sewage Disposal Rules](#).
21. Adequate Heat, Light, and Ventilation. A daycare facility must have adequate heat, light and ventilation. Windows and doors must be screened if used for ventilation.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

05. Animals and Pets. Any pet or animal present at the facility, indoors or outdoors, must be in good health, show no evidence of carrying disease, and be a friendly companion of the children. The operator must maintain the animal's vaccinations and vaccination records which will be made available to the

Department upon request.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:**

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to.

IDAPA 16.06.03.380. HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

01. Food. Must be from an approved source under IDAPA 16.02.19, Idaho Food Code. Food must not be served past expiration or use by date.

02. Food Preparation. Food for use in daycare facilities must be prepared and served in a sanitary manner with sanitized utensils and on surfaces that have been cleaned, rinsed, and sanitized prior to use to prevent cross-contamination.

a. Frozen food must be thawed in the refrigerator, under cold running water, or as part of the cooking process and cooked to proper temperatures under IDAPA 16.02.19, Idaho Food Code.

b. Individuals preparing food must use proper hand-washing techniques, minimize bare hand contact with food, and wear clean clothes.

03. Food Temperatures. Potentially hazardous foods must be kept refrigerated at forty-one degrees Fahrenheit (41°F) or below, held hot at one hundred thirty-five degrees Fahrenheit (135°F) or more, and reheated or cooled at safe temperatures under IDAPA 16.02.19, Idaho Food Code. Refrigerators must be equipped with an accurate thermometer.

04. Food Storage. All food that is served in daycare facilities must be stored in such a manner that protects it from potential contamination. There must be no evidence of pests present in the daycare facility.

05. Food Contact Surfaces. Must be kept clean and sanitized, including counters, serving tables, high chair trays, and cutting boards.

06. Dishwashing Sanitizing. Dishes, glasses, utensils, silverware, and all other

objects used for food preparation and eating must be sanitized using appropriate sanitizing procedures.

07. Utensil Storage. Clean utensils must be stored on clean shelves or drawers and not subject to recontamination, and sharp knives and other sharp objects be kept out of reach of children.

08. Garbage. Must be kept covered or inaccessible to children.

09. Hand Washing. Children and facility staff must be provided with individual or disposable towels for hand drying, and the hand washing area be equipped with soap and warm and cold running water.

10. Diaper Changing. Must be conducted in such a manner as to prevent the spread of communicable diseases, be separate from food preparation and serving areas, and have easy access to a hand washing sink.

11. Sleeping Areas. Children sleeping at the facility must have separate cots, mats, or beds and blankets.

13. Restrooms, Water Supply, and Sewage. All daycare facilities must have restrooms.

a. Each facility must have at least one (1) flushable toilet and one (1) hand washing sink with warm and cold water per restroom.

b. Plumbing and bathroom fixtures must be in good condition.

c. All daycare facilities and homes must comply with IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

14. Water Supply. The facility's water supply must meet one (1) of the following requirements:

a. Be from a public water system that is maintained under IDAPA 58.01.08, [Idaho Rules for Public Drinking Water Systems](#), at the time of initial or renewal application; or

b. Be from a private source, such as well or spring, be tested annually for bacteria and nitrate, and be approved by the Department.

c. Water used for consumption at a daycare facility is from an acceptable source. Temporary use of bottled water or boiled water may be allowed for a period specified by the Department.

15. Sewage Disposal. Facility sewage must be disposed of through a public system, or in the absence of a public system, in a manner approved by the local health authority under IDAPA 58.01.03 [Individual/Subsurface Sewage Disposal Rules](#).

21. Adequate Heat, Light, and Ventilation. A daycare facility must have adequate heat, light and ventilation. Windows and doors must be screened if used for ventilation.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

05. Animals and Pets. Any pet or animal present at the facility, indoors or outdoors, must be in good health, show no evidence of carrying disease, and be a friendly companion of the children. The operator must maintain the animal's vaccinations and vaccination records which will be made available to the Department upon request.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:
- 05. Child's Health Information.**
- a. Immunization record or waiver of exemption form or statement;
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04. Food Storage. All food that is served in daycare facilities must be stored in such a manner that protects it from potential contamination. There must be no evidence of pests present in the daycare facility.

05. Food Contact Surfaces. Must be kept clean and sanitized, including counters, serving tables, high chair trays, and cutting boards.

06. Dishwashing Sanitizing. Dishes, glasses, utensils, silverware, and all other objects used for food preparation and eating must be sanitized using appropriate sanitizing procedures.

07. Utensil Storage. Clean utensils must be stored on clean shelves or drawers and not subject to recontamination, and sharp knives and other sharp objects be kept out of reach of children.

08. Garbage. Must be kept covered or inaccessible to children.

09. Hand Washing. Children and facility staff must be provided with individual or disposable towels for hand drying, and the hand washing area be equipped with soap and warm and cold running water.

10. Diaper Changing. Must be conducted in such a manner as to prevent the spread of communicable diseases, be separate from food preparation and serving

areas, and have easy access to a hand washing sink.

11. Sleeping Areas. Children sleeping at the facility must have separate cots, mats, or beds and blankets.

13. Restrooms, Water Supply, and Sewage. All daycare facilities must have restrooms.

a. Each facility must have at least one (1) flushable toilet and one (1) hand washing sink with warm and cold water per restroom.

b. Plumbing and bathroom fixtures must be in good condition.

c. All daycare facilities and homes must comply with IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

14. Water Supply. The facility's water supply must meet one (1) of the following requirements:

a. Be from a public water system that is maintained under IDAPA 58.01.08, [Idaho Rules for Public Drinking Water Systems](#), at the time of initial or renewal application; or

b. Be from a private source, such as well or spring, be tested annually for bacteria and nitrate, and be approved by the Department.

c. Water used for consumption at a daycare facility is from an acceptable source. Temporary use of bottled water or boiled water may be allowed for a period specified by the Department.

15. Sewage Disposal. Facility sewage must be disposed of through a public system, or in the absence of a public system, in a manner approved by the local health authority under IDAPA 58.01.03 [Individual/Subsurface Sewage Disposal Rules](#).

21. Adequate Heat, Light, and Ventilation. A daycare facility must have adequate heat, light and ventilation. Windows and doors must be screened if used for ventilation.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

05. Animals and Pets. Any pet or animal present at the facility, indoors or outdoors, must be in good health, show no evidence of carrying disease, and be a friendly companion of the children. The operator must maintain the animal's vaccinations and vaccination records which will be made available to the Department upon request.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

a. Immunization record or waiver of exemption form or statement;

b. Any medical conditions or allergies that could affect the care of the child; and

c. Medications the child is taking or may be allergic to.

IDAPA 16.06.03.380. HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

01. Food. Must be from an approved source under IDAPA 16.02.19, *Idaho Food Code*. Food must not be served past expiration or "use by" date.

02. Food Preparation. Food for use in daycare facilities must be prepared and served in a sanitary manner with sanitized utensils and on surfaces that have been cleaned, rinsed, and sanitized prior to use to prevent cross-contamination.

a. Frozen food must be thawed in the refrigerator, under cold running water, or as part of the cooking process and cooked to proper temperatures under IDAPA 16.02.19, *Idaho Food Code*.

b. Individuals preparing food must use proper hand-washing techniques, minimize bare hand contact with food, and wear clean clothes.

03. Food Temperatures. Potentially hazardous foods must be kept refrigerated at forty-one degrees Fahrenheit (41°F) or below, held hot at one hundred thirty-five degrees Fahrenheit (135°F) or more, and reheated or cooled at safe temperatures under IDAPA 16.02.19, *Idaho Food Code*. Refrigerators must be equipped with an accurate thermometer.

04. Food Storage. All food that is served in daycare facilities must be stored in such a manner that protects it from potential contamination. There must be no evidence of pests present in the daycare facility.

05. Food Contact Surfaces. Must be kept clean and sanitized, including counters, serving tables, high chair trays, and cutting boards.

06. Dishwashing Sanitizing. Dishes, glasses, utensils, silverware, and all other objects used for food preparation and eating must be sanitized using appropriate sanitizing procedures.

07. Utensil Storage. Clean utensils must be stored on clean shelves or drawers and not subject to recontamination, and sharp knives and other sharp objects be kept out of reach of children.

08. Garbage. Must be kept covered or inaccessible to children.

09. Hand Washing. Children and facility staff must be provided with individual or disposable towels for hand drying, and the hand washing area be equipped with soap and warm and cold running water.

10. Diaper Changing. Must be conducted in such a manner as to prevent the spread of communicable diseases, be separate from food preparation and serving areas, and have easy access to a hand washing sink.

11. Sleeping Areas. Children sleeping at the facility must have separate cots, mats, or beds and blankets.

13. Restrooms, Water Supply, and Sewage. All daycare facilities must have restrooms.

a. Each facility must have at least one (1) flushable toilet and one (1) hand washing sink with warm and cold water per restroom.

b. Plumbing and bathroom fixtures must be in good condition.

c. All daycare facilities and homes must comply with IDAPA 24.39.30, *Rules of Building Safety (Building Code Rules)*.

14. Water Supply. The facility's water supply must meet one (1) of the following requirements:

- a. Be from a public water system that is maintained under IDAPA 58.01.08, **Idaho Rules for Public Drinking Water Systems**, at the time of initial or renewal application; or
- b. Be from a private source, such as well or spring, be tested annually for bacteria and nitrate, and be approved by the Department.
- c. Water used for consumption at a daycare facility is from an acceptable source. Temporary use of bottled water or boiled water may be allowed for a period specified by the Department.

15. Sewage Disposal. Facility sewage must be disposed of through a public system, or in the absence of a public system, in a manner approved by the local health authority under IDAPA 58.01.03 **Individual/Subsurface Sewage Disposal Rules**.

21. Adequate Heat, Light, and Ventilation. A daycare facility must have adequate heat, light and ventilation. Windows and doors must be screened if used for ventilation.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

05. Animals and Pets. Any pet or animal present at the facility, indoors or outdoors, must be in good health, show no evidence of carrying disease, and be a friendly companion of the children. The operator must maintain the animal's vaccinations and vaccination records which will be made available to the Department upon request.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

- b. Provide the standards, appropriate to the provider setting and age of children, that address that children attending child care programs under CCDF are age-appropriately immunized, according to the latest recommendation for childhood immunizations of the respective State public health agency, for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.02.11.101. COMPLIANCE.** The parent, custodian, or guardian of a child must comply with the provisions contained in this chapter within fourteen (14) days of initial attendance to any licensed daycare facility in Idaho.
IDAPA 16.02.11.102. EVIDENCE OF IMMUNIZATION STATUS. The Within the deadlines established in Section 101 of these rules, a parent, custodian, or guardian of each child must present to the licensed daycare facility operator an immunization record. A child who has received at least one (1) dose of each required vaccine and is currently on schedule for subsequent immunizations may conditionally attend daycare when a schedule of intended immunizations form is provided. The licensed daycare facility operator must have a schedule of intended immunizations form completed by a parent, custodian, or guardian for any child who is not immunized, excepted, or exempted, and who is in the process of

receiving, or has been scheduled to receive, the required immunizations.

IDAPA 16.02.11.110. EXEMPTIONS TO IMMUNIZATION REQUIREMENT. In Idaho, families may exempt children from immunization requirements for life or health endangering circumstances, religious exemptions, or other exemptions. Providers must retain a copy of the signed exemption statement in lieu of immunization records.

IDAPA 16.06.12.105. ELIGIBLE CHILD. A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions: (3-17-22) 01. Immunizations Requirements. A child must be immunized in accordance with IDAPA 16.02.11, **Immunization Requirements for Licensed Daycare Facility Attendees.** Child care benefits can continue during a reasonable period necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

IDAPA 16.06.12.810 DOCUMENTATION OF SERVICES AND ACCESS TO RECORDS.

01. Documentation of Services. Providers must generate documentation at the time of service sufficient to support the reimbursement for child care services. Documentation must be legible and retained for a period of three (3) years from the date the child care was provided. Documentation to support child care services includes:

b. Immunization records, conditional admittance form, or exemption form according to IDAPA 16.02.11, **Immunization Requirements for Licensed Daycare Facility Attendees.**

IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

a. Immunization record or waiver of exemption form or statement;

b. Any medical conditions or allergies that could affect the care of the child; and

c. Medications the child is taking or may be allergic to.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.02.11.101. COMPLIANCE.** The parent, custodian, or guardian of a child must comply with the provisions contained in this chapter within fourteen (14) days of initial attendance to any licensed daycare facility in Idaho.
- IDAPA 16.02.11.102. EVIDENCE OF IMMUNIZATION STATUS.** The Within the deadlines established in Section 101 of these rules, a parent, custodian, or guardian of each child must present to the licensed daycare facility operator an immunization record. A child who has received at least one (1) dose of each required vaccine and is currently on schedule for subsequent immunizations may conditionally attend daycare when a schedule of intended immunizations form is provided. The licensed daycare facility operator must have a schedule of intended immunizations form completed by a parent, custodian, or guardian for any child

who is not immunized, excepted, or exempted, and who is in the process of receiving, or has been scheduled to receive, the required immunizations. IDAPA 16.02.11.110. EXEMPTIONS TO IMMUNIZATION REQUIREMENT. In Idaho, families may exempt children from immunization requirements for life or health endangering circumstances, religious exemptions, or other exemptions. Providers must retain a copy of the signed exemption statement in lieu of immunization records.

IDAPA 16.06.12.105. ELIGIBLE CHILD. A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions:

01. Immunizations Requirements. A child must be immunized in accordance with IDAPA 16.02.11, ☐ Immunization Requirements for Licensed Daycare Facility Attendees.☐ Child care benefits can continue during a reasonable period necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

IDAPA 16.06.12.810 DOCUMENTATION OF SERVICES AND ACCESS TO RECORDS.

01. Documentation of Services. Providers must generate documentation at the time of service sufficient to support the reimbursement for child care services. Documentation must be legible and retained for a period of three (3) years from the date the child care was provided. Documentation to support child care services includes:

b. Immunization records, conditional admittance form, or exemption form according to IDAPA 16.02.11, ☐ Immunization Requirements for Licensed Daycare Facility Attendees.☐

IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: IDAPA 16.06.12.105. ELIGIBLE CHILD. A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions:

01. Immunizations Requirements. A child must be immunized in accordance with IDAPA 16.02.11, ☐ Immunization Requirements for Licensed Daycare Facility Attendees.☐ Child care benefits can continue during a reasonable period

necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

IDAPA 16.06.12.810 DOCUMENTATION OF SERVICES AND ACCESS TO RECORDS.

01. Documentation of Services. Providers must generate documentation at the time of service sufficient to support the reimbursement for child care services. Documentation must be legible and retained for a period of three (3) years from the date the child care was provided. Documentation to support child care services includes:

b. Immunization records, conditional admittance form, or exemption form according to IDAPA 16.02.11, ☐Immunization Requirements for Licensed Daycare Facility Attendees.☐

IDAPA 16.02.11.101. COMPLIANCE. The parent, custodian, or guardian of a child must comply with the provisions contained in this chapter within fourteen (14) days of initial attendance to any licensed daycare facility in Idaho.

IDAPA 16.02.11.102. EVIDENCE OF IMMUNIZATION STATUS. The Within the deadlines established in Section 101 of these rules, a parent, custodian, or guardian of each child must present to the licensed daycare facility operator an immunization record. A child who has received at least one (1) dose of each required vaccine and is currently on schedule for subsequent immunizations may conditionally attend daycare when a schedule of intended immunizations form is provided. The licensed daycare facility operator must have a schedule of intended immunizations form completed by a parent, custodian, or guardian for any child who is not immunized, excepted, or exempted, and who is in the process of receiving, or has been scheduled to receive, the required immunizations.

IDAPA 16.02.11.110. EXEMPTIONS TO IMMUNIZATION REQUIREMENT. In Idaho, families may exempt children from immunization requirements for life or health endangering circumstances, religious exemptions, or other exemptions. Providers must retain a copy of the signed exemption statement in lieu of immunization records.

IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.02.11.101. COMPLIANCE.** The parent, custodian, or guardian of a child must comply with the provisions contained in this chapter within fourteen (14) days of initial attendance to any licensed daycare facility in Idaho.
IDAPA 16.02.11.102. EVIDENCE OF IMMUNIZATION STATUS. The Within the deadlines established in Section 101 of these rules, a parent, custodian, or guardian of each child must present to the licensed daycare facility operator an immunization record. A child who has received at least one (1) dose of each

required vaccine and is currently on schedule for subsequent immunizations may conditionally attend daycare when a schedule of intended immunizations form is provided. The licensed daycare facility operator must have a schedule of intended immunizations form completed by a parent, custodian, or guardian for any child who is not immunized, excepted, or exempted, and who is in the process of receiving, or has been scheduled to receive, the required immunizations.

IDAPA 16.02.11.110. EXEMPTIONS TO IMMUNIZATION REQUIREMENT. In Idaho, families may exempt children from immunization requirements for life or health endangering circumstances, religious exemptions, or other exemptions. Providers must retain a copy of the signed exemption statement in lieu of immunization records.

IDAPA 16.06.12.105. ELIGIBLE CHILD. A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions: (3-17-22) 01. Immunizations Requirements. A child must be immunized in accordance with IDAPA 16.02.11, **Immunization Requirements for Licensed Daycare Facility Attendees.** Child care benefits can continue during a reasonable period necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

IDAPA 16.06.12.810 DOCUMENTATION OF SERVICES AND ACCESS TO RECORDS. 01. Documentation of Services. Providers must generate documentation at the time of service sufficient to support the reimbursement for child care services. Documentation must be legible and retained for a period of three (3) years from the date the child care was provided. Documentation to support child care services includes:

b. Immunization records, conditional admittance form, or exemption form according to IDAPA 16.02.11, **Immunization Requirements for Licensed Daycare Facility Attendees.**

IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.02.11.101. COMPLIANCE.** The parent, custodian, or guardian of a child must comply with the provisions contained in this chapter within fourteen (14) days of initial attendance to any licensed daycare facility in Idaho.

IDAPA 16.02.11.102. EVIDENCE OF IMMUNIZATION STATUS. The Within the deadlines established in Section 101 of these rules, a parent, custodian, or guardian of each child must present to the licensed daycare facility operator an immunization record. A child who has received at least one (1) dose of each required vaccine and is currently on schedule for subsequent immunizations may

conditionally attend daycare when a schedule of intended immunizations form is provided. The licensed daycare facility operator must have a schedule of intended immunizations form completed by a parent, custodian, or guardian for any child who is not immunized, excepted, or exempted, and who is in the process of receiving, or has been scheduled to receive, the required immunizations.

IDAPA 16.02.11.110. EXEMPTIONS TO IMMUNIZATION REQUIREMENT. In Idaho, families may exempt children from immunization requirements for life or health endangering circumstances, religious exemptions, or other exemptions. Providers must retain a copy of the signed exemption statement in lieu of immunization records.

IDAPA 16.06.12.105. ELIGIBLE CHILD. A family can only receive child care benefits for eligible children. A child is eligible for child care benefits under the following conditions:

01. Immunizations Requirements. A child must be immunized in accordance with IDAPA 16.02.11, **Immunization Requirements for Licensed Daycare Facility Attendees.** Child care benefits can continue during a reasonable period necessary for the child to be immunized. Parents must provide evidence that the child has been immunized unless the child is attending school.

IDAPA 16.06.12.810 DOCUMENTATION OF SERVICES AND ACCESS TO RECORDS.

01. Documentation of Services. Providers must generate documentation at the time of service sufficient to support the reimbursement for child care services. Documentation must be legible and retained for a period of three (3) years from the date the child care was provided. Documentation to support child care services includes:

b. Immunization records, conditional admittance form, or exemption form according to IDAPA 16.02.11, **Immunization Requirements for Licensed Daycare Facility Attendees.**

IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;**
- b. Any medical conditions or allergies that could affect the care of the child; and**
- c. Medications the child is taking or may be allergic to.**

- 5.3.2 Prevention of sudden infant death syndrome and the use of safe-sleep practices health and safety standard

Provide the standards, appropriate to the provider setting and age of children, that address the prevention of sudden infant death syndrome and use of safe sleeping practices for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be

conducted annually by a qualified inspector designated by the Department and will be unannounced.

12. Safe Sleep. Providers must place newborn infants up to twelve (12) months old in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified sleep space.

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

14. Safe Sleep. Providers must place newborn infants to twelve (12) months in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified crib.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

12. Safe Sleep. Providers must place newborn infants up to twelve (12) months old in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified sleep space.

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

14. Safe Sleep. Providers must place newborn infants to twelve (12) months in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified crib.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department

and will be unannounced.

12. Safe Sleep. Providers must place newborn infants up to twelve (12) months old in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified sleep space.

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

14. Safe Sleep. Providers must place newborn infants to twelve (12) months in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified crib.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

12. Safe Sleep. Providers must place newborn infants up to twelve (12) months old in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified sleep space.

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

14. Safe Sleep. Providers must place newborn infants to twelve (12) months in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified crib.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

12. Safe Sleep. Providers must place newborn infants up to twelve (12) months old in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified sleep space.

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

14. Safe Sleep. Providers must place newborn infants to twelve (12) months in a safe sleep environment. Safe sleep practices include alone, on their backs, and in a Consumer Product Safety Commission (CPSC) certified crib.

5.3.3 Administration of medication, consistent with standards for parental consent health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the administration of medication for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.
 - iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x] Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children
- b. Provide the standards, appropriate to the provider setting and age of children, that address obtaining permission from parents to administer medications to children for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS.** Each daycare facility licensed by the

Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.

- iii. All CCDF-eligible licensed in-home care. Provide the standard:
[x]Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.**
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.
- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.**
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.**
20. Medication. No person can administer any medication to a child without it first being authorized by a parent or caretaker. All medications, refrigerated or unrefrigerated, must be in a locked box or otherwise in inaccessible to children.

5.3.4 Prevention of and response to emergencies due to food and allergic reactions health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the *prevention* of emergencies due to food and allergic reactions for the following

CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.03.201**
Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:
05. Child's Health Information.
 - a. Immunization record or waiver of exemption form or statement;
 - b. Any medical conditions or allergies that could affect the care of the child;
and
 - c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.03.201** **Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:**
05. Child's Health Information.
 - a. Immunization record or waiver of exemption form or statement;
 - b. Any medical conditions or allergies that could affect the care of the child;
and
 - c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**
[x] Not applicable.
- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:

IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

- d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

- d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
- b. Any medical conditions or allergies that could affect the care of the child; and
- c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- b. Provide the standards, appropriate to the provider setting and age of children, that address the *response* to emergencies due to food and allergic reactions for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
b. Any medical conditions or allergies that could affect the care of the child;
and
c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.03.201** Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;
b. Any medical conditions or allergies that could affect the care of the child;
and
c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

[x] Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:**

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;**
b. Any medical conditions or allergies that could affect the care of the child;
and
c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:**

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;**
b. Any medical conditions or allergies that could affect the care of the child;
and
c. Medications the child is taking or may be allergic to

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event

that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.03.201 Each owner or operator of a daycare facility must maintain records for each child in attendance covering the previous twelve-month period. The record must contain the following:**

05. Child's Health Information.

- a. Immunization record or waiver of exemption form or statement;**
- b. Any medical conditions or allergies that could affect the care of the child; and**
- c. Medications the child is taking or may be allergic to**

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

19. Disaster and Emergency Planning. Providers must have documented plans for emergencies resulting from a natural disaster, or human-caused event that include:

d. Procedures for the prevention of and response to emergencies due to food and allergic reactions

- 5.3.5 Building and physical premises safety, including the identification of and protection from hazards, bodies of water, and vehicular traffic health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from building and physical premises hazards for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

16. Alcohol and Illegal Drugs. Must not be used by providers, owners, operators, staff, volunteers, children, or visitors at daycare facilities, in the presence of children during hours of operation, or in vehicles while transporting children.

a. Any individual under the influence of alcohol or drugs is not permitted at or in the daycare facility.

b. Illegal drugs are prohibited by law and therefore are not allowed on the premises of any licensed daycare facility at any time.

17. Smoke-Free Environment. Children must be afforded a smoke-free environment during all daycare hours, whether indoors or outdoors. While children are in care, the operator and all staff must ensure that no smoking or other tobacco use occurs within the facility, in outdoor areas, or in vehicles when children are present.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

02. Heat-Producing Equipment. A furnace, fireplace, wood-burning stove, water heater, and other flame or heat-producing equipment must be installed and maintained as recommended by the manufacturer and protected on all surfaces by screens or other means.

03. Portable Heating Devices. Must be limited and approved for use and location by the Fire Inspector prior to use within a facility and will not be used during sleeping hours.

04. Storage of Weapons, Firearms, and Ammunition. Firearms or other weapons stored at a daycare facility must be kept in a locked cabinet, gun safe, or other container that is inaccessible to children, while children are in attendance. Keys to these containers must also be inaccessible to children

a. Ammunition must be stored in a locked container separate from firearms.

b. Matches, lighters, and any other means of starting fires must be kept away from and out of the reach of children.

c. Other weapons that could cause harm must be stored out of reach of children.

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

01. Appliances and Electrical Cords. All appliances, lamp cords, exposed light sockets, and electrical outlets will be protected to prevent electrocution.

02. Balconies and Stairways. Balconies and stairways accessible to children will have substantial railings as required by IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

03. Stairway Protection. Where an operator cares for children less than three (3) years old, stairways will be protected to prevent child access to stairs.

04. Hazardous Area Restrictions. Based on the age and functioning level of children in care and the type of hazard and the area surrounding the hazard, the area will be restricted to prevent easy access to the hazard.

05. Fueled Equipment. Fueled equipment including motorcycles, mopeds, lawncare equipment, and portable cooking equipment. This equipment will not be stored or repaired in areas where children are present.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

b. Outdoor equipment, such as climbing apparatus, slides, and swings will be anchored firmly and placed in a safe location and according to the manufacturer's instructions.

c. Outdoor play areas will be designed so that all parts are always visible and easily supervised by staff.

d. Toys, play equipment, and any other equipment used by the children will be of substantial construction and free from rough edges and sharp corners. Unguarded ladders on slides will be kept in good repair and well-maintained.

e. Toys and objects with a diameter of less than one (1) inch (two point five (2.5) centimeters), objects with removable parts that have a diameter of less than one (1) inch (two point five (2.5) centimeters), plastic bags, styrofoam objects, and balloons will not be accessible to children ages three (3) and under, or children who are known to place such objects in their mouths.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard:
16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

16. Alcohol and Illegal Drugs. Must not be used by providers, owners, operators, staff, volunteers, children, or visitors at daycare facilities, in the presence of children during hours of operation, or in vehicles while transporting children.

a. Any individual under the influence of alcohol or drugs is not permitted at or in the daycare facility.

b. Illegal drugs are prohibited by law and therefore are not allowed on the premises of any licensed daycare facility at any time.

17. Smoke-Free Environment. Children must be afforded a smoke-free

environment during all daycare hours, whether indoors or outdoors. While children are in care, the operator and all staff must ensure that no smoking or other tobacco use occurs within the facility, in outdoor areas, or in vehicles when children are present.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

02. Heat-Producing Equipment. A furnace, fireplace, wood-burning stove, water heater, and other flame or heat-producing equipment must be installed and maintained as recommended by the manufacturer and protected on all surfaces by screens or other means.

03. Portable Heating Devices. Must be limited and approved for use and location by the Fire Inspector prior to use within a facility and will not be used during sleeping hours.

04. Storage of Weapons, Firearms, and Ammunition. Firearms or other weapons stored at a daycare facility must be kept in a locked cabinet, gun safe, or other container that is inaccessible to children, while children are in attendance. Keys to these containers must also be inaccessible to children

- a. Ammunition must be stored in a locked container separate from firearms.
- b. Matches, lighters, and any other means of starting fires must be kept away from and out of the reach of children.
- c. Other weapons that could cause harm must be stored out of reach of children.

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

01. Appliances and Electrical Cords. All appliances, lamp cords, exposed light sockets, and electrical outlets will be protected to prevent electrocution.

02. Balconies and Stairways. Balconies and stairways accessible to children will have substantial railings as required by IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

03. Stairway Protection. Where an operator cares for children less than three (3) years old, stairways will be protected to prevent child access to stairs.

04. Hazardous Area Restrictions. Based on the age and functioning level of children in care and the type of hazard and the area surrounding the hazard, the area will be restricted to prevent easy access to the hazard.

05. Fueled Equipment. Fueled equipment including motorcycles, mopeds, lawncare equipment, and portable cooking equipment. This equipment will not be stored or repaired in areas where children are present.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other

health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

- a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.
- b. Outdoor equipment, such as climbing apparatus, slides, and swings will be anchored firmly and placed in a safe location and according to the manufacturer's instructions.
- c. Outdoor play areas will be designed so that all parts are always visible and easily supervised by staff.
- d. Toys, play equipment, and any other equipment used by the children will be of substantial construction and free from rough edges and sharp corners. Unguarded ladders on slides will be kept in good repair and well-maintained.
- e. Toys and objects with a diameter of less than one (1) inch (two point five (2.5) centimeters), objects with removable parts that have a diameter of less than one (1) inch (two point five (2.5) centimeters), plastic bags, styrofoam objects, and balloons will not be accessible to children ages three (3) and under, or children who are known to place such objects in their mouths.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

16. Alcohol and Illegal Drugs. Must not be used by providers, owners, operators, staff, volunteers, children, or visitors at daycare facilities, in the presence of children during hours of operation, or in vehicles while transporting children.

- a. Any individual under the influence of alcohol or drugs is not permitted at or in the daycare facility.
- b. Illegal drugs are prohibited by law and therefore are not allowed on the premises of any licensed daycare facility at any time.

17. Smoke-Free Environment. Children must be afforded a smoke-free environment during all daycare hours, whether indoors or outdoors. While children are in care, the operator and all staff must ensure that no smoking or other tobacco use occurs within the facility, in outdoor areas, or in vehicles when children are present.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

02. Heat-Producing Equipment. A furnace, fireplace, wood-burning stove, water heater, and other flame or heat-producing equipment must be installed and maintained as recommended by the manufacturer and protected on all surfaces by screens or other means.

03. Portable Heating Devices. Must be limited and approved for use and location by the Fire Inspector prior to use within a facility and will not be used during sleeping hours.

04. Storage of Weapons, Firearms, and Ammunition. Firearms or other weapons stored at a daycare facility must be kept in a locked cabinet, gun safe, or other container that is inaccessible to children, while children are in attendance. Keys to these containers must also be inaccessible to children

- a. Ammunition must be stored in a locked container separate from firearms.
- b. Matches, lighters, and any other means of starting fires must be kept away from and out of the reach of children.

- c. Other weapons that could cause harm must be stored out of reach of children.

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

01. Appliances and Electrical Cords. All appliances, lamp cords, exposed light sockets, and electrical outlets will be protected to prevent electrocution.

02. Balconies and Stairways. Balconies and stairways accessible to children will have substantial railings as required by IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

03. Stairway Protection. Where an operator cares for children less than three (3) years old, stairways will be protected to prevent child access to stairs.

04. Hazardous Area Restrictions. Based on the age and functioning level of children in care and the type of hazard and the area surrounding the hazard, the area will be restricted to prevent easy access to the hazard.

05. Fueled Equipment. Fueled equipment including motorcycles, mopeds, lawncare equipment, and portable cooking equipment. This equipment will not be stored or repaired in areas where children are present.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-

appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

b. Outdoor equipment, such as climbing apparatus, slides, and swings will be anchored firmly and placed in a safe location and according to the manufacturer's instructions.

c. Outdoor play areas will be designed so that all parts are always visible and easily supervised by staff.

d. Toys, play equipment, and any other equipment used by the children will be of substantial construction and free from rough edges and sharp corners. Unguarded ladders on slides will be kept in good repair and well-maintained.

e. Toys and objects with a diameter of less than one (1) inch (two point five (2.5) centimeters), objects with removable parts that have a diameter of less than one (1) inch (two point five (2.5) centimeters), plastic bags, styrofoam objects, and balloons will not be accessible to children ages three (3) and under, or children who are known to place such objects in their mouths.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

16. Alcohol and Illegal Drugs. Must not be used by providers, owners, operators, staff, volunteers, children, or visitors at daycare facilities, in the presence of children during hours of operation, or in vehicles while transporting children.

a. Any individual under the influence of alcohol or drugs is not permitted at or in the daycare facility.

b. Illegal drugs are prohibited by law and therefore are not allowed on the premises of any licensed daycare facility at any time.

17. Smoke-Free Environment. Children must be afforded a smoke-free environment during all daycare hours, whether indoors or outdoors. While children are in care, the operator and all staff must ensure that no smoking

or other tobacco use occurs within the facility, in outdoor areas, or in vehicles when children are present.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

02. Heat-Producing Equipment. A furnace, fireplace, wood-burning stove, water heater, and other flame or heat-producing equipment must be installed and maintained as recommended by the manufacturer and protected on all surfaces by screens or other means.

03. Portable Heating Devices. Must be limited and approved for use and location by the Fire Inspector prior to use within a facility and will not be used during sleeping hours.

04. Storage of Weapons, Firearms, and Ammunition. Firearms or other weapons stored at a daycare facility must be kept in a locked cabinet, gun safe, or other container that is inaccessible to children, while children are in attendance. Keys to these containers must also be inaccessible to children

- a. Ammunition must be stored in a locked container separate from firearms.
- b. Matches, lighters, and any other means of starting fires must be kept away from and out of the reach of children.
- c. Other weapons that could cause harm must be stored out of reach of children.

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

01. Appliances and Electrical Cords. All appliances, lamp cords, exposed light sockets, and electrical outlets will be protected to prevent electrocution.

02. Balconies and Stairways. Balconies and stairways accessible to children will have substantial railings as required by IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

03. Stairway Protection. Where an operator cares for children less than three (3) years old, stairways will be protected to prevent child access to stairs.

04. Hazardous Area Restrictions. Based on the age and functioning level of children in care and the type of hazard and the area surrounding the hazard, the area will be restricted to prevent easy access to the hazard.

05. Fueled Equipment. Fueled equipment including motorcycles, mopeds, lawncare equipment, and portable cooking equipment. This equipment will not be stored or repaired in areas where children are present.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained

free from hazards such as wells, machinery, and animal waste.

- a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.
- b. Outdoor equipment, such as climbing apparatus, slides, and swings will be anchored firmly and placed in a safe location and according to the manufacturer's instructions.
- c. Outdoor play areas will be designed so that all parts are always visible and easily supervised by staff.
- d. Toys, play equipment, and any other equipment used by the children will be of substantial construction and free from rough edges and sharp corners. Unguarded ladders on slides will be kept in good repair and well-maintained.
- e. Toys and objects with a diameter of less than one (1) inch (two point five (2.5) centimeters), objects with removable parts that have a diameter of less than one (1) inch (two point five (2.5) centimeters), plastic bags, styrofoam objects, and balloons will not be accessible to children ages three (3) and under, or children who are known to place such objects in their mouths.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

16. Alcohol and Illegal Drugs. Must not be used by providers, owners, operators, staff, volunteers, children, or visitors at daycare facilities, in the presence of children during hours of operation, or in vehicles while transporting children.

a. Any individual under the influence of alcohol or drugs is not permitted at or in the daycare facility.

b. Illegal drugs are prohibited by law and therefore are not allowed on the premises of any licensed daycare facility at any time.

17. Smoke-Free Environment. Children must be afforded a smoke-free environment during all daycare hours, whether indoors or outdoors. While children are in care, the operator and all staff must ensure that no smoking or other tobacco use occurs within the facility, in outdoor areas, or in vehicles when children are present.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare

facility licensed by the Department must comply with the following:

02. Heat-Producing Equipment. A furnace, fireplace, wood-burning stove, water heater, and other flame or heat-producing equipment must be installed and maintained as recommended by the manufacturer and protected on all surfaces by screens or other means.

03. Portable Heating Devices. Must be limited and approved for use and location by the Fire Inspector prior to use within a facility and will not be used during sleeping hours.

04. Storage of Weapons, Firearms, and Ammunition. Firearms or other weapons stored at a daycare facility must be kept in a locked cabinet, gun safe, or other container that is inaccessible to children, while children are in attendance. Keys to these containers must also be inaccessible to children

- a. Ammunition must be stored in a locked container separate from firearms.
- b. Matches, lighters, and any other means of starting fires must be kept away from and out of the reach of children.

- c. Other weapons that could cause harm must be stored out of reach of children.

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

01. Appliances and Electrical Cords. All appliances, lamp cords, exposed light sockets, and electrical outlets will be protected to prevent electrocution.

02. Balconies and Stairways. Balconies and stairways accessible to children will have substantial railings as required by IDAPA 24.39.30, [Rules of Building Safety \(Building Code Rules\)](#).

03. Stairway Protection. Where an operator cares for children less than three (3) years old, stairways will be protected to prevent child access to stairs.

04. Hazardous Area Restrictions. Based on the age and functioning level of children in care and the type of hazard and the area surrounding the hazard, the area will be restricted to prevent easy access to the hazard.

05. Fueled Equipment. Fueled equipment including motorcycles, mopeds, lawncare equipment, and portable cooking equipment. This equipment will not be stored or repaired in areas where children are present.

07. Indoor Play Areas and Toys. The indoor play areas will be clean, have age-appropriate toys, and be free from accumulation of dirt, rubbish, or other health hazards.

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

- a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high

without any holes or spaces greater than four (4) inches in diameter.

b. Outdoor equipment, such as climbing apparatus, slides, and swings will be anchored firmly and placed in a safe location and according to the manufacturer's instructions.

c. Outdoor play areas will be designed so that all parts are always visible and easily supervised by staff.

d. Toys, play equipment, and any other equipment used by the children will be of substantial construction and free from rough edges and sharp corners. Unguarded ladders on slides will be kept in good repair and well-maintained.

e. Toys and objects with a diameter of less than one (1) inch (two point five (2.5) centimeters), objects with removable parts that have a diameter of less than one (1) inch (two point five (2.5) centimeters), plastic bags, styrofoam objects, and balloons will not be accessible to children ages three (3) and under, or children who are known to place such objects in their mouths.

- b. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from bodies of water for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.400 400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

06. Water Hazards. Above and below ground pools, hot tubs, ponds, and other bodies of water that are on the daycare facility premises must provide the following safeguards:

a. The area surrounding the body of water must be fenced and locked in a manner that prevents access by children and meets the following

i. The fence will be at least four (4) feet high with no vertical opening more than four (4) inches wide and designed so that a young child cannot climb or squeeze under or through the fence. The fence will surround all sides of the pool and have a self-closing gate that has a self-latching mechanism in proper working order that is out of the reach of young children.

ii. If the house forms one (1) side of the barrier for the pool, all doors that provide unrestricted access to the pool will have alarms that produce an audible sound when the door is opened.

b. Furniture or other large objects will not be left near the fence in a manner that would enable a child to climb on the furniture or other large object and gain access to the pool. If the area surrounding a pool, hot tub, pond, or other body of water is not fenced and locked, there will be a secured protective covering that prevents access by a child.

- c. Wading pools and buckets will be empty when not in use.
- d. Children will be under direct supervision of adult staff who are certified in pediatric CPR and pediatric first aid while using a bathtub, pool, hot tub, pond, or other body of water.
- e. A minimum of a four (4) foot high fence that prevents access from the daycare facility premises if the daycare premises are adjacent to a body of water.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard:
16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.400 400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

06. Water Hazards. Above and below ground pools, hot tubs, ponds, and other bodies of water that are on the daycare facility premises must provide the following safeguards:

- a. The area surrounding the body of water must be fenced and locked in a manner that prevents access by children and meets the following
 - i. The fence will be at least four (4) feet high with no vertical opening more than four (4) inches wide and designed so that a young child cannot climb or squeeze under or through the fence. The fence will surround all sides of the pool and have a self-closing gate that has a self-latching mechanism in proper working order that is out of the reach of young children.
 - ii. If the house forms one (1) side of the barrier for the pool, all doors that provide unrestricted access to the pool will have alarms that produce an audible sound when the door is opened.
- b. Furniture or other large objects will not be left near the fence in a manner that would enable a child to climb on the furniture or other large object and gain access to the pool. If the area surrounding a pool, hot tub, pond, or other body of water is not fenced and locked, there will be a secured protective covering that prevents access by a child.
- c. Wading pools and buckets will be empty when not in use.
- d. Children will be under direct supervision of adult staff who are certified in pediatric CPR and pediatric first aid while using a bathtub, pool, hot tub, pond, or other body of water.
- e. A minimum of a four (4) foot high fence that prevents access from the daycare facility premises if the daycare premises are adjacent to a body of water.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

[x] Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**
- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.**

IDAPA 16.06.03.400 400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

06. Water Hazards. Above and below ground pools, hot tubs, ponds, and other bodies of water that are on the daycare facility premises must provide the following safeguards:

- a. The area surrounding the body of water must be fenced and locked in a manner that prevents access by children and meets the following
 - i. The fence will be at least four (4) feet high with no vertical opening more than four (4) inches wide and designed so that a young child cannot climb or squeeze under or through the fence. The fence will surround all sides of the pool and have a self-closing gate that has a self-latching mechanism in proper working order that is out of the reach of young children.
 - ii. If the house forms one (1) side of the barrier for the pool, all doors that provide unrestricted access to the pool will have alarms that produce an audible sound when the door is opened.
- b. Furniture or other large objects will not be left near the fence in a manner that would enable a child to climb on the furniture or other large object and gain access to the pool. If the area surrounding a pool, hot tub, pond, or other body of water is not fenced and locked, there will be a secured protective covering that prevents access by a child.
- c. Wading pools and buckets will be empty when not in use.
- d. Children will be under direct supervision of adult staff who are certified in pediatric CPR and pediatric first aid while using a bathtub, pool, hot tub, pond, or other body of water.
- e. A minimum of a four (4) foot high fence that prevents access from the daycare facility premises if the daycare premises are adjacent to a body of water.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with:

applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.400 400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

06. Water Hazards. Above and below ground pools, hot tubs, ponds, and other bodies of water that are on the daycare facility premises must provide the following safeguards:

- a. The area surrounding the body of water must be fenced and locked in a manner that prevents access by children and meets the following
 - i. The fence will be at least four (4) feet high with no vertical opening more than four (4) inches wide and designed so that a young child cannot climb or squeeze under or through the fence. The fence will surround all sides of the pool and have a self-closing gate that has a self-latching mechanism in proper working order that is out of the reach of young children.
 - ii. If the house forms one (1) side of the barrier for the pool, all doors that provide unrestricted access to the pool will have alarms that produce an audible sound when the door is opened.
- b. Furniture or other large objects will not be left near the fence in a manner that would enable a child to climb on the furniture or other large object and gain access to the pool. If the area surrounding a pool, hot tub, pond, or other body of water is not fenced and locked, there will be a secured protective covering that prevents access by a child.
- c. Wading pools and buckets will be empty when not in use.
- d. Children will be under direct supervision of adult staff who are certified in pediatric CPR and pediatric first aid while using a bathtub, pool, hot tub, pond, or other body of water.
- e. A minimum of a four (4) foot high fence that prevents access from the daycare facility premises if the daycare premises are adjacent to a body of water.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.400 400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

06. Water Hazards. Above and below ground pools, hot tubs, ponds, and other bodies of water that are on the daycare facility premises must provide the following safeguards:

- a. The area surrounding the body of water must be fenced and locked in a manner that prevents access by children and meets the following
 - i. The fence will be at least four (4) feet high with no vertical opening more than four (4) inches wide and designed so that a young child cannot climb or squeeze under or through the fence. The fence will surround all sides of the pool and have a self-closing gate that has a self-latching mechanism in proper working order that is out of the reach of young children.
 - ii. If the house forms one (1) side of the barrier for the pool, all doors that provide unrestricted access to the pool will have alarms that produce an audible sound when the door is opened.
- b. Furniture or other large objects will not be left near the fence in a manner that would enable a child to climb on the furniture or other large object and gain access to the pool. If the area surrounding a pool, hot tub, pond, or other body of water is not fenced and locked, there will be a secured protective covering that prevents access by a child.
- c. Wading pools and buckets will be empty when not in use.
- d. Children will be under direct supervision of adult staff who are certified in pediatric CPR and pediatric first aid while using a bathtub, pool, hot tub, pond, or other body of water.
- e. A minimum of a four (4) foot high fence that prevents access from the daycare facility premises if the daycare premises are adjacent to a body of water.

- c. Provide the standards, appropriate to the provider setting and age of children, that address the identification of and protection from vehicular traffic hazards for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: 16.06.12.800.

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:

16.06.12.800.

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid

driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

5.3.6 Prevention of shaken baby syndrome, abusive head trauma, and maltreatment health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the prevention of shaken baby syndrome and abusive head trauma and indicate the age of children it applies to for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.**
15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:
06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.**
15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All

providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience

maltreatment under 45 CFR 98.41.

- b. Provide the standards, appropriate to the provider setting and age of children, that address the prevention of child maltreatment and indicate the age of children it applies to for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
- 15. Behavior Management and Discipline.** Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
- 15. Behavior Management and Discipline.** Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA**

16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

IDAPA

16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA**

16.06.12.802.02. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

15. Behavior Management and Discipline. Methods of behavior management and discipline for children must be positive, consistent, and based on each child's needs, stage of development, and behavior. Discipline is to promote self-control, self-esteem, and independence. Providers must certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

IDAPA 16.06.03.121. 121. APPLICATION FOR DAYCARE LICENSE OR RENEWAL. Any individual applying for licensure as a daycare facility must be at least eighteen (18) years old, apply, and provide information required by the Department under this rule, to include:

06. Statement to Comply. As part of the application, the applicant must thoroughly read and review these rules and agree that they are prepared to comply with all provisions. Providers must also certify that they will not harm, shake, or abuse children, and that children in their care will not experience maltreatment under 45 CFR 98.41.

5.3.7 Emergency preparedness and response planning standard

Identify by checking below that the emergency preparedness and response planning due to natural disasters and human-caused events standard includes procedures in the following areas:

- i. ☒ Evacuation
- ii. ☒ Relocation
- iii. ☒ Shelter-in-place
- iv. ☒ Lock down
- v. Staff emergency preparedness
 - ☒ Training
 - ☒ Practice drills
- vi. Volunteer emergency preparedness
 - ☒ Training
 - ☒ Practice drills
- vii. ☒ Communication with families
- viii. ☒ Reunification with families
- ix. ☒ Continuity of operations
- x. Accommodation of
 - ☒ Infants
 - ☒ Toddlers
 - ☒ Children with disabilities
 - ☒ Children with chronic medical conditions
- xi. If any of the above are not checked, describe:

5.3.8 Handling and storage of hazardous materials and the appropriate disposal of biocontaminants health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the handling and storage of hazardous materials for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers**

must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS.** All providers

must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS.** All providers

must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying

with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

- b. Provide the standards, appropriate to the provider setting and age of children, that address the disposal of bio contaminants for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **IDAPA 16.06.12.802.**

HEALTH AND SAFETY REQUIREMENTS. All providers

must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.

Biocontaminants must be disposed of appropriately.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **IDAPA 16.06.12.802.**

HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children.

Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:
06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.
Biocontaminants must be disposed of appropriately.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children.
Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:
06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children.
Biocontaminants must be disposed of appropriately.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying

with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children. Biocontaminants must be disposed of appropriately.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **IDAPA 16.06.12.802. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
04. Hazardous Substances. Medicines, cleaning supplies, and other hazardous substances must be handled safely and stored out of the reach of children. Biocontaminants must be disposed of appropriately.

IDAPA 16.06.03.381. MISCELLANEOUS SAFETY REQUIREMENTS. Each daycare facility licensed by the Department must comply with the following:

06. Hazardous Materials. Cleaning materials, flammable liquids, detergents, aerosol cans, pesticides, and other poisonous and toxic materials must be kept in their original containers and in a place inaccessible to children. They must be used in such a way that will not contaminate play surfaces, food, food preparation areas, or constitute a hazard to the children. Biocontaminants must be disposed of appropriately.

5.3.9 Precautions in transporting children health and safety standard

Provide the standards, appropriate to the provider setting and age of children, that address precautions in transporting children for the following CCDF-eligible providers:

- i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must

comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard: **CCDF-eligible license-exempt in-home care providers are trained on the following standards at the location where care is provided:**

16.06.12.800.

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

IDAPA 16.06.03.380 HEALTH STANDARDS. Each daycare facility licensed by the Department, must comply with the following. Health inspections must be conducted annually by a qualified inspector designated by the Department and will be unannounced.

18. Transportation. Provider that transports a child(ren) will possess a valid driver's license, be insured under Idaho Law, and abide by all traffic laws including the requirement that all children are in proper safety restraints while being transported under Section 49-672, Idaho Code, and Section 49-673, Idaho Code. Vehicles used to transport children will be properly maintained and in good working condition.

IDAPA 16.06.03.400. BUILDINGS, GROUNDS, FURNISHINGS, AND EQUIPMENT. Each daycare facility licensed by the Department must comply with the following:

08. Outdoor Play Areas and Toys. Any outdoor play area must be maintained free from hazards such as wells, machinery, and animal waste.

a. If any part of the play area is adjacent to a busy roadway, drainage or irrigation ditch, stream, large holes, or other hazardous areas, the play area will be enclosed with a fence in good repair that is at least four (4) feet high without any holes or spaces greater than four (4) inches in diameter.

5.3.10 Pediatric first aid and pediatric cardiopulmonary resuscitation (CPR) health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address pediatric first aid for all staff for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these

certifications will not count in child-staff ratios.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard:
16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

[x] Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except

for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard:
16.06.12.800. CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho

Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

b. Provide the standards, appropriate to the provider setting and age of children, that address pediatric cardiopulmonary resuscitation for all staff for the following CCDF-eligible providers:

i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho

Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

- ii. All CCDF-eligible licensed family child care homes. Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

[x] Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **16.06.12.800.**

CHILD CARE PROVIDER LICENSING. All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state

requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard:
16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.800. CHILD CARE PROVIDER LICENSING.** All providers of child care who receive a Department subsidy must be licensed or must comply with: applicable State Daycare licensing requirements in Title 39, Chapter 11, Idaho Code; these rules; local licensing ordinances; or tribal ordinances. If both state requirements and ordinances apply to a provider, the provider must comply with the stricter requirement. A provider operating outside Idaho must comply with the licensing laws of their state or locality.

16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with

these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

08. CPR/First Aid. All providers must have current certification in pediatric rescue breathing (CPR) and pediatric first aid treatment from a certified instructor.

IDAPA 16.06.03.300.03. CHILD-STAFF RATIO. Under Section 39-1109, Idaho Code, the Department determines the maximum allowable child-staff ratio based on a point system.

b. All providers, owners, and staff who provide direct care to children must have current certification in pediatric rescue breathing (CPR) and pediatric first aid from a certified instructor. Providers who do not have these certifications will not count in child-staff ratios.

5.3.11 Identification and reporting of child abuse and neglect health and safety standard

- a. Provide the standards, appropriate to the provider setting and age of children, that address the identification of child abuse and neglect for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.801. HEALTH AND SAFETY TRAINING. All child care providers must complete a series of health and safety trainings during an orientation period of not more than ninety (90) days, in addition to ongoing annual training that address each of the following topics:**
06. Child Abuse Prevention. Prevention of shaken baby syndrome, abusive head trauma, child maltreatment, and recognition and reporting of child abuse and neglect.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **16.06.12.801. HEALTH AND SAFETY TRAINING. All child care providers must complete a series of health and safety trainings during an orientation period of not more than ninety (90) days, in addition to ongoing annual training that address each of the following topics:**
06. Child Abuse Prevention. Prevention of shaken baby syndrome, abusive head trauma, child maltreatment, and recognition and reporting of child abuse and neglect.
 - iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**
[x] Not applicable.
 - iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**
 - v. All CCDF-eligible license-exempt family child care homes. Provide the standard: **16.06.12.801. HEALTH AND SAFETY TRAINING. All child care providers must complete a series of health and safety trainings during an orientation period of not more than ninety (90) days, in addition to ongoing annual training that address each of the following topics:**

06. Child Abuse Prevention. Prevention of shaken baby syndrome, abusive head trauma, child maltreatment, and recognition and reporting of child abuse and neglect.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard:
16.06.12.801. HEALTH AND SAFETY TRAINING. All child care providers must complete a series of health and safety trainings during an orientation period of not more than ninety (90) days, in addition to ongoing annual training that address each of the following topics:
06. Child Abuse Prevention. Prevention of shaken baby syndrome, abusive head trauma, child maltreatment, and recognition and reporting of child abuse and neglect.
- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.801. HEALTH AND SAFETY TRAINING. All child care providers must complete a series of health and safety trainings during an orientation period of not more than ninety (90) days, in addition to ongoing annual training that address each of the following topics:**
06. Child Abuse Prevention. Prevention of shaken baby syndrome, abusive head trauma, child maltreatment, and recognition and reporting of child abuse and neglect.
- b. Provide your standards, appropriate to the provider setting and age of children, that address the reporting of child abuse and neglect for the following CCDF-eligible providers:
 - i. All CCDF-eligible licensed center care. Provide the standard: **16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.**
10. Child Abuse. Providers must report suspected child abuse to the appropriate authority.

16.06.03.421. MANDATORY REPORTING OF ABUSE, ABANDONMENT, OR NEGLECT. Under Section 16-1605, Idaho Code, daycare owners, operators, staff, and any other person who has reason to believe that a child has been abused, abandoned, or neglected, or is being subjected to conditions or circumstances which would reasonably result in abuse, abandonment, or neglect, must report or cause to be reported within twenty-four (24) hours, such conditions or circumstances to the Department or the proper law enforcement agency.
 - ii. All CCDF-eligible licensed family child care homes. Provide the standard: **16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except**

for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

10. Child Abuse. Providers must report suspected child abuse to the appropriate authority.

16.06.03.421. MANDATORY REPORTING OF ABUSE, ABANDONMENT, OR NEGLECT. Under Section 16-1605, Idaho Code, daycare owners, operators, staff, and any other person who has reason to believe that a child has been abused, abandoned, or neglected, or is being subjected to conditions or circumstances which would reasonably result in abuse, abandonment, or neglect, must report or cause to be reported within twenty-four (24) hours, such conditions or circumstances to the Department or the proper law enforcement agency.

- iii. All CCDF-eligible licensed in-home care. Provide the standard: **Idaho does not have licensed in-home care.**

☒ Not applicable.

- iv. All CCDF-eligible license-exempt center care. Provide the standard: **Idaho does not have license-exempt center care.**

- v. All CCDF-eligible license-exempt family child care homes. Provide the standard:
16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
10. Child Abuse. Providers must report suspected child abuse to the appropriate authority.

16.06.03.421. MANDATORY REPORTING OF ABUSE, ABANDONMENT, OR NEGLECT. Under Section 16-1605, Idaho Code, daycare owners, operators, staff, and any other person who has reason to believe that a child has been abused, abandoned, or neglected, or is being subjected to conditions or circumstances which would reasonably result in abuse, abandonment, or neglect, must report or cause to be reported within twenty-four (24) hours, such conditions or circumstances to the Department or the proper law enforcement agency.

- vi. All CCDF-eligible license-exempt in-home care. Provide the standard:
16.06.12.802. HEALTH AND SAFETY REQUIREMENTS. All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying

with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.

10. Child Abuse. Providers must report suspected child abuse to the appropriate authority.

16.06.03.421. MANDATORY REPORTING OF ABUSE, ABANDONMENT, OR NEGLECT. Under Section 16-1605, Idaho Code, daycare owners, operators, staff, and any other person who has reason to believe that a child has been abused, abandoned, or neglected, or is being subjected to conditions or circumstances which would reasonably result in abuse, abandonment, or neglect, must report or cause to be reported within twenty-four (24) hours, such conditions or circumstances to the Department or the proper law enforcement agency.

- vii. All CCDF-eligible out-of-school programs (afterschool programs, summer camps, day camps, etc.). Provide the standard: **16.06.12.802. HEALTH AND SAFETY REQUIREMENTS.** All providers must comply with the health and safety requirements under this rule. All providers must agree to an annual, unannounced health and safety inspection, except for in-home child care under Section 401 of these rules. Compliance with these standards does not exempt a provider from complying with stricter health and safety standards under state law, tribal law, local ordinance, or other applicable law.
10. Child Abuse. Providers must report suspected child abuse to the appropriate authority.

16.06.03.421. MANDATORY REPORTING OF ABUSE, ABANDONMENT, OR NEGLECT. Under Section 16-1605, Idaho Code, daycare owners, operators, staff, and any other person who has reason to believe that a child has been abused, abandoned, or neglected, or is being subjected to conditions or circumstances which would reasonably result in abuse, abandonment, or neglect, must report or cause to be reported within twenty-four (24) hours, such conditions or circumstances to the Department or the proper law enforcement agency.

- c. Confirm if child care providers must comply with the [Lead Agency's](#) procedures for reporting child abuse and neglect as required by the Child Abuse Prevention and Treatment Act (42 U.S.C. 5106a(b)(2)(B)(i):

☒ Yes, confirmed.

☐ No. If no, describe:

5.3.12 Additional optional standards

In addition to the required health and safety standards, does the Lead Agency require providers to comply with the following optional standards?

☐ Yes.

☒ No. If no, skip to Section 5.4

If yes, describe the standard(s).

- i. Nutrition. Describe:
- ii. Access to physical activity. Describe:
- iii. Caring for children with special needs. Describe:
- iv. Any other areas determined necessary to promote child development or to protect children's health and safety. Describe:

5.4 Pre-Service or Orientation Training on Health and Safety Standards

Lead Agencies must have requirements for all caregivers, teachers, and directors at CCDF providers to complete pre-service or orientation training (within 3 months of starting) on all CCDF health and safety standards and child development. The training must be appropriate to the setting and the age of children served. This training must address the required health and safety standards and the content area of child development. Lead Agencies have flexibility in determining the minimum number of training hours to require, and are encouraged to consult with Caring for our Children Basics for best practices.

Exemptions for relative providers' training requirements are addressed in question 5.8.1.

5.4.1 Health and safety pre-service/orientation training requirements

Lead Agencies must certify staff have pre-service or orientation training on each standard that is appropriate to different settings and age groups. Lead Agencies may require pre-service or orientation to be completed before staff can care for children unsupervised. In the table below, check the boxes for which you have training requirements.

	Is this standard addressed in the pre-service or orientation training?	Is the pre-service or orientation training on this standard appropriate to different settings and age groups?	Does the Lead Agency require staff to complete the training before caring for children unsupervised?
a. Prevention and control of infectious diseases (including immunizations)	☒	☒	☐
b. SIDS prevention and use of safe sleep practices	☒	☒	☐
c. Administration of medication	☒	☒	☐
d. Prevention and response to food and	☒	☒	☐

allergic reactions			
e. Building and physical premises safety, including identification of and protection from hazards, bodies of water, and vehicular traffic	☒	☒	☐
f. Prevention of shaken baby syndrome, abusive head trauma and child maltreatment	☒	☒	☐
g. Emergency preparedness and response planning and procedures	☒	☒	☐
h. Handling and storage of hazardous materials and disposal of biocontaminants	☒	☒	☐
i. Appropriate Precautions in transporting children, if applicable	☒	☒	☐
j. Pediatric first aid and pediatric CPR (age-appropriate)	☒	☒	☒
k. Child abuse and neglect recognition and reporting	☒	☒	☐
l. Child development including major domains of cognitive, social, emotional, physical development and approaches to learning.	☒	☒	☐

m. If the Lead Agency does not certify implementation of all the health and safety pre-service/orientation training requirements for staff in programs serving children receiving CCDF assistance, please describe: **NA**

n. Are there any provider categories to whom the above pre-service or orientation training requirements do not apply?

☒ No

☐ Yes. If yes, describe:

5.5 Monitoring and Enforcement of Licensing and Health and Safety Requirements

5.5.1 Inspections for licensed CCDF providers

Licensing inspectors must perform at least one annual, unannounced inspection of each licensed CCDF provider for compliance with all child care licensing standards, including an inspection for compliance with health and safety and fire standards. Lead Agencies must conduct at least one pre-licensure inspection for compliance with health, safety, and fire standards of each child care provider and facility in the State/Territory.

a. Licensed CCDF center-based providers

- i. Does your pre-licensure inspection for licensed center-based providers assess compliance with health standards, safety standards, and fire standards?
☒ Yes.
☐ No. If no, describe:
- ii. Identify the frequency of annual unannounced inspections for licensed center-based providers addressing compliance with health, safety, and fire standards:
☒ Annually.
☐ More than once a year. If more than once a year, describe:
☐ Other. If other, describe:
- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed center-based providers?
☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.
☒ No. If no, describe: **All providers receive the same annual unannounced inspection.**
- iv. Identify which department or agency is responsible for completing the inspections for licensed center-based providers. **Central District Health Department (CDH)**

b. Licensed CCDF family child care providers

- i. Does your pre-licensure inspection for licensed family child care homes assess compliance with health standards, safety standards, and fire standards?
☒ Yes.
☐ No. If no, describe:
- ii. Identify the frequency of annual unannounced inspections for licensed family child care homes addressing compliance with health, safety, and fire standards:
☒ Annually.
☐ More than once a year. If more than once a year, describe:
☐ Other. If other, describe:
- iii. Does the Lead Agency implement a differential monitoring approach when

monitoring licensed family child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No. If no, describe: **All providers receive the same annual unannounced inspection.**

- iv. Identify which department or agency is responsible for completing the inspections for licensed family child care providers. **Central District Health Department (CDH)**

c. Licensed in-home CCDF child care providers

- i. Does your Lead Agency license CCDF in-home child care (care in the child's own home) providers?

☒ No.

☐ Yes. If yes, does your pre-licensure inspection for licensed in-home providers assess compliance with health, safety, and fire standards?

☐ Yes.

☐ No. If no, describe:

- ii. Identify the frequency of annual unannounced inspections for licensed in-home child care providers for compliance with health, safety, and fire standards completed:

☒ Annually.

☐ More than once a year. If more than once a year, describe:

☐ Other. If other, describe:

- iii. Does the Lead Agency implement a differential monitoring approach when monitoring licensed in-home child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iv. Identify which department or agency is responsible for completing the inspections for licensed in-home providers. **Central District Health Department (CDH)**

5.5.2 Inspections for license-exempt providers

Licensing inspectors must perform at least one annual monitoring visit of each license-exempt CCDF provider for compliance with health, safety, and fire standards. Inspections for relative providers will be addressed in subsection 5.8.

Describe the policies and practices for the annual monitoring of:

a. License-exempt CCDF center-based child care providers

- i. Identify the frequency of inspections for compliance with health, safety, and fire standards for license-exempt center-based providers:

☐ Annually.

☐ More than once a year. If more than once a year, describe:

☒ Other. If other, describe: **Idaho does not have license-except center-based providers.**

- ii. Does the Lead Agency implement a differential monitoring approach when monitoring license-exempt center-based providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iii. Identify which department or agency is responsible for completing the inspections for license-exempt center-based CCDF providers. **Idaho does not have license-except center-based providers.**

b. License-exempt CCDF family child care providers

- i. Identify the frequency of the inspections of license-exempt family child care providers to determine compliance with health, safety, and fire standards:

☒ Annually.

☐ More than once a year. If more than once a year, describe:

☐ Other. If other, describe:

- ii. Does the Lead Agency implement a differential monitoring approach when monitoring license-exempt family child care providers?

☐ Yes. If yes, describe how the differential monitoring approach is representative of the full complement of health and safety requirements.

☒ No.

- iii. Identify which department or agency is responsible for completing the inspections for license-exempt family child care providers. **Central District Health Department (CDH)**

5.5.3 Inspections for CCDF license-exempt in-home child care providers

Lead Agencies may develop alternate monitoring requirements for care provided in the child's home that are appropriate to the setting. This flexibility cannot be used to bypass the monitoring requirement altogether.

- a. Describe the requirements for the annual monitoring of CCDF license-exempt in-home child care (care in the child's own home) providers, including if monitoring is announced or unannounced, occurs more frequently than once per year, and if differential monitoring procedures are used. **Each in-home care provider must annually complete health and safety training/monitoring provided by the local Health District, conducted by an Environmental Health Specialist. The training follows the same standards as monitoring CCDF providers. The training is announced as it is conducted at the location where child care is provided and reviews 31 health, safety, and fire standards detailed in the District Health Department's Idaho Child Care Management Manual. The manual includes policy citations for each standard.**

Currently, Idaho conducts health and safety trainings, rather than health and safety inspections of in-home child care providers. This was an identified noncompliance in the April 4, 2023 Preliminary Notice of Possible Non-Compliance. IDHW needs additional time to develop policies and processes to address inspection of in-home child care providers while limiting programmatic burdens to children authorized to receive child care in their own home and providers who care for them. IDHW will request technical assistance regarding the full implementation of this requirement.

- b. List the entity(ies) in your State/Territory responsible for conducting inspections of license-exempt CCDF in-home child care (care in the child's own home) providers: **Central District Health Department (CDH)**

5.5.4 Posting monitoring and inspection reports

Lead Agencies must post monitoring and inspection reports on their consumer education website for each licensed and CCDF child care provider, except in cases where the provider is related to all the children in their care. These reports must include the results of required annual monitoring visits and visits due to major substantiated complaints about a provider's failure to comply with health and safety requirements and child care policies. A full report covers everything in the monitoring visit, including areas of compliance and non-compliance. If the Lead Agency does not produce any reports that include areas of compliance, the website must include information about all areas covered by a monitoring visit.

The reports must be in plain language or provide a plain language summary Lead Agency and be timely to ensure that the results of the reports are available and easily understood by parents when they are deciding on a child care provider. Lead Agencies must post at least 3 years of monitoring and inspection reports.

- a. Does the Lead Agency post:
 - i. ☒ Pre-licensing inspection reports for licensed programs.
 - ii. ☐ Full monitoring and inspection reports that include areas of compliance and non-compliance for all non-relative providers eligible to provide CCDF services.
 - iii. ☐ Monitoring and inspection reports that include areas of non-compliance only, with information about all areas covered by a monitoring visit posted separately on the website (e.g., a blank checklist used by monitors) for all non-relative providers eligible to provide CCDF services. If checked, provide a direct URL/website link to the website where a blank checklist is posted:
 - iv. ☐ Other. Describe:
- b. Check if the monitoring and inspection reports and any related plain language summaries include:
 - i. ☒ Date of inspection.
 - ii. ☒ Health and safety violations, including those violations that resulted in fatalities or serious injuries occurring at the provider. Describe how these health

and safety violations are prominently displayed: **Idaho Child Care Check details all inspection criteria, the pass or fail status of each criteria, the date of correction for that criteria, as well as any corrective actions taken by the provider.**

- iii. ☒ Corrective action plans taken by the Lead Agency and/or child care provider.
Describe: **These published inspections explain the actions the facility and/or provider took to correct their violations.**
- iv. ☒ A minimum of 3 years of results, where available.
- v. If any of the components above are not selected, please explain:
- c. Lead Agencies must post monitoring and inspection reports and/or any related summaries in a timely manner.
 - i. Provide the direct URL/website link to where the reports are posted:
<https://www.idahochildcarecheck.org/>
 - ii. Identify the Lead Agency's established timeline for posting monitoring reports and describe how it is timely: **After an inspection, all monitoring documentation is reviewed by the licensing or child care specialist to ensure completeness and accuracy. The report is then posted to the on-line system within 60 days after the inspection visit.**
- d. Does the Lead Agency certify that the monitoring and inspection reports or the summaries are in plain language that is understandable to parents and other consumers?
☒ Yes.
☐ No. If no, describe:
- e. Does the Lead Agency certify that there is a process for correcting inaccuracies in the monitoring and inspection reports?
☒ Yes.
☐ No. If no, describe:
- f. Does the Lead Agency maintain monitoring and inspection reports on the consumer education website?
☒ Yes.
☐ No. If no, describe:

5.5.5 Qualifications and training of licensing inspectors

Lead Agencies must ensure that individuals who are hired as licensing inspectors (or qualified monitors designated by the Lead Agency) are qualified to inspect child care providers and facilities and have received health and safety training appropriate to the provider setting and age of the children served.

Describe how the Lead Agency ensures that licensing inspectors (or qualified monitors designated by the Lead Agency) are qualified and have received training on health and safety requirements that are appropriate to the age of the children in care and the type of provider setting. (C#: WC089900.I.E., C#: WC089900.VI.A)

As required in our contract with Central District Health, child care health and safety inspectors must be, at minimum, the equivalent of an Environmental Health Specialist, and shall meet professional licensure, certification, or be eligible for licensure, in order to conduct specified child care health and safety inspections. Additionally required in the contract, the health and safety inspector's professional license, professional certification, or professional training status for licensure or certification must be in a health-related field that is valid in the State of Idaho. Inspectors must receive training in related health and safety requirements appropriate to provider settings and age of children served.

The Health District Operations Manual clarifies that inspections must also be appropriate to the age of the children in care and the type of setting where care is being provided. Inspectors must receive training in related health and safety requirements appropriate to provider settings and age of children served. Environmental Health Specialists who conduct inspections of child care facilities in Idaho have completed a course in child development, highlighting the age of the children in care and the type of setting where care is being provided. Compliance with this requirement is maintained in each inspector's personnel file.

All Child Care Health and Safety Inspectors receive and review the Idaho Child Care Management Manual. The manual outlines the minimum statewide procedures for conducting inspections of child care facilities receiving public assistance from the Idaho Department of Health and Welfare's Idaho Child Care Program (ICCP) and facilities that have a basic day care license. Use of the manual helps to ensure consistent interpretation and application of the regulations and rules that govern child care in the state. Also, it helps to prevent gaps in services provided. The manual details all relevant statutes, rules, policies, and processes for the completion of Child Care Health and Safety inspections in Idaho, and includes customer service standards that incorporate timely responses, polite service, and time to educate providers on health and safety standards. Additionally, the manual details all aspects of the State's licensure requirements and all aspects of the State's requirements for provider participation with ICCP. Each inspector is trained using the manual, has access to the most current version of the manual, which is maintained internally by the Health Districts in collaboration with the Idaho Child Care Program and State Daycare Licensing.

5.5.6 Ratio of licensing inspectors

Lead Agencies must ensure the ratio of licensing inspectors to child care providers and facilities in the State/Territory are maintained at a level sufficient to enable the Lead Agency to conduct effective inspections of child care providers and facilities on a timely basis in accordance with federal, State, and local laws.

Provide the ratio of licensing inspectors to child care providers (i.e., number of inspectors per number of child care providers) and facilities in the State/Territory and include how the ratio is sufficient to conduct effective inspections on a timely basis. **The Idaho Department of Health and Welfare (IDHW) has a statewide contract with Central District Health Department, which subcontracts to each subsequent health district to appropriately administer and monitor health and safety inspections throughout Idaho for all child care providers who are licensed and/or participate with the Idaho Child Care Program (ICCP). Per the contractual requirements, "The Contractor shall maintain a sufficient inspector to provider ratio to ensure timeliness of all inspections per contract requirements." The number of needed inspectors varies by region as the inspectors may share duties or conduct other types of inspections. The contract explicitly identifies timelines associated to inspections, complaint referral site visits, and follow up for which**

the Health District must comply. To date, standards for timeliness have been met and follow contract requirements outlined by IDHW.

5.6 Ongoing Health and Safety Training

Lead Agencies must have ongoing training requirements for all caregivers, teachers, and directors of eligible CCDF providers for health and safety standards but have discretion on frequency and training content (e.g., pediatric CPR refresher every year and recertification every 2 years). Lead Agencies have discretion on which health and safety standards are subject to ongoing training. Lead Agencies may exempt relative providers from these requirements.

5.6.1 Required ongoing training of health and safety standards

Describe any required ongoing training of health and safety standards for caregivers, teachers, and directors of the following CCDF eligible provider types.

- a. Licensed child care centers: **After completion of the initial 12 hours of trainings in orientation period, 12-hours of annual trainings are required for all providers and staff at facilities participating with the Idaho Child Care Program. The annual training requirements include: A two-hour training targeting a specific health and safety topic, commonly referred to as a "refresher course" mandatory for all providers, and 10 elective hours of approved training topics selected individually. As the initial training series is updated every three years, providers may complete the updated trainings and receive credit for those training hours. The annual "refresher course," mandatory for all providers and staff at facilities participating with the Idaho Child Care Program, must be fully aligned to at least one of the CCDF health and safety training topics, ensuring that all providers receive training on at least one federally required topic annually after completing their initial trainings.**
- b. License-exempt child care centers: **Idaho does not have license-exempt child care centers.**
- c. Licensed family child care homes: **After completion of the initial 12 hours of trainings in orientation period, 12-hours of annual trainings are required for all providers and staff at facilities participating with the Idaho Child Care Program. The annual training requirements include: A two-hour training targeting a specific health and safety topic, commonly referred to as a "refresher course" mandatory for all providers, and 10 elective hours of approved training topics selected individually. As the initial training series is updated every three years, providers may complete the updated trainings and receive credit for those training hours. The annual "refresher course," mandatory for all providers and staff at facilities participating with the Idaho Child Care Program, must be fully aligned to at least one of the CCDF health and safety training topics, ensuring that all providers receive training on at least one federally required topic annually after completing their initial trainings.**
- d. License-exempt family child care homes: **After completion of the initial 12 hours of trainings in orientation period, 12-hours of annual trainings are required for all providers and staff at facilities participating with the Idaho Child Care Program. The annual training requirements include: A two-hour training targeting a specific health and safety topic, commonly referred to as a "refresher course" mandatory for all providers, and 10 elective hours of approved training topics selected individually. As the initial training series is updated every three years, providers may complete the updated trainings and receive**

credit for those training hours. The annual “refresher course,” mandatory for all providers and staff at facilities participating with the Idaho Child Care Program, must be fully aligned to at least one of the CCDF health and safety training topics, ensuring that all providers receive training on at least one federally required topic annually after completing their initial trainings.

- e. Regulated or registered in-home child care: After completion of the initial 12 hours of trainings in orientation period, 12-hours of annual trainings are required for all providers and staff at facilities participating with the Idaho Child Care Program. The annual training requirements include: A two-hour training targeting a specific health and safety topic, commonly referred to as a “refresher course” mandatory for all providers, and 10 elective hours of approved training topics selected individually. As the initial training series is updated every three years, providers may complete the updated trainings and receive credit for those training hours. The annual “refresher course,” mandatory for all providers and staff at facilities participating with the Idaho Child Care Program, must be fully aligned to at least one of the CCDF health and safety training topics, ensuring that all providers receive training on at least one federally required topic annually after completing their initial trainings.
- f. Non-regulated or registered in-home child care: **Idaho does not license in-home child care. Idaho does not regulate in-home care providers who do not receive CCDF.**

5.7 Comprehensive Background Checks

Lead Agencies must conduct comprehensive background checks for all child care staff members (including prospective staff members) of all child care providers that are (1) licensed, regulated, or registered under State/Territory law, regardless of whether they receive CCDF funds; or (2) all other child care providers eligible to deliver CCDF services (e.g., license-exempt CCDF eligible child care providers). Family child care home providers must also submit background check requests for all household members age 18 or older.

A comprehensive background check must include: three in-state checks, two national checks, and three interstate checks if the individual resided in another State or Territory in the preceding 5 years. The background check components must be completed at least once every five years.

All child care staff members must receive a qualifying result from either the FBI criminal background check or an in-state fingerprint criminal history check before working (under supervision) with or near children. Lead Agencies must apply a CCDF-specific list of disqualifying crimes for child care providers serving families participating in CCDF.

These background check requirements do not apply to individuals who are related to all children for whom child care services are provided. Exemptions for relative providers will be addressed in subsection 5.8.

5.7.1 In-state criminal history check with fingerprints

- a. Does the Lead Agency conduct in-state criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

[] Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct in-state criminal background checks with fingerprints. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho
Chubbuck, Idaho
Coeur d'Alene, Idaho
Idaho Falls, Idaho
Moscow, Idaho
Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C "Criminal History and Background Checks"

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 "License Application; Criminal History Check"

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 "Background and Criminal History Checks"

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct in-state criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers) other than relative providers?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct in-state criminal background checks with fingerprints. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for

licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho
Chubbuck, Idaho
Coeur d'Alene, Idaho
Idaho Falls, Idaho
Moscow, Idaho
Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C "Criminal History and Background Checks"
https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217
-Chubbuck- 6.04.150 "License Application; Criminal History Check"
https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477
-Coeur d'Alene- 5.68.050:
https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060
-Idaho Falls- 6-3-3 and 6-3-2
<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>
-Moscow- Title 9, Chapter 10-7
<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>
-Pocatello- 5.28.060 "Background and Criminal History Checks"
https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct the in-state criminal background check with fingerprints for all individuals age 18 or older who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an in-state criminal background check with fingerprints. **In Idaho, individuals age 18 or older who reside in a family child care home ALL receive an in-state criminal background check with fingerprints. Cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho
Chubbuck, Idaho

Coeur d’Alene, Idaho
Idaho Falls, Idaho
Moscow, Idaho
Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C ☐Criminal History and Background Checks☐

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 ☐License Application; Criminal History Check☐

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d’Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

5.7.2 National Federal Bureau of Investigation (FBI) criminal history check with fingerprints

- a. Does the Lead Agency conduct FBI criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct FBI criminal background checks with fingerprints. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be “at least as stringent” as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho
Chubbuck, Idaho
Coeur d’Alene, Idaho
Idaho Falls, Idaho
Moscow, Idaho
Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C ☐ Criminal History and Background Checks

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 ☐ License Application; Criminal History Check

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐ Background and Criminal History Checks

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct FBI criminal history background checks with fingerprints for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct FBI criminal background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be ☐at least as stringent☐ as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho

Chubbuck, Idaho

Coeur d'Alene, Idaho

Idaho Falls, Idaho

Moscow, Idaho

Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C ☐ Criminal History and Background Checks

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 ☐ License Application; Criminal History Check

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐ Background and Criminal History Checks

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct the FBI criminal background check with fingerprints for all individuals age 18 or older who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an FBI criminal background check with fingerprints. **All individuals age 18 or older who reside in a family child care home receive an FBI criminal background check with fingerprints. In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be ☐ at least as stringent ☐ as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho

Chubbuck, Idaho

Coeur d'Alene, Idaho

Idaho Falls, Idaho

Moscow, Idaho

Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C ☐ Criminal History and Background Checks

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 ☐ License Application; Criminal History Check

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

[https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-](https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060)

2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐ Background and Criminal History Checks

5.7.3 National Crime Information Center (NCIC) National Sex Offender Registry (NSOR) name-based check

The majority of NCIC NSOR records are fingerprint records and are automatically included in the FBI fingerprint criminal background check. But a small percentage of NCIC NSOR records are only name-based records and must be accessed through the required name-based search of the NCIC NSOR.

- a. Does the Lead Agency conduct NCIC NSOR name-based background checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?
- ☐ Yes.
- ☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct NCIC NSOR name-based background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho
Chubbuck, Idaho
Coeur d'Alene, Idaho
Idaho Falls, Idaho
Moscow, Idaho
Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C "Criminal History and Background Checks"

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 "License Application; Criminal History Check"

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 **Background and Criminal History Checks**
https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct NCIC NSOR name-based background checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct NCIC NSOR name-based background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho

Chubbuck, Idaho

Coeur d'Alene, Idaho

Idaho Falls, Idaho

Moscow, Idaho

Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C **Criminal History and Background Checks**

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 **License Application; Criminal History Check**

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 **Background and Criminal History Checks**

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct the NCIC NSOR name-based background check for all individuals age 18 or older who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive a NCIC NSOR name-based background check. **All individuals age 18 or older who reside in a family child care home receive a NCIC NSOR name-based background check. In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

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-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 "Background and Criminal History Checks"

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

5.7.4 In-state sex offender registry (SOR) check

- a. Does the Lead Agency conduct in-state SOR checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct in-state SOR background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

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Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

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https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 "License Application; Criminal History Check"

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 "Background and Criminal History Checks"

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct in-state SOR background checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct in-state SOR background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho
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Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C ☐Criminal History and Background Checks☐

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 ☐License Application; Criminal History Check☐

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d’Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct the in-state SOR background check for all individuals age 18 or older who reside in a family child care home?

[] Yes.

[x] No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive an in-state SOR background check. **All individuals age 18 or older who reside in a family child care home receive an in-state SOR background check. In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be “at least as stringent” as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

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-Boise- 3-6-11-C ☐Criminal History and Background Checks☐

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217
 -Chubbuck- 6.04.150 ☐ License Application; Criminal History Check☐
https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477
 -Coeur d’Alene- 5.68.050:
https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060
 -Idaho Falls- 6-3-3 and 6-3-2
<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>
 -Moscow- Title 9, Chapter 10-7
<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>
 -Pocatello- 5.28.060 ☐ Background and Criminal History Checks☐
https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

5.7.5 In-state child abuse and neglect (CAN) registry check

- a. Does the Lead Agency conduct CAN registry checks for all child care staff members (including prospective staff members) of licensed, regulated, or registered child care providers, regardless of CCDF participation?
☒ Yes.
☐ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct CAN registry checks.
- b. Does the Lead Agency conduct CAN registry checks for all child care staff members (including prospective staff members) of all other child care providers eligible for CCDF participation (i.e., license-exempt providers)?
☒ Yes.
☐ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct CAN registry checks.
- c. Does the Lead Agency conduct the CAN registry check for all individuals age 18 or older who reside in a family child care home?
☒ Yes.
☐ No. If no, describe individuals age 18 or older who reside in a family child care home who do not receive a CAN registry check.

5.7.6 Interstate criminal history check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate criminal history background checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate criminal history background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

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Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C "Criminal History and Background Checks"

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 "License Application; Criminal History Check"

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 "Background and Criminal History Checks"

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct interstate criminal history background checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate criminal history background checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the

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Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

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-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct interstate criminal history background checks for all individuals age 18 or older who reside in a family child care home and resided in other state(s) in the past 5 years.

[] Yes.

[x] No. If no, describe why individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive an interstate criminal history background check. **All individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home receive an interstate criminal history background check. In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

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-Chubbuck- 6.04.150 ☐License Application; Criminal History Check☐

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

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-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

5.7.7 Interstate Sex Offender Registry (SOR) check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate SOR checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate SOR checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

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-Chubbuck- 6.04.150 ☐License Application; Criminal History Check☐

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

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-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct interstate SOR checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate SOR checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

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 -Coeur d’Alene- 5.68.050:
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 -Idaho Falls- 6-3-3 and 6-3-2
<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>
 -Moscow- Title 9, Chapter 10-7
<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>
 -Pocatello- 5.28.060 ☐Background and Criminal History Checks☐
https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct the interstate SOR checks for all individuals age 18 or older who resided in other state(s) in the past 5 years who reside in a family child care home?

[] Yes.

[x] No. If no, describe individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive an interstate SOR check. **All individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home receive an interstate SOR check. In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be “at least as stringent” as the background checks required by state licensing.

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 -Coeur d’Alene- 5.68.050:
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 -Idaho Falls- 6-3-3 and 6-3-2
<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

5.7.8 Interstate child abuse and neglect (CAN) registry check

These questions refer to requirements for a Lead Agency to conduct an interstate check for a child care staff member (including prospective child care staff members) who currently lives in their State or Territory but has lived in another State, Territory, or Tribal land within the previous 5 years.

- a. Does the Lead Agency conduct interstate CAN registry checks for any staff member (or prospective staff member) that resided in other state(s) in the past 5 years of licensed, regulated, or registered child care providers, regardless of CCDF participation?

☐ Yes.

☒ No. If no, describe any categories of licensed, regulated, or registered child care providers for whom you do not conduct interstate CAN registry checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

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-Coeur d'Alene- 5.68.050:

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-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day->

Care-Ordinance-PDF

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency conduct interstate CAN registry checks for any staff member (or prospective staff member) who resided in other state(s) in the past 5 years eligible for CCDF participation (i.e., license-exempt providers)?

☐ Yes.

☒ No. If no, describe any categories of child care providers eligible for CCDF participation for whom you do not conduct interstate CAN registry checks. **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

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https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 ☐Background and Criminal History Checks☐

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- c. Does the Lead Agency conduct the interstate CAN registry checks for all individuals age 18 or older who resided in other state(s) in the past 5 years who reside in a family child care home?

☐ Yes.

☒ No. If no, describe individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home that do not receive interstate CAN registry checks. **All individuals age 18 or older that resided in other state(s) in the past 5 years who reside in a family child care home receive interstate CAN registry checks. In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho

Chubbuck, Idaho

Coeur d'Alene, Idaho

Idaho Falls, Idaho

Moscow, Idaho

Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C "Criminal History and Background Checks"

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 "License Application; Criminal History Check"

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 "Background and Criminal History Checks"

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

5.7.9 Disqualifications for child care employment

The Lead Agency must prohibit employment of individuals with child care providers receiving CCDF subsidy payment if they meet any of the following disqualifying criteria:

- Refused to consent to a background check.
- Knowingly made materially false statements in connection with the background check.
- Are registered, or are required to be registered, on the State/Territory sex offender registry or repository or the National Sex Offender Registry.

- Have been convicted of a felony consisting of murder, child abuse or neglect, crimes against children (including child pornography), spousal abuse, crimes involving rape or sexual assault, kidnapping, arson, physical assault, or battery.
 - Have a violent misdemeanor committed as an adult against a child, including the following crimes: child abuse, child endangerment, sexual assault, or any misdemeanor involving child pornography.
 - Convicted of a felony consisting of a drug-related offense committed during the preceding 5 years.
- a. Does the Lead Agency disqualify the employment of child care staff members (including prospective staff members) by child care providers receiving CCDF subsidy payment for CCDF-identified disqualifying criteria?

☐ Yes.

☒ No. If no, describe the disqualifying criteria: **In Idaho, cities may conduct their own background checks, or accept an IDHW background check.**

Idaho Code 39-1108 explains that if a city or county has adopted an ordinance for licensing the requirements for background checks must be "at least as stringent" as the background checks required by state licensing.

The following cities maintain their own background check requirements that are at least as stringent as the state of Idaho:

Boise, Idaho

Chubbuck, Idaho

Coeur d'Alene, Idaho

Idaho Falls, Idaho

Moscow, Idaho

Pocatello, Idaho

Per their codes all cities are accessing IDHW background checks or conducting more stringent background checks. City codes for each city are linked here:

-Boise- 3-6-11-C "Criminal History and Background Checks"

https://codelibrary.amlegal.com/codes/boise_id/latest/boise/0-0-0-2217

-Chubbuck- 6.04.150 "License Application; Criminal History Check"

https://codelibrary.amlegal.com/codes/chubbuckid/latest/chubbuck_id/0-0-0-1477

-Coeur d'Alene- 5.68.050:

https://codelibrary.amlegal.com/codes/coeurdaleneid/latest/coeurdalene_id/0-0-0-2737#JD_5.68.060

-Idaho Falls- 6-3-3 and 6-3-2

<https://www.idahofallsidaho.gov/DocumentCenter/View/152/Chapter-3---Day-Care-PDF>

-Moscow- Title 9, Chapter 10-7

<https://www.ci.moscow.id.us/DocumentCenter/View/1341/Chapter-10---Moscow-Day-Care-Ordinance-PDF>

-Pocatello- 5.28.060 "Background and Criminal History Checks"

https://codelibrary.amlegal.com/codes/pocatelloid/latest/pocatello_id/0-0-0-687

- b. Does the Lead Agency use the same criteria for licensed, regulated, and registered child care providers regardless of CCDF participation?
- ☒ Yes.
- ☐ No. If no, describe any disqualifying criteria used for licensed, regulated, and registered child care providers:
- c. How does the Lead Agency use results from the in-state child abuse and neglect registry check?
- ☐ Does not use them to disqualify employment.
- ☒ Uses them to disqualify employment. If checked, describe: **Unconditional denials are issued for:**
- b. A relevant record on any Child Protection Registry for the classes of individuals under Section 126 of these rules;**
- c. A relevant record on the Idaho Child Protection Central Registry with a Level one (1) or Level two (2) designation for all other applicants covered by these rules (16.05.06.200)**
- d. How does the Lead Agency use results from the interstate child abuse and neglect registry check?
- ☐ Does not use them to disqualify employment.
- ☒ Uses them to disqualify employment. If checked, describe: **Unconditional denials are issued for:**
- b. A relevant record on any Child Protection Registry for the classes of individuals under Section 126 of these rules;**
- c. A relevant record on the Idaho Child Protection Central Registry with a Level one (1) or Level two (2) designation for all other applicants covered by these rules (16.05.06.200)**

5.7.10 Privacy

Lead Agencies must ensure the privacy of a prospective staff member by notifying child care providers of the individual's eligibility or ineligibility for child care employment based on the results of the comprehensive background check without revealing any documentation of criminal history or disqualifying crimes or other related information regarding the individual.

Does the Lead Agency certify they ensure the privacy of child care staff members (including prospective child care staff member) when providing the results of the comprehensive background check?

☒ Yes.

☐ No. If no, describe the current process of notification:

5.7.11 Appeals processes for background checks

Lead Agencies must provide for a process that allows child care provider staff members (and prospective staff members) to appeal the results of a background check to challenge the accuracy or completeness of the information contained in the individual's background check report.

Does the appeals process:

- i. Provide the affected individual with information related to each disqualifying

crime in a report, along with information/notice on the opportunity to appeal.

☒ Yes.

☐ No. Describe:

- ii. Provide the affected individual with clear instructions about how to complete the appeals process for each background check component if they wish to challenge the accuracy or completeness of the information contained in such individual's background report.

☒ Yes.

☐ No. Describe:

- iii. Ensure the Lead Agency attempts to verify the accuracy of the information challenged by the individual, including making an effort to locate any missing disposition information related to the disqualifying crime.

☒ Yes.

☐ No. Describe:

- iv. Get completed in a timely manner.

☒ Yes.

☐ No. Describe:

- v. Ensure the affected individual receives written notice of the decision. In the case of a negative determination, the decision must indicate (1) the Lead Agency's efforts to verify the accuracy of information challenged by the individual, (2) any additional appeals rights available to the individual, and (3) information on how the individual can correct the federal or State records at issue in the case.

☒ Yes.

☐ No. Describe:

- vi. Facilitate coordination between the Lead Agency and other agencies in charge of background check information and results (such as the Child Welfare office and the State Identification Bureau), to ensure the appeals process is conducted in accordance with the Act.

☒ Yes.

☐ No. Describe:

5.7.12 Provisional hiring of prospective staff members

Lead Agencies must at least complete and receive a qualifying result for either the FBI criminal background check or a fingerprint-based in-state criminal background check where the individual resides before prospective staff members may provide services or be in the vicinity of children.

Until all the background check components have been completed, the prospective staff member must be supervised at all times by someone who has already received a qualifying result on a background check within the past five years.

Check all background checks for which the Lead Agency requires a qualifying result before a prospective child care staff member begins work with children.

- a. FBI criminal background check.

☒ Yes.

☐ No. If no, describe:

- b. In-state criminal background check with fingerprints.

☒ Yes.

☐ No. If no, describe:

- c. In-state Sex Offender Registry.

☒ Yes.

☐ No. If no, describe:

- d. In-state child abuse and neglect registry.

☒ Yes.

☐ No. If no, describe:

- e. Name-based national Sex Offender Registry (NCIC NSOR).

☒ Yes.

☐ No. If no, describe:

- f. Interstate criminal background check, as applicable.

☒ Yes.

☐ No. If no, describe:

- g. Interstate Sex Offender Registry check, as applicable.

☒ Yes.

☐ No. If no, describe:

- h. Interstate child abuse and neglect registry check, as applicable.

☐ Yes.

☒ No. If no, describe: **If an applicant receives qualifying results from the FBI fingerprint based check as well as the in-state fingerprint criminal background check, and completion of interstate child abuse and neglect registry checks would cause a delay in the applicant's ability to work, the Background Check Unit may issue a Notice of Preliminary Clearance. A Notice of Preliminary Clearance explains to the employer that the applicant has successfully met the requirements to begin working under the supervision of a staff with a cleared Enhanced Background Check while the results of the interstate child abuse registry checks are compiled and reviewed.**

- i. Does the Lead Agency require provisional hires to be supervised by a staff member who received a qualifying result on the comprehensive background check while awaiting results from the provisional hire's full comprehensive background check?

☒ Yes.

☐ No. If no, describe:

5.7.13 Completing the criminal background check within a 45-day timeframe

The Lead Agency must carry out a request from a child care provider for a criminal background check as expeditiously as possible, and no more than 45 days after the date on which the provider submitted the request

- a. Does the Lead Agency ensure background checks are completed within 45 days (after the date on which the provider submits the request)?

☐ Yes.

☒ No. If no, describe the timeline for completion for categories of providers, including which background check components take more than 45 days. **Background checks conducted for applicants who have resided in states outside of Idaho in the previous five years have experienced processing delays while the results of child abuse and neglect registry checks are compiled and reviewed from other states. In those situations, a Notice of Preliminary Clearance is provided to eligible child care staff so that they may be working under supervision while the remaining results are received.**

- b. Does the Lead Agency ensure child care staff receive a comprehensive background check when they work in your State but reside in a different State?

☒ Yes.

☐ No. If no, describe the current policy:

5.7.14 Responses to interstate background check requests

Lead Agencies must respond as expeditiously as possible to requests for interstate background checks from other States/Territories/Tribes in order to meet the 45-day timeframe.

- a. Does your State participate in the National Crime Prevention and Privacy Compact or National Fingerprint File programs?

☒ Yes.

☐ No.

- b. Describe how the State/Territory responds to interstate criminal history, Sex Offender Registry, and Child Abuse and Neglect Registry background check requests from another state. **Idaho is an NFF participating state, per that Compact our state makes its III-indexed records available on an interstate and Federal-State basis for both criminal justice and noncriminal justice purposes.**

Interstate Screening Requests: Idaho State Police is responsible for conducting and responding to interstate screening requests. Idaho allows screening for employment purposes, and the Lead Agency provides information and instructions on the Background Check Unit website to link requestors to the Idaho State Police.

Sex Offender Registry: The Idaho Sex Offender Registry is publicly available at: https://apps.isp.idaho.gov/sor_id/ States and Territories can check the registry by visiting

the website and entering the applicant's last name. Any registered offender with the same last name will be listed along with first names and other details about the individual and the registered offenses.

Child Abuse and Neglect Registry: The Lead Agency (Idaho Department of Health and Welfare) is responsible for conducting and responding to individuals requesting to be screened for substantiated reports of abuse or neglect. The Lead Agency provides information and instructions on Child Care Provider Forms available on the Background Check Unit website to link requestors to the form required for screening. The results are returned to the individual or agency who requested the screening. Instructions for requesting an Idaho Child Abuse and Neglect Registry Check are available at the Background Check Unit website: <https://healthandwelfare.idaho.gov/bcu> Applicants must consent to the release of their information via an IDHW provided form, pay a \$20 fee for the cost of conducting the check, and results are provided as expeditiously as possible.

- c. Does your State/Territory have a law or policy that prevents a response to CCDF interstate background check requests from other States/Territories/Tribes?
- ☐ Yes. If yes, describe the current policy.
- ☒ No.

5.7.15 Consumer education website links to interstate background check processes

Lead Agencies must include on their consumer education website and the website of local Lead Agencies if the CCDF program is county-run, the policies and procedures related to comprehensive background checks. This includes the process by which a child care provider or other State or Territory may submit a background check request.

- a. Provide the direct URL/website link that contains instructions on how child care providers and other States and Territories should initiate background check requests for prospective and current child care staff members: <https://healthandwelfare.idaho.gov/bcu>
- Check to certify that the required elements are included on the Lead Agency's consumer and provider education website for each interstate background check component.
- b. Interstate criminal background check:
- i. ☒ Agency name
 - ii. ☒ Address
 - iii. ☒ Phone number
 - iv. ☒ Email
 - v. ☒ Website
 - vi. ☒ Instructions
 - vii. ☒ Forms
 - viii. ☒ Fees
 - ix. ☒ Is the State a National Fingerprint File (NFF) State?

- x. ☒ Is the State a National Crime Prevention and Privacy Compact State?
- xi. If not all boxes above are checked, describe: **NA**
- c. Interstate sex offender registry (SOR) check:
 - i. ☒ Agency name
 - ii. ☒ Address
 - iii. ☒ Phone number
 - iv. ☒ Email
 - v. ☒ Website
 - vi. ☒ Instructions
 - vii. ☒ Forms
 - viii. ☒ Fees
 - ix. If not all boxes above are checked, describe: **NA**
- d. Interstate child abuse and neglect (CAN) registry check:
 - i. ☒ Agency name
 - ii. ☒ Is the CAN check conducted through a county administered registry or centralized registry?
 - iii. ☒ Address
 - iv. ☒ Phone number
 - v. ☒ Email
 - vi. ☒ Website
 - vii. ☒ Instructions
 - viii. ☒ Forms
 - ix. ☒ Fees
 - x. If not all boxes above are checked, describe: **NA**

5.7.16 Background check fees

The Lead Agency must ensure that fees charged for completing the background checks do not exceed the actual cost of processing and administration.

Does the Lead Agency certify that background check fees do not exceed the actual cost of processing and administering the background checks?

☒ Yes.

☐ No. If no, describe what is currently in place and what elements still need to be implemented:

5.7.17 Renewal of the comprehensive background check

Does the Lead Agency conduct the background check at least every 5 years for all components?

☐ Yes.

☒ No. If no, what is the frequency for renewing each component? **The lead agency conducts background checks at least every 5 years for all components. Although not specifically addressed in this question, the lead agency notes that the agency received a Preliminary Notice of Non-Compliance dated December 14, 2021 related to background checks for child care staff members that have been separated from employment from a child care provider for more than 180 days, but who's background check is not yet expired. Idaho will need additional time to come into compliance with this requirement while not adding additional burden to child care providers staffing their facilities and has requested technical assistance on this topic.**

5.8 Exemptions for Relative Providers

Lead Agencies may exempt relatives (defined in CCDF regulations as grandparents, great-grandparents, siblings if living in a separate residence, aunts, and uncles) from certain health and safety requirements. This exception applies only if the individual cares only for relative children.

5.8.1 Exemptions for relative providers

Does the Lead Agency exempt any federally defined relative providers from licensing requirements, the CCDF health and safety standards, preservice/orientation training, ongoing training, inspections, or background checks?

☒ No.

☐ Yes. If yes, which type of relatives do you exempt, and from what requirements (licensing requirements, CCDF health and safety standards, preservice/orientation training, ongoing training, inspections, and/or background checks) do you exempt them?

6 Support for a Skilled, Qualified, and Compensated Child Care Workforce

A skilled child care workforce with adequate wages and benefits underpins a stable high-quality child care system that is accessible and reliable for working parents and that meets their needs and promotes equal access. Positive interactions between children and caregivers provide the cornerstone of quality child care experiences. Responsive caregiving and rich interactions support healthy socio-emotional, cognitive, and physical development in children. Strategies that successfully support the child care workforce address key challenges, including low wages, poor benefits, and difficult job conditions. Lead Agencies can help mitigate some of these challenges through various CCDF policies, including through ongoing professional development and supports for all provider types and embedded in the payment policies and practices covered in Section 4. Lead Agencies must have a framework for training, professional development, and post-secondary education. They must also incorporate health and safety training into their professional development. Lead Agencies should also implement policies that focus on improving wages and access to benefits for the child care workforce. When implemented as a cohesive approach, the initiatives support the recruitment and retention of a qualified and effective child care workforce, and improve opportunities for caregivers, teachers, and directors to advance on their progression of training, professional development, and postsecondary education.

This section addresses Lead Agency efforts to support the child care workforce, the components and implementation of the professional development framework, and early learning and developmental guidelines.

6.1 Supporting the Child Care Workforce

Lead Agencies have broad flexibility to implement policies and practices to support the child care workforce.

6.1.1 Strategies to improve recruitment, retention, compensation, and well-being

- a. Identify any Lead Agency activities related to strengthening workforce recruitment and retention of child care providers. Check all that apply:
 - i. ☐ Providing program-level grants to support investments in staff compensation.
 - ii. ☐ Providing bonuses or stipends paid directly to staff, like sign-on or retention bonuses.
 - iii. ☐ Connecting family child care providers and center-based child care staff to health insurance or supporting premiums in the Marketplace.
 - iv. ☐ Subsidizing family child care provider and center-based child care staff retirement benefits.
 - v. ☐ Providing paid sick, personal, and parental leave for family child care providers and center-based child care staff.
 - vi. ☐ Providing student loan debt relief or loan repayment for family child care providers and center-based child care staff.
 - vii. ☒ Providing scholarships or tuition support for center-based child care staff and family child care providers.
 - viii. ☒ Other. Describe: **Early Childhood Care and Education career pathways for high school CTE (Career Technical Education) students have been updated and students participating in these pathways in their high school programs have the opportunity for freshman year college scholarships (when pursuing degree or certificate in ECE) which can be renewed indefinitely upon employment in a center or family child care setting and continuation in an early childhood degree pathway.**
- b. Describe any Lead Agency ongoing efforts and future plans to assess and improve the compensation of the child care workforce in the State or Territory, including increasing wages, bonuses, and stipends. **Collaborations established with external organizations presently offer living expense stipends to child care providers participating in the IdahoSTARS Academic Scholarship program, ensuring they can sustain at least part-time employment while pursuing their education on a part-time basis. IdahoSTARS intends to pursue the continuation of this initiative should the external funding cease.**

Career pathways acknowledge the accomplishments of child care providers in their professional development efforts and grants a financial bonus based on their level of attainment in professional development once they complete 20 hours of ongoing training

annually. A restructure of the current pathway slated for launch in 2025 will ensure the financial bonuses tied to PD are more equitable and accessible to all child care providers participating in professional development activities.

Available business practice training and targeted technical assistance opportunities from IdahoSTARS provide child care owners with the tools and knowledge necessary to manage finances, staff, and planning more effectively, potentially leading to higher wages and compensation for their staff.

- c. Describe any Lead Agency ongoing efforts and future plans to expand access to benefits, including health insurance, paid sick, personal, and parental leave, and retirement benefits. **IdahoSTARS provides business cohorts and other professional development and technical assistance to programs in an effort to increase knowledge and resources on all aspects of successful child care business practices, including benefits. IdahoSTARS staff in the northern region of the state (Region 1) are partnering with the local Early Learning Collaborative to explore implementation of shared services in that region which may include access to benefits for child care providers.**
- d. Describe any Lead Agency ongoing efforts and future plans to support the mental health and well-being of the child care workforce. **IdahoSTARS ongoing efforts and future plans to support the mental health and well-being of the child care workforce are inclusive of a reflective consultation program, mental health awareness training, and professional development opportunities.**

IdahoSTARS has 5 team members that have been training to become eligible to offer reflective consultation to others. Our goal is to begin working with providers in the Spring of 2025.

IdahoSTARS offers professional development on mental health awareness such as Caring for the Caregiver which provides knowledge and awareness addressing caregiver mental health and prevention of child abuse.

- e. Describe any other strategies the Lead Agency is developing and/or implementing to support providers' recruitment and retention of the child care workforce. **IdahoSTARS partners and collaborates with high school CTE programs to expose students to early childhood education, fostering student interest in pursuing careers working with young children and broadening recruitment efforts to attract a diverse pool of aspiring educators. Additionally, IdahoSTARS provides professional development opportunities for FCS (Family and Consumer Sciences) CTE teachers to effectively prepare students for careers in ECE. Program supports including technical assistance and resources for curriculum articulation to the CDA (Child Development Associate credential) facilitate students' credentialing pathways and establish clear career trajectories in the field after graduation. Engagement with Family, Career and Community Leaders of America (FCCLA) student organization allows IdahoSTARS to introduce high school students to early childhood education careers, offering educational resources and opportunities to cultivate their interest and encourage recruitment into the industry. College scholarships available from IdahoSTARS to graduating high school seniors who are pursuing a degree or certificate at the post-secondary level lead to retention in the field.**

6.1.2 Strategies to support provider business practices

- a. Describe other strategies that the Lead Agency is developing and/or implementing to strengthen child care providers' business management and administrative practices. **Business practice training and targeted technical assistance opportunities are available across the state. These opportunities include topics such as budgeting, record keeping, marketing and staffing. IdahoSTARS regional office staff coach providers on general business practice topics.**
- b. Check the topics addressed in the Lead Agency's strategies for strengthening child care providers' administrative business practices. Check all that apply:
 - i. ☐ Fiscal management.
 - ii. ☒ Budgeting.
 - iii. ☒ Recordkeeping.
 - iv. ☐ Hiring, developing, and retaining qualified staff.
 - v. ☐ Risk management.
 - vi. ☐ Community relationships.
 - vii. ☐ Marketing and public relations.
 - viii. ☒ Parent-provider communications.
 - ix. ☒ Use of technology in business administration.
 - x. ☒ Compliance with employment and labor laws.
 - xi. ☐ Other. Describe any other efforts to strengthen providers' administrative business:

6.1.3 Strategies to support provider participation

Lead Agencies must facilitate participation of child care providers and staff with limited English proficiency and disabilities in the child care subsidy system. Describe how the Lead Agency will facilitate this participation, including engagement with providers to identify barriers and specific strategies used to support their participation:

- a. Providers and staff with limited English proficiency: **The IdahoSTARS Customer Support Team is bilingual and offers support and services directly in our three largest languages: English, Spanish & Swahili. We also support child care providers with distinct languages practices in navigating our systems through document translation, the ability to request interpreters for in person appointments/visits and phone interpretation.**
- b. Providers and staff who have disabilities: **IdahoSTARS customer support team works with providers and staff who have disabilities to meet their individual needs. IdahoSTARS actively supports individuals with disabilities in personal and professional inclusion. During training registration, a provider can request accommodations for training. All documents, live training slides presentations, and in-house developed online trainings are reviewed for best practices in accessibility. The IdahoSTARS website strives to meet web content accessibility guidelines to allow access to all users.**

6.2 Professional Development Framework

A Lead Agency must have a professional development framework for training, professional development, and post-secondary education for caregivers, teachers, and directors in child care programs that serve children of all ages. The framework must include these components:

(1) professional standards and competencies, (2) career pathways, (3) advisory structures, (4) articulation, (5) workforce information, and (6) financing. CCDF provides Lead Agencies flexibility on the strategies, breadth, and depth of the framework. The professional development framework must be developed in consultation with the State Advisory Council on Early Childhood Education and Care or a similar coordinating body.

6.2.1 Updates and consultation

- a. Did the Lead Agency make any updates to the professional development framework since the FFY 2022-2024 CCDF Plan was submitted?

[x] Yes. If yes, describe the elements of the framework that were updated and describe if and how the State Advisory Council on Early Childhood Education and Care (if applicable) or similar coordinating body was consulted: **IdahoSTARS partners and collaborates with high school Career and Technical Education (CTE) programs to expose students to early childhood education, fostering student interest in pursuing careers working with young children and broadening recruitment efforts to attract a diverse pool of aspiring educators. Additionally, IdahoSTARS provides professional development opportunities for FCS (Family and Consumer Sciences) CTE teachers to effectively prepare students for careers in Early Childhood Education (ECE). Program supports including technical assistance and resources for curriculum articulation to the CDA (Child Development Associate credential) facilitate students' credentialing pathways and establish clear career trajectories in the field after graduation. Engagement with Family, Career and Community Leaders of America (FCCLA) student organization allows IdahoSTARS to introduce high school students to early childhood education careers, offering educational resources and opportunities to cultivate their interest and encourage recruitment into the industry. College scholarships available from IdahoSTARS to graduating high school seniors who are pursuing a degree or certificate at the post-secondary level lead to retention in the field.**

The ICCP Panel, serving as the advisory committee for IdahoSTARS, was involved in the decision making process to add the **“Future Educator’s Scholarship”** targeted to graduating Idaho High School Seniors who have committed to pursuing a degree or certificate in Early Childhood Education, Child Development, Family and Consumer Sciences or a closely related field at an accredited Idaho institution of higher education. The advisory committee was consulted in the decision making process to add this new scholarship as this is only one of two scholarships which does not require current employment in child care for the duration of the 1-year freshman scholarship. They were crucial in the conversation to allow this employment exemption, which is a deviation to IdahoSTARS typical incentive distribution requirements in order to address workforce recruitment and retention issues. Those Future Educator Scholarship recipients who become employed in child care after their freshman year have the ability to transition to a renewable scholarship which does require continued employment in child care to maintain scholarship eligibility.

☐ No.

- b. Did the Lead Agency consult with other key groups in the development of their professional development framework?

☒ Yes. If yes, identify the other key groups: **Child care providers were the primary driver of revising the pathway as providers expressed frustration with stagnation on the pathway especially due to the pause at Level 4 due to the associate degree requirement. Idaho's Early Childhood Higher Education Consortium, which includes faculty from all Idaho institutions of higher education with early childhood coursework and the Idaho Child Care Program (ICCP) Advisory Panel, which serves as the state's CCDF advisory council was consulted in the discussion and decision making process.**

☐ No.

6.2.2 Description of the professional development framework

- a. Describe how the Lead Agency's framework for training and professional development addresses the following required elements:

- i. Professional standards and competencies. For example, Lead Agencies can include information about which roles in early childhood education are included (such as teachers, directors, infant and toddler specialists, mental health consultants, coaches, licensors, QIS assessors, family service workers, home visitors). **The IdahoSTARS Quality Domains and Subcategories, which align with national standards, are used to categorize all professional development opportunities, training, targeted technical assistance and coaching. The Idaho Early Learning eGuidelines are the most current, research-based standards in Idaho. The eGuidelines are utilized in all IdahoSTARS approved trainings.**
- ii. Career pathways. For example, Lead Agencies can include information about professional development registries, career ladders, and levels. **The IdahoSTARS Early Childhood Care and Education Career Pathway contains six levels beginning with Level 1 of the Professional Development System (PDS) which reflects entry and completion of a high school diploma or equivalency. Level 2 requires a CDA (Child Development Associate) credential, Montessori credential, or 12 ECE-related college credits. Level 3 requires a technical certificate in ECE. Level 4 requires an associate degree in ECE or 30 ECE-related college credits. Level 5 requires a bachelor's degree in ECE or a bachelor's degree in any field with 36 ECE-related college credits; Level 6 requires a master's degree or higher in ECE. International transcripts and diplomas are translated and reviewed individually to determine placement on the pathway.**
- iii. Advisory structure. For example, Lead Agencies can include information about how the professional development advisory structure interacts with the State Advisory Council on Early Childhood Education and Care. **The Idaho Child Care Program (ICCP) Advisory Panel provides guidance and recommendations to both ICCP and the IdahoSTARS program. IdahoSTARS leadership works with Idaho's child care administrator and team to determine agenda items and plan for the Advisory Panel meetings, scheduled**

quarterly and held either in person and/or via a web-based platform to allow participation by all members of the panel. The Advisory Panel provides guidance and recommendations on the annual ICCP required training, updates and revisions to Steps to Quality, the initial and ongoing development of the RISE Application (IdahoSTARS' web-based database), revisions to the career pathway, incentives and opportunities that support the professional development for child care providers, and ongoing efforts to embed equity in our processes and practices.

- iv. **Articulation.** For example, Lead Agencies can include information about articulation agreements, and collaborative agreements that support progress in degree acquisition. **The partnership and collaboration between IdahoSTARS and 2 and 4-year higher education institutions are beneficial for child care providers in Idaho. IdahoSTARS participates in the Early Childhood Education Consortium, which convenes representatives from higher education institutions and state agencies annually. These meetings provide opportunities for program updates, sharing of resources, and alignment of training and educational offerings with state frameworks and priorities. Collaborations from participation in the Higher Education Consortium have resulted in new degree programs, articulation agreements between 2 and 4-year institutions, clearer degree pathways between institutions and alignment of curriculum not only to professional standards, but to the needs of the workforce.**
- v. **Workforce information.** For example, Lead Agencies can include information about workforce demographics, educator well-being, retention/turnover surveys, actual wage scales, and/or access to benefits. **IdahoSTARS collects information from the early care and education workforce via our RISE Application. Beyond the management of program licensing and subsidy eligibility, the RISE Application collects demographic data on Idaho's child care workforce. Data is collected on the workforce in areas of both employment and demographics. Data collected on the workforce's employment include wage, geographic location, child care setting and ages served, position title, and dates of employment. Personal demographic data points include highest level of education, race/ethnicity, language(s) spoken, sex, and age. Data collected from the system informs the management and equity of our programs. Where appropriate, this information is aggregated, disseminated, and utilized to inform policy makers, business owners, directors, educators, parents, child care providers, service providers, partners, among others across Idaho. Data from the system informs IdahoSTARS annual "State of the Child Care Workforce Report".**
- vi. **Financing.** For example, Lead Agencies can include information about strategies including scholarships, apprenticeships, wage enhancements, etc. **The IdahoSTARS professional development system (PDS) offers several financial supports and incentives for eligible providers. The Early Childhood Care and Education Career Pathway offers an annual financial incentive "anniversary award" for the completion of 20 or more hours of IdahoSTARS approved training or one Early Childhood Education (ECE) college credit or**

related credit within their anniversary cycle. The amount of the anniversary award is tied to the provider's level on the career pathway and increases as the provider moves up levels on the pathway. Financial support of \$200 in the form of training scholarships is available to every provider enrolled in the PDS and who work 15 hours or more in a center or home based child care facility. Training scholarships are used for approved professional development trainings, conferences, and other educational opportunities with an associated cost. GED testing scholarships reimburse providers for costs associated with completing GED testing. Child Development Associate (CDA) Assessment and Renewal scholarships are used to pay for costs associated with obtaining or renewing the CDA credential. Academic degree scholarships are available for providers who are working toward a degree in early childhood education at an approved Idaho college or university. Single course scholarships cover the cost of one ECE or closely related course at an approved Idaho college or university. Badge incentives are a financial recognition available to providers who complete targeted, IdahoSTARS approved professional development activities to earn an IdahoSTARS Early Childhood Badge. Seven different badges are available and providers may earn up to 2 associated financial recognitions for earning badges each fiscal year.

- b. Does the Lead Agency use additional elements?

☒ Yes.

If yes, describe the element(s). Check all that apply.

- i. ☒ Continuing education unit trainings and credit-bearing professional development. Describe: **IdahoSTARS uses the International Accreditors of Continuing Education and Training (IACET) accreditor to determine training hour equivalents to continuing education units (CEU) and academic credits. IACET-approved companies equate one CEU per ten contact/training hours, academic credits equate as one credit per 15 contact/training hours. Waldorf and Montessori programs have a variable equivalent that is based on the category of class offered.**
- ii. ☒ Engagement of training and professional development providers, including higher education, in aligning training and educational opportunities with the Lead Agency's framework. Describe: **The IdahoSTARS Training Office oversees the ongoing training and professional development for child care providers in Idaho. The Training Office utilizes a system to approve trainers and training in each region, conferences, and e-learning opportunities. IdahoSTARS approved trainers and trainings reflect current research, best practice, and specialized topics. Trainings are accessible throughout the state in a variety of formats that includes live in-person, live webinar, online (e-learning), and through academic based course work. Idaho strives to meet the needs of a diverse provider and child care population by offering training in a variety of formats and utilizing the data system as a means to obtain language preferences and/or adaptations needed that allow all providers to successfully complete training classes. Idaho provides a variety of training in**

English and Spanish and offers interpreters for languages other than English, translation of materials for IdahoSTARS developed training, and adaptations upon request.

iii. ☐ Other. Describe:

☐ No.

6.2.3 Impact of the Professional Development Framework

Describe how the framework improves the quality, diversity, stability, and retention of caregivers, teachers, and directors and identify what data are available to assess the impact.

- a. Professional standards and competencies. For example, do the professional standards and competencies reflect the diversity of providers across role, child care setting, or age of children served? **Idaho's state standards, the Idaho Early Learning eGuidelines are designed to assist in guiding children's development and learning for a diversity of providers, across roles, different child care settings and is based on developmental ages Birth through Grade 3. The standards are a focus in all IdahoSTARS approved training.**

- Children are active learners who learn through play.
- Each child learns in unique ways.
- Children receive learning opportunities that support their unique needs in inclusive environments.
- Learning is most meaningful when it is integrated across all areas of development.
- Domain 1: Approaches to Learning, Executive Functioning, and Cognitive Development
- Domain 2: Physical Well-Being, Health and Motor Development
- Domain 3: Social and Emotional Development
- Domain 4: General Knowledge
- Domain 5: Communication, Language, and Literacy

IdahoSTARS Quality Domains and Subcategories are used to categorize approved trainings, target technical assistant, and coaching to ensure best practices are with the professional development offerings. The seven domains provide information for all child care providers and director/owner in a variety of settings. These domains contain content for ages Birth through 12 years.

- Curriculum & Instruction
- Child Development
- Inclusion & Diversity
- Business Practices
- Health, Safety & Wellness
- Environments
- Partnerships with Families & Communities

- b. Career pathways. For example, has the Lead Agency developed a wage ladder that provides progressively higher wages as early educators gain more experience and credentials? What types of child care settings and staff roles are addressed in career pathways, such as licensed centers and family child care homes? **The Career Pathway outlines specific requirements for obtaining credentials such as the Child Development Associate (CDA) credential, technical certificates in Early Childhood Education (ECE),**

associate degrees, bachelor's degrees, and master's degrees. By facilitating access to credentialing programs and incentivizing credential attainment, IdahoSTARS promotes diversity and professionalism within the early childhood education workforce. While IdahoSTARS may not have developed a formal wage ladder, the Career Pathway's emphasis on higher education, training, and credentials indirectly supports wage progression. As early educators gain more experience and qualifications, they become eligible for higher-paying positions within the field, contributing to stability and retention by providing opportunities for career advancement. The Career Pathway encompasses various child care settings, including licensed centers and family child care homes, as well as all staff roles. This comprehensive approach ensures that career advancement opportunities are accessible across all segments of the child care workforce. Data collected through the RISE Application includes credential attainment, workforce demographics and employment information such as wage and turnover. Analysis of the data in relation to Career Pathway participation and advancement provides insights into the diversity and inclusivity of the workforce and can assist in evaluation of the effectiveness of career advancement initiatives in retaining qualified professionals within the field.

- c. Advisory structure. For example, has the advisory structure identified goals for child care workforce compensation, including types of staff and target compensation levels? Does the Lead Agency have a Preschool Development Birth-to-Five grant and is part of its scope of work child care compensation activities? Are they represented in the advisory structure? **The Idaho Child Care Program (ICCP) Advisory Panel provides guidance and recommendations to both ICCP and the IdahoSTARS program. IdahoSTARS leadership works with Idaho's child care administrator and team to determine agenda items and plan for the Advisory Panel meetings, scheduled quarterly and held either in person and/or via a web-based platform to allow participation by all members of the panel. The Advisory Panel provides guidance and recommendations on the annual ICCP required training, updates and revisions to Steps to Quality, the initial and ongoing development of the RISE Application (IdahoSTARS' web-based database), revisions to the career pathway, incentives and opportunities that support the professional development for child care providers, and ongoing efforts to embed equity in our processes and practices.**

The Lead Agency is not the grantee for the Preschool Development Birth-to-Five grant, however the grantee is represented on the Idaho Child Care Advisory Panel. We do not work with any advisory group, ICCP, PDG or other regarding target compensation levels. Note that PDG is not directly aligned with CCDF recognitions, but PDG partnership activities around academic and CDA supports have been provided.

- d. Articulation. For example, how does the advisory structure include training and professional development for providers, including higher education, to assist in aligning training and education opportunities? **The Idaho Child Care Program (ICCP) Advisory Panel comprises representatives from various stakeholders, including higher education institutions, training providers, child care administrators, and IdahoSTARS leadership. This diverse representation allows for collaboration and input from key entities involved in training and education. The panel provides guidance and recommendations on a range of topics related to professional development, including training content, curriculum development, and educational opportunities. Higher education representatives contribute their expertise to ensure that training and educational offerings align with academic**

standards and best practices.

Idaho's legislature recently (effective January 2024) passed measures allowing and encouraging Idaho institutions of higher education to award "Credit for Prior Learning" academic credits to individuals who have achieved industry credentials Individualized assessments or experiential learning, particularly portfolio based assessments whereby students demonstrate learning acquired through experiences including but not limited to:

- a) Work or employment
- b) Employer training programs
- c) Independent study
- d) Non-credit courses
- e) Non-college courses or seminars
- f) Apprenticeships

These rules apply to every school in Idaho who offers an Early Childhood Education degree or certificate pathway; decisions on how each school implements this opportunity. At this time, opportunities exist for ECE prior learning credits at the College of Western Idaho (credits for CDA credential) and Lewis Clark State College (credit for prior learning awarded through a portfolio process). IdahoSTARS continues to work on articulation and credit for prior learning through collaboration with the Idaho ECE Higher Education Consortium.

- e. Workforce information. For example, does the Lead Agency have data on the existing wages and benefits available to the child care workforce? Do any partners such as the Quality Improvement System, child care resource and referral agencies, Bureau of Labor Statistics, and universities and research organizations collect compensation and benefits data? Does the Lead Agency monitor child care workforce wages and access to benefits through ongoing data collection and evaluation? Can the data identify any disparities in the existing compensation and benefits (by geography, role, child care setting, race, ethnicity, gender, or age of children served)? **The Lead Agency, through IdahoSTARS, monitors child care workforce wages and workforce turnover through ongoing data collection and evaluation. This involves collecting data on wages and other employment-related factors through the RISE Application and through periodic surveys of the workforce. Regular evaluation of this data allows for tracking trends over time and identifying areas for improvement. The IdahoSTARS annual publication 'State of the Child Care Workforce Report' examines issues of wages, turnover, and other workforce-related factors.**
- f. Financing. For example, has the Lead Agency set a minimum or living wage as a floor for all child care staff? Do Lead Agency-provider subsidy agreements contain requirements for staff compensation levels? Do Lead Agencies provide program-level compensation grants to support staff base salaries and benefits? Does the Lead Agency administer bonuses or stipends directly to workers? **IdahoSTARS does not explicitly set a minimum or living wage as a floor for all child care staff, the financing supports offered do indirectly contribute to improving wages and benefits for the workforce. By offering financial supports such as training scholarships, GED testing scholarships, academic degree scholarships, and compensation grants, IdahoSTARS helps providers increase their qualifications and potentially negotiate higher wages. Direct financial payments (bonuses) to child care workers as incentives for professional development are directly tied to retention, and**

quality improvement efforts. For example, the anniversary award tied to completion of training hours or college credits provides direct financial incentives to providers for investing in their professional growth. Data collected in the RISE Application includes information on wages, turnover rates, and other employment-related factors. Analysis of this data allows IdahoSTARS to assess the impact of financing initiatives on workforce stability, retention, and diversity.

6.3 Ongoing Training and Professional Development

6.3.1 Required hours of ongoing training

Provide the number of hours of ongoing training required annually for CCDF-eligible providers in the following settings:

- a. Licensed child care centers: **Twelve (12) hours annually**
- b. License-exempt child care centers: **Idaho does not have license-exempt center child care.**
- c. Licensed family child care homes: **Twelve (12) hours annually**
- d. License-exempt family child care homes: **Twelve (12) hours annually**
- e. Regulated or registered in-home child care: **Twelve (12) hours annually**
- f. Non-regulated or registered in-home child care: **Idaho does not license in-home child care.**

6.3.2 Accessibility of professional development for Tribal organizations

Describe how the Lead Agency's training and professional development are accessible to providers supported through Indian tribes or Tribal organizations receiving CCDF funds (as applicable). **Idaho's training and professional development system are open to all child care professionals, which includes all providers supported through Indian tribes or tribal organizations and who are in the field of early care and education. Within the IdahoSTARS data system training offerings are posted on an electronic training calendar as well as within the training catalog for provider's training registration and training requests. Offerings include live in-person, live webinar, online (e-learning), approved company, and targeted technical assistance/coaching for professional development hours. Individuals who are unaware of or do not utilize the data system are able to access the IdahoSTARS website where information is available on how to join the Professional Development System, how to create an account in the data system, and how to access other opportunities.**

6.3.3 Professional development appropriate for the diversity of children, families, and child care providers

Describe how the Lead Agency's training and professional development requirements reflect the diversity of children, families, and child care providers participating in CCDF. To the extent practicable, how does professional development include specialized training or credentials for providers who care for infants or school-age children; individuals with limited English proficiency; children who are bilingual; children with developmental delays or disabilities; and/or Native Americans, including Indians, as the term is defined in Section 900.6 in subpart B of the Indian Self-Determination and Education Assistance Act (including Alaska Natives) and Native Hawaiians? **IdahoSTARS training (live, webinar, and eLearning), targeted technical assistance, and coaching are**

offered within the Quality Domain of Inclusion & Diversity: Access-provides a wide range of activities and environments for every child by removing physical barriers and offering multiple ways to promote learning, Participation-uses a range of instructional approaches to promote engagement in play and learning activities, and a sense of belonging for every child, Supports-refers to broader aspects of the system such as professional development, incentives for inclusion, and opportunities for communication and collaboration among families and professionals to assure high quality inclusion, and Diversity-the program support children's social and cultural identity as it relates to race, color, religion, sex, income level, national origin, and home language.

IdahoSTARS live in-person and webinars offer child care providers interpreters as needed to ensure a better learning experience.

Infant and Toddler Mental Health: Recognizing the critical importance of mental health in early childhood, providers are encouraged to pursue specialized training and recognition to obtain the Infant and Family Associate (IFA) Endorsement category recognized by the Alliance.

- In partnership with AIM Early Idaho, IdahoSTARS IFA endorsed team members support child care providers in reviewing their professional development record (PDR) to identify gaps and recognize strengths. These IdahoSTARS team members develop training plans and support the provider through the application process. IdahoSTARS endorsed team members also act as endorsement reviewers, reviewing and recommending applicants from other entities in Idaho for endorsement. Providers are also assisted with a Pyramid Model Crosswalk and RISE Application crosswalk of approved trainings that meet the required competencies required to earn endorsement.

- Through intentional training opportunities IdahoSTARS highly trained team members offer training, coaching, and Targeted Technical Assistance (TTA) on infant and toddler mental health through the Pyramid Model, Zero to Three Critical Competency training, and other identified training in our RISE Application.

IdahoSTARS equips providers with the knowledge and skills to promote healthy social-emotional development in infants, identify early signs of mental health concerns, and support families in nurturing their child's emotional wellbeing.

IdahoSTARS live in-person and webinars offer child care providers interpreters as needed as well as any accommodation (Visual, Hearing, or Mobility) to ensure the best learning experience. All IdahoSTARS developed training, eLearning or PowerPoint, are developed using accessibility best practice for all learners. Every training developer must create their training with the lens of adaptations for the content to support all children and deliver these supports during the training. IdahoSTARS also offers many eLearning opportunities in Spanish with more being translated each year.

To better address the needs of specific populations, IdahoSTARS offers a comprehensive array of professional development opportunities that include specialized training and credentials tailored to providers who care for infants, school-age children, individuals with limited English proficiency, children who are bilingual, children with developmental disabilities or delays, and Native American children, including Indians, as the term is defined in Section 900.6 in subpart B of the Indian Self-Determination and Education Assistance Act (including Alaska Natives) and Native Hawaiians.

Here's a more detailed and specific description of how IdahoSTARS incorporates these elements into its training and support services:

IdahoSTARS Training and Support for Diverse Populations

General Overview:

IdahoSTARS provides a robust professional development framework within the Quality Domain of Inclusion & Diversity, encompassing Access, Participation, Supports, and Diversity. These initiatives are designed to ensure that every child has access to high-quality early childhood education, irrespective of their background or developmental needs. The training is delivered through live sessions, webinars, and eLearning, with additional targeted technical assistance and coaching.

Specialized Training and Credentials:

1. Infants and Toddlers:

Infant and Family Associate (IFA) Endorsement: IdahoSTARS encourages providers who work with infants to pursue specialized training in infant mental health, including the IFA Endorsement recognized by the Alliance. This endorsement emphasizes the importance of early childhood mental health and equips providers with the skills to support infants' social-emotional development.

Partnership with Aim Early Idaho: IFA-endorsed team members assist providers in reviewing their Professional Development Record (PDR) to identify gaps, develop training plans, and support the application process for endorsement. These team members also serve as endorsement reviewers, ensuring that applicants from across Idaho meet the required competencies.

Pyramid Model and Zero to Three Training: IdahoSTARS provides training on the Pyramid Model and Zero to Three Critical Competencies, focusing on promoting healthy social-emotional development, identifying early signs of mental health concerns, and supporting families in nurturing their infants' emotional well-being.

2. School-Age Children:

Developmentally Appropriate Practices: IdahoSTARS offers training on developmentally appropriate practices for school-age children, emphasizing engagement in learning activities that foster a sense of belonging. Providers learn to create inclusive environments that accommodate the varied needs of school-age children, ensuring their participation and success in child care settings.

3. Individuals with Limited English Proficiency and Bilingual Children:

Culturally Responsive Training: IdahoSTARS provides specialized training to equip providers with strategies for supporting children who have limited English proficiency or are bilingual. This includes offering interpreters during live and webinar training sessions to enhance learning experiences for providers. Training emphasizes creating inclusive environments that honor and support the linguistic and cultural identities of these children.

Language Support: The training and technical assistance offered by IdahoSTARS focus on developing instructional approaches that facilitate language acquisition and support bilingualism, promoting participation and engagement in the learning process.

Diverse Classroom Materials: Providers are trained on selecting culturally diverse books and materials that reflect the backgrounds of the children they serve, promoting a sense of belonging and cultural identity.

4. Children with Developmental Disabilities or Delays:

Inclusion and Accessibility Training: IdahoSTARS offers professional development on inclusive practices that address the needs of children with developmental disabilities or delays. This

includes training on modifying environments, adapting instructional methods, and utilizing assistive technologies to remove barriers to participation and learning.

Collaboration with Families: Providers are trained to collaborate closely with families and other professionals to develop individualized support plans that promote the full inclusion of children with disabilities in all aspects of the child care environment.

EC Series by SESTA: This series includes Early Care and Education Environment Indicators for Inclusion, which guide providers in adapting their environments to meet the needs of all children.

5. Native American Children, Including Indigenous Populations:

IdahoSTARS offers training that emphasizes the importance of understanding and respecting the cultural identities. This includes fostering an environment that supports these children's social and cultural identity, including their language, traditions, and values.

Continuous Support and Professional Development

Technical Assistance and Coaching: IdahoSTARS offers ongoing support through targeted technical assistance (TTA) and coaching to help providers apply what they have learned in their professional development. This ensures that training is not just theoretical but also practical and impactful in everyday childcare settings.

Collaborative Opportunities: Providers are encouraged to engage in communication and collaboration with families and other professionals to ensure the highest quality of inclusive care, reflecting the diverse backgrounds and needs of the children in their care.

6.3.4 Child developmental screening

Describe how all providers receive, through training and professional development, information about: (1) existing resources and services the State/Territory can make available in conducting developmental screenings and providing referrals to services when appropriate for children who receive assistance under this part, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program (42 U.S.C. 1396 et seq.) and developmental screening services available under section 619 and part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.); and (2) how child care providers may utilize these resources and services to obtain developmental screenings for children who receive assistance and who may be at risk for cognitive or other developmental delays, which may include social, emotional, physical, or linguistic delays: **Child Care Providers are introduced to developmental screenings through the Idaho Child Care Program In-office Orientation (IIO) as they decide to become an ICCP provider. IdahoSTARS has embedded an emerging indicator in our state quality and rating improvement system inclusive of the use of developmental monitoring and screening. Child care providers engaged with the Idaho Pyramid Model Collaborative begin using developmental screeners as a component of their roadmap in Year 2 or Year 3 of our three year roadmap. Providers additional have access to training and Targeted Technical Assistance (TTA) associated with developmental screeners specifically the ASQ 3 and ASQ SE 2. General information on developmental screening is located on the IdahoSTARS website in the Basics of Child Development tab.**

6.4 Early Learning and Developmental Guidelines

Lead Agencies must develop, maintain, or implement early learning and developmental guidelines appropriate for children from birth to kindergarten entry. Early learning and developmental guidelines should describe what children should know and be able to do at different ages and cover the essential domains of early childhood development, which at a minimum includes cognition, including language arts and mathematics; social, emotional, and physical development; and approaches toward learning.

6.4.1 Early learning and developmental guidelines

- a. Check the boxes below to certify the Lead Agency's early learning and developmental guidelines are:
 - i. ☒ Research-based.
 - ii. ☒ Developmentally appropriate.
 - iii. ☒ Culturally and linguistically appropriate.
 - iv. ☒ Aligned with kindergarten entry.
 - v. ☒ Appropriate for all children from birth to kindergarten entry.
 - vi. ☒ Implemented in consultation with the educational agency and the State Advisory Council on Early Childhood Education and Care or similar coordinating body.
 - vii. If any components above are not checked, describe: **NA**
- b. Check the boxes below to certify that the required domains are included in the Lead Agency's early learning and developmental guidelines.
 - i. ☒ Cognition, including language arts and mathematics.
 - ii. ☒ Social development.
 - iii. ☒ Emotional development.
 - iv. ☒ Physical development.
 - v. ☒ Approaches toward learning.
 - vi. ☐ Other optional domains. Describe any optional domains:
 - vii. If any components above are not checked, describe: **NA**
- c. When were the Lead Agency's early learning and developmental guidelines most recently updated and for what reason? **The Office of Head Start is currently the lead on development and updates for the Early Learning and Development Guidelines. The Idaho Early Learning Guidelines were revised in 2020-2021 and are reviewed every 5 years.**
- d. Provide the Web link to the Lead Agency's early learning and developmental guidelines. **<https://healthandwelfare.idaho.gov/services-programs/about-idaho-early-learning-guidelines>**

6.4.2 Use of early learning and developmental guidelines

- a. Describe how the Lead Agency uses its early learning and developmental guidelines. **The state of Idaho created the Idaho Early Learning eGuidelines, the latest set of high quality,**

research-based Idaho early learning standards for children from birth through kindergarten. The latest revision of the eGuidelines (2019-2020) are a culmination of a two-year study and revision process by a statewide team of early childhood experts, professionals, and higher education faculty.

The eGuidelines provides a common language and framework for discussing and describing young children's typical growth.

The eGuidelines are used to guide policy, pre-service and in-service development, and best practice at all levels of early childhood. These eGuidelines have been adopted as Idaho's Early Learning Standards for Idaho's Special Education Preschool programs. In addition, they are carefully aligned with Head Start/Early Head Start School Readiness goals, and the Head Start Early Learning Framework. The eGuidelines offer guidance on child development from birth to kindergarten for schools, Head Start programs, parents, child care providers, and professional development organizations. In addition, the eGuidelines expand and support the K-12 Common Core Standards by addressing early social and emotional development and approaches to learning and cognitive development through the primary grades. These address key developmental issues that are necessary for successful learning and are used extensively by our early special educators in the development of individual education plans. Each domain addressed in the eGuidelines specifically highlight "Supporting Individual Differences, Languages, and Diversity" noting that cultural and linguistic experiences shape development, Caregiver Strategies offer guidance on cultural variations in behavior and language.

These guidelines were vetted through the Idaho Child Care Program Advisory Panel. Through the support of the Early Childhood Coordinating Council (EC3) and the Head Start Collaboration office, collateral products were developed for infants and toddlers and for preschool aged children. These materials were disseminated to child care providers across the state by IdahoAEYC and Child Care Resource and Referral staff. IdahoSTARS approved training are required to reference the Early Learning eGuidelines and trainers include the connection of their training to the eGuidelines during their presentations. The intention is to help build capacity of child care providers to access the online tool when planning their classroom activities.

- b. Check the boxes below to certify that CCDF funds are not used to develop or implement an assessment for children that:
- i. ☒ Will be the primary or sole basis to determine a child care provider ineligible to participate in the CCDF.
 - ii. ☒ Will be used as the primary or sole basis to provide a reward or sanction for an individual provider.
 - iii. ☒ Will be used as the primary or sole method for assessing program effectiveness.
 - iv. ☒ Will be used to deny children eligibility to participate in CCDF.
 - v. If any components above are not checked, describe: **NA**

7 Quality Improvement Activities

The quality of child care directly affects children’s safety and healthy development while in care settings, and high-quality child care can be foundational across the lifespan. Lead Agencies may use CCDF for quality improvement activities for all children in care, not just those receiving child care subsidies. OCC will collect the most detailed Lead Agency information about quality improvement activities in annual reports instead of this Plan.

Lead Agencies must report on CCDF child care quality improvement investments in three ways:

1. In this Plan, Lead Agencies will describe the types of activities supported by quality investments over the 3-year period.
2. An annual expenditure report (the ACF-696). Lead Agencies will provide data on how much CCDF funding is spent on quality activities. This report will be used to determine compliance with the required quality and infant and toddler spending requirements.
3. An annual Quality Progress Report (the ACF-218). Lead Agencies will provide a description of activities funded by quality expenditures, the measures used to evaluate its progress in improving the quality of child care programs and services within the State/Territory, and progress or barriers encountered on those measures.

In this section of the Plan, Lead Agencies will describe their quality activities needs assessment and identify the types of quality improvement activities where CCDF investments are being made using quality set-aside funds.

7.1 Quality Activities Needs Assessment

7.1.1 Needs assessment process and findings

- a. Describe the Lead Agency needs assessment process for expending CCDF funds on activities to improve the quality of child care, including the frequency of assessment, how a diverse range of parents and providers were consulted, and how their views are incorporated: **IdahoSTARS engages child care providers through a variety of beneficiary voice activities to support the project’s program development, guide our implementation, and gauge our impact. It is through beneficiary voice activities that we understand the needs of child care providers in our state. Beneficiary voice activities can involve general or targeted groups of child care providers participating in listening sessions, interviews, surveys, and the completion of feedback loops. Steps to Quality, the state’s Quality Rating and Improvement System (QRIS), has utilized all forms of beneficiary voice activities to inform a QRIS restructure. Utilizing beneficiary voice activities with child care providers has contributed to our understanding of how our QRIS can better meet the needs of child care programs.**

IdahoSTARS is committed to utilizing beneficiary voice in our process to identify the needs of child care providers. We regularly hold listening session wherein we engage child care providers with broad, open-ended questions about our supports and services. We respect their expertise as users of our systems and use their feedback to inform our decision making. Providers are invited to listening session via email and phone call. The transcripts

of the listening sessions are reviewed by teams of staff members to identify common themes, the themes are then sent out to the listening session participants to confirm that we are correctly interpreting their feedback. Then we use those themes to inform our decision making.

- b. Describe the findings of the assessment, including any findings related to needs of different populations and types of providers, and if any overarching goals for quality improvement were identified: **Through our Beneficiary Voice Activities, we have identified the following overarching needs from providers:**
1. Child care providers want their work to be valued and supported by their peers, directors, families and IdahoSTARS.
 2. Child care providers value professional development as a Providers have distinct communication preferences and when IdahoSTARS is unaware of those preferences, users do not 'receive' information that is important to them.
 3. Child care providers value the RISE Application as a tool. They have concerns about the ease of navigation based on language preference, access to technology & technological fluency.
 4. Providers also have recommendations for improvement ways to learn new information, refresh knowledge and improve their skills/practice in order to better serve their families.
 5. Financial incentives are helpful and motivating. Incentives make providers feel recognized and appreciated.
 6. Providers find value in relationship based supports like coaching and TTA. These supports are valuable when they are consistent, responsive, reliable, well-informed and able to provide personalized support.
 7. Providers struggle to navigate IdahoSTARS systems without individualized support from an IdahoSTARS staff member.

During our listening sessions we have identified the following needs:

1. Child care providers want their work to be valued and supported by their peers, directors, families and IdahoSTARS.
2. Child care providers value professional development as a way to learn new information, refresh knowledge and improve their skills/practice in order to better serve their families.
3. Financial incentives are helpful and motivating. Incentives make providers feel recognized and appreciated.
4. Providers find value in relationship based supports like coaching and TTA. These supports are valuable when they are consistent, responsive, reliable, well-informed and able to provide personalized support.
5. Providers struggle to navigate IdahoSTARS systems without individualized support from an IdahoSTARS staff member.
6. Providers have distinct communication preferences and when IdahoSTARS is unaware of those preferences, users do not 'receive' information that is important to them.
7. Child care providers value the RISE Application as a tool. They have concerns about the ease of navigation based on language preference, access to technology & technological fluency. Providers also have recommendations for improvement.

7.2 Use of Quality Set-Aside Funds

Lead Agencies must use a portion of their CCDF expenditures for activities designed to improve the quality of child care services and to increase parental options for and access to high-quality child care. They must use the quality set-aside funds on at least one of 10 activities described in CCDF and the quality activities must be aligned with a Statewide or Territory-wide assessment of the State's or Territory's need to carry out such services and care.

7.2.1 Quality improvement activities

- a. Describe how the Lead Agency will make its Quality Progress Report (ACF – 218) and expenditure reports, available to the public. Provide a link if available. **Idaho's annual Quality Progress Report is published annually at this website:**
<https://healthandwelfare.idaho.gov/providers/child-care-providers/child-care-resources>

Annual program expenditures are reported in Idaho's Fact, Figures, and Trends reports published here:
<https://healthandwelfare.idaho.gov/about-dhw/facts-figures-and-trends>

- b. Identify Lead Agency plans, if any, to spend CCDF funds for each of the following quality improvement activities. If an activity is checked "yes", describe the Lead Agency's current and/or future plans for this activity.
- i. Supporting the training and professional development of the child care workforce, including birth to five and school-age providers.
- ☐ No plans to spend in this category of activities at this time.
- ☒ Yes. If yes, describe current and future investments. **Professional Development Specialist staff positions for development and delivery of training and targeted technical assistance across the state. Professional Development System Annual incentives for recognition of child care providers achievements. Training Scholarships to assist child care providers access of training opportunities. Badge Recognitions to acknowledge child care providers professional development attainment. Cohorts Recognitions to enhance the knowledge and skills obtained. Regional office Quality Consultant staff coach providers in early childhood best practices. New targeted technical assistance modules are developed and delivered each year.**
- ii. Developing, maintaining, or implementing early learning and developmental guidelines.
- ☒ No plans to spend in this category of activities at this time.
- ☐ Yes. If yes, describe current and future investments.
- iii. Developing, implementing, or enhancing a quality improvement system.
- ☐ No plans to spend in this category of activities at this time.
- ☒ Yes. If yes, describe current and future investments. **IdahoSTARS will continue to utilize the state's current Quality Rating and Improvement System (QRIS), Steps to Quality. Steps to Quality is a six-step block system available to licensed and subsidy-certified child care facilities and Head Starts. IdahoSTARS has been engaging in internal and external listening sessions and workgroups for several**

years with the eventual goal of a launching a restructured QRIS system.

- iv. Improving the supply and quality of child care services for infants and toddlers.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **Our current and future investments in activities such as Infant and Toddler Mental Health endorsement, Reflective Consultation, and Critical Competency training are crucial steps towards enhancing the quality of child care for our youngest learners. By allocating resources to support our staff in these specialized areas, we are not only investing in their professional development but also ensuring that they are equipped with the latest knowledge and skills. Collaborating with national partners further amplifies these efforts, allowing us to provide cutting-edge training and resources that ultimately benefit the well-being and development of infants and toddlers in Idaho.**

- v. Establishing or expanding a statewide system of CCR&R services.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **Idaho maintains a robust statewide system of CCR&R services. We would like to expand our Professional Development Specialists Position to include 1 to 2 more positions. This will support the delivery of targeted technical assistance and training.**

- vi. Facilitating compliance with Lead Agency child care licensing, monitoring, inspection and health and safety standards.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **Regional offices deliver Initial Idaho Child Care Program Orientations (IIO) that focus on compliance with licensing, monitoring, inspection, and health and safety standards. Child care Health Consultants respond to referrals for follow up on coaching from Health Inspectors and work with Providers to come into compliance. Idaho Child Care Program providers are required to take initial trainings focusing on Health and Safety Standards. These trainings are updated every three years. Providers are also required to take the annual training which IdahoSTARS develops each year and typically focuses on Health and Safety. General coaching from child care Health Consultants is often related to best practice in health and safety including compliance standards.**

- vii. Evaluating and assessing the quality and effectiveness of child care services within the State/Territory.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **IdahoSTARS uses a six-step Quality Rating and Improvement System (QRIS) to evaluate and assess the quality of participating child care facilities. Participation is voluntary and requires the facility is licensed and subsidy certified. The QRIS, Steps to Quality, utilizes a suite of Environment Rating Scales (ERS) to assess the participating programs, plus an annual verification of the programs quality practices that results in a quality**

rating. All programs at Steps 2-5 receive an annual ERS assessment. Minimum scores are required for rating at Steps 3-5. The block verification for a step rating consists of 79 quality indicators that result in a rating of Steps 1-5. Step 6 is a fast-track rating for programs with a current accreditation or Head Start grant.

viii. Accreditation support.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **IdahoSTARS supports programs on the way to and in maintaining national accreditation in a couple of ways. IdahoSTARS offers Quality Improvement grants for programs rated at Steps 2-6 requiring materials or training to meet their quality improvement goals, including but not limited to creating an environment that meets accreditation standards. IdahoSTARS also offers an accreditation grant for programs at Steps 3-6 that covers the cost of fees associated with national accreditation including self-study, application for accreditation or renewal.**

ix. Supporting State/Territory or local efforts to develop high-quality program standards relating to health, mental health, nutrition, physical activity, and physical development.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **The IdahoSTARS QRIS system, Steps to Quality, currently uses 79 quality indicators to verify a program's quality step. The quality indicators span all seven of IdahoSTARS quality domains: Business Practices, Health, Safety and Wellness, Environments, Child Development, Curriculum and Instruction, Inclusion and Diversity, and Partnerships with Families and Communities.**

x. Other activities determined by the Lead Agency to improve the quality of child care services and the measurement of outcomes related to improved provider preparedness, child safety, child well-being, or kindergarten entry.

☐ No plans to spend in this category of activities at this time.

☒ Yes. If yes, describe current and future investments. **Child Care Health Consultants coach providers in filling out emergency plans. IdahoSTARS coaches work individually with providers in emergency planning, safety, and well-being. These coaching sessions are logged into RISE Application. IdahoSTARS also has an emergency planning targeted technical assistance module that providers can request.**

8 Lead Agency Coordination and Partnerships to Support Service Delivery

Coordination and partnerships help ensure that the Lead Agency's efforts accomplish CCDF goals effectively, leverage other resources, and avoid duplication of effort. Such coordination and partnerships can help families better access child care, can assist in providing consumer education to parents, and can be used to improve child care quality and the stability of child care providers. Such coordination can also be particularly helpful in the aftermath of disasters when the provision of emergency child care services and the rebuilding and restoring of child care infrastructure are an essential part of ensuring the well-being of children and families in recovering communities.

This section identifies who the Lead Agency collaborates with to implement services, how match and maintenance-of-effort (MOE) funds are used, coordination with child care resource and referral (CCR&R) systems, and efforts for disaster preparedness and response plans to support continuity of operations in response to emergencies.

8.1 Coordination with Partners to Expand Accessibility and Continuity of Care

Lead Agencies must coordinate child care services supported by CCDF with other federal, State/Territory, and local level programs. This includes programs for the benefit of Indian children, infants and toddlers, children with disabilities, children experiencing homelessness, and children in foster care.

8.1.1 Coordination with required and optional partners

Describe how the Lead Agency coordinates and the results of this coordination of the provision of child care services with the organizations and agencies to expand accessibility and continuity of care and to assist children enrolled in early childhood programs in receiving full-day services that meet the needs of working families.

The Lead Agency must coordinate with the following agencies:

- a. State Advisory Council on Early Childhood Education and Care or similar coordinating body (pursuant to 642B(b)(1)(A)(i) of the Head Start Act). Describe the coordination and results of the coordination: **The Idaho Child Care Program (ICCP) Advisory Panel serves as the state council for both CCDF and Head Start to improve the quality of child care by linking services for children when possible and aligning systems to enhance services. The results of aligning systems is a more efficient and accessible program for Idaho families and providers, who access ICCP subsidy and IdahoSTARS services.**
- b. Indian Tribe(s) and/or Tribal organization(s), at the option of the Tribe or Tribal organization. Describe the coordination and results of the coordination, including which Tribe(s) was (were) involved: **The State will continue to partner with all Tribes that share borders with Idaho. The Idaho Department of Health and Welfare (IDHW) meets quarterly with all Tribes to ensure communication is happening both ways around programmatic information and policy changes. We use both the IDHW-wide meetings as well as the Idaho Child Care Program Advisory Panel meeting as avenues for sharing information, gathering feedback and providing opportunities for alignment where necessary. These actions results in stronger relationships with Tribal Nations which in turn allow us both to serve children and families to the best of our collective services.**

☐ Not applicable. Check here if there are no Indian Tribes and/or Tribal organizations in the State/Territory.
- c. State/Territory agency(ies) responsible for programs for children with disabilities, including early intervention programs authorized under the Individuals with Disabilities Education Act. Describe the coordination and results of the coordination: **The Lead Agency sits on the Infant Toddler Advisory Council with both Part C and 619 state leads. The CCDF administrator sits on the Infant Toddler Sub Committee for transitioning of children and coordinates inter-program efforts to share information and develop plans for linking comprehensive services between infant toddler, child care and school transitions. This results in a shared language across programs and clear expectations of programs for**

parents who participate in the programs identified.

- d. State/Territory office/director for Head Start State collaboration. Describe the coordination and results of the coordination: **The Head Start collaboration director sits on the ICCP Advisory Panel. The CCDF administrator and Head Start collaboration director meet regularly to discuss information between both programs, problem solve and develop long range plans for partnership. The CCDF administrator worked in partnership with the Head Start collaboration director to develop both the CCDF and Head Start grant applications with the focus on collaboration and alignment of activities to strengthen knowledge and understanding around CCDF and Head Start systems. Long term goals include: (1) Increasing enrollment of Head Start programs within Idaho's Steps to Quality program; and (2) aligning Head Start and Subsidy as an expanded effort to improve the standards around health and safety and staff qualifications of child care staff to better align with Head Start.**
- e. State/Territory agency responsible for public health, including the agency responsible for immunizations. Describe the coordination and results of the coordination: **Quarterly meetings are held with the Health District state lead, the CCDF program, Licensing, and 2-1-1 Careline to share information on changes, problem solving situations, and coordination of efforts for a more streamlined process for providers and families. IdahoSTARS child care health consultants (CCHC) work closely with each regional public health office to coordinate efforts in training, outreach and Idaho technical assistance regarding public health issues including immunizations, health and safety inspections, etc.**
- f. State/Territory agency responsible for employment services/workforce development. Describe the coordination and results of the coordination: **Both Employment Services and Child Care share the goal of ensuring that employment opportunities taking place within Idaho are being developed in tandem with the child care infrastructure to support families seeking care for their children as they relocate for these jobs. Reviewing workforce data and poverty levels as a component to community assessments will be critical in placement of jobs and recruitment of businesses. These efforts results in more families having access to child care programming as well as workforce and child care working together to recruit new child care businesses in areas of need, as identified through shared data evaluation.**
- g. State/Territory agency responsible for public education, including pre-Kindergarten. Describe the coordination and results of the coordination: **The Lead Agency is part of efforts to align and coordinate services where possible with both McKinney-Vento and 619, both programs within the State Department of Education (SDE). State funded prekindergarten is not available in Idaho, so the partnership with the SDE pertains only to McKinney-Vento and 619. The goal is to improve access to and alignment with McKinney-Vento services between the Idaho Department of Health and Welfare and the SDE. The process will be accomplished via cross training staff and teams throughout the state and providing professional development opportunities for teachers on how best to serve families experiencing homelessness. These efforts result in more unhoused children having access to child care and school districts knowing what resources are available for serving families.**
- h. State/Territory agency responsible for child care licensing. Describe the coordination and results of the coordination: **Idaho state licensing is housed under the Idaho Child Care**

Program (ICCP). Alignment of programming and services is a comprehensive aspect of the daily work of ICCP and all aspects of provider management. IdahoSTARS assists providers in starting and renewing child care licenses and Health Districts monitor to the health and safety requirements. Together in coordination, all three entities (DHW, IdahoSTARS, Health Districts) contribute to the execution of a license. Idaho's data system is coordinated, so providers upload all content in one location, even though various entities contribute to the components of compliance. This results in streamlined efforts for the provider, through simplified review and processing of all required documents, clear communication and availability of information at any point for a provider.

- i. State/Territory agency responsible for the Child and Adult Care Food Program (CACFP) and other relevant nutrition programs. Describe the coordination and results of the coordination: **CACFP sponsoring organizations such as St. Vincent de Paul administer the CACFP for child care homes, affiliated and unaffiliated child care centers, after school programs, and youth emergency shelters. For independent center sponsors, the responsibility for the CACFP lies with the State Department of Education, Child Nutrition Programs. The Lead Agency works with CACFP sponsoring organizations to promote and articulate CACFP education for providers. The goal of this coordination is to ensure all providers have nutritional information and/or subsidies in order to ensure children are provided nutritious snacks and meals and that parents are educated and have access to adequate nutrition. The Lead Agency ensures that all subsidized providers receive information about voluntary CACFP participation.**
- j. McKinney-Vento State coordinators for homeless education and other agencies providing services for children experiencing homelessness and, to the extent practicable, local McKinney-Vento liaisons. Describe the coordination and results of the coordination: **The CCDF administrator and McKinney-Vento coordinator sit on various state councils together, and they strive to coordinate meetings when mutually beneficial to share information and develop resources and tools that can assist both statewide coordinators in the schools and community partners serving families in local communities \who are experiencing homelessness.**
- k. State/Territory agency responsible for the TANF program. Describe the coordination and results of the coordination: **The Lead Agency is a part of the same state Department and Division responsible for TANF administration. In Idaho, the CCDF Administrator also serves as the TANF Administrator. Our goal is to coordinate meetings when necessary to improve access and continuity of care for children served by our two programs. Child care assistance is currently provided to families participating in the work services program mandatory for cash assistance families.**
- l. State/Territory agency responsible for Medicaid and the State Children's Health Insurance Program. Describe the coordination and results of the coordination: **The Lead Agency is a part of the same state Department and Division responsible for the state Children's Health Insurance Program and Medicaid eligibility determination. Our goal is to coordinate meetings when necessary to share information and develop plans on a per-topic basis.**
- m. State/Territory agency responsible for mental health services. Describe the coordination and results of the coordination: **The Lead Agency is a part of the same state Department responsible for Mental Health. Our goal is to coordinate meetings when necessary to improve access and continuity of care for children, served by our two programs.**

- n. Child care resource and referral agencies, child care consumer education organizations, and providers of early childhood education training and professional development. Describe the coordination and results of the coordination: **Child care resource centers (CCRC) are present in all seven regions of Idaho targeted at providing resources and education to both families and child care providers. CCRC staff provides technical assistance and coaching to child care providers enrolled in the state's Quality Ratings and Improvement System (QRIS) to improve the quality of care, expand accessibility and continuity of care, and assist children enrolled in early childhood programs to receive full-day services. The Lead Agency partners with the CCRCs through the implementation of activities as defined within the statewide contract with the University of Idaho. CCRCs are a part of IdahoSTARS, the state project working to improve the quality of child care, as defined by the Department.**
- o. Statewide afterschool network or other coordinating entity for out-of-school time care (if applicable). Describe the coordination and results of the coordination: **The Director of the Idaho Out-of-School Network sits on the ICCP Advisory Panel. The coordination goals for Idaho's Out-of-School Network are the following:**
 Increase awareness about and access for out-of-school programs
 Organize policy and partnerships for out-of-school programs
 Nurture quality and professional development for out-of-school programs
 This is completed through partnership between a statewide coalition, made up of leaders in Idaho, for which the Child Care Administrator is one participant. The process of aligning professional development for out-of-school programs with child care's Step to Quality (STQ) Program ensures consistency and a clear pathway for all programs serving children birth to age 13. STQ is a voluntary system for child care centers and home-based programs. Programs participating in STQ work with regional quality consultants to assess, improve, and communicate quality practices
- p. Agency responsible for emergency management and response. Describe the coordination and results of the coordination: **The Lead Agency is a part of the state's emergency management and response team. Our goal is to coordinate meetings with our Lead Agency coordinator when necessary to share information and develop plans on a per-topic basis.**
- q. The following are examples of optional partners a Lead Agency might coordinate with to provide services. Check which optional partners the Lead Agency coordinates with and describe the coordination and results of the coordination.
- i. ☒ State/Territory/local agencies with Early Head Start – Child Care Partnership grants. Describe: **The Lead Agency works alongside the Head Start office to continue education and knowledge building around the benefits of the Early Head Start - Child Care partnership. The long range goal is to increase the number of partnerships if given the opportunity to expand in future years.**
 - ii. ☒ State/Territory institutions for higher education, including community colleges. Describe: **IdahoSTARS is a joint project between the University of Idaho's (UIdaho) Center on Disabilities and Human Development (CDHD) and the Idaho Association for the Education of Young Children (Idaho AEYC). The project is funded by the Idaho Department of Health and Welfare (IDHW) through the Child Care and**

Development Block Grant (CCDBG). A function of the responsibilities of IdahoSTARS is to advance scholarships and incentives for students in early childhood education across Idaho's two- and four-year higher education institutions, headed by the director of academics and certifications on the IdahoSTARS project.

- iii. ☐ Other federal, State, local, and/or private agencies providing early childhood and school-age/youth-serving developmental services. Describe:
- iv. ☒ State/Territory agency responsible for implementing the Maternal, Infant, and Early Childhood Home Visiting (MIECHV) programs grant. Describe: **The CCDF administrator sits on the statewide advisory panel for the Maternal, Infant, and Early Childhood Home Visiting (MIECHV) Program. The goal is to increase the number of children receiving early intervention programming and continuity of care as families make progress through the years with their children. We are striving to increase collaboration and coordination of shared resources to professionals in the field of early care and education.**
- v. ☐ Agency responsible for Early and Periodic Screening, Diagnostic, and Treatment Program. Describe:
- vi. ☒ State/Territory agency responsible for child welfare. Describe: **The Lead Agency is a part of the same state Department responsible for Child Welfare. Our goal is to coordinate meetings when necessary to improve access and continuity of care for children served by our two programs. Child Care partners with Child Welfare in determining eligibility for foster children in need of child care services. Specifically, children in state licensed foster homes have a streamlined eligibility process with a simplified application that does not count the foster parents' income when determining co-pays or eligibility for the foster child. This partnership is defined in an agreement, and when processes or procedures change, both divisions coordinate communication and training strategies.**
- vii. ☐ Child care provider groups or associations. Describe:
- viii. ☐ Parent groups or organizations. Describe:
- ix. ☐ Title IV B 21st Century Community Learning Center Coordinators. Describe:
- x. ☐ Other. Describe:

8.2 Optional Use of Combined Funds, CCDF Matching, and Maintenance-of-Effort Funds

Lead Agencies may combine CCDF funds with other Federal, State, and local child care and early childhood development programs, including those in 8.1.1. These programs include preschool programs, Tribal child care programs, and other early childhood programs, including those serving infants and toddlers with disabilities, children experiencing homelessness, and children in foster care.

Combining funds may include blending multiple funding streams, pooling funds, or layering funds from multiple funding streams to expand and/or enhance services for infants, toddlers, preschoolers, and school-age children and families to allow for the delivery of comprehensive quality care that meets the needs of children and families. For example, Lead Agencies may use

multiple funding sources to offer grants or contracts to programs to deliver services; a Lead Agency may allow a county/local government to use coordinated funding streams; or policies may be in place that allow local programs to layer CCDF funds with additional funding sources to pay for full-day, full-year child care that meets Early Head Start/Head Start Program Performance Standards or State/Territory pre-Kindergarten requirements in addition to State/Territory child care licensing requirements.

As a reminder, CCDF funds may be used in collaborative efforts with Head Start and Early Head Start programs to provide comprehensive child care and development services for children who are eligible for both programs.

8.2.1 Combining funding for CCDF services

Does the Lead Agency combine funding for CCDF services with Title XX of the Social Services Block Grant (SSBG), Title IV B 21st Century Community Learning Center Funds, State-only child care funds, TANF direct funds for child care not transferred into CCDF, Title IV-B, IV-E funds, or other federal or State programs?

☐ No. (If no, skip to question 8.2.2)

☒ Yes.

- i. If yes, describe which funds you will combine. Combined funds may include, but are not limited to:

☒ Title XX (Social Services Block Grant, SSBG)

☐ Title IV B 21st Century Community Learning Center Funds (Every Student Succeeds Act)

☐ State- or Territory-only child care funds

☒ TANF direct funds for child care not transferred into CCDF

☐ Title IV-B funds (Social Security Act)

☐ Title IV-E funds (Social Security Act)

☐ Other. Describe:

- ii. If yes, what does the Lead Agency use combined funds to support, such as extending the day or year of services available (i.e., full-day, full-year programming for working families), smoothing transitions for children, enhancing and aligning quality of services, linking comprehensive services to children in child care, or developing the supply of child care for vulnerable populations? **Idaho defines "combine" as using CCDF in combination with other allocated funding sources to serve eligible children. Idaho combines the Social Services Block Grant (SSBG), Temporary Assistance for Needy Families (TANF) program, and State General Funds.**

8.2.2 Funds used to meet CCDF matching and MOE requirements

Lead Agencies may use public funds and donated funds to meet CCDF match and maintenance of effort (matching MOE) requirements.

Note: Lead Agencies that use State pre-Kindergarten funds to meet matching requirements must check State pre-Kindergarten funds and public and/or private funds.

Use of private funds for match or maintenance-of-effort: Donated funds do not need to be under the administrative control of the Lead Agency to qualify as an expenditure for federal match. However, Lead Agencies must identify and designate in the State/Territory CCDF Plan the donated funds given to public or private entities to implement the CCDF child care program.

☐ Not applicable. The Lead Agency is a Territory (skip to 8.3.1).

a. Does the Lead Agency use public funds to meet match requirements?

☒ Yes. If yes, describe which funds are used: **To meet the CCDF MOE and match requirements, the Lead Agency will use state general funds spent on child care expenses beyond those covered by the Idaho Child Care Program for children in foster care. Additionally, state general funds spent by local school districts on the administration of non-compulsory kindergarten and early literacy initiatives will be used for match in accordance with the federal limitations on public pre-kindergarten funds.**

☐ No.

b. Does the Lead Agency use donated funds to meet match requirements?

☒ Yes. If yes, identify the entity(ies) designated to receive donated funds:

i. ☐ Donated directly to the state.

ii. ☒ Donated to a separate entity(ies) designated to receive donated funds. If checked, identify the name, address, contact, and type of entities designated to receive private donated funds: **The Idaho Workforce Development Council (IWDC) administered grants for investments that expand high quality child care capacity using non-CCDF funds. The grants were intended to offset start-up costs for employers providing onsite/near-site child care , encourage child care provider/employer partners to expand number of children served, and support with recruiting and training of qualified childcare staff. Childcare grant participants demonstrated private investments of at least 50% of the grant funds requested, these private investments have been recorded and verified by the Idaho Workforce Development Council (514 W Jefferson St. Ste. 131 Boise, ID 83702) which financial information provided to the lead agency in full alignment with our record retention policies for matching purposes.**

☐ No.

c. Does the Lead Agency certify that, if State expenditures for pre-Kindergarten programs are used to meet the MOE requirements, the following is true:

- The Lead Agency did not reduce its level of effort in full-day/full-year child care services.
- The Lead Agency ensures that pre-Kindergarten programs meet the needs of working parents.
- The estimated percentage of the MOE requirement that will be met with pre-Kindergarten expenditures (does not to exceed 20 percent).
- If the percentage is more than 10 percent of the MOE requirement, the State will coordinate its pre-Kindergarten and child care services to expand the availability of child

care.

Public pre-Kindergarten funds may also serve as MOE funds as long as the State can describe how it will coordinate pre-Kindergarten and child care services to expand the availability of child care while using public pre-Kindergarten funds as no more than 20 percent of the State's MOE or 30 percent of its matching funds in a single fiscal year.

If expenditures for pre-Kindergarten services are used to meet the MOE requirement, does the Lead Agency certify that the State or Territory has not reduced its level of effort in full-day/full-year child care services?

☒ Yes.

☐ No. If no, describe:

8.3 Coordination with Child Care Resource and Referral Systems

Lead Agencies may use CCDF funds to establish or support a system or network of local or regional child care resource and referral (CCR&R) organizations that is coordinated, to the extent determined by the Lead Agency, by a statewide public or private non-profit, community-based or regionally based, lead child care resource and referral organization (such as a statewide CCR&R network).

If Lead Agencies use CCDF funds for local CCR&R organizations, the local or regional CCR&R organizations supported by those funds must, at the direction of the Lead Agency:

- Provide parents in the State with consumer education information concerning the full range of child care options (including faith-based and community-based child care providers), analyzed by provider, including child care provided during non-traditional hours and through emergency child care centers, in their area.
- To the extent practicable, work directly with families who receive assistance to offer the families support and assistance to make an informed decision about which child care providers they will use to ensure that the families are enrolling their children in the most appropriate child care setting that suits their needs and one that is of high quality (as determined by the Lead Agency).
- Collect data and provide information on the coordination of services and supports, including services under Part B, Section 619 and Part C of the Individuals with Disabilities Education Act.
- Collect data and provide information on the supply of and demand for child care services in areas of the State and submit the information to the Lead Agency.
- Work to establish partnerships with public agencies and private entities, including faith-based and community-based child care providers, to increase the supply and quality of child care services in the State and, as appropriate, coordinate their activities with the activities of the Lead Agency and local agencies that administer funds made available through CCDF.

8.3.1 Funding a system or network of CCR&R organization(s)

Does the Lead Agency fund a system or network of local or regional CCR&R organization(s)?

☐ No. The Lead Agency does not fund a system or network of local or regional CCR&R organization(s) and has no plans to establish one.

☐ No, but the Lead Agency has plans to develop a system or network of local or regional CCR&R organization(s).

☒ Yes. The Lead Agency funds a system or network of local or regional CCR&R organization(s) with all the responsibilities outlined above. If yes, describe the activities outlined above carried out by the CCR&R organization(s), as directed by the Lead Agency:

CCR&R services are provided statewide through a contract with the University of Idaho (Uldaho), which employs staff and/or contracts with other agencies to carry out the CCR&R scope of work. The scope of work for the CCR&R offices includes:

(1) Working directly with families who receive child care assistance to offer the families support and assistance in making an informed decision about child care options to ensure families are enrolling their children in the most appropriate child care setting to suit their needs and that is of high quality as determined by IdahoSTARS. IdahoSTARS does this by discussing options for childcare with parents and sending resources to parents with referral information. These resources include when applicable, breastfeeding and safe sleep information, what to look for when searching for care, questions to ask providers, crying baby plan, developmental screening, act early information, keeping children safe abuse and neglect information, and guidelines for human milk storage. IdahoSTARS offers standard, customized, and enhanced referrals.

(2) Collecting data and providing information on the coordination of services and supports including services provided through the Individuals with Disabilities Education Improvement Act (IDEA) for children with disabilities;

(3) Collecting data from referrals though follow up surveys. When unmet needs are reported, this information gets documented in the RISE Application.

(4) Engaging in services to increase the supply and quality of child care services in the state through business practice training and coaching to improve sustainability and quality;

(5) working with other agencies through community events and partnerships to identify and support work around expanding childcare access and as appropriate, coordinating their activities with the activities of the lead agency and/or local agencies that administer Child Care and Development Funds (CCDF);

(6) Reviewing rates and contacting a minimum of 250 providers annually whose data stands out as outliers, working with the provider to correct any discrepancies for market rate quality assurance, and encouraging and assisting providers in updating their Provider Information Report; For such contracts as Uldaho, the State monitors administrative implementation and responsibilities through the Division of Welfare's Contracts and External Resource Management (CERM) team. CERM evaluated and ensures that the standards, techniques, and criteria identified in the scope of work are consistent and timely.

8.4 Public-Private Partnerships

Lead Agencies must demonstrate how they encourage partnerships among other public agencies, Tribal organizations, private entities, faith-based organizations, businesses, or organizations that promote business involvement, and/or community-based organizations to leverage existing service delivery (i.e., cooperative agreement among providers to pool resources to pay for shared fixed costs and operation) to leverage existing child care and early education service delivery systems and to increase the supply and quality of child care services for children younger than

age 13.

8.4.1 Lead Agency public-private partnerships

Identify and describe any public-private partnerships encouraged by the Lead Agency to leverage public and private resources to further the goals of CCDF: **The Idaho Department of Health and Welfare is providing review and support of the Idaho Workforce Development Council's administration of Child Care Expansion Grants. This funding, administered by the Workforce Development Council (WDC), is increasing the number of child care seats available to working families by targeting employers and/or child care providers willing to build new or expand existing facilities. Grant participants are developing and coordinating local business partnerships in support of the expansion of available child care services in each community. With an emphasis on high-quality care, these partnerships are homegrown, diverse opportunities meeting the unique needs of Idaho's workforce, child care providers, and children.**

8.5 Disaster Preparedness and Response Plan

Lead Agencies must establish a Statewide Child Care Disaster Plan and demonstrate how they will address the needs of children—including the need for safe child care before, during, and after a state of emergency declared by the Governor or a major disaster or emergency (as defined by Section 102 of the Robert T. Stafford Disaster Relief and Emergency Assistance Act, 42 U.S.C. 5122)—through a Statewide Disaster Plan.

8.5.1 Statewide Disaster Plan updates

- a. When was the Lead Agency's Child Care Disaster Plan most recently updated and for what reason? **The Idaho Department of Health and Welfare's Continuity of Operations Plan is reviewed and maintained on an official biennial plan review cycle, the most recent review and updates were completed December 2023.**
- b. Please certify compliance by checking the required elements the Lead Agency includes in the current State Disaster Preparedness and Response Plan.
 - i. The plan was developed in collaboration with the following required entities:
 - ☒ State human services agency.
 - ☒ State emergency management agency.
 - ☒ State licensing agency.
 - ☒ State health department or public health department.
 - ☒ Local and State child care resource and referral agencies.
 - ☒ State Advisory Council on Early Childhood Education and Care or similar coordinating body.
 - ii. ☒ The plan includes guidelines for the continuation of child care subsidies.
 - iii. ☒ The plan includes guidelines for the continuation of child care services.
 - iv. ☒ The plan includes procedures for the coordination of post-disaster recovery of child care services.

- v. The plan contains requirements for all CCDF providers (both licensed and license-exempt) to have in place:
 - ☒ Procedures for evacuation.
 - ☒ Procedures for relocation.
 - ☒ Procedures for shelter-in-place.
 - ☒ Procedures for communication and reunification with families.
 - ☒ Procedures for continuity of operations.
 - ☒ Procedures for accommodations of infants and toddlers.
 - ☒ Procedures for accommodations of children with disabilities.
 - ☒ Procedures for accommodations of children with chronic medical conditions.
- vi. ☒ The plan contains procedures for staff and volunteer emergency preparedness training.
- vii. ☒ The plan contains procedures for staff and volunteer practice drills.
- viii. If any of the above are not checked, describe:
- ix. If available, provide the direct URL/website link to the website where the Statewide Child Care Disaster Plan is posted:

9 Family Outreach and Consumer Education

CCDF consumer education requirements facilitate parental choice in child care arrangements, support parents as child care consumers who need information to make informed choices regarding the services that best suit their family's needs, and the delivery of resources that can support child development and well-being. Lead Agency consumer education activities must provide information for parents receiving CCDF assistance, the general public, and, when appropriate, child care providers. Lead Agencies should use targeted strategies for each group to ensure tailored consumer education information and take steps to ensure they are effectively reaching all individuals, including those with limited English proficiency and those with disabilities.

In this section, Lead Agencies address their consumer education practices, including details about their child care consumer education website, and the process for collecting and maintaining a record of parental complaints.

9.1 Parental Complaint Process

Lead Agencies must maintain a record of substantiated parental complaints against child care providers and make information regarding such complaints available to the public on request. Lead Agencies must also provide a detailed description of the hotline or similar reporting process for parents to submit complaints about child care providers; the process for substantiating complaints; the manner in which the Lead Agency maintains a record of substantiated parental complaints; and ways that the Lead Agency makes information on such parental complaints available to the public on request. Lead Agencies are not required to limit the complaint process to parents.

9.1.1 Parental complaint process

- a. Describe the Lead Agency's hotline or similar reporting process through which parents can submit complaints about child care providers, including a link if it is a Web-based process: **Parents, families, and concerned community members can submit child care complaints via email (careline@dhw.idaho.gov), fax (208-334-5531), or telephone by contacting 2-1-1 Idaho CareLine (1-800-926-2588). An online complaint form is available at the following web address: <https://healthandwelfare.idaho.gov/services-programs/children-families/file-child-care-complaint>**
- b. Describe how the parental complaint process ensures broad access to services for families that speak languages other than English: **Language assistance services are available pre request for families that speak languages other than English.**
- c. Describe how the parental complaint process ensures broad access to services for persons with disabilities: **The Idaho CareLine (211) phone line offers a Telecommunications Device for the Deaf (TDD) phone line and accepts text messages. Complaints can also be made in writing, by fax, or electronically.**
- d. For complaints about providers, including CCDF providers and non-CCDF providers, does the Lead Agency have a process and timeline for screening, substantiating, and responding to complaints, including information about whether the process includes monitoring?

☒ Yes. If yes, describe: **The complaint process is the same for all child care providers in Idaho. Parents, families, and concerned community members can submit child care complaints via email(careline@dhw.idaho.gov), fax (208-334-5531), or telephone by contacting 2-1-1 IdahoCareLine (1-800-926- 2588). Once received, the complaints are entered into a single database called the Idaho Child Care Incident Tracker. Every complaint is reviewed by the state Child Care Program specialist or the State Daycare Licensing specialist to determine the correct resources and agencies for referral. Complaints involving health and safety violations, the operation of a facility without a required license, and ratio/supervision violations may all result in an unannounced health and safety inspection. Complaints related to business practices, professionalism, billing, toilet training, meals, and activities are referred to voluntary consultation through the Child Care Resource Center. Allegations of fraud are forwarded to an Idaho Department of Health and Welfare (DHW)'s Integrity Unit that specializes in the review of child care facilities. Any complaint alleging any type of abuse or illegal activity is forwarded to local law enforcement agencies, and a follow up report is requested from the responding agency within 30 days.**

☐ No.
- e. For substantiated parental complaints, who maintains the record for CCDF and non-CCDF providers? **Records of substantiated complaints against licensed and/or providers participating with the Idaho Child Care Program and all supporting documentation are maintained indefinitely in the Idaho Child Care Concern Tracker, and summaries of substantiated complaints are published for three years on IdahoChildCareCheck.org.**
- f. Describe how information about substantiated parental complaints is made available to the public; this information can include the consumer education website discussed in subsection 9.2: **Summaries of all substantiated complaints, and the response to each complaint, are published for three years on IdahoChildCareCheck.org.**

9.2 Consumer Education Website

Lead Agencies must provide information to parents, the general public, and child care providers through a State or Territory website, which is consumer-friendly and easily accessible for families who speak languages other than English and persons with disabilities. The website must:

- Include information to assist families in understanding the Lead Agency's policies and procedures, including licensing child care providers;
- Include monitoring and inspection reports for each provider and, if available, the quality of each provider;
- Provide the aggregate number of deaths, serious injuries, and the number of cases of substantiated child abuse that have occurred in child care settings;
- Include contact information for local CCR&R organizations to help families access additional information on finding child care; and
- Include information on how parents can contact the Lead Agency and other organizations to better understand the information on the website.

9.2.1 Consumer-friendly website

Does the Lead Agency ensure that its consumer education website is consumer-friendly and easily accessible?

- Provide the URL for the Lead Agency's consumer education website homepage:
<https://idahostars.org/>
- Does the Lead Agency certify that the consumer education website ensures broad access to services for families who speak languages other than English?
☒ Yes.
☐ No. If no, describe:
- Does the Lead Agency certify that the consumer education website ensures broad access to services for persons with disabilities?
☒ Yes.
☐ No. If no, describe:

9.2.2 Additional consumer education website links

Provide the direct URL/website link for the following:

- Provide the direct URL/website link to how the Lead Agency licenses child care providers: **<https://idahostars.org/Child-Care-Providers/Child-Care-Licensing>**
- Provide the direct URL/website link to the processes for conducting monitoring and inspections of child care providers: **<https://idahostars.org/Child-Care-Providers/Child-Care-Licensing>**
- Provide the direct URL/website link to the policies and procedures related to criminal background checks for staff members of child care providers:
<https://healthandwelfare.idaho.gov/bcu>

- iv. Provide the direct URL/website link to the offenses that prevent individuals from being employed by a child care provider:
<https://adminrules.idaho.gov/rules/current/16/160506.pdf>

9.2.3 Searchable list of providers

- a. The consumer education website must include a list of all licensed providers searchable by ZIP code.
- i. Does the Lead Agency certify that the consumer education website includes a list of all licensed providers searchable by ZIP code?
☒ Yes.
☐ No. If no, describe:
- ii. Provide the direct URL/website link to the list of child care providers searchable by ZIP code: <https://idahostars.org/Families?page13735=1&size13735=12#Search>
- iii. In addition to the licensed child care providers that must be included in the searchable list, are there additional providers included in the Lead Agency's searchable list of child care providers? Check all that apply:
- ☐ License-exempt center-based CCDF providers.
☒ License-exempt family child care CCDF providers.
☐ License-exempt non-CCDF providers.
☐ Relative CCDF child care providers.
☐ Other (e.g., summer camps, public pre-Kindergarten). Describe:
- b. Identify what additional (optional) information, if any, is available in the searchable results by ZIP code. Check the box when information is provided.

Provider Information Available in Searchable Results					
	All licensed providers	License-exempt CCDF center-based providers	License-exempt CCDF family child care home providers	License-exempt non-CCDF providers	Relative CCDF providers
Contact information	☒	☐	☒	☐	☒
Enrollment capacity	☒	☐	☒	☐	☐
Hours, days, and months of operation	☐	☐	☐	☐	☐
Provider education and training	☒	☐	☒	☐	☐

Languages spoken by the caregiver	☒	☐	☒	☐	☐
Quality information	☒	☐	☒	☐	☐
Monitoring reports	☒	☐	☒	☐	☐
Willingness to accept CCDF certificates	☒	☐	☒	☐	☐
Ages of children served	☐	☐	☐	☐	☐
Specialization or training for certain populations	☐	☐	☐	☐	☐
Care provided during nontraditional hours	☐	☐	☐	☐	☐

- c. Identify any other information searchable on the consumer education website for the child care provider type listed below and then, if checked, describe the searchable information included on the website.
- i. ☐ All licensed providers. Describe:
 - ii. ☐ License-exempt CCDF center-based providers. Describe:
 - iii. ☐ License-exempt CCDF family child care providers. Describe:
 - iv. ☐ License-exempt, non-CCDF providers. Describe:
 - v. ☐ Relative CCDF providers. Describe:
 - vi. ☐ Other. Describe:

9.2.4 Provider-specific quality information

Lead Agencies must identify specific quality information on each child care provider for whom they have this information. Provider-specific quality information must only be posted on the consumer education website if it is available for the individual child care provider.

- a. What specific quality information does the Lead Agency provide on the website?
 - i. ☒ Quality improvement system.
 - ii. ☒ National accreditation.
 - iii. ☐ Enhanced licensing system.
 - iv. ☐ Meeting Head Start/Early Head Start Program Performance Standards.
 - v. ☐ Meeting pre-Kindergarten quality requirements.
 - vi. ☐ School-age standards.
 - vii. ☐ Quality framework or quality improvement system.
 - viii. ☐ Other. Describe:
- b. For what types of child care providers is quality information available?
 - i. ☒ Licensed CCDF providers. Describe the quality information: **The current quality**

rating, license status, subsidy certification for each facility.

- ii. ☐ Licensed non-CCDF providers. Describe the quality information:
- iii. ☐ License-exempt center-based CCDF providers. Describe the quality information:
- iv. ☐ License-exempt FCC CCDF providers. Describe the quality information:
- v. ☐ License-exempt non-CCDF providers. Describe the quality information:
- vi. ☐ Relative child care providers. Describe the quality information:
- vii. ☐ Other. Describe:

9.2.5 Aggregate data on serious injuries, deaths, and substantiated abuse

Lead Agencies must post aggregate data on serious injuries, deaths, and substantiated cases of child abuse that have occurred in child care settings each year on the consumer education website. This aggregate data must include information about any child in the care of a provider eligible to receive CCDF, not just children receiving subsidies.

This aggregate information on serious injuries and deaths must be separated by category of care (e.g., centers, family child care homes, and in-home care) and licensing status (i.e., licensed or license-exempt) for all eligible CCDF child care providers in the State/Territory. The information on instances of substantiated child abuse does not have to be organized by category of care or licensing status. Information must also include the total number of children in care by provider type and licensing status, so that families can better understand the data presented on serious injuries, deaths, and substantiated cases of abuse.

- a. Certify by checking below that the required elements are included in the Aggregate Data Report on serious incident data that have occurred in child care settings each year.
 - i. ☒ The total number of serious injuries of children in care by provider category and licensing status.
 - ii. ☒ The total number of deaths of children in care by provider category and licensing status.
 - iii. ☒ The total number of substantiated instances of child abuse in child care settings.
 - iv. ☒ The total number of children in care by provider category and licensing status.
 - v. If any of the above elements are not included, describe: **NA**
- b. Certify by providing:
 - i. The designated entity to which child care providers must submit reports of any serious injuries or deaths of children occurring in child care and describe how the Lead Agency obtains the aggregate data from the entity: **Providers must self report any serious injuries or deaths associated with care in their facility to the Lead Agency by contacting IDHW via phone at the Idaho CareLine or via email at the Child Care inbox (ChildCare@dhw.idaho.gov). Child care staff add the information into Idaho Child Care Incident Tracker, the Lead Agency is able to obtain aggregate data from the Incident Tracker system.**

- ii. The definition of “substantiated child abuse” used by the Lead Agency for this requirement: **Substantiated Incidents are investigated and determined to have occurred by Law Enforcement in the region where the incident occurred.**
- iii. The definition of “serious injury” used by the Lead Agency for this requirement: **Category for incidents regarding children that required hospitalization or a doctor’s care. This category is not used for minor injuries such as scrapes, diaper rash, or child-to-child biting.**
- c. Provide the direct URL/website link to the page where the aggregate number of serious injuries, deaths, and substantiated child abuse, and the total number of children in care by provider category and licensing status are posted:
<https://healthandwelfare.idaho.gov/providers/child-care-providers/child-care-resources>

9.2.6 Contact information on referrals to local child care resource and referral organizations

The Lead Agency consumer education website must include contact information on referrals to local CCR&R organizations.

- a. Does the consumer education website include contact information on referrals to local CCR&R organizations?
☒ Yes.
☐ No.
☐ Not applicable. The Lead Agency does not have local CCR&R organizations.
- b. Provide the direct URL/website link to this information:
<https://idahostars.org/Families?page13735=1&size13735=12#Search>

9.2.7 Lead Agency contact information for parents

The Lead Agency consumer and provider education website must include information on how parents can contact the Lead Agency or its designee and other programs that can help the parent understand information included on the website.

- a. Does the website provide directions on how parents can contact the Lead Agency or its designee and other programs to help them understand information included on the website?
☒ Yes.
☐ No.
- b. Provide the direct URL/website link to this information: **<https://idahochildcarecheck.org>**

9.2.8 Posting sliding fee scale, co-payment amount, and policies for waiving co-payments

The consumer education website must include the sliding fee scale for parent co-payments, including the co-payment amount a family may expect to pay and policies for waiving co-payments.

- a. Does the Lead Agency certify that their consumer education website includes the sliding fee scale for parent co-payments, including the co-payment amount a family may expect to pay and policies for waiving co-payments?

☒ Yes.

☐ No.

- b. Provide the direct URL/website link to the sliding fee scale.

<https://publicdocuments.dhw.idaho.gov/WebLink/DocView.aspx?id=4671&dbid=0&repo=PUBLIC-DOCUMENTS>

9.3 Increasing Engagement and Access to Information

Lead Agencies must collect and disseminate information about the full range of child care services to promote parental choice to parents of children eligible for CCDF, the general public, and child care providers.

9.3.1 Information about CCDF availability and eligibility

Describe how the Lead Agency shares information with eligible parents, the general public, and child care providers about the availability of child care services provided through CCDF and other programs for which the family may be eligible. The description should include, at a minimum, what is provided (e.g., written materials, the website, and direct communications) and what approaches are used to tailor information to parents, the general public, and child care providers.
For Parents and the General Public-

The Idaho Department of Health and Welfare (DHW) utilizes videos to highlight the Idaho Child Care Program and its benefits to families, as well as how families can seek assistance; these videos are played statewide in the lobbies of each local office.

IDHW provides information on income, eligible activities and other criteria needed for eligibility via websites

(<http://healthandwelfare.idaho.gov/Children/ChildCareAssistance/tabid/292/Default.aspx> ; <https://www.livebetteridaho.org/>), vendor booths, blogs (<https://dhwblog.com/>), and social media tools (<https://www.facebook.com/IdahoHealthandWelfare/> and

<https://www.facebook.com/IdahoSTARS-252266474799407/?fref=ts>) and brochures

(<https://publicdocuments.dhw.idaho.gov/WebLink/Browse.aspx?id=2892&dbid=0&repo=PUBLIC-DOCUMENTS&cr=1>) are disseminated to community partners who serve families with children.

IdahoSTARS has a family portal that provides information about the ICCP program

<https://idahostars.org/Families/Financial-Assistance-ICCP>, with relevant links to the application.

<https://healthandwelfare.idaho.gov/services-programs/children-families/apply-child-care-assistance> Additionally, IdahoSTARS has created a fact sheet about qualifications and income guidelines that is passed out at public events and given to child care providers for distribution to families.

Families applying for the Idaho Child Care Program are made aware of other assistance programs such as SNAP, TANF, and Medicaid at application and redetermination. Families and the general public may access program information for other assistance programs at the IDHW Website:

<https://healthandwelfare.idaho.gov/>

Families also have a dedicated webpage with family specific resources at <https://idahostars.org/>

Child Care Providers-

The Lead Agency maintains a webpage specifically targeted to Child Care Providers including provider licensing and ICCP participation information here:

<https://healthandwelfare.idaho.gov/providers/child-care-providers/becoming-child-care-provider>

Additionally, child care providers also have a dedicated webpage with provider specific resources at <https://idahostars.org/> Also at the IdahoSTARS webpage providers may access subsidy enrollment requirements and additional resources for providers here:

<https://idahostars.org/Child-Care-Providers/ICCP> Providers are encouraged to work with locally operating Child Care Resource Centers as needed for any additional information.

9.3.2 Information about child care and other services available for parents

Does the Lead Agency certify that it provides information described in 9.3.1 for the following required programs?

- Temporary Assistance for Needy Families (TANF) program.
- Head Start and Early Head Start programs.
- Low Income Home Energy Assistance Program (LIHEAP)
- Supplemental Nutrition Assistance Program (SNAP).
- Women, Infants, and Children Program (WIC) program.
- Child and Adult Care Food Program (CACFP).
- Medicaid and Children's Health Insurance Program (CHIP).
- Programs carried out under IDEA Part B, Section 619 and Part C.

☒ Yes.

☐ No. If no, describe:

9.3.3 Consumer statement for parents receiving CCDF services

Lead Agencies must provide parents receiving CCDF services with a consumer statement in hard copy or electronically that contains general information about the CCDF program and specific information about the child care provider they select.

Please certify if the Lead Agency provides parents receiving CCDF services a consumer statement that contains the following 8 requirements:

1. Health and safety requirements met by the provider
2. Licensing or regulatory requirements met by the provider

3. Date the provider was last inspected
4. Any history of violations of these requirements
5. Any voluntary quality standards met by the provider
6. How CCDF subsidies are designed to promote equal access
7. How to submit a complaint through the hotline
8. How to contact a local resource and referral agency or other community-based organization to receive assistance in finding and enrolling in quality child care

Does the Lead Agency provide to families, either in hard copy or electronically, a consumer statement that contains the required information about the provider they have selected, including the eight required elements above?

☒ Yes.

☐ No. If no, describe:

9.3.4 Informing families about best practices on child development

Describe how the Lead Agency makes information available to parents, providers, and the general public on research and best practices concerning children's development, including physical health and development, and information about successful parent and family engagement. At a minimum, the description should include what information is provided; how the information is provided; any distinct activities for sharing this information with parents, providers, the general public; and any partners in providing this information. **For Parents**

- **Referral and Support:** Parents seeking child care referrals and information on child development can connect with a Child Care Health Consultant or Quality Consultant. These professionals offer resources and guidance on research and best practices.
- **Online Resources:** Parents have access to the IdahoSTARS website, which provides a wealth of information and connections to local organizations catering to children birth to school age.
- **Parent Education:** Our goal is to expand our current work to be more inclusive of families. Hosting or supporting providers in hosting parent education events focusing on topics like developmental screening and monitoring. These events aim to educate and empower parents as experts of their children and support education on key aspects of children's development. These events can be hosted at the local child care or through virtual events in connection with Developmental Screening using the ASQ 3 and ASQ SE 2.

For Providers

- **Training Opportunities:** Providers can participate in training, sessions focused on child development, physical health and development and other domains and subcategories through the online training system housed in our RISE Application, live training sessions, or virtual options.
- **Coaching and Technical Assistance:** Coaching and additional technical assistance is available in these categories which offers providers individualized or group coaching and training tied to specific goals of the provider. Coaching and Technical Assistance is provided on-site or virtual.
- **Local Resources:** Providers can access lending library resources from their local Child Care Resource and Referral (CCR&R) offices located regionally. The CCR&R offices also provide flyers, brochures, and handouts with valuable information tailored to specific communities served by each region. These resources are provided in home language as much as possible.

- Quality Consultants coach providers in the domains of child development, health safety and wellness, and partnerships with families and communities

For General Public

- **Website Resources:** The Parent Page of the IdahoSTARS website offer a range of family health and wellness resources. This includes general information aimed at promoting healthy environments at home and in child care settings.

9.3.5 Unlimited parental access to their children

Does the Lead Agency have procedures to ensure that parents have unlimited access to their children whenever their children are in the care of a provider who receives CCDF funds:

☒ Yes.

☐ No. If no, describe:

9.3.6 Informing families about best practices in social and emotional health

Describe how the Lead Agency shares information with families, providers, and the general public regarding the social-emotional and behavioral and mental health of young children, including positive behavioral intervention and support models based on research and best practices for those from birth to school age: **Parents**

Parents who contact the Child Care Resource and Referral office (CCR&R) seeking a referral for child care and who request information on children's mental health may be connected with their child care health consultant who can provide contacts for additional information and resources pertaining to local providers and organizations focusing on children's mental health. Parents may also access the IdahoSTARS Facebook page and the IdahoSTARS website for additional resources. Parents who contact their CCR&R may also receive information through flyers, brochures, and other handouts contained in the CCR&R resource packets. Resource packets are specific to the local communities and covers birth to school-age children.

Parents at programs in the Idaho Pyramid Collaborative At every tier of the Pyramid Model, programs and child care providers consider what strategies might be used to welcome and support diverse families, how to create opportunities to learn from families, strategies for partnering with families to promote child outcomes, and provide supports and services that families might need to promote their child's development. These strategies are evaluated using the program level Benchmarks of Quality which provides each program with key components to incorporate into their program level and classroom level ways to meet implementation fidelity. Some strategies are providing information directly to families on how to build strong relationships, teaching social and emotional skills at home, using the National Center for Pyramid Model Innovation (NCPMI) resources (The Backpack Series, Making Life Easier, Social Stories, Family Articles), asking parents for input into Program Wide Expectations, and partnering with families on classroom specific initiatives.

Providers

Training opportunities are available through IdahoSTARS to providers who seek to gain information on early childhood mental health. Idaho's QRIS, Steps to Quality, has embedded essential training which includes specific content on this topic area, and additional lending library

resources are available from their local CCR&Rs.

IdahoSTARS is a lead partner of the Idaho Pyramid Model Collaborative State Team working to implement positive behavioral intervention and supports to Idaho early childhood programs through the Pyramid Model framework. The Idaho Pyramid Model Collaborative provides training, coaching, and technical assistance supports to child care programs, Head Start programs, and school district IDEA Part B, Section 619 programs. Programs enrolled in the collaborative have access to over 18 hours of Live or self-paced online training on implementation of Pyramid Model practices which include Nurturing and Responsive Environments, High Quality Environments, Teaching Social and Emotional Skills, Teaching Problem Solving, Teaching Friendship Skills, and Tier 3 interventions (Prevent Teach Reinforce) for children exhibiting persistent challenging behavior in their first year. The second year of the road map includes training, coaching, and technical assistance including training on implicit bias and individualized supports (visual supports, peer mediated skills.) The final year on the Idaho Pyramid Model Professional Development road map, includes coaching, training, and technical assistance on trauma informed care and diversity, equity, inclusion topics, and provider wellness. At the Program level, program leaders and their leadership teams attend monthly program calls with IdahoSTARS to support the development of Systems, policies, and procedures to support the implementation fidelity of Pyramid Model practices in their programs including policies and procedures on how to respond to challenging behavior and suspension, expulsion, and exclusion. A statewide network of Pyramid Model coaches, which include IdahoSTARS Quality Consultants, are supported by a state program coach trained in behavioral intervention and support. Program practices are evaluated using the Teach Pyramid Observation Tool (TPOT) for Preschool classrooms, the Teaching Pyramid Infant and Toddler Observation Tool (TPITOS), which guides program coaching plans and assists in progress monitoring. Programs practices are evaluated using the Benchmarks of Quality (BoQ).

Providers can also become specialized in infant/toddler mental health through enhanced credentials.

- Child care Health Consultants are available statewide to assist providers with addressing questions/concerns that focus on early childhood mental health.

General public

The Parent pages of the IdahoSTARS website contains information on [Aim Early Idaho](http://www.aimearlyidaho.org) (www.aimearlyidaho.org) and on children with disabilities. The Provider pages includes the child care health consultant information with links to early childhood mental health. The IdahoSTARS Facebook contains posts with resources and information on early childhood mental health. CCR&R staff participate in community events and disseminate information/resources using brochures, flyers, and/or handouts.

9.3.7 Policies on the prevention of the suspension and expulsion of children

- a. The Lead Agency must have policies to prevent the suspension and expulsion of children from birth to age 5 in child care and other early childhood programs receiving CCDF funds. Describe those policies and how those policies are shared with families, providers, and the general public: **IdahoSTARS does promote education and training to child care providers on addressing and understanding challenging behaviors and has professional development opportunities as an option for providers inclusive of training**

and targeted technical assistance (TTA) on challenging behavior and implicit bias. Providers can engage in general training, coaching, and targeted technical assistance (TTA) on addressing and preventing challenging behavior, implicit bias, and developing social emotional competencies in children. Providers engaged in the Idaho Pyramid Collaborative that serve these populations will have enhanced training, coaching, and technical assistance that extends beyond classroom practices that support understanding of nurturing and responsive relationships with children, families, and co-workers, universal practices that support high quality environments, social and emotional competencies, and individualized supports for children with persistent challenging behaviors. The Framework also addresses program systems, policies, and procedures to explicitly address implicit bias, implement culturally responsive practices, use data systems to understand potential discipline equity issues and develop policies and procedures around challenging behavior and suspension and expulsion. IdahoSTARS also promotes and provides families with an outline of Steps to Choosing Safe and Healthy Child Care which encompasses 3 steps - Look, Check, Ask - pertaining to finding quality child care to meet their child's needs. A child care checklist, in both English and Spanish, is available and promoted when conducting child care referrals to parents, including which questions to ask when conducting interviews with child care providers. This checklist includes how to ask about and request documents that support positive, clear, consistent guidance practices.

- b. Describe what policies, if any, the Lead Agency has to prevent the suspension and expulsion of school-age children from child or youth care settings receiving CCDF funds: Providers that engage with IdahoSTARS and serve school-age children have access to training, coaching, and targeted technical assistance in addressing and preventing challenging behavior, implicit bias, and social emotional competencies. Providers engaged in the Idaho Pyramid Collaborative that serve these populations will have enhanced training, coaching, and technical assistance that extends beyond classroom practices that support understanding of nurturing and responsive relationships with children, families, and co-workers, universal practices that support high quality environments, social and emotional competencies, and individualized supports for children with persistent challenging behaviors. The Framework also addresses program systems, policies, and procedures to explicitly address implicit bias, implement culturally responsive practices, use data systems to understand potential discipline equity issues and develop policies and procedures around challenging behavior and suspension and expulsion.

9.4 Providing Information on Developmental Screenings

Lead Agencies must provide information on developmental screenings to parents as part of the intake process for families participating in CCDF and to child care providers through training and education. This information must include:

- Existing resources and services that the State can make available in conducting developmental screenings and providing referrals to services when appropriate for children who receive child care assistance, including the coordinated use of the Early and Periodic Screening, Diagnosis, and Treatment program under the Medicaid program carried out under Title XIX of the Social Security Act and developmental screening services available under IDEA Part B, Section 619 and Part C; and,

- A description of how a family or child care provider can use these resources and services to obtain developmental screenings for children who receive subsidies and who might be at risk of cognitive or other developmental delays, which can include social, emotional, physical, or linguistic delays.

Information on developmental screenings, as in other consumer education information, must be accessible for individuals with limited English proficiency and individuals with disabilities.

9.4.1 Developmental screenings

Does the Lead Agency collect and disseminate information on the following:

- Existing resources and services available for obtaining developmental screening for parents receiving CCDF, the general public, and child care providers.
☒ Yes.
☐ No. If no, describe:
- Early and Periodic Screening, Diagnosis, and Treatment program under the Medicaid program—carried out under Title XIX of the Social Security Act (42 U.S.C. 1396 et seq.)—and developmental screening services available under Part B, Section 619 and Part C of the Individuals with Disabilities Education Act (20 U.S.C. 1419, 1431 et seq.).
☒ Yes.
☐ No. If no, describe:
- Developmental screenings to parents receiving a subsidy as part of the intake process.
☒ Yes. If yes, include the information provided, ways it is provided, and any partners in this work: **All notices related to ICCP eligibility and participation include information for parents related to resources for accessing developmental screenings through partnered programs at IDHW.**

Child Care Providers are introduced to developmental screenings through the ICCP in Office Orientation as they decide to become an ICCP provider. IdahoSTARS has embedded an emerging indicator in our state quality and rating improvement system inclusive of the use of developmental monitoring and screening. Child care providers engaged with the Idaho Pyramid Model Collaborative begin using developmental screeners as a component of their roadmap in Year 2 or Year 3 of our three year roadmap. Providers additionally have access to training and Targeted Technical Assistance (TTA) associated with developmental screeners specifically the ASQ 3 and ASQ SE 2. General information on developmental screening is located on the IdahoSTARS website in the Basics of Child Development tab.

☐ No. If no, describe:
- How families receiving CCDF services or child care providers receiving CCDF can use the available resources and services to obtain developmental screenings for children at risk for cognitive or other developmental delays.
☒ Yes.
☐ No. If no, describe:

10 Program Integrity and Accountability

Program integrity and accountability activities are integral to the effective administration of the CCDF program. As stewards of federal funds, Lead Agencies must ensure strong and effective internal controls to prevent fraud and maintain continuity of services to meet the needs of children and families. In order to operate and maintain a strong CCDF program, regular evaluation of the program's internal controls as well as comprehensive training for all entities involved in the administration of the program are imperative. In this section, Lead Agencies will describe their internal controls and how those internal controls effectively ensure integrity and accountability. These accountability measures should address reducing fraud, waste, and abuse, including program violations and administrative errors and should apply to all CCDF funds.

10.1 Effective Internal Controls

Lead Agencies must ensure the integrity of the use of CCDF funds through effective fiscal management and must ensure that financial practices are in place. Lead Agencies must have effective fiscal management practices in place for all CCDF expenditures.

10.1.1 Organizational structure to support integrity and internal controls

Describe how the Lead Agency's organizational structure ensures the oversight and implementation of effective internal controls that promote and support program integrity and accountability. Describe: **1. Assignment of authority and responsibilities related to program integrity**

In Idaho, the CCDF Program is administered by the CCDF Administrator known locally as the Child Care Program Manager. CCDF is administered within the Division of Self Reliance as part of the Idaho Department of Health and Welfare (IDHW). Within IDHW, the CCDF administrator works with other divisions, bureaus, and programs to ensure the program is ran with effective internal controls with specific attention to program integrity. IDHW's organizational structure is designed to ensure rigorous oversight and implementation of internal controls that bolster program integrity and accountability.

2. and 3. Delegation of duties and coordination of activities

Duties are delegated to staff based on their job title and essential job functions. Staff are trained on items such as Confidentiality, Program Integrity, HIPAA policies, grant administration and security training to ensure and promote program integrity. The CCDF administrator meets routinely and as needed with the managers supervising and the staff maintaining the following efforts:

Self Reliance Operations- overseeing the Customer Support Staff and Self Reliance Specialists conducting subsidy eligibility determinations.

Self Reliance Integrity Unit- The Idaho Department of Health and Welfare's Division of Welfare houses a program integrity unit. The Self-Reliance Integrity Unit (SRIU) staff are responsible for program integrity and completion of all aspects of case work. They review programs including Medicaid, Supplemental Nutrition Assistance Program, Idaho Child Care Program, Temporary Assistance in Idaho, and Aid to the Aged, Blind and Disabled. They must review of all facets of casework to include interviewing, data collection, fact gathering, case set up, error detection, case correction, processing eligibility, processing overpayments, review all actions for intent to receive benefits without entitlement, and completion of all fraud actions. Fraud actions include

completing intentional program violations, hearings, overpayments, and preparing cases for prosecution. All actions must align to state and federal guidelines.

Self Reliance Resolutions Unit- managing client and provider overpayments, fair hearings and appeals, escalations, and public records requests.

Bureau of Compliance and Support including CERM (Contracts and External Resource Management) Team- This team monitors administrative implementation and responsibilities and ensures that the standards, techniques, and criteria identified in the contract's scope of work are consistent and timely. This team also reviews and processes the division's invoices.

Division of Management Services- provides administrative services to support the department. The division oversees units such as facilities, criminal history background checks, internal audit, and investigation services.

Division of Financial Services- ensures adequate cash is available for the department to meet its financial obligations and functions as the financial liaison to human services programs by drawing federal funds from the U S Treasury to meet immediate cash needs of federally funded programs; Requesting state general and dedicated funds through the Office of the State Controller; Preparing expenditure reports; Preparing and submitting the department's annual budget request to the Division of Financial Management and Legislative Services; Distributing appropriated funding to operating budgets within the department; Monitoring program expenditure trends to allocated funding; Preparing various financial analysis and reporting for division and executive management; Monitoring established positions; Researching and compiling historical expenditure and revenue information.

4. Communication between fiscal and program staff.

All invoices are reviewed for allowable uses and contract compliance prior to payment. Program and Fiscal staff meet each month to review expenditures and monitor budgets. The Idaho Department of Health and Welfare's finance department tracks expenditures to specific accounts and funding sources. Monthly reports are provided to divisions for review as well as quarterly budget meetings take place to review budgets.

5. Segregation of duties.

All contracted entities submit monthly expenditures for which the Idaho Department of Health and Welfare's Contracts and External Resource Management (CERM) reviews for accuracy and then submits to the fiscal department for processing, approval, and payment. Contract monitoring tools are developed in coordination with the program, but monitoring is conducted independently by CERM team staff. Findings are submitted to the Child Care Program Manager for review and approval prior to submission to the contractor. If any items were found deficient, the monitor team meets with the contract manager to discuss possible adverse conditions and determine the severity of each. Developing and organizing deficiencies is critical to the process. It will assist in determining whether all pertinent information was obtained during the monitor and facilitate discussions and decisions related to the potential findings. Each finding is supported with specific examples and concrete details. The CERM team submits final results to contractor, contract manager, Division of Welfare administrator, and bureau chief over Compliance and Support. If there were findings, the contractor will be given a date by which to submit reimbursement, performance improvement, or corrective action plan.

6. Establishment of checks and balances to identify potential fraud risks.

Our review process is set up to find any areas in which our performance is at risk, pinpoint the area of concern (whether policy, process, or training), and quickly respond to the field needs

either at the local level or roll out statewide based on the findings. For each processing center/field office reviewed, the case reviews incorporate initial contact with the Idaho Department of Health and Welfare through approval/denial as well as acting on changes. We review program access, integrity, system keying standards, and maintenance. Our reviews also include customer service standards and timeliness. Our review profile captures error causes and we have reports which help us to identify any error trends. Any errors discovered are sent for immediate correction. Errors are shared with the field program managers for individual performance management as well as used for identifying statewide trends. The bureau chiefs for operations and operations design identify areas in which we can provide statewide training when necessary which supports our statewide business model.

Include the following elements in your description:

1. Assignment of authority and responsibilities related to program integrity.
2. Delegation of duties.
3. Coordination of activities.
4. Communication between fiscal and program staff.
5. Segregation of duties.
6. Establishment of checks and balances to identify potential fraud risks.
7. Other activities that support program integrity.

10.1.2 Fiscal management practices

Describe how the Lead Agency ensures effective fiscal management practices for all CCDF expenditures, including:

- a. Fiscal oversight of CCDF funds, including grants and contracts. Describe: **The Idaho Department of Health and Welfare (DHW) has real time verification standards embedded into initial eligibility processes. In real time, self-reliance specialists can use our various interfaces to verify information necessary to determine eligibility and begin services. Monthly dependent care forms are submitted, outlining payment billed amounts completed in partnership with parents and signed by parents and providers, verifying the amounts articulated are verified by the parent and provider. The form is then submitted to DHW for payment. Each year, all providers sign a provider agreement for child care which outlines the requirements for participating in the program including appropriate record keeping.**

The Lead Agency maintains overall control of expenditures by monitoring performance-based contracts for compliance with federal regulations and the scope of work for all written contracts and agreements. This includes, but is not limited to, contracts for IdahoSTARS and the Health Districts. Contracts contain language that requires the contractor to certify that federal and state guidelines are followed. In addition to on-site record reviews conducted by the CERM team, contractors and Grantees are required to submit semi-annual performance reports on specific performance indicators. Additionally, all contracted entities submit monthly expenditures that are reviewed for accuracy, and

contractual compliance prior to submission to the fiscal division for processing, approval, and payment.

- b. Tracking systems that ensure reasonable and allowable costs and allow for tracing of funds to a level of expenditure adequate to establish that such funds have not been used in violation of the provision of this part. Describe: **The Idaho Department of Health and Welfare's finance department tracks expenditures to specific accounts and funding sources. Monthly reports are provided to divisions for review as well as quarterly budget meetings take place to review budgets.**
- c. Processes and procedures to prepare and submit required state and federal fiscal reporting. Describe: **The ACF-696 report is completed quarterly by the Grant Reporter within the Division of Financial Services using financial information compiled in our statewide financial management system, Luma, as well as the reporting amounts using the Child Care & Development Block Grant (CCDBG) Quarterly Workpapers. These workpapers are created using a report from Fiscal and are tied to Fiscal each month to ensure accuracy. The Grant Reporter tracks the match and MOE requirements based upon ACF annual requirements posted online. The Quarterly Workpapers are broken out by their sources, which are the Mandatory Fund, the Matching Fund and the Discretionary Fund, which includes TANF Transfer (Grant 1262TF). This aids the Lead Agency in meeting their multiple compliance requirements. The majority of the worksheets are pre-formulated and automated to prevent inaccurate information from being entered. The Child Care Program Manager provides secondary a review of the work papers each month and there is a double check at the bottom of the spreadsheet to confirm there are no variances.**

The report is then routed for review and approval to the Cash Analyst, Budget Analyst, and Financial Manager, all staff in the Division of Financial Services under the leadership of the Lead Agency's Chief Financial Officer. Review of the report includes comparing reported data to supporting records in the grant folder. The Unit Supervisor will also review prior quarter and year-to-date amounts to verify that amounts are reasonable. Documentation of the persons preparing, reviewing, and approving the ACF-696 are done by initialing and dating the routing slip. The "Report Electronically Filed" sign-off on the routing slip is not the date the report was filed; it is the date data entry was done. The Financial Executive Officer does a final review of the report to the supporting excel file CCDBG, the support in the report folder, and additional notes in the report folder, and if there are no errors noted, signs and submits the report.

- d. Other. Describe:

10.1.3 Effectiveness of fiscal management practices

Describe how the Lead Agency knows there are effective fiscal management practices in place for all CCDF expenditures, including:

- a. How the Lead Agency defines effective fiscal management practices. Describe: **Effective fiscal management practices involve regular monitoring, transparent accounting procedures, and adherence to federal and state guidelines to ensure proper allocation and use of CCDF funds. This includes detailed monthly reviews, rigorous expenditure tracking, and multiple controls for payouts, supported by advanced fiscal projection tools.**
- b. How the Lead Agency measures and tracks results of their fiscal management practices.

Describe: The Idaho Child Care Program measures the effectiveness of fiscal management through performance indicators such as expenditure rates, compliance with budget allocations, and audit results. Automated resources such as expenditure tracking systems and monthly financial reporting are utilized, in addition to frequent staff and leadership review of reports, and on-site monitoring of contracted partners measures and tracks indicators consistently. All audit results are reviewed at each level of leadership within the Division of Self Reliance and the Division of Management Services to develop improvements based on all results.

- c. How the results inform implementation. Describe: Both the Statewide Single Audit and the Internal Audit teams publish findings resulting from their audits. These findings indicate what caused the error, and each finding requires a management response in the form of a corrective action plan. Results of monitoring or auditing are shared with program staff and department leadership and used to impact staff training and organizational decisions. At the grant and program level, similar informal procedures are performed to correct issues that are discovered as part of the review process.
- d. Other. Describe: NA

10.1.4 Identifying risk

Describe the processes the Lead Agency uses to identify risk in the CCDF program including:

- a. Each process used by the Lead Agency to identify risk (including entities responsible for implementing each process). Describe: For subsidy payments, Supervisory reviews are completed annually. Our review process is set up to find any areas in which our performance is at risk, pinpoint the area of concern (whether policy, process, or training), and quickly respond to the field needs either at the local level or roll out statewide based on the findings. For each processing center/field office reviewed, the case reviews incorporate initial contact with the Idaho Department of Health and Welfare through approval/denial as well as acting on changes. We review program access, integrity, system keying standards, and maintenance. Our reviews also include customer service standards and timeliness. Additionally, all eligibility staff are able to submit any subsidy case or provider for additional review by the Self Reliance Integrity Unit if concerns are identified.

All contracts contain language that requires the contractor to certify that federal and state guidelines are followed. In addition to on-site record reviews conducted by the CERM team, contractors and Grantees are required to submit semi-annual performance reports on specific performance indicators. Additionally, all contracted entities submit monthly expenditures that are reviewed for accuracy, and contractual compliance prior to submission to the fiscal division for processing, approval, and payment. Contract monitoring tools are developed in coordination with the program, but monitoring is conducted independently by CERM team staff. Findings are submitted to the Child Care Program Manager for review and approval prior to submission to the contractor. If any items were found deficient, the monitor team meets with the contract manager to discuss possible adverse conditions and determine the severity of each. Any finding is supported with specific examples and concrete details. The CERM team submits final results to contractor, contract manager, Division of Welfare administrator, and bureau chief over Compliance and Support. If there were findings, the contractor is given a date by which to submit reimbursement, performance improvement, or corrective action plan.

- b. The frequency of each risk assessment. Describe: Risk assessment is completed on an ongoing basis. Daily evaluation occurs in eligibility determination. Ongoing supervisory quality control reviews are conducted on subsidy cases as well.

A thorough risk assessment is completed prior to awarding any subgrant or contract and upon each annual renewal.

All child care providers complete an updated Provider Agreement annually validating that they are not on the Office of Inspector General's Exclusion List.

The Lead Agency conducts the triennial improper payments review in addition to frequent legislative audits of all levels of programming.

- c. How the Lead Agency uses risk assessment results to inform program improvement. Describe: Our review profile captures error causes, and we have reports which help us to identify any error trends. Any errors discovered are sent for immediate correction. Errors are shared with the field program managers for individual performance management as well as used for identifying statewide trends. The bureau chiefs for operations and operations design identify areas in which we can provide statewide training when necessary, which supports our statewide business model. For contracts and subgrants, the Lead Agency requires risk assessments to evaluate potential subrecipients' capacity to comply with federal and state regulations and the terms of the subaward. The aim is to identify the appropriate allocation of monitoring resources towards subrecipients based on their risk of noncompliance with Federal statutes, regulations, and the terms and conditions of specific subawards. This process is intended to optimize resource allocation, minimize waste, and enhance decision-making in the monitoring of subrecipients. Programs document assessment results and decisions regarding monitoring activities for each subrecipient. These records must be signed and dated by monitoring staff, supervisors or managers, and designated compliance officers. Clear communication of results to all relevant parties is essential to ensure understanding and alignment on planned monitoring activities.

- d. How the Lead Agency knows that the risk assessment processes utilized are effective. Describe: The Lead Agency requires all subgranting and contracting programs to develop a monitoring plan that covers every subrecipient. Considerations such as staffing resources and the geographic distribution of subrecipients across the State, guide the development of monitoring activities. Adequate oversight is crucial for maintaining performance standards, particularly amid staffing changes and other influencing factors. Early issue resolution can preempt problems and minimize the need for extensive technical assistance later on.

The Lead Agency maintains overall control of expenditures by monitoring performance-based contracts for compliance with federal regulations and the scope of work for all written contracts and agreements. This includes, but is not limited to, contracts for IdahoSTARS and the Health Districts. Contracts contain language that requires the

contractor to certify that federal and state guidelines are followed. In addition to on-site record reviews conducted by the CERM team, contractors and Grantees are required to submit semi-annual performance reports on specific performance indicators. Additionally, all contracted entities submit monthly expenditures that are reviewed for accuracy, and contractual compliance prior to submission to the fiscal division for processing, approval, and payment. The results of the submitted reports and on-site monitoring evaluation continue to demonstrate our program's success with minimal findings, expeditious resolutions, and compliance rates exceeding the contractual requirements.

The Idaho Child Care Program has received successful results in triannual error rate reviews, annual Legislative audits, and semi-annual monitoring of CCDF funded contracts indicates successful program implementation and risk management.

e. Other. Describe: **NA**

10.1.5 Processes to train about CCDF requirements and program integrity

Describe the processes the Lead Agency uses to train staff of the Lead Agency and other agencies engaged in the administration of CCDF, and child care providers about program requirements and integrity.

- a. Describe how the Lead Agency ensures that all staff who administer the CCDF program (including through MOUs, grants, and contracts) are informed and trained regarding program requirements and integrity.
 - i. Describe the training provided to staff members around CCDF program requirements and program integrity: **The Idaho Department of Health and Welfare's Division of Self Reliance trains all self-reliance specialists on each program administered within the division. Training includes videos on program rules, policies, general program details, process documents, helpful tools, release notices, and monthly management reviews.**
 - ii. Describe how staff training is evaluated for effectiveness: **The Bureau Chief over Operations monitors various reports on a daily, weekly, and monthly basis. Program managers receive reports on their region to identify and review results and create training opportunities in areas of the business requiring additional assistance to meet targets. Monthly supervisor reviews aid in the identification and prevention of agency errors because it allows for a review of more eligibility cases and training where needed.**
 - iii. Describe how the Lead Agency uses program integrity data (e.g., error rate results, risk assessment data) to inform ongoing staff training needs: **Idaho uses program integrity data to identify risk areas for training and or policy clarifications. Idaho conducts error rate reviews in real time allowing any potential errors to immediately become learning and training opportunities for staff to learn from supervisors and case reviewers. When not conducting case reviews for the error rate, staff receive regular ongoing training on any policy or rule changes made to the program and materials are updated on a regular basis using feedback from partners to improve process documents and policy guidance.**

- b. Describe how the Lead Agency ensures all providers for children receiving CCDF funds are informed and trained regarding CCDF program requirements and program integrity:
 - i. Describe the training for providers around CCDF program requirements and program integrity: **Every provider who starts a child care business that accepts CCDF funds receives an Idaho Child Care Program (ICCP) orientation with a CCRC Resource Specialist. The provider is provided slides and a provider guidebook with a follow-up visit after they have completed the process and has received a vendor number. The content of the ICCP orientation, supporting materials, and follow-up contact informs the provider of ICCP rules, required processes and support with implementing the requirements. Every new provider is introduced to their local CCRC staff, their customer support team, the 2-1-1 Idaho Careline, and the IdahoSTARS website. Local and state resources are provided and opportunities for professional development are shared and encouraged. The Idaho Child Care Program (ICCP) notifies providers of policy changes through required annually updated provider agreements, email and mailed policy notifications, IdahoSTARS newsletters, and pop-ups in the RISE database system.**
 - ii. Describe how provider training is evaluated for effectiveness: **Provider training is evaluated through audit outcomes, training requests, and based on informally extracting recurrent themes in informal and formal technical assistance provided to child care providers within their regions.**
 - iii. Describe how the Lead Agency uses program integrity data (e.g., error rate results, risk assessment data) to inform ongoing provider training needs: **Staff from the Self Reliance Integrity Unit and Self Reliance Fair Hearing Unit meet regularly with Child Care Program staff to review data, challenging cases, policy questions, and process needs for all child care cases. Any consistent findings associated to providers are reviewed and shared with IdahoSTARS leadership for referral for technical assistance or updated trainings. Additionally, consistent findings can be used to inform annual updates to the Provider Agreement for improved clarity and compliance.**

10.1.6 Evaluate internal control activities

Describe how the Lead Agency uses the following to regularly evaluate the effectiveness of Lead Agency internal control activities for all CCDF expenditures.

- a. Error rate review triennial report results (if applicable). Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **Idaho conducts error rate reviews in real time allowing any potential errors to immediately become learning and training opportunities for staff to learn from supervisors and case reviewers. ACF-404 results are shared with division leadership, child care program staff, and quality control team and Lead Agency staff may use error rate results if a pattern is identified concerning incorrect eligibility decisions and is promptly addressed through training or policy updates. Final results are shared with legislative auditors when requested and inform staff training, organizational decisions, policy development and ongoing refinements.**
- b. Audit results. Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **Idaho's Legislative**

Services Office conducts routine audits of IDHW programs including this Idaho Child Care Program. Audits are either fiscal or performance in nature during which state auditors review the Lead Agency's policies, procedures, fiscal records, systems and databases, and relevant documents to determine compliance with state and federal requirements. Audits provide the Lead Agency with regular outside analysis of program operations and potential weaknesses in the system. Audit outcomes are made publicly available, presented to the legislature annually, and shared with leadership from the Divisions of Self Reliance and Management Services, and child care program staff. Lead Agency staff work to promptly address any findings through staff training, policy updates, and/or system improvements.

- c. Other. Describe who this information is shared with and how the Lead Agency uses the information to evaluate the effectiveness of its internal controls: **NA**

10.1.7 Identified weaknesses in internal controls

Has the Lead Agency or other entity identified any weaknesses in its internal controls?

- a. ☒ No. If no, describe when and how it was most recently determined that there were no weaknesses in the Lead Agency's internal controls. **The Lead Agency routinely works to improve internal controls via data management, process improvements, and consistent training. The child care program participates in annual audits conducted by Legislative Services Office, any potential findings are promptly addressed and resolved. At this time there are no concerns with internal controls.**
- b. ☐ Yes. If yes, what were the indicators? How did you use the information to strengthen your internal controls?

10.2 Fraud Investigation, Payment Recovery, and Sanctions

Lead Agencies must have the necessary controls to identify fraud and other program violations to ensure program integrity. Program violations can include both intentional and unintentional client and/or provider violations, as defined by the Lead Agency. These violations and errors, identified through the error-rate review process and other review processes, may result in payment or nonpayment (administrative) errors and may or may not be the result of fraud, based on the Lead Agency definition.

10.2.1 Strategies used to identify and prevent program violations

Check the activities the Lead Agency employs to ensure program integrity, and for each checked activity, identify what type of program violations the activity addresses, describe the activity and the results of these activities based on the most recent analysis.

- a. ☒ Share/match data from other programs (e.g., TANF program, Child and Adult Care Food Program, Food and Nutrition Service (FNS), Medicaid) or other databases (e.g., State Directory of New Hires, Social Security Administration, Public Assistance Reporting Information System (PARIS)).
 - i. ☒ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Idaho currently shares data with the between the following programs: Supplemental Nutrition Assistance Program (SNAP), Temporary Assistance for Needy Families (TANF), Aid to the Aged, Blind,**

and Disabled (AABD), and Medicaid as all of these programs share the same eligibility system.

The sharing and matching of data with other programs helps prevent intentional program violations because it can show information that is required to be reported for the Child Care program but may have not been reported, although ICCP only acts on information from these other programs if the recipient is required to report it or if it will benefit the household. The results of this activity also reduce the risk of incorrect eligibility determinations.

The Idaho Department of Health and Welfare (DHW) has an integrity unit who monitors PARIS matches monthly. The report is SNAP related and used across all programs. The report reflects households who may be receiving benefits in more than one state. This information is reviewed and if an IPV has been committed, a waiver of disqualification hearing is sent to the customer. If not signed, an administrative hearing is requested. If an Intentional Program Violation (IPV) was acknowledged, the party who committed the IPV loses eligibility for ICCP and an overpayment may be processed. If fraud is detected, it is routed to a Fraud Investigator to review and determine the proper course of action.

Program staff meet with staff from the Self Reliance Resolutions Unit and Self Reliance Integrity Unit to review the outcomes of data sharing and reports and routinely monitors for patterns and errors to learn about opportunities to improve processes and policies to prevent negative client or provider outcomes such as IPV's, unintentional program violations, and agency errors.

- ii. **[x] Unintentional program violations.** Describe the activities, the results of these activities, and how they inform better practice: **Idaho currently shares data with the between the following programs: Supplemental Nutrition Assistance Program (SNAP), Temporary Assistance for Needy Families (TANF), Aid to the Aged, Blind, and Disabled (AABD), and Medicaid as all of these programs share the same eligibility system.**

The sharing and matching of data with other programs helps prevent unintentional program violations because it can show information that is required to be reported for the Child Care program but may have not been reported, although ICCP only acts on information from these other programs if the recipient is required to report it or if it will benefit the household. The results of this activity also reduce the risk of incorrect eligibility determinations.

The Idaho Department of Health and Welfare (DHW) has an integrity unit who monitors PARIS matches monthly. The report is SNAP related and used across all programs. The report reflects households who may be receiving benefits in more than one state.

If an unintentional program violation is determined, an overpayment is processed.

Program staff meet with staff from the Self Reliance Resolutions Unit and Self Reliance Integrity Unit to review the outcomes of data sharing and reports and routinely monitors for patterns and errors to learn about opportunities to improve processes and policies to prevent negative client or provider outcomes such as IPV's, unintentional program violations, and agency errors.

- iii. ☒ Agency errors. Describe the activities, the results of these activities, and how they inform better practice:: **Idaho currently shares data with the between the following programs: Supplemental Nutrition Assistance Program (SNAP), Temporary Assistance for Needy Families (TANF), Aid to the Aged, Blind, and Disabled (AABD), and Medicaid as all of these programs share the same eligibility system.**

The sharing and matching of data with other programs helps prevent agency errors because it can show information that is required to be reported for the Child Care program but may have not been reported, although ICCP only acts on information from these other programs if the recipient is required to report it or if it will benefit the household. The results of this activity also reduce the risk of incorrect eligibility determinations.

The Idaho Department of Health and Welfare (DHW) has an integrity unit who monitors PARIS matches monthly. The report is SNAP related and used across all programs. The report reflects households who may be receiving benefits in more than one state.

If an agency error is determined, an overpayment or supplement is processed.

Program staff meet with staff from the Self Reliance Resolutions Unit and Self Reliance Integrity Unit to review the outcomes of data sharing and reports and routinely monitors for patterns and errors to learn about opportunities to improve processes and policies to prevent negative client or provider outcomes such as IPV's, unintentional program violations, and agency errors.

- b. ☒ Run system reports that flag errors (include types).
 - i. ☐ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice:
 - ii. ☒ Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Our automated eligibility system flags missing or conflicting data that may contribute to unintentional program violations highlighting it to eligibility workers, and can prevent the processing of eligibility without required eligibility data. Additionally, the Bureau Chief over operations monitors various reports on a daily, weekly, and monthly basis. Information obtained is provided to program managers relating to their region. Additionally, the information identified assists in training opportunities and identifying gaps. These automated eligibility system components and report reviews, combined with trained, skilled eligibility staff prevents many**

unintentional program violations.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Our automated eligibility system flags missing or conflicting data that may contribute to unintentional program violations highlighting it to eligibility workers, and can prevent the processing of eligibility without required eligibility data. Additionally, the Bureau Chief over operations monitors various reports on a daily, weekly, and monthly basis. Information obtained is provided to program managers relating to their region. Additionally, the information identified assists in training opportunities and identifying gaps. These automated eligibility system components and report reviews, combined with trained, skilled eligibility staff prevents many agency errors.**

- c. **[x]** Review enrollment documents and attendance or billing records.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **IDHW Integrity Unit and Child Care staff may request enrollment documents, attendance records, and billing records at any time, and often do to review and identify potentially inaccurate provider billing practices. Any resulting overpayment is established using these records, and they are used to identify potential fraud. The review of attendance and billing records is an ongoing practice allowing IDHW to measure discrepancies and validate billing. Audits and investigations are conducted in response to provider concerns, when possible issues are identified by IDHW staff, and based on complaints reported related to cases of suspected fraud or billing inaccuracies.**

As a result of reviewing enrollment documents and attendance and billing records staff are able to accurately attribute overpayments, IPVs, UPVs, and agency errors. Additionally, staff review of these documents and information has lead to document improvements, and improvement in provider trainings and technical assistance to prevent additional negative client or provider outcomes such as IPV's, unintentional program violations, and agency errors.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **IDHW Integrity Unit and Child Care staff may request enrollment documents, attendance records, and billing records at any time, and often do to review and identify potentially inaccurate provider billing practices. Any resulting overpayment is established using these records, and they are used to identify potential fraud. The review of attendance and billing records is an ongoing practice allowing IDHW to measure discrepancies and validate billing. Audits and investigations are conducted in response to provider concerns, when possible issues are identified by IDHW staff, and based on complaints reported related to cases of suspected fraud or billing inaccuracies.**

As a result of reviewing enrollment documents and attendance and billing records staff are able to accurately attribute overpayments, IPVs, UPVs, and agency errors. Additionally, staff review of these documents and information has lead to document improvements, and improvement in provider trainings and technical assistance to prevent additional negative client or provider outcomes such as IPV's, unintentional program violations, and agency errors.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **IDHW Integrity Unit and Child Care staff may request enrollment documents, attendance records, and billing records at any time, and often do to review and identify potentially inaccurate provider billing practices. Any resulting overpayment is established using these records, and they are used to identify potential fraud. The review of attendance and billing records is an ongoing practice allowing IDHW to measure discrepancies and validate billing. Audits and investigations are conducted in response to provider concerns, when possible issues are identified by IDHW staff, and based on complaints reported related to cases of suspected fraud or billing inaccuracies.**

As a result of reviewing enrollment documents and attendance and billing records staff are able to accurately attribute overpayments, IPV's, UPV's, and agency errors. Additionally, staff review of these documents and information has lead to document improvements, and improvement in provider trainings and technical assistance to prevent additional negative client or provider outcomes such as IPV's, unintentional program violations, and agency errors.

- d. **[x]** Conduct supervisory staff reviews or quality assurance reviews.
- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Our review process is set up to find any areas in which our performance is at risk, pinpoint the area of concern (policy, process, or training) , and quickly respond to the field needs either at the local level or roll out statewide based on the findings. For each processing center/field office reviewed, the case reviews incorporate initial contact with the Idaho Department of Health and Welfare (DHW) through approval/denial as well as acting on changes. We review program access, integrity, system keying standards, and maintenance. Our reviews also include customer service standards and timeliness. Our review profile captures error causes and we have reports which help us to identify any error trends. Any errors discovered are sent for immediate correction. Errors are shared with the field program managers for individual performance management as well as used for identifying statewide trends. The bureau chiefs for Operations and Operations Design identify areas in which we can provide statewide training when necessary which supports our statewide business model.**
- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **Our review process is set up to find any areas in which our performance is at risk, pinpoint the area of concern (policy, process, or training) , and quickly respond to the field needs either at the local level or roll out statewide based on the findings. For each processing center/field office reviewed, the case reviews incorporate initial contact with the Idaho Department of Health and Welfare (DHW) through approval/denial as well as acting on changes. We review program access, integrity, system keying standards, and maintenance. Our reviews also include customer service standards and timeliness. Our review profile captures error causes and we have reports which help us to identify any error trends. Any errors discovered are sent for immediate correction. Errors are shared with the field program managers for**

individual performance management as well as used for identifying statewide trends. The bureau chiefs for Operations and Operations Design identify areas in which we can provide statewide training when necessary which supports our statewide business model.

- iii. **[x]** Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **Our review process is set up to find any areas in which our performance is at risk, pinpoint the area of concern (policy, process, or training) , and quickly respond to the field needs either at the local level or roll out statewide based on the findings. For each processing center/field office reviewed, the case reviews incorporate initial contact with the Idaho Department of Health and Welfare (DHW) through approval/denial as well as acting on changes. We review program access, integrity, system keying standards, and maintenance. Our reviews also include customer service standards and timeliness. Our review profile captures error causes and we have reports which help us to identify any error trends. Any errors discovered are sent for immediate correction. Errors are shared with the field program managers for individual performance management as well as used for identifying statewide trends. The bureau chiefs for Operations and Operations Design identify areas in which we can provide statewide training when necessary which supports our statewide business model.**

e. **[x]** Audit provider records.

- i. **[x]** Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **IDHW Integrity Unit and Child Care staff may request enrollment documents, attendance records, and billing records at any time, and often do to review and identify potentially inaccurate provider billing practices. Any resulting overpayment is established using these records, and they are used to identify potential fraud. The review of attendance and billing records is an ongoing practice allowing IDHW to measure discrepancies and validate billing. Audits and investigations are conducted in response to provider concerns, when possible issues are identified by IDHW staff, and based on complaints reported related to cases of suspected fraud or billing inaccuracies.**

Any audits resulting in findings are reviewed by the Program Manager, division leadership, and have been requested by legislative auditors. Patterns identified in audit findings have been the impetus for agency document and process improvements, and improvement in provider trainings and technical assistance to prevent ongoing audit findings.

- ii. **[x]** Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **IDHW Integrity Unit and Child Care staff may request enrollment documents, attendance records, and billing records at any time, and often do to review and identify potentially inaccurate provider billing practices. Any resulting overpayment is established using these records, and they are used to identify potential fraud. The review of attendance and billing records is an ongoing practice allowing IDHW to measure discrepancies and validate billing. Audits and investigations are conducted in response to provider concerns, when possible issues are identified by IDHW staff, and based on complaints reported related to cases of suspected fraud or billing inaccuracies.**

Any audits resulting in findings are reviewed by the Program Manager, division leadership, and have been requested by legislative auditors. Patterns identified in audit findings have been the impetus for agency document and process improvements, and improvement in provider trainings and technical assistance to prevent ongoing audit findings.

- iii. ☒ Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **IDHW Integrity Unit and Child Care staff may request enrollment documents, attendance records, and billing records at any time, and often do to review and identify potentially inaccurate provider billing practices. Any resulting overpayment is established using these records, and they are used to identify potential fraud. The review of attendance and billing records is an ongoing practice allowing IDHW to measure discrepancies and validate billing. Audits and investigations are conducted in response to provider concerns, when possible issues are identified by IDHW staff, and based on complaints reported related to cases of suspected fraud or billing inaccuracies.**

Any audits resulting in findings are reviewed by the Program Manager, division leadership, and have been requested by legislative auditors. Patterns identified in audit findings have been the impetus for agency document and process improvements, and improvement in provider trainings and technical assistance to prevent ongoing audit findings.

- f. ☒ Train staff on policy and/or audits.
 - i. ☒ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Idaho Department of Health and Welfare's Division of Self Reliance trains all self-reliance specialists on each program administered within the division. Training includes videos on program rules, policies, general program details, process documents, helpful tools, release notices, and monthly management reviews.**
 - ii. ☒ Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice: **The Idaho Department of Health and Welfare's Division of Self Reliance trains all self-reliance specialists on each program administered within the division. Training includes videos on program rules, policies, general program details, process documents, helpful tools, release notices, and monthly management reviews.**
 - iii. ☒ Agency errors. Describe the activities, the results of these activities, and how they inform better practice: **The Idaho Department of Health and Welfare's Division of Self Reliance trains all self-reliance specialists on each program administered within the division. Training includes videos on program rules, policies, general program details, process documents, helpful tools, release notices, and monthly management reviews.**
- g. ☐ Other. Describe the activity(ies):
 - i. ☐ Intentional program violations. Describe the activities, the results of these activities, and how they inform better practice:

- ii. ☐ Unintentional program violations. Describe the activities, the results of these activities, and how they inform better practice:
- iii. ☐ Agency errors. Describe the activities, the results of these activities, and how they inform better practice:

10.2.2 Identification and recovery of misspent funds

Lead Agencies must identify and recover misspent funds that are a result of fraud, and they have the option to recover any misspent funds that are a result of unintentional program violations or agency errors.

- a. Identify which agency is responsible for pursuing fraud and overpayments (e.g., State Office of the Inspector General, State Attorney): **The Idaho Department of Health and Welfare in partnership with the State Office of Inspector General are responsible for the pursuit of fraud and overpayments.**
- b. Check and describe all activities, including the results of such activity, that the Lead Agency uses to investigate and recover improper payments due to fraud. Consider in your response potential fraud committed by providers, clients, staff, vendors, and contractors. Include in the description how each activity assists in the investigation and recovery of improper payment due to fraud or intentional program violations. Activities can include, but are not limited to, the following:
 - i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The Department is required to pursue all overpayments when they reach the threshold of \$100 dollars. If a case has been determined fraud and an improper payment was made, the Department would apply the overpayment as well as possibly take action to disqualify the provider or family from the program, as referenced in IDAPA section 700 of 16.06.12. The result of setting a minimum overpayment threshold helps to ensure that the agency is not spending more money on administration costs to collect an overpayment than what the overpayment collection would yield, while still holding participants accountable for the responsible use of the funds.**
 - ii. ☐ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis:
 - iii. ☒ Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **Financial services works to establish repayment plans with identified families or providers. To assist in the recovery of improper payments due to fraud or intentional program violations, a process for payment is established and signed between both parties. The result of recovering funds through repayment plans helps to ensure that participants are accountable for full repayment and the responsible use of the funds while limiting the financial burden of repayment and potentially maintaining eligibility for needed programs or provider participation.**
 - iv. ☐ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis:

- v. ☒ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis: **Financial Services works with the state controller's office to comply when individuals or providers have overpayments that necessitate collection through intercepts after other means of collection have been unsuccessful. As a result of recovering funds through tax intercepts the child care program demonstrates ongoing fiscal accountability and maintains program sustainability.**
 - vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
 - vii. ☒ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis: **Allegations of fraud are forwarded to an Idaho Department of Health and Welfare (DHW)'s Integrity Unit that specializes in the review of child care payments, if fraud is discovered the case is forwarded to the Idaho Department of Health and Welfare (IDHW) maintained unit for welfare fraud investigations. Trained investigators actively pursue allegations of program abuse by clients and child care providers. Once their findings are complete, the Revenue Operations unit at DHW actively pursues the collection of various outstanding monies owed to DHW. They pursue the recovery of program benefit overpayments whether stemming from intentional program violations on the part of DHW clientele or from simple human error. To assist in the recovery of payments, the Revenue Operations pursue the capture of any monies receivable as negotiated through a repayment agreement or as awarded through fair hearing and legal proceedings, and then systematically process the receipt and distribution of those funds to ensure that they are deposited into the appropriate financial accounts.**
 - viii. ☐ Other. Describe the activities and the results of these activities:
- c. Does the Lead Agency investigate and recover improper payments due to unintentional program violations?
- ☐ No.
- ☒ Yes.
- If yes, check and describe below any activities that the Lead Agency will use to investigate and recover improper payments due to unintentional program violations. Include in the description how each activity assists in the investigation and recovery of improper payments due to unintentional program violations. Include a description of the results of such activity.
- i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The Department is required to pursue all overpayments when they reach the threshold of \$100 dollars. If an Unintentional Program Violation occurs, the department would pursue to the overpayment but the family would not be disqualified from receiving services nor would it count toward the penalties associated to Intentional Program Violations. The result of setting a minimum overpayment threshold helps to ensure that the**

agency is not spending more money on administration costs to collect an overpayment than what the overpayment collection would yield, while still holding participants accountable for the responsible use of the funds.

- ii. ☐ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis:
 - iii. ☒ Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **Financial Services works to establish repayment plans with identified families or providers. A process for payment is established and signed between both parties. The result of recovering funds through repayment plans helps to ensure that participants are accountable for full repayment and the responsible use of the funds while limiting the financial burden of repayment and potentially maintaining eligibility for needed programs or provider participation.**
 - iv. ☐ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis:
 - v. ☒ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis: **Financial Services works with the state controller's office to comply when individuals or providers have overpayments that necessitate collection through intercepts after other means of collection have been unsuccessful. As a result of recovering funds through tax intercepts the child care program demonstrates ongoing fiscal accountability and maintains program sustainability.**
 - vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
 - vii. ☒ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis: **Concerns about improper payments are forwarded to an Idaho Department of Health and Welfare (DHW)'s Integrity Unit that specializes in the review of child care payments, if an overpayment is discovered it must be repaid. The Revenue Operations unit at DHW actively pursues the collection of various outstanding monies owed to DHW. They pursue the recovery of program benefit overpayments whether stemming from intentional program violations on the part of DHW clientele or from simple human error. To assist in the recovery of payments, the Revenue Operations pursue the capture of any monies receivable as negotiated through a repayment agreement or as awarded through fair hearing and legal proceedings, and then systematically process the receipt and distribution of those funds to ensure that they are deposited into the appropriate financial accounts.**
 - viii. ☐ Other. Describe the activities and the results of these activities:
- d. Does the Lead Agency investigate and recover improper payments due to agency errors?
- ☐ No.
- ☒ Yes.

If yes, check and describe all activities that the Lead Agency will use to investigate and recover improper payments due to agency errors. Include in the description how each activity assists in the investigation and recovery of improper payments due to administrative errors. Include a description of the results of such activity.

- i. ☒ Require recovery after a minimum dollar amount of an improper payment and identify the minimum dollar amount. Describe the activities and the results of these activities based on the most recent analysis: **The Department is required to pursue all overpayments when they reach the threshold of \$100 dollars. If an agency error occurs, the department would pursue the overpayment but the family would not be disqualified from receiving services nor would it count toward the penalties associated to Intentional Program Violations. The result of setting a minimum overpayment threshold helps to ensure that the agency is not spending more money on administration costs to collect an overpayment than what the overpayment collection would yield, while still holding participants accountable for the responsible use of the funds.**
- ii. ☐ Coordinate with and refer to the other State/Territory agencies (e.g., State/Territory collection agency, law enforcement agency). Describe the activities and the results of these activities based on the most recent analysis:
- iii. ☒ Recover through repayment plans. Describe the activities and the results of these activities based on the most recent analysis: **Financial Services works to establish repayment plans with identified families or providers. A process for payment is established and signed between both parties. The result of recovering funds through repayment plans helps to ensure that participants are accountable for full repayment and the responsible use of the funds while limiting the financial burden of repayment and potentially maintaining eligibility for needed programs or provider participation.**
- iv. ☐ Reduce payments in subsequent months. Describe the activities and the results of these activities based on the most recent analysis:
- v. ☒ Recover through State/Territory tax intercepts. Describe the activities and the results of these activities based on the most recent analysis: **Financial Services works with the state controller's office to comply when individuals or providers have overpayments that necessitate collection through intercepts after other means of collection have been unsuccessful. As a result of recovering funds through tax intercepts the child care program demonstrates ongoing fiscal accountability and maintains program sustainability.**
- vi. ☐ Recover through other means. Describe the activities and the results of these activities based on the most recent analysis:
- vii. ☒ Establish a unit to investigate and collect improper payments and describe the composition of the unit. Describe the activities and the results of these activities based on the most recent analysis: **Concerns about agency errors are forwarded to an Idaho Department of Health and Welfare (DHW)'s Integrity Unit that specializes in the review of child care subsidies, if an overpayment is discovered it must be repaid. The Revenue Operations unit at DHW actively pursues the collection of various outstanding monies owed to DHW. They pursue the recovery**

of program benefit overpayments whether stemming from intentional program violations on the part of DHW clientele or from simple human error. To assist in the recovery of payments, the Revenue Operations pursue the capture of any monies receivable as negotiated through a repayment agreement or as awarded through fair hearing and legal proceedings, and then systematically process the receipt and distribution of those funds to ensure that they are deposited into the appropriate financial accounts.

viii. ☐ Other. Describe the activities and the results of these activities:

e. What type of sanction will the Lead Agency place on clients and providers to help reduce improper payments due to intentional program violations or fraud? Check and describe all that apply:

i. ☒ Disqualify the client. Describe this process, including a description of the appeal process for clients who are disqualified. Describe the activities and the results of these activities based on the most recent analysis: **Intentional Program Violations (IPV) - (IDAPA: 16.06.12.702))-** An IPV is an intentionally false or misleading action or statement as identified below in Subsections 702.01 through 702.08 of this rule. An IPV is established when a family member or the child care provider admits the IPV in writing and waives the right to an administrative hearing, or when determined by an administrative hearing, a court decision, or through deferred adjudication. Deferred adjudication exists when the court defers a determination of guilt because the accused family member or child care provider meets the terms of a court order or an agreement with the prosecutor.

Penalties for an IPV - (IDAPA: 16.06.12.703) When the the Idaho Department of Health and Welfare (DHW) determines an IPV was committed, the party who committed the IPV loses eligibility for the Idaho Child Care Program (ICCP). If an individual has committed an IPV, the entire family is ineligible for child care benefits. If a child care provider has committed an IPV, the provider is ineligible to receive payments. The period of ineligibility for each offense, for both participants and providers, is as follows:

First Offense. 12 months for the first IPV or fraud offense, or the length of time specified by the court

Second Offense. 24 months for the second IPV or fraud offense, or the length of time specified by the court

Third Offense. Permanent ineligibility for the third or subsequent IPV or fraud offense, or the length of time specified by the court

Fair Hearing Process - (Benefits Process Manual: "Fair Hearing Process") Clients have 30 days from the date of the decision to request a fair hearing. Fair Hearing Officers review and respond to all requests within five calendar days of the "fair hearing requested" task being set. They communicate the status of reinstatement/continuance of benefits pending the hearing result to the customer within five calendar days of the "fair hearing requested" task being set. Then work with the client to coordinate and manage all aspects of a fair hearing to completion.

The result of client disqualification temporary or permanently prevents ongoing misuse of program funds and maintains program sustainability.

- ii. ☒ Disqualify the provider. Describe this process, including a description of the appeal process for providers who are disqualified. Describe the activities and the results of these activities based on the most recent analysis: **Termination of Provider Status - (Idaho Code 56-209h)**

The Idaho Department of Health and Welfare (DHW) may terminate the provider agreement of, or otherwise deny provider status for a period of up to five years from the date DHW's action becomes final to any individual or entity providing the Idaho Child Care Program.

Provider Notification - (IDAPA 16.06.17.750)

When the Idaho Department of Health and Welfare (DHW) determines actions defined in Sections 701 through 705, 750, and 751 of these rules are appropriate, it will send written notice of the decision to the provider or person. The notice will state the basis for the action, the length of the action, the effect of the section on that person's ability to provide services under state and federal programs, and the person's appeal rights.

Notice to State Licensing Authorities - (Benefits Process Manual: "Fair Hearing Process")

DHW will promptly notify all appropriate licensing authorities having responsibility for licensing of a DHW action, and the facts and circumstances of that action. DHW may request certain actions be taken and that DHW be informed of actions taken. A certified letter serves as written notice of the decision. A decision issued by DHW in the Division of Welfare program will be final and effective unless an individual or representative files a written notice of appeal within 30 days from the date the decision was mailed.

Appeals-

Appeals can be sent to: Idaho Department of Health and Welfare Administrative Procedures Section (APS) P.O. Box 83720, Boise, Idaho 83720-0036 or APS@dhw.idaho.gov (Letter of termination). Fair hearing officers review and respond to all requests within 14 calendar days of receipt of the appeal. They communicate the status of reinstatement of the provider agreement pending the hearing result to the provider, then work with the provider to coordinate and manage all aspects of a fair hearing to completion.

The result of provider disqualification temporary or permanently prevents ongoing misuse of program funds and maintains program sustainability.

- iii. ☒ Prosecute criminally. Describe the activities and the results of these activities based on the most recent analysis: **Idaho would fully cooperate with law enforcement if an individual or provider was being prosecuted.**

- iv. ☐ Other. Describe the activities and the results of these activities based on the most recent analysis:

Appendix 1: Lead Agency Implementation Plan

The Appendix will be available for Lead Agencies to use in CARS after the Plan approval letter is issued.

For each non-compliance, Lead Agencies must describe the following:

- **Action Steps:** List the action steps needed to correct the finding (e.g., update policy manual, legislative approval, IT system changes, etc.). For each action step list the:
 - ***Responsible Entity:*** Indicate the entity (e.g., agency, team, etc.) responsible for completing the action step.
 - ***Expected Completion Date:*** List the expected completion date for the action step.
- **Overall Target Date for Compliance:** List date Lead Agency anticipates completing implementation, achieving full compliance with all aspects of the findings. (Note: Compliance will not be determined until the FFY 2025-2027 CCDF Plan is amended and approved).

Appendix 1: Form

[Plan question with non-compliance and associated provision will pre-populate based on preliminary notice of non-compliance]

A. Action Steps for Implementation	B. Responsible Entity(ies)	C. Expected Completion Date
Step 1:		
Step 2 (as necessary):		
[Additional steps added as necessary]		
Overall Target Date for Compliance:		